

Student Support Persona Cards

A K–12 Teacher Resource Grounded in UDL and IDEA Frameworks
60 cards · 11 categories · 3 grade bands each

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Web version: <https://fyvr.net/student-personas/>

Frameworks referenced: Universal Design for Learning (UDL) · Individuals with Disabilities Education Act (IDEA) · Section 504 of the Rehabilitation Act · SAMHSA Trauma-Informed Principles

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Learning Disabilities

Learning Disabilities

Dyslexia

Dyslexia is a language-based learning disability that affects reading fluency, decoding, and spelling. It is neurological in origin and unrelated to intelligence. Students with dyslexia often have strong reasoning and verbal skills but struggle significantly with print.

<https://fyvr.net/student-personas/dyslexia>

Elementary (K–5)

A bright, verbally expressive student who struggles to sound out simple words, reverses letters, and avoids reading aloud. May appear frustrated or disengaged during literacy activities despite strong comprehension when content is read to them.

What You Might Notice

- Difficulty sounding out unfamiliar words; guesses based on first letter
- Slow, labored oral reading with frequent self-corrections
- Reversals of letters (b/d, p/q) beyond age 7
- Avoids reading-related tasks; may complain of headaches or stomachaches

Student Strengths

- Strong listening comprehension and verbal reasoning
- Creative storytelling and imaginative thinking
- Excellent memory for information heard or experienced

Recommended Accommodations

- Provide audiobooks or text-to-speech for all assigned reading
- Allow oral responses in place of written ones
- Extended time (1.5x) on reading and writing tasks
- Reduce copying requirements; provide printed notes
- Use multisensory phonics instruction (Orton-Gillingham approach)

Assistive Technology

- Learning Ally or Bookshare for audiobooks
- Read&Write; for Google Chrome (text-to-speech)
- Speech-to-text tools (Google Voice Typing)

UDL Strategies

- Offer multiple means of representation: audio, visual, and tactile materials
- Pre-teach vocabulary with visual word walls before reading units
- Provide graphic organizers to scaffold written expression

IEP Considerations

- Present levels should document reading fluency (words correct per minute) and decoding accuracy via DIBELS or AIMSweb data
- Annual goal: Student will read grade-level decodable passages at X wcpm with Y% accuracy as measured by monthly curriculum-based measures
- Recommend specialized reading intervention (e.g., Orton-Gillingham, Wilson) at least 3x/week
- Accommodations language: extended time, text-to-speech, oral testing, reduced written output

Who to Collaborate With

- Reading Specialist / Literacy Coach
- Special Education Teacher (resource room)

Family Partnership Tip

Read aloud together every night using audiobooks — let your child follow along with the text. This builds fluency and comprehension without the pressure of decoding.

Middle School (6–8)

A student who has often developed coping strategies but still struggles significantly with reading speed and written output. May appear lazy or unmotivated; is often exhausted from the cognitive effort of decoding. Content knowledge may be strong but assessments rarely reflect it.

What You Might Notice

- Reads significantly below grade level; avoids independent reading
- Written work is brief and lacks detail compared to verbal ability
- Difficulty with multi-step written directions
- Misspellings are phonetically plausible but inconsistent

Student Strengths

- Strong verbal participation and discussion skills
- Creative problem-solving and out-of-the-box thinking
- Resilience developed from years of navigating challenges

Recommended Accommodations

- Text-to-speech for all textbooks and tests
- Speech-to-text or scribe for written assignments
- Extended time on all assessments
- Spell-check and word prediction tools permitted
- Tests in a separate, low-distraction setting

Assistive Technology

- Kurzweil 3000 or NaturalReader for textbook access
- Co:Writer for word prediction and spelling support
- Google Docs voice typing for written work

UDL Strategies

- Chunk reading assignments with built-in audio alternatives
- Allow students to demonstrate knowledge through podcasts, videos, or oral presentations
- Use anchor charts and visual summaries instead of text-heavy notes

IEP Considerations

- Present levels must address both decoding and reading comprehension separately
- Goals should target reading fluency AND written expression (not just one domain)
- Consider transition planning language about self-advocacy skills for high school
- Document need for audio versions of standardized assessments (state testing accommodations)

Who to Collaborate With

- Special Education Case Manager
- School Psychologist (for re-evaluation if needed)

Family Partnership Tip

Encourage your teen to use audiobooks for pleasure reading — it counts and builds vocabulary. Help them practice asking teachers for accommodations themselves.

High School (9–12)

A student who is bright and capable but whose reading and writing deficits are now masked by avoidance and workarounds. May be at risk of failing courses that rely heavily on written output. Needs strong self-advocacy skills and preparation for post-secondary accommodations.

What You Might Notice

- Turns in incomplete or very short written assignments
- Avoids reading independently; often hasn't done assigned readings
- Strong in class discussion but performance doesn't match on tests
- May be anxious about timed writing tasks like AP exams or SAT

Student Strengths

- Articulate verbal communicator; excels in discussions and presentations
- Entrepreneurial, big-picture thinking
- Has developed strong self-awareness about learning needs

Recommended Accommodations

- Extended time (1.5x or 2x) on all tests and major assignments
- Text-to-speech and speech-to-text permitted on all tasks
- Alternative formats for demonstrating knowledge (oral exams, presentations)
- Access to notes and outlines during tests
- Reduced or modified written output requirements where feasible

Assistive Technology

- Snap&Read; for PDF and web text-to-speech
- Dragon NaturallySpeaking for extended speech-to-text
- Grammarly or built-in spell/grammar check

UDL Strategies

- Provide choice in how students demonstrate mastery of content
- Use flipped classroom models so students can pre-listen to content at home
- Build explicit self-advocacy instruction into the curriculum

IEP Considerations

- Transition goals must address post-secondary education: how to register with disability services, how to self-advocate with professors
- Document all accommodations clearly for the college/university disability services office
- Annual goal may focus on self-advocacy: student will independently request accommodations in X out of Y opportunities
- Consider whether student qualifies for College Board or ACT testing accommodations (begin process early)

Who to Collaborate With

- Transition Coordinator
- Guidance Counselor (for college planning with disability documentation)

Family Partnership Tip

Help your student research disability services at colleges they're interested in — requirements vary widely. Having a strong paper trail from their IEP is invaluable.

Resources: [Yale Center for Dyslexia](#) | [Understood — Dyslexia](#) | [IDA — Dyslexia in the Classroom](#)

Dysgraphia

Dysgraphia is a learning disability affecting written expression, including handwriting, spelling, and the organization of written language. It is not a motor problem alone — it affects the automatic production of writing, which consumes cognitive resources that should be available for composing ideas.

<https://fyvr.net/student-personas/dysgraphia>

Elementary (K–5)

A student whose handwriting is illegible or extremely labored, who avoids writing tasks, and whose written work is far shorter than their verbal ideas suggest. May have a strong pencil grip, press very hard, or frequently erase.

What You Might Notice

- Handwriting is slow, effortful, inconsistent in size and spacing
- Written sentences are much shorter than the student can express orally
- Difficulty remembering how to form letters; mixes print and cursive
- Avoids or resists writing tasks; may have emotional outbursts around writing

Student Strengths

- Rich oral storytelling and strong verbal ideas
- Visual-spatial reasoning and artistic creativity
- Strong comprehension and oral language skills

Recommended Accommodations

- Allow keyboarding as an alternative to handwriting
- Reduce written output requirements; accept shorter written responses
- Provide graphic organizers and sentence starters
- Allow extra time for all written assignments
- Accept speech-to-text or dictated responses

Assistive Technology

- Chromebook or iPad with keyboard for all written work
- Speech-to-text (Google Voice Typing, Apple Dictation)
- Clicker 8 or Co:Writer for word prediction

UDL Strategies

- Offer typing, drawing, or verbal recording as alternative expression formats
- Teach keyboarding skills explicitly as early as second grade
- Use interactive notebooks with pre-printed sections to minimize copying

IEP Considerations

- Present levels should include handwriting fluency data (letters per minute) and written expression samples
- Annual goal: Student will produce a structured paragraph of X sentences using keyboarding in Y minutes with Z% legibility/coherence
- Occupational therapy evaluation may be warranted to rule out fine motor component
- Document explicit keyboarding accommodation for all classroom and state testing

Who to Collaborate With

- Occupational Therapist
- Special Education Teacher

Family Partnership Tip

Let your child dictate stories to you at home. Scribe their ideas without correcting them — this separates the act of composing from the mechanics of writing.

Middle School (6–8)

A student who has often found workarounds but whose limitations become more visible as writing demands increase. May use extremely simple vocabulary in writing to avoid spelling errors, or submit work that doesn't reflect their actual knowledge.

What You Might Notice

- Written work is brief, poorly organized, and filled with spelling errors inconsistent with verbal ability
- Avoids note-taking; loses information from lectures
- Slow to copy from the board; often has incomplete notes
- May refuse writing tasks or melt down when faced with lengthy essays

Student Strengths

- Strong verbal participation; can explain concepts clearly when asked
- Creative thinking that comes through in discussion
- Often highly motivated when allowed to express knowledge differently

Recommended Accommodations

- Allow all written work to be typed
- Provide teacher or peer notes for all lectures
- Reduce note-taking requirements; allow partial notes or outlines
- Extended time on written assessments
- Allow voice memos or recordings in lieu of written notes

Assistive Technology

- Otter.ai or Rev for lecture transcription
- Notability or OneNote for typed/hybrid notes
- Grammarly for editing support

UDL Strategies

- Provide structured writing templates for every major assignment
- Teach and model the writing process explicitly (planning → drafting → revising)
- Use collaborative writing tools where students can contribute verbally or in short bursts

IEP Considerations

- Goal should address written expression quality (organization, ideas) separately from mechanics
- Address note-taking explicitly in the IEP — it is a functional access issue
- Consider 504 vs. IEP if only written expression is affected
- Include accommodation for state writing assessments (extended time, keyboarding)

Who to Collaborate With

- Special Education Case Manager
- Occupational Therapist (if fine motor component persists)

Family Partnership Tip

Help your student set up a voice-to-text workflow at home for homework. Practice using the same tools they use at school so the habit becomes automatic.

High School (9–12)

A student who may be failing writing-heavy courses despite strong content knowledge. The gap between their oral and written performance is striking. Urgently needs assistive technology proficiency and self-advocacy skills for college.

What You Might Notice

- Essays are skeletal and don't reflect the student's depth of understanding
- Avoids large writing projects until the last minute; panic-writes
- Timed writing (in-class essays, AP exams) is particularly distressing
- May have internalized the belief that they are 'bad at writing'

Student Strengths

- Outstanding verbal communication; excels in debate or discussion
- Analytical thinking that shows in oral responses
- Has developed persistence and problem-solving around writing barriers

Recommended Accommodations

- All assessments and assignments typed; no handwriting required
- Extended time (1.5x–2x) for all written work including in-class essays
- Access to speech-to-text tools during writing
- Separate setting for written assessments to allow dictation
- Annotated outlines accepted in place of full first drafts

Assistive Technology

- Dragon NaturallySpeaking Professional for extended dictation

- Ginger or ProWritingAid for grammar support
- Mind-mapping tools (MindMeister, XMind) for planning

UDL Strategies

- Allow multimodal products: video essays, podcasts, or narrated slide decks
- Teach explicit planning strategies (TREE, POWER) before each writing unit
- Provide writing conferences as an alternative to written feedback

IEP Considerations

- Transition planning must include instruction in AT tools student will use independently in college
- Document all writing accommodations for College Board/ACT (apply 12–18 months before testing)
- Annual goal should address written expression in functional contexts (college application essay, email communication)
- Note in IEP that oral exams or verbal demonstrations are acceptable alternatives

Who to Collaborate With

- Transition Coordinator
- Writing Center staff or English teacher for co-planning

Family Partnership Tip

Help your student practice dictating to Dragon or Google Docs at home. The more they use it for homework, the faster and more fluent they'll become before college.

Resources: [Understood — Dysgraphia](#) | [NICHD — Writing & Dysgraphia](#) | [LD Online — Dysgraphia](#)

Dyscalculia

Dyscalculia is a specific learning disability in mathematics that affects number sense, arithmetic fluency, and mathematical reasoning. Students with dyscalculia often struggle with basic fact retrieval, place value, and understanding quantities, despite average or above-average intelligence in other areas.

<https://fyvr.net/student-personas/dyscalculia>

Elementary (K–5)

A student who cannot reliably count on or back from a number, confuses similar-looking numerals, and cannot recall basic addition or subtraction facts even with extensive practice. May have anxiety around math class.

What You Might Notice

- Cannot reliably recall basic addition/subtraction facts after typical instruction
- Confuses numerals (6/9, 2/5) and struggles with place value concepts
- Counts on fingers for all computation; loses track easily
- Avoids math activities; shows distress when called on during math

Student Strengths

- Strong verbal reasoning and language skills
- Good at understanding patterns in non-numerical contexts
- Often creative and strong in arts or storytelling

Recommended Accommodations

- Allow use of a number line, multiplication chart, and calculator for computation
- Provide graph paper for alignment of multi-digit problems
- Reduce the number of problems on assignments (demonstrate mastery with fewer)
- Allow extended time for math tests
- Use manipulatives (base-ten blocks, counters) for all arithmetic concepts

Assistive Technology

- Mathway or Photomath for step-by-step problem solving
- ModMath app for graph-paper-style typed math
- Number Pieces or Number Frames (Math Learning Center apps)

UDL Strategies

- Teach math concepts with concrete → representational → abstract (CRA) sequence
- Use color-coding for place value and operation steps
- Anchor new math concepts to real-world, meaningful contexts

IEP Considerations

- Present levels should include math fluency probe data (single-digit facts per minute) and computation accuracy
- Annual goal: Student will solve single-digit addition/subtraction facts with X% accuracy in Y seconds using a reference chart
- Specialized math intervention (e.g., Number Rockets, TouchMath) recommended
- Document calculator accommodation for computation — student's goal is mathematical reasoning, not fact retrieval

Who to Collaborate With

- Math Specialist or Interventionist
- Special Education Teacher

Family Partnership Tip

Play math games at home that use dice, cards, or physical objects. Keep it low-stakes and fun — the goal is building number sense, not drilling facts.

Middle School (6–8)

A student who enters middle school math without automaticity in basic facts, making multi-step algebra and fractions extremely difficult. The cognitive load of basic computation leaves no capacity for the reasoning level of the work. Anxiety around math is often significant.

What You Might Notice

- Struggles with fractions, ratios, and proportional reasoning
- Makes consistent errors in multi-step computation even when the concept is understood
- Cannot do mental math; relies on fingers or tallying
- Shuts down or becomes avoidant when math gets difficult

Student Strengths

- Often strong in logical verbal reasoning and word problems when decoding isn't an issue
- Persistence and creativity in finding alternative solution paths
- Strong in non-quantitative areas of STEM (science concepts, earth science)

Recommended Accommodations

- Calculator permitted for all computation
- Formula sheet and reference card for all tests
- Extended time on all math assessments
- Problems read aloud or provided in audio format
- Reduced assignment length focused on conceptual items

Assistive Technology

- Desmos graphing calculator (free, accessible)
- Khan Academy for self-paced concept review
- Photomath for step-by-step worked examples

UDL Strategies

- Allow multiple representations of math: tables, graphs, equations, and verbal explanations
- Use technology-based manipulation tools for fractions and geometry
- Provide worked examples and partially completed problems as scaffolds

IEP Considerations

- Goal should address functional numeracy and conceptual understanding, not just fact fluency
- Specify which calculator is allowed and for which tasks in the accommodations section
- Consider co-teaching or resource room support during math class
- Address math anxiety explicitly — may warrant counseling support

Who to Collaborate With

- Math Teacher (co-planning accommodations)
- School Counselor (for math anxiety support)

Family Partnership Tip

Let your student use a calculator freely at home. The goal is understanding math concepts — using tools is a life skill, not cheating.

High School (9–12)

A student whose math disability significantly limits course options and may affect graduation requirements. Needs explicit instruction in functional math skills and strong advocacy for appropriate course placement that reflects goals, not just deficit.

What You Might Notice

- Struggles in algebra and higher-level courses despite hard work
- Cannot complete timed math tests even when allowed extended time
- Has strong conceptual understanding verbally but computation errors undermine grades
- May be reluctant to take math-related electives or career paths due to math fear

Student Strengths

- Understands mathematical concepts when given time to reason through them
- Often strong in data interpretation, statistics, and applied math
- Has developed strong self-awareness about math needs

Recommended Accommodations

- Calculator for all computation tasks
- Formula sheet on all assessments
- Extended time (1.5x–2x)
- Access to visual models and graph paper
- Option to demonstrate mastery through verbal explanation or alternative formats

Assistive Technology

- Desmos for all graphing tasks
- Wolfram Alpha for step-by-step algebra support
- Microsoft Math Solver for visual problem solving

UDL Strategies

- Connect math to real-world applications relevant to student's career interests
- Use statistics and data science as an alternative pathway for students avoiding algebra II
- Allow project-based assessments in math (budgeting, data analysis projects)

IEP Considerations

- Transition planning must address math requirements for diploma, college entrance, and career pathways
- Consider functional math alternatives if traditional algebra II is a barrier to graduation
- Annual goal: Student will use calculator-assisted computation to solve multi-step real-world problems with X% accuracy
- Document calculator accommodation explicitly for SAT/ACT accommodation requests

Who to Collaborate With

- Guidance Counselor (for course planning)
- Transition Coordinator

Family Partnership Tip

Talk with your student about what they want to do after graduation — this conversation should drive math course decisions, not a one-size-fits-all sequence.

Resources: [Understood — Dyscalculia](#) | [NCLD — Dyscalculia](#) | [Math Learning Disabilities — LD Online](#)

Reading Comprehension Disorder

Reading Comprehension Disorder (sometimes called Specific Comprehension Deficit or poor comprehension) affects a student's ability to understand what they read, despite adequate decoding skills. These students can read words aloud accurately but fail to build meaning, make inferences, or recall details from text.

<https://fyvr.net/student-personas/reading-comprehension-disorder>

Elementary (K–5)

A student who can read words aloud with reasonable accuracy but cannot answer questions about what they just read. Appears to 'read' but retains little. May not realize they haven't understood — metacognitive monitoring is weak.

What You Might Notice

- Reads text aloud accurately but cannot summarize or answer questions
- Struggles with making inferences or identifying the main idea
- Appears confused after silent reading; cannot recall key details
- Does not ask clarifying questions; unaware of comprehension breakdown

Student Strengths

- Often has adequate or strong decoding and word-reading skills
- Can understand content well when it is read aloud and discussed
- Strong listening comprehension relative to reading comprehension

Recommended Accommodations

- Pre-teach vocabulary and background knowledge before reading assignments
- Provide guided reading with stopping points and check-in questions
- Allow oral retelling instead of written responses to demonstrate comprehension
- Pair text with visual supports (diagrams, illustrations, timelines)
- Read text aloud to student or provide audio version

Assistive Technology

- Actively Learn or Newsela (embedded comprehension supports)
- Google Read&Write; for text highlighting and annotation
- Epic! Books with read-aloud feature

UDL Strategies

- Teach explicit comprehension strategies: visualizing, questioning, summarizing
- Use think-alouds to model metacognitive monitoring during shared reading
- Provide graphic organizers for story elements and informational text structures

IEP Considerations

- Distinguish clearly between decoding and comprehension data in present levels
- Annual goal: Student will identify the main idea and two supporting details from a grade-level informational passage with X% accuracy
- Recommend evidence-based comprehension intervention (e.g., Collaborative Strategic Reading, SRA Corrective Reading comprehension strand)
- Distinguish this from dyslexia in evaluation documentation — intervention is different

Who to Collaborate With

- Reading Specialist
- Speech-Language Pathologist (if language comprehension component)

Family Partnership Tip

After your child reads a page, pause and ask 'What just happened?' or 'What was that section mostly about?' This builds the habit of self-monitoring for meaning.

Middle School (6–8)

A student who reads independently but gains little from the reading. Struggles with content-area texts, especially when inference or synthesis is required. Often does well on factual recall but fails on higher-order questions. Gap widens as texts become denser.

What You Might Notice

- Can answer literal questions but struggles with inference, theme, or author's purpose
- Does not annotate or interact with text during reading
- Written responses to reading lack textual evidence
- Reading comprehension on assessments is consistently below grade level

Student Strengths

- Good word-level reading; fluent oral reader
- Strong in classes with discussion-based or experiential learning
- Often strong in math, science labs, or arts

Recommended Accommodations

- Provide chapter summaries or study guides before assigning reading
- Allow partner or shared reading for comprehension-heavy texts
- Use annotation guides with specific focus questions
- Accept verbal or recorded responses to reading assignments
- Reduce reading volume while maintaining comprehension focus

Assistive Technology

- Actively Learn with teacher-embedded questions and notes
- CommonLit with scaffolded guided reading supports
- Otter.ai for recording verbal responses

UDL Strategies

- Use reciprocal teaching (predict, question, clarify, summarize) as a classroom routine
- Teach text structure explicitly for narrative and informational genres
- Incorporate discussion as a comprehension-building strategy before written tasks

IEP Considerations

- Goals should be specific to comprehension skills: inferencing, summarizing, identifying text structure
- Consider speech-language evaluation if language comprehension weakness is suspected
- Note in accommodations: pre-reading supports, reduced reading volume for homework, oral response options
- Track progress using curriculum-based comprehension probes, not just fluency

Who to Collaborate With

- Speech-Language Pathologist
- Content area teachers (for co-planning text supports)

Family Partnership Tip

Watch documentaries or educational videos together about topics your student is reading in class — building background knowledge dramatically improves comprehension.

High School (9–12)

A student who has masked this deficit through multiple-choice test-taking strategies, class participation, and avoiding assigned reading. Struggles significantly with complex literary analysis, research papers, and content-area reading that requires synthesis across texts.

What You Might Notice

- Participates well verbally but cannot write analytically about texts
- Avoids reading; uses Sparknotes or summaries rather than primary texts
- Struggles to find and cite evidence from texts in written work
- Comprehension collapses on dense, complex, or multi-text tasks

Student Strengths

- Strong verbal reasoning and debate skills
- Excellent at understanding content presented in video, lecture, or discussion
- Has often developed strong listening and social comprehension skills

Recommended Accommodations

- Provide annotated texts, chapter summaries, or teacher-created study guides
- Allow audio versions of all literary and content texts
- Extended time for reading-heavy assessments
- Reduce textual evidence requirements while maintaining analytical expectations
- Allow oral discussion or recorded response as alternative to written literary analysis

Assistive Technology

- Speechify or NaturalReader for all assigned texts
- Hypothesis for collaborative online annotation
- Newsela for leveled nonfiction alternatives

UDL Strategies

- Provide Socratic seminars as an alternative comprehension demonstration
- Teach academic vocabulary explicitly across all content areas
- Use film adaptations and primary source audio alongside texts

IEP Considerations

- Annual goal should address synthesis skills: student will compare arguments across two texts with teacher-provided graphic organizer at X% accuracy
- Transition goals: student will use comprehension support tools independently in college coursework
- Include explicit accommodation for standardized tests: extended time on reading sections
- Document the distinction between this disability and low motivation in evaluation and present levels

Who to Collaborate With

- English/ELA Teacher (co-planning reading scaffolds)
- School Librarian (for alternate text format sourcing)

Family Partnership Tip

Encourage your student to listen to audiobooks or podcasts about topics they're passionate about — comprehension is a skill that transfers when built with engaging content.

Resources: [Reading Rockets — Comprehension](#) | [Understood — Reading Comprehension](#) | [NICHD — Reading Research](#)

Nonverbal Learning Disability

Nonverbal Learning Disability (NVLD) is a neurological disorder characterized by strong verbal skills alongside significant deficits in visual-spatial processing, math, motor coordination, and social perception. Students with NVLD often appear highly capable due to advanced vocabulary but struggle deeply with tasks requiring visual-spatial reasoning, novel problem-solving, and reading social cues.

<https://fyvr.net/student-personas/nonverbal-learning-disability>

Elementary (K–5)

A verbally advanced student who seems mature and articulate but struggles with puzzles, maps, math concepts, handwriting, and navigating new situations. May seem socially awkward or misread peer cues. Often struggles in recess and group work despite strong classroom participation.

What You Might Notice

- Strong reader and verbal communicator; poor math problem-solver
- Struggles with visual-spatial tasks: maps, diagrams, puzzles, geometry
- Handwriting is labored and poorly organized on the page
- Has difficulty with transitions, new routines, or unstructured time

Student Strengths

- Advanced vocabulary and verbal reasoning
- Excellent rote memory and decoding skills
- Highly verbal and eager to please adults

Recommended Accommodations

- Provide verbal explanations to accompany all visual materials
- Use graph paper and organizers for math and spatial tasks
- Pre-teach transitions and new routines with explicit verbal scripts
- Allow extra time for visual-spatial tasks
- Provide verbal directions in addition to written or visual ones

Assistive Technology

- Google Maps or visual navigation tools for spatial tasks
- Virtual manipulatives for math concepts
- Typing tools to bypass handwriting barriers

UDL Strategies

- Pair all visual materials with verbal descriptions
- Break visual-spatial tasks into step-by-step verbal procedures
- Use social stories and explicit social skills instruction for peer interactions

IEP Considerations

- Present levels must include both verbal and nonverbal cognitive data from psychoeducational evaluation
- Goals should address math reasoning, written expression, and if needed, social skills
- Recommend Occupational Therapy evaluation for visual-motor and spatial difficulties
- Note: NVLD is not a DSM-5 diagnosis — document as Other Health Impairment or SLD in the eligible category based on state criteria

Who to Collaborate With

- Occupational Therapist
- School Psychologist

Family Partnership Tip

When your child is confused by a visual task (map, diagram), narrate it aloud step by step. Their brain processes verbal information far more efficiently than visual.

Middle School (6–8)

A student whose verbal strengths mask significant struggles in math, spatial reasoning, and social understanding. Increasingly isolated from peers due to difficulty reading social cues. Written work that requires organization, diagrams, or charts is particularly challenging.

What You Might Notice

- Struggles with geometry, graphs, and any math requiring spatial reasoning
- Social interactions are often awkward; misreads sarcasm, jokes, and tone
- Gets lost in multi-step problems that require visual organization
- Written work is verbose but poorly organized visually

Student Strengths

- Rich verbal language; often excels in language arts, history, and debate
- Strong factual memory and recall for information learned verbally
- Genuinely curious and intellectually engaged

Recommended Accommodations

- Verbal instructions for all spatial and visual tasks
- Calculator and formula sheet for math
- Templates and structured formats for all written and visual assignments
- Social skills group or explicit social coaching
- Extended time for tests involving diagrams or visual-spatial processing

Assistive Technology

- Desmos for visual graphing with verbal labeling
- OneNote for structured written organization
- Social skills apps (e.g., Model Me Kids, Social Express)

UDL Strategies

- Present geographic, mathematical, and scientific content with verbal narration
- Use verbal mnemonics and step-by-step scripts for spatial procedures
- Build cooperative learning structures with explicit social roles and scripts

IEP Considerations

- Goals should address math reasoning AND social skills (two distinct areas)
- Social skills goal: Student will identify and respond appropriately to peer social cues in X out of Y observed interactions
- Counseling or social skills group should be listed as a related service
- Math goal should not just target computation — address visual-spatial math reasoning specifically

Who to Collaborate With

- School Counselor
- Math Specialist

Family Partnership Tip

Help your student debrief social situations at home without judgment — 'What do you think the other kid was feeling?' helps build perspective-taking over time.

High School (9–12)

A student whose high verbal ability has led others to underestimate the severity of their disability. Now facing serious struggles in geometry, chemistry, physics, and any course requiring spatial visualization or social-emotional navigation. Transition planning is critical.

What You Might Notice

- Fails or nearly fails visual-spatial courses (geometry, physics, art) despite strong verbal performance
- Has few close peer relationships; appears lonely or isolated
- Struggles with executive function for long-term project management
- May be overlooked for services because verbal IQ is high

Student Strengths

- Exceptional verbal reasoning and writing (in text-heavy domains)
- Strong in history, literature, social studies, and language
- Self-aware about areas of challenge when given safe opportunity to discuss

Recommended Accommodations

- Verbal-only alternatives to diagram or map-based assessments
- Explicit organization tools (templates, checklists, timelines) for all projects
- Social skills coaching and counseling as related services
- Course modifications for spatial-heavy requirements (e.g., geometry waiver or alternative)
- Extended time and chunked deadlines for complex projects

Assistive Technology

- Grammarly and outline tools for written organization
- GoodNotes or Notability for structured note-taking
- Google Calendar with explicit event reminders for executive function

UDL Strategies

- Offer verbal or essay-based alternatives to spatial demonstrations of knowledge
- Build explicit social-emotional learning into advisory or homeroom
- Teach time management and project planning as explicit skills

IEP Considerations

- Transition planning must address career environments that leverage verbal strengths
- Annual goal: Student will independently use an organizational planning tool to meet X deadlines per semester
- Social skills goal should reflect real-world post-secondary context: job interviews, college roommate communication
- Document NVLD profile clearly for post-secondary disability services — colleges need to understand the profile

Who to Collaborate With

- Transition Coordinator
- School Counselor or Psychologist

Family Partnership Tip

Help your student identify careers and environments that play to their verbal strengths — law, writing, history, social work. Knowing their strengths is as important as knowing their challenges.

Resources: NVLD Project | Understood — NVLD | LD Online — NLD

Attention & Executive Function

Attention & Executive Function

ADHD - Inattentive Type

ADHD Inattentive Type is characterized primarily by difficulty sustaining attention, following through on tasks, organizing work, and avoiding distraction — without the prominent hyperactivity seen in other ADHD presentations. These students are often overlooked because they are quiet and compliant, yet their academic performance is undermined by persistent attentional challenges.

<https://fyvr.net/student-personas/adhd-inattentive-type>

Elementary (K–5)

A quiet, often daydreaming student who seems to be in their own world. Loses materials, forgets to turn in homework, misses steps in multi-part directions, and is easily distracted by their own thoughts. Teachers often assume the student isn't trying.

What You Might Notice

- Frequently off-task during independent work; stares into space
- Loses books, folders, assignments, and materials regularly
- Misses steps in multi-part instructions even when given slowly
- Starts tasks but does not finish; work is incomplete at bell

Student Strengths

- Often highly creative and imaginative (their 'daydreaming' is active thinking)
- Kind, empathetic, and well-liked by peers
- Can hyperfocus deeply on topics of genuine interest

Recommended Accommodations

- Preferential seating near the teacher, away from high-traffic areas
- Break long tasks into small, chunked steps with check-ins
- Provide visual checklists for multi-step tasks and routines
- Allow movement breaks every 15–20 minutes
- Use timers (visual or auditory) to support task completion

Assistive Technology

- Time Timer app or physical Time Timer clock
- Google Classroom task lists with checkboxes
- ClassDojo or Class Charts for behavior/task monitoring (teacher-facing)

UDL Strategies

- Use 'parking lot' sticky notes so students can capture ideas without derailing focus
- Incorporate movement into instruction (stand-and-share, gallery walks)
- Provide immediate, specific feedback to redirect attention positively

IEP Considerations

- Present levels must include data on task completion rate and attention duration
- Consider eligibility under Other Health Impairment (OHI) — ADHD is specifically named in IDEA under OHI
- Annual goal: Student will complete X% of in-class assignments within allotted time using a visual checklist
- Coordinate with parents about medication management and its effects on school performance

Who to Collaborate With

- School Counselor (for organizational coaching)
- Pediatrician or prescribing physician (re: medication)

Family Partnership Tip

Help your child build a 'launch pad' — one spot by the door where everything needed for school lives. Reducing morning chaos reduces the chance items get forgotten.

Middle School (6–8)

A student whose grades have begun to slip as organizational demands increase. Forgets to record assignments, loses papers between classes, and struggles with long-term projects. The unstructured nature of middle school transitions exacerbates inattentive symptoms.

What You Might Notice

- Assignment notebook or planner is rarely filled in correctly
- Turns in incomplete work or forgets to submit digital assignments
- Struggles to initiate long-term projects; starts the night before
- Appears present in class but retains little from lectures without notes

Student Strengths

- Highly creative; often the 'idea generator' in group projects
- Strong in hands-on, project-based learning
- Curious and intrinsically motivated when content connects to interests

Recommended Accommodations

- Provide guided notes or partially completed note templates for all lectures — do not rely on the student to capture everything independently
- Reduce overall workload where possible: mark which problems or questions are required and cross off the rest; circle the work to complete rather than assigning a full set
- Provide printed worksheets or tests for math when the standard delivery involves video-gated or re-watch-dependent digital programs
- Allow additional time on assignments, not to exceed one week from the due date and not beyond the end of the quarter
- Support student in building a task checklist at the start of in-class assignments or projects — student initiates with the teacher, not the other way around

- Allow different modes of demonstrating proficiency — oral check-ins, verbal explanation, or annotated work in place of written tests where appropriate
- Preferential seating near quieter peers and away from high-traffic areas when possible

Assistive Technology

- Google Calendar with shared family or teacher access for deadlines (free, Chromebook-native)
- Google Tasks or Google Keep for checklist and task management (free, no app install required)
- Google Docs with voice typing enabled for note-taking (built into Chromebook)
- Noise-buffering headphones with music access from a non-wifi device during independent work — configured so the student can still hear direct teacher instruction

UDL Strategies

- Teach organizational strategies explicitly — do not assume students with ADHD develop them through observation or maturity
- Use backwards-planning scaffolds for any assignment longer than one week: break into named milestones with check-in dates
- Reduce cognitive load on assessments by separating the skill being tested from the delivery format — a student who understands probability should not be failing because of an inaccessible 8-minute video gateway
- Allow in-progress submissions for feedback before the final deadline
- Build student agency into task initiation — let the student co-create the checklist rather than receiving one pre-made

IEP Considerations

- Address organization, initiation, and homework completion as explicit IEP goals — not just academic content targets
- Annual goal example: Student will record and complete X% of assignments using a shared digital task list with bi-weekly teacher review (not daily check-in, which is rarely sustainable at scale)
- Workload reduction and flexible submission formats should be written as specific accommodations, not left to teacher discretion
- Be realistic about check-in accommodations: 'teacher checks in daily' is frequently unimplemented in classes of 30+ students — write goals the student can initiate themselves
- Ensure all content-area teachers have read and acknowledged the IEP accommodations — inconsistency across teachers is one of the most common failure points

Who to Collaborate With

- School Counselor
- Advisory/Homeroom Teacher
- Special Education Case Manager (if IEP)

Family Partnership Tip

Ask your student to show you their task list at the end of each school day — not to check up on them, but to help them hear themselves say what's due. A consistent 5-minute debrief at pickup or dinner is more effective than occasional reminders.

High School (9–12)

A student who may be failing multiple classes despite understanding the material, solely due to executive function breakdowns. The compounding effect of years of missed deadlines, incomplete work, and lost credit is now threatening graduation. Self-esteem and motivation are often significantly affected.

What You Might Notice

- Has multiple missing assignments and zero-grade penalties despite knowing the content
- Starts studying but gets sidetracked; falls behind on independent coursework
- Frequently overwhelmed by large projects; paralyzed rather than productive
- May have given up on some classes and overcompensates in areas of interest

Student Strengths

- Often deeply knowledgeable about specific areas of passion
- Creative and innovative when engaged
- Responds strongly to mentorship and authentic relationship with teachers

Recommended Accommodations

- Provide guided notes or structured note templates for all lecture-based content
- Reduce assignment volume where possible — prioritize quality of demonstrated understanding over quantity of completed items
- Extended deadline for assignments, arranged in advance (not ad hoc): not to exceed one week from the due date, not beyond the end of the grading period
- Provide printed or static-format versions of assessments when the standard format is delivered through video-gated or re-watch-dependent digital platforms
- Allow flexible demonstration of mastery: oral interview, annotated draft, or recorded explanation in place of timed written tests
- Support student in creating a task checklist at the start of major assignments or projects — student leads this process with the teacher
- Preferential seating near quieter peers; access to noise-buffering headphones during independent work

Assistive Technology

- Google Calendar and Google Tasks for deadline and assignment tracking (free, Chromebook/school account native)
- Google Docs voice typing for drafting written work
- Canvas or school LMS notification settings — configure deadline alerts via email rather than requiring students to check the platform
- Noise-buffering headphones with music access from a non-wifi or offline device for focus during work sessions

UDL Strategies

- Allow flexible deadlines with proactive planning agreements rather than reactive extensions
- Provide choice in assignment formats to increase buy-in and initiation
- Build in self-assessment check-ins midway through all projects

IEP Considerations

- Transition goals must explicitly address executive function strategies for post-secondary settings — college disability services and employers do not provide the same scaffolding as K-12
- Assess whether credit deficits are disability-related; if so, address compensatory strategies in the IEP rather than treating them as motivation or effort failures
- Annual goal: Student will use a self-maintained digital planning system to submit X% of assignments within the extended deadline window across all enrolled classes
- Late-work and grading accommodation policies must be written explicitly in the IEP — 'teacher discretion' is not an accommodation
- Write accommodations that the student can activate themselves — check-in accommodations that depend solely on teacher initiation are the most commonly unimplemented in large high schools
- If a re-watch-dependent or video-gated digital platform is used for core instruction or assessment, an accessible alternative format must be available as an accommodation

Who to Collaborate With

- Guidance Counselor (credit recovery planning)
- Case Manager or Advisory Teacher

Family Partnership Tip

Help your teen identify one trusted adult at school — a counselor, case manager, or advisory teacher — and make sure they know who that person is and how to reach them. The relationship matters more than the system.

Resources: [CDC — ADHD in Schools](#) | [CHADD — Inattentive ADHD](#) | [Understood — Inattentive ADHD](#)

ADHD - Hyperactive-Impulsive Type

ADHD Hyperactive-Impulsive Type is characterized by excessive motor activity, impulsive responses, difficulty waiting, and behavioral disinhibition. These students are often physically restless, interrupt frequently, and act before thinking. While often misread as defiant or disruptive, their behavior is neurologically driven and responds well to structured, proactive classroom strategies.

<https://fyvr.net/student-personas/adhd-hyperactive-impulsive-type>

Elementary (K–5)

A high-energy student who is out of their seat, calling out answers, grabbing materials from others, and struggling to wait in line. Often not malicious — simply cannot hold back. Teachers spend significant time redirecting this student, which affects the whole class.

What You Might Notice

- Frequently out of seat during work time; fidgets constantly when seated
- Calls out answers or comments without raising hand
- Difficulty waiting in line, for a turn, or during transitions
- Grabs, touches, or pushes peers impulsively without apparent intent to harm

Student Strengths

- Enthusiastic and energetic; brings excitement and spontaneity to the class
- Often the first to volunteer, engage, and try new things
- Genuine, open-hearted, and honest in relationships

Recommended Accommodations

- Provide a flexible seating option (wobble stool, standing desk, floor cushion)
- Use a movement pass — student can take a brief break without asking
- Allow fidget tools that don't distract others (fidget band on chair leg)
- Provide visual cue cards for classroom expectations (raise hand, stay seated)
- Give frequent, specific praise for on-task and prosocial behavior

Assistive Technology

- Visual timer displayed on classroom screen
- Vibrating smartwatch (e.g., Watchminder) for discreet reminders
- GoNoodle for whole-class movement breaks

UDL Strategies

- Incorporate movement into lessons (stand up to answer, move to stations)
- Use signal systems (thumbs up/down, whiteboard responses) instead of hand-raising
- Structure lessons with frequent transitions and activity changes every 10–15 minutes

IEP Considerations

- Behavior Intervention Plan (BIP) may be needed if behaviors are impeding learning
- Annual goal: Student will raise hand and wait to be called on in X out of Y observed opportunities
- Functional Behavioral Assessment (FBA) should precede any BIP
- ADHD must be documented as IDEA eligibility under OHI with documentation of educational impact

Who to Collaborate With

- School Psychologist (for FBA/BIP)
- Special Education Behavior Specialist

Family Partnership Tip

Make sure your child gets significant physical activity before school — even 15–20 minutes of running or play dramatically reduces hyperactivity in class.

Middle School (6–8)

A student whose hyperactivity may look different at this age — more internal restlessness, excessive talking, risky social behavior, and impulsive decision-making rather than running around. Peer relationships are strained by impulsive comments. Academic work suffers from impulsive errors.

What You Might Notice

- Talks excessively; dominates group conversations
- Makes impulsive social comments that alienate peers
- Rushes through work with careless errors; doesn't review before submitting
- Jumps into tasks before hearing full directions

Student Strengths

- Adventurous, funny, and entertaining — often a natural leader
- Quick to start tasks and generate ideas
- Highly engaged when activities involve energy, debate, or novelty

Recommended Accommodations

- Preferential seating away from high-traffic, high-distraction areas
- Teach and reinforce a stop-check-submit routine before turning in work
- Provide social coaching around turn-taking in conversation
- Allow brief movement breaks between periods or tasks
- Written reminders on desk for behavioral expectations

Assistive Technology

- Timer apps to pace work and build in review time
- Flipgrid or Padlet for channeling verbal energy into structured participation
- Calm or Headspace for self-regulation check-ins

UDL Strategies

- Provide discussion protocols that structure turn-taking (talking chips, timer-based sharing)

- Use project-based learning to channel high energy into meaningful tasks
- Build in brief physical activity transitions between academic segments

IEP Considerations

- BIP should shift focus from punishment to proactive strategies and skill-building
- Annual goal: Student will use a self-monitoring checklist to review work before submission on X% of assignments
- Social skills goals should be specific: taking turns in conversation, using appropriate volume
- Involve student in developing their own BIP strategies — buy-in is essential at this age

Who to Collaborate With

- School Counselor
- Special Education Behavior Specialist

Family Partnership Tip

Help your child learn the 'pause button' at home — practice a 3-second pause before responding in conversations. Name it together: 'Let's use the pause button.'

High School (9–12)

A student whose impulsivity now manifests in risky decision-making, social conflict, and academic self-sabotage (saying the wrong thing at the wrong time, getting in trouble when they were just being funny). May have discipline history that has eroded relationships with school adults.

What You Might Notice

- Gets in trouble for outbursts or comments that seemed impulsive, not malicious
- Has a discipline history that includes many minor incidents
- Rushes through tests and scores lower than ability would suggest
- Difficulty sustaining long projects; either abandons them or panics at the end

Student Strengths

- Charismatic, energetic, and genuinely engaging in the right context
- Takes initiative and gets things started quickly
- Often skilled in physical, hands-on, or performance-based activities

Recommended Accommodations

- Rebuild trust through restorative practices rather than punitive discipline
- Provide scripts for self-advocacy (how to excuse yourself when overwhelmed)
- Extended time on tests to allow review of impulsive errors
- Check-in/check-out behavioral support with a trusted adult
- Advance notice of agenda changes or high-stakes situations

Assistive Technology

- Brainscape or Anki flashcard apps for active study (prefer action-based learning)

- Revver or Loom for video-based project work (channels energy into creation)
- Mindfulness apps (Wim Hof, Headspace) for self-regulation

UDL Strategies

- Provide project-based, action-oriented assessments that match energy and strengths
- Use positive behavior contracts co-created with the student
- Teach self-monitoring and self-regulation skills explicitly in counseling or advisory

IEP Considerations

- FBA and BIP must be updated to reflect adolescent presentation of hyperactive-impulsive ADHD
- Transition goals: student will identify two self-regulation strategies to use in workplace or college settings
- Address any suspension/discipline record in the IEP — manifestation determination reviews are a right
- Consider whether restraint or seclusion has been used and ensure compliance with IDEA regulations

Who to Collaborate With

- Restorative Practices Coordinator
- School Counselor or Psychologist

Family Partnership Tip

When an incident happens at school, lead with curiosity rather than punishment at home: 'Tell me what happened from the beginning.' Getting the full story is step one.

Resources: CDC — ADHD in Schools | CHADD — Hyperactive ADHD | Understood — Hyperactive ADHD

ADHD - Combined Type

ADHD Combined Type includes significant symptoms of both inattention and hyperactivity-impulsivity. It is the most common ADHD presentation. These students are simultaneously distracted, restless, impulsive, and disorganized — a combination that creates substantial challenges across nearly every aspect of school. With the right supports, they can thrive.

<https://fyvr.net/student-personas/adhd-combined-type>

Elementary (K–5)

A student who is both easily distracted and physically restless. Moves around the classroom, blurts out answers, loses everything, forgets instructions, and has great difficulty completing work independently. Teachers often feel this student requires constant supervision.

What You Might Notice

- Frequently off-task AND out of seat during the same lesson
- Blurts out answers, interrupts, and struggles to wait
- Has poor follow-through on multi-step tasks
- Loses materials daily; homework is rarely completed and submitted

Student Strengths

- High energy and enthusiasm when engaged
- Creative, lateral thinking that generates novel ideas
- Genuine warmth and desire to connect with adults and peers

Recommended Accommodations

- Flexible seating with movement options (wobble stool, standing option)
- Visual daily schedule and task checklists
- Break tasks into very short segments with brief movement breaks
- Preferential seating near the teacher
- Behavior-specific praise and a simple token system for task completion

Assistive Technology

- Visual timer (Time Timer app or physical clock)
- ClassDojo for real-time positive feedback
- PBS Kids or ABCmouse apps for structured academic engagement

UDL Strategies

- Use active learning structures — minimize passive listening
- Provide predictable, visual-supported routines for every transition
- Offer choices in activities, seating, and response format to increase buy-in

IEP Considerations

- Combined type often requires both behavioral and academic goals
- Consider FBA if behaviors are significantly impacting learning or safety
- Annual goal (behavioral): Student will complete X% of in-class work using a visual checklist and movement break schedule
- Annual goal (academic): Specify targeted academic area (e.g., reading fluency, math facts)

Who to Collaborate With

- Special Education Teacher
- School Psychologist (for FBA)

Family Partnership Tip

Work with the teacher to use the same visual schedule structure at home that's used at school. Predictability reduces the constant negotiating that exhausts everyone.

Middle School (6–8)

A student whose combined type now looks like constant disorganization, social impulsivity, academic avoidance, and restlessness during longer class periods. The demands of navigating multiple teachers, subjects, and social dynamics all at once are overwhelming. Grades are inconsistent — strong on some days, missing work on others.

What You Might Notice

- Binder or locker is a chaotic mix of every subject
- Has the knowledge in class discussions but fails tests due to careless errors and rushing
- Gets in social trouble due to impulsive comments, then is distracted by the aftermath
- Homework completion is highly variable and unrelated to difficulty level

Student Strengths

- Entrepreneurial energy; loves leading projects and generating ideas
- Highly social; knows everyone and is well-liked when impulsivity is managed
- Excellent in project-based work when it involves physical or hands-on elements

Recommended Accommodations

- Daily teacher check-in (2 minutes at start or end of class)
- Consistent organizational system across all classes (same binder structure)
- Chunked assignments with milestone check-ins
- Extended time and reduced question sets on tests
- Written reminders of behavioral expectations posted visibly

Assistive Technology

- Google Classroom for all assignment tracking
- Remind app for teacher-to-student deadline nudges
- Pomodoro timer apps for work-session structure

UDL Strategies

- Provide project-based learning with real-world stakes
- Use interactive note-taking methods (Cornell notes, graphic organizers)
- Incorporate peer collaboration structures that leverage social energy productively

IEP Considerations

- Both organizational skills and behavioral regulation goals are typically needed
- Team coordination is essential — all teachers must implement accommodations consistently
- Annual goal: Student will use Google Classroom task list to submit X% of assignments on time with weekly case manager check-in
- Check-in/Check-out (CICO) behavioral support should be formalized in the BIP if behaviors are significant

Who to Collaborate With

- Case Manager (coordinating across all teachers)
- School Counselor

Family Partnership Tip

Have a standing 10-minute 'school download' each evening — not about homework, just about the day. This helps your student process what happened and lets you catch issues early.

High School (9–12)

A student at significant risk of failure or dropout if the right supports aren't in place. The combination of inattention and impulsivity at the high school level can derail credit accumulation, relationships, and self-concept. Also highly capable of success with the right environment, mentorship, and structures.

What You Might Notice

- Multiple incomplete or missing assignments across classes simultaneously
- Impulsive decisions that have led to school consequences or strained relationships
- Engages brilliantly in class discussion but can't transfer that to written assessments
- Bounces between hyperfocus on passions and complete avoidance of anything else

Student Strengths

- Visionary, entrepreneurial, often charismatic leader
- Intense passion and deep knowledge in areas of genuine interest
- Genuine relationships with teachers who 'get' them

Recommended Accommodations

- Mentoring relationship with one trusted adult who provides daily or weekly check-in
- Flexible deadline structures with agreed-upon plans rather than zero policies
- Extended time on all summative assessments
- Reduced homework volume with focus on in-class mastery
- Access to a quiet workspace for extended work sessions

Assistive Technology

- Notion for project management and deadlines
- Focusmate for virtual co-working accountability
- Canvas or Google Classroom with push notifications

UDL Strategies

- Allow student to connect coursework to personal projects or career interests
- Use contract grading or portfolio-based assessment to reward growth
- Build explicit self-regulation skill instruction into IEP services

IEP Considerations

- Transition plan must address both post-secondary environment (college vs. work) and self-management skills
- Annual goals should include self-advocacy: student will independently request accommodations in X out of Y situations
- Address grading policies in IEP — zero policies may be discriminatory for disability-related missed work
- Consider whether student has co-occurring anxiety, depression, or learning disabilities that need separate goals

Who to Collaborate With

- Guidance Counselor (transition and credit planning)
- Mentor teacher or advisory teacher

Family Partnership Tip

Help your student identify what conditions they focus best in — music or silence? Morning or evening? Short bursts or long sessions? Self-knowledge is the most powerful ADHD tool.

Resources: [CDC — ADHD Overview](#) | [CHADD — Combined Presentation](#) | [OSEP — ADHD & IDEA](#)

Executive Function Challenges

Executive function challenges affect the brain's management system: planning, prioritizing, initiating tasks, managing time, shifting between tasks, and regulating emotions. These difficulties can occur with or without a formal ADHD diagnosis. Students often know what they need to do but cannot get started, stay on track, or finish independently.

<https://fyvr.net/student-personas/executive-function-challenges>

Elementary (K–5)

A student who seems capable but cannot get started on tasks, transitions poorly between activities, loses track of steps in routines, and falls apart emotionally when plans change unexpectedly. Teachers may label them as lazy or oppositional when the real issue is neurological.

What You Might Notice

- Sits with a blank page long after classmates have begun
- Struggles with transitions; needs extra prompting to shift activities
- Forgets steps of multi-part routines even after repeated practice
- Has emotional reactions disproportionate to task difficulty when stuck

Student Strengths

- Strong ideas when given a structured starting point
- Creative thinker who benefits enormously from scaffolding
- Warm, relational learner who responds well to supportive adult connection

Recommended Accommodations

- Provide first-step prompts for all tasks ('Start by writing your name and date')
- Use visual task lists broken into very small steps
- Warn about transitions 5 minutes in advance
- Establish predictable, consistent daily routines
- Offer choice within structure to build initiation skills

Assistive Technology

- iPrompts or First-Then visual schedule apps
- Time Timer for transition warnings
- Seesaw for structured task submission with scaffolded prompts

UDL Strategies

- Front-load the 'what to do first' for every assignment and transition
- Use anchor activities — consistent, known tasks students can begin independently
- Teach planning strategies using think-aloud and worked examples

IEP Considerations

- Executive function goals can be included in IEPs under multiple eligibility categories
- Annual goal: Student will independently begin assigned tasks within 2 minutes of direction using a visual task card on X% of opportunities
- Occupational therapy evaluation may be warranted for task initiation and planning
- Distinguish executive function challenges from anxiety, oppositional behavior, or intellectual disability in evaluation

Who to Collaborate With

- Occupational Therapist
- Special Education Teacher

Family Partnership Tip

At home, use 'first-then' language for all tasks: 'First shoes and backpack, then breakfast.' Visual first-then boards significantly reduce morning battles.

Middle School (6–8)

A student whose difficulty with planning, prioritizing, and initiating is now becoming academically devastating. Long-term projects are left until the last night. Study habits are nonexistent. The student may be smart and self-aware but unable to bridge the gap between knowing and doing.

What You Might Notice

- Cannot break large projects into steps independently; needs explicit coaching
- Studies for tests by rereading rather than using active strategies
- Has difficulty shifting attention when a task changes unexpectedly
- Homework is either perfect and late or incomplete and on time — rarely both

Student Strengths

- Deep thinker who produces excellent work when adequately scaffolded
- Self-aware about challenges when given language to name them
- Strong in individual projects with clear structure and regular check-ins

Recommended Accommodations

- Backward planning calendar provided for every project over one week
- Weekly goal-setting meeting with teacher or case manager
- Checkpoint grades at milestone stages of projects
- Reduce transitions between unrelated tasks within a class period
- Extended time on assessments that require planning and organization

Assistive Technology

- Trello or Asana for project tracking with due dates
- Showbie or Seesaw for structured assignment submission
- Reminders app synced with parent for homework tracking

UDL Strategies

- Teach and model backwards planning explicitly for every major unit project
- Provide planning templates as a permanent accommodation — never a one-time scaffold
- Use metacognitive journaling: 'What's my plan for today?'

IEP Considerations

- Executive function goals should be specific and measurable: planning, initiation, task completion
- Annual goal: Student will create and follow a backward-planned timeline for projects of more than 3 days, meeting X% of intermediate deadlines
- Consider whether school-based counseling focused on EF skill-building is appropriate
- Homework accommodation language: reduced volume, alternative submission formats, deadline flexibility

Who to Collaborate With

- Case Manager
- School Counselor

Family Partnership Tip

Every Sunday evening, spend 10 minutes looking at the week ahead together — pull up Google Classroom, check the backpack, and name one thing due each day. Predictability is the antidote to EF paralysis.

High School (9–12)

A student who understands material deeply but whose executive function challenges create a growing gap between potential and performance. College applications, long research papers, and senior projects are particularly challenging. Urgently needs tools and strategies they can use independently after graduation.

What You Might Notice

- Struggles significantly with the college application process (initiation, organization)
- Long-term projects are often submitted late, incomplete, or in the wrong format
- Appears capable in class discussion but performance on take-home work doesn't reflect it
- May have adopted learned helplessness — stops asking for help because it hasn't worked

Student Strengths

- Excellent analytical thinking when focused and supported
- Strong self-awareness developed through years of struggle with EF
- Highly creative and capable when given structure and a trusted adult relationship

Recommended Accommodations

- Individualized project timelines with negotiated milestone deadlines
- Weekly 1:1 check-in with case manager or advisor
- Late submission policy that allows work without catastrophic grade loss
- Reduced homework with mastery demonstrated in other ways
- Quiet, structured workspace available during free periods

Assistive Technology

- Notion for comprehensive personal organization system
- Cal Newport's time-blocking method implemented via Google Calendar
- Focusmate or similar for accountability work sessions

UDL Strategies

- Teach EF strategies as explicit curriculum: planning, prioritizing, task initiation
- Allow portfolio-based assessment to replace single high-stakes projects
- Build regular reflection into all courses: 'What's working? What needs adjustment?'

IEP Considerations

- Transition plan must explicitly address executive function skill-building for independence
- Annual goal: Student will independently create a project management plan for assignments over 5 days, using a digital tool, with minimal adult prompting
- Consider whether student needs EF coaching as a related service (some districts provide this)
- Document grading accommodations to prevent zeros for disability-related late work

Who to Collaborate With

- Transition Coordinator
- College counselor or advisor

Family Partnership Tip

Help your student build an 'external brain' — a single trusted system (notebook, app, board) that captures everything. Finding what works for them is the most important EF investment.

Resources: [CAST — Executive Function & UDL](#) | [Understood — Executive Function](#) | [Harvard — Executive Function](#)

Processing Speed Differences

Processing speed differences refer to a significantly slower rate at which a student takes in, integrates, and responds to information. Students with slow processing speed are not less intelligent — they simply need more time to complete cognitive tasks. They are often penalized by timed tests and fast-paced instruction, creating a persistent and unfair performance gap.

<https://fyvr.net/student-personas/processing-speed-differences>

Elementary (K–5)

A thoughtful, careful student who is always the last to finish tasks. While the class moves on, this student is still working. They do excellent work when given enough time but consistently appear behind or incomplete. Often mistaken for a struggling student when they are simply a slow but accurate worker.

What You Might Notice

- Consistently last to finish — by a significant margin
- Copying from the board is labored; falls far behind during note-taking
- Transitions between tasks are slow; takes extra time to put materials away and get ready
- Produces high-quality work but requires more time than peers to do so

Student Strengths

- Careful, accurate, and thorough when given sufficient time
- Deep thinker who considers multiple angles before responding
- Often highly persistent and hardworking

Recommended Accommodations

- Extended time (1.5x–2x) on all timed tasks and assessments
- Reduce copying requirements; provide printed materials
- Allow work to be finished in a subsequent period or at home without penalty
- Reduce the quantity of problems while maintaining difficulty level
- Provide advance copies of materials to preview before instruction

Assistive Technology

- Text-to-speech for reading tasks to reduce processing load
- Word prediction to speed up written output
- Pre-loaded graphic organizers to reduce setup time on writing tasks

UDL Strategies

- Avoid cold-calling; use think-pair-share with processing time built in
- Provide wait time (10+ seconds) before expecting responses
- Offer choice of quantity: 'Do 5 of these 10 problems to show you understand the concept'

IEP Considerations

- Processing speed should be documented from cognitive assessment (Coding, Symbol Search on WISC)
- Extended time is a primary accommodation — must be explicitly stated as 1.5x or 2x
- Annual goal should address functional task completion rate, not just speed
- Note: processing speed is not a diagnostic category by itself — identify the underlying eligibility (e.g., SLD, OHI)

Who to Collaborate With

- School Psychologist (for assessment interpretation)
- Special Education Teacher

Family Partnership Tip

Never rush your child through homework. A slower pace does not mean worse work — it often means better work. Protect extended time at home just as the IEP protects it at school.

Middle School (6–8)

A student who is increasingly penalized by the pace of middle school instruction — faster transitions, more note-taking, timed assessments, and less adult support. The gap between their knowledge and their demonstrated performance grows as the curriculum accelerates. Frustration and self-doubt often emerge.

What You Might Notice

- Notes are incomplete at end of class; misses key information
- Timed tests significantly underestimate knowledge compared to untimed work
- Transitions between classes cause delays that result in tardiness or disorganization
- Has detailed, accurate understanding of material but cannot demonstrate it quickly

Student Strengths

- Deep, careful thinker who produces excellent work when time is protected
- Persistent and determined; rarely gives up
- Strong verbal understanding when instruction is paced appropriately

Recommended Accommodations

- Extended time on all tests and quizzes
- Provide teacher-generated notes or note-taking templates
- Allow recording of lectures for review
- Reduce total number of homework problems while maintaining depth
- Advance notice of timed activities so student can prepare strategies

Assistive Technology

- Otter.ai or Microsoft Transcribe for lecture recording and transcription
- Notability for annotating pre-loaded slides and notes
- Snap Type Pro for taking photos of board work instead of copying

UDL Strategies

- Post all materials digitally before class so students can follow along rather than copy
- Use asynchronous options where possible (record instruction for review)
- Explicitly teach note-taking shortcuts and abbreviations

IEP Considerations

- Accommodation language: 'extended time at 1.5x standard; may complete in next available period'
- Address note-taking as a specific accommodation — teacher notes, pre-filled outlines, or peer notes
- Goal may address self-advocacy: student will independently request extended time in X out of Y situations
- State testing accommodations must be documented and applied for annually in most states

Who to Collaborate With

- Case Manager (coordinating across all teachers)
- All content-area teachers (consistent implementation is key)

Family Partnership Tip

Help your student build a 'done is better than perfect' mantra — perfectionism combined with slow processing is a recipe for paralysis. Partially done, submitted work is better than perfect, unfinished work.

High School (9–12)

A student whose processing speed differences now significantly affect their performance on AP exams, SAT/ACT, and timed in-class assessments. They may have strong knowledge but face the repeated experience of not finishing tests. This has significant impact on grades, confidence, and college planning.

What You Might Notice

- Consistently doesn't finish timed tests; last several questions are blank
- AP exam performance doesn't reflect class performance
- Takes significantly longer on every task — homework, notes, essays
- Understands and retains material well but underperforms under time pressure

Student Strengths

- Thorough, accurate, and deeply reflective work when time is protected
- Strong comprehension and retention of material
- Highly self-aware and motivated; advocating for time is a skill they've developed

Recommended Accommodations

- Extended time (typically 1.5x–2x) on all assessments
- Access to untimed or extended-time College Board accommodations
- Reduced homework volume without grade penalty
- Flexible test-taking schedule (complete across two periods if needed)
- Pre-provided outlines, templates, and notes for all classes

Assistive Technology

- Read&Write; Gold for all reading and writing tasks

- Otter.ai for all lectures and note review
- Dragon NaturallySpeaking for faster written output

UDL Strategies

- Eliminate unnecessary timed conditions that don't reflect learning goals
- Use mastery-based assessment with multiple submission opportunities
- Teach test-taking strategies: skip, return, prioritize high-value items

IEP Considerations

- College Board and ACT accommodations require documentation — begin the application process by junior year
- Transition goals: student will independently use and advocate for extended time in all post-secondary settings
- Annual goal should document progress in self-advocacy, not just academic skills
- Ensure extended time is consistently applied on ALL tests — partial accommodation is inequitable

Who to Collaborate With

- Guidance Counselor (for testing accommodation applications)
- Case Manager

Family Partnership Tip

Help your student apply for extended time on the SAT/ACT early — the documentation process can take months. Start conversations with the school psychologist in 10th grade.

Resources: [Understood — Processing Speed](#) | [LD Online — Processing Speed](#) | [NCLD — Processing Speed](#)

Autism Spectrum

Autism - High Support Needs

Students with autism requiring high support have significant challenges with communication, daily living skills, behavior regulation, and sensory processing that require intensive, individualized educational supports throughout the school day. Many use AAC devices or have limited spoken language. Learning occurs best in highly structured, predictable, low-stimulation environments with consistent adult support.

<https://fyvr.net/student-personas/autism-high-support-needs>

Elementary (K–5)

A student who may have limited spoken language or use an AAC device, who requires a one-on-one paraprofessional for most of the day, and who benefits from a highly structured, visual schedule in a small, calm environment. Transitions and unexpected changes frequently trigger significant behavioral responses.

What You Might Notice

- Limited or absent spoken language; may use AAC device, PECS, or gestures
- Engages in repetitive or self-stimulatory behaviors (rocking, hand-flapping, vocalizing)
- Significant distress during unexpected transitions or changes in routine
- Limited eye contact and social-communicative initiation with peers or adults

Student Strengths

- Often has strong visual memory and pattern recognition
- Can demonstrate learning in alternative ways (matching, sorting, pointing) that may not be captured in verbal assessments
- Deep focus and engagement with preferred topics or activities

Recommended Accommodations

- Individualized visual schedule using photographs or symbols updated daily
- Designated quiet space for sensory regulation breaks
- Consistent daily routine with minimal unexpected changes; advance visual preparation for changes
- AAC device or PECS system accessible at all times
- Paraprofessional support throughout the day with planned fading where appropriate

Assistive Technology

- Proloquo2Go or TouchChat for AAC
- PECS (Picture Exchange Communication System)
- First-Then visual schedule apps (iPrompts, ChoiceWorks)

UDL Strategies

- Use task analysis and chaining to break all academic and life skills into small, teachable steps
- Incorporate sensory preferences and high-interest topics as motivators

- Use visual supports consistently for all instructions and expectations

IEP Considerations

- IEP must include functional communication goals — this is a primary focus
- Annual goal (communication): Student will use AAC device to make a request or comment in X out of Y opportunities across settings
- Behavior goals should be based on FBA and address replacement behaviors, not just reduction of challenging behavior
- Extended School Year (ESY) services should be considered to prevent regression

Who to Collaborate With

- Speech-Language Pathologist (AAC focus)
- Board-Certified Behavior Analyst (BCBA) or Behavior Specialist

Family Partnership Tip

Use the same visual schedule and communication system at home that's used at school. Consistency across environments is the single most powerful thing families can do.

Middle School (6–8)

A student who benefits from a special day class or substantially separate placement with integrated inclusion opportunities in preferred, structured settings. Daily living skills, communication, and behavior regulation are primary educational goals alongside modified academic content.

What You Might Notice

- Requires significant adult support to navigate school building and routines
- Communication is supported through AAC, sign, or limited verbal output
- May have challenging behaviors (aggression, self-injury, elopement) that require a safety plan
- Peer interactions are limited but meaningful connections are possible with structured support

Student Strengths

- Often responds well to preferred adults who understand their communication
- Can make meaningful choices and demonstrate preferences clearly
- May have specific areas of strength or strong procedural memory for known routines

Recommended Accommodations

- Modified curriculum aligned with grade-level themes but at individual functional level
- Structured, consistent special day class environment with predictable schedule
- Sensory diet developed with OT and embedded throughout the day
- Crisis/safety plan in place for behaviors that pose safety concerns
- Supported inclusion in activities where participation is meaningful (electives, lunch, specials)

Assistive Technology

- High-tech AAC updated with vocabulary relevant to current curriculum

- Adapted iPad apps for access to modified curriculum content
- Video modeling for daily living and social routines

UDL Strategies

- Embed instruction in natural routines and contexts rather than isolated drill
- Use systematic instruction with data collection on all IEP goals
- Provide choice-making opportunities as a core instructional strategy

IEP Considerations

- IEP must address all IDEA areas: academic, communication, social/behavioral, daily living, transition (beginning at 16 or earlier if appropriate)
- Present levels should include data from multiple environments: school, home, community
- Placement decisions must follow LRE mandate — justify any restrictive placement in writing
- Begin transition planning focused on daily living skills, community access, and post-school environments

Who to Collaborate With

- Special Education Teacher (special day class)
- SLP, OT, BCBA (full multidisciplinary team)

Family Partnership Tip

Bring your knowledge of your child's communication to every IEP meeting. You know what 'that look' means — teach the team your child's communication signals.

High School (9–12)

A student for whom transition planning is now the central focus of education. The IEP team is working toward the most independent and fulfilling post-school life possible, which may include supported employment, residential options, and community participation. Academic goals are functional and community-referenced.

What You Might Notice

- Requires high support to navigate community and workplace environments
- Communication needs remain significant; AAC or alternative communication is ongoing
- Daily living skills (hygiene, meal preparation, money management) are active instructional areas
- Social behaviors in work and community settings are an active focus of instruction

Student Strengths

- Has established meaningful relationships with familiar adults and some peers
- Shows preferences and makes choices that should guide transition planning
- Has developed some predictable routines that support independence in specific tasks

Recommended Accommodations

- Community-based instruction (CBI) as part of the school program
- Job sampling and vocational exploration with supported experiences
- Daily living skills instruction embedded in natural settings

- Continued paraprofessional and adult support with systematic fading plan
- Safety planning for community navigation

Assistive Technology

- AAC continues as primary communication system
- Visual schedule apps for independent navigation of daily routines
- Task analysis apps (Job Coach, Ablenet) for supported employment

UDL Strategies

- All instruction should be functional and community-referenced
- Use self-determination curriculum (e.g., *Whose Future Is It Anyway?*) adapted to support level
- Center student's preferences and choices in all planning

IEP Considerations

- Transition planning is legally required by age 16 (or earlier in many states)
- Student must be invited to their own IEP meeting; participation adapted to communication needs
- Coordinate with adult services agencies (DDS, vocational rehabilitation) well before age 22
- Annual goal: Student will complete X vocational task steps with Y level of prompting in community/work setting

Who to Collaborate With

- Transition Specialist
- Adult services agency (DDS, VR) representative

Family Partnership Tip

Start researching adult services now — waitlists for adult disability services can be years long. Your IEP team can connect you with the right agencies before graduation.

Resources: [Autism Speaks — School Support](#) | [OSEP — Autism & IDEA](#) | [AANE — Autism in Education](#)

Autism - Moderate Support Needs

Students with autism requiring moderate support have challenges in communication, social interaction, and behavioral regulation that require substantial daily support but allow for meaningful participation in general education settings with appropriate accommodations. They often have functional speech but struggle with the pragmatics of communication, social reciprocity, and sensory regulation in busy school environments.

<https://fyvr.net/student-personas/autism-moderate-support-needs>

Elementary (K–5)

A student who has functional speech but struggles with conversation, peer interaction, and adapting behavior to social expectations. Routines are critical — changes trigger distress. May have significant sensory sensitivities. Participates in general education with paraprofessional support and pull-out services.

What You Might Notice

- Speaks in full sentences but conversation is one-sided or scripted
- Significant distress during unstructured time (recess, lunch, transitions)
- Strong preferences for specific topics; difficulty engaging with others' interests
- Sensory overload in loud, bright, or crowded settings

Student Strengths

- Deep knowledge and enthusiasm about areas of special interest
- Often very honest, rule-following, and consistent
- Strong visual memory and recall of facts

Recommended Accommodations

- Visual daily schedule updated with any changes in advance
- Access to quiet, calm space during high-stimulation times (lunch, recess)
- Pre-teach transitions with verbal and visual advance notice
- Social skills group for structured peer interaction practice
- Sensory accommodations (noise-canceling headphones, lighting adjustments)

Assistive Technology

- ChoiceWorks or iPrompts visual schedule apps
- Noise-canceling headphones for high-stimulation environments
- Social stories apps (Stories2Learn)

UDL Strategies

- Use student's special interests as entry points for all academic learning
- Provide visual and written instructions alongside verbal ones
- Teach social expectations explicitly — do not assume neurotypical social scripts transfer

IEP Considerations

- IEP should include social communication goals as a primary focus
- Annual goal: Student will initiate a topic-appropriate comment or question with a peer in X out of Y structured opportunities
- Sensory needs should be addressed in the IEP with an OT-developed sensory plan
- Consider the least restrictive environment — partial general education inclusion with support is often appropriate

Who to Collaborate With

- Speech-Language Pathologist
- Occupational Therapist

Family Partnership Tip

Practice 'conversation turns' at home using your child's favorite topic — even if it's train schedules. Two turns each, then switch. It builds the back-and-forth skill in a safe, motivating context.

Middle School (6–8)

A student who is increasingly aware of social differences and may be experiencing anxiety, bullying, or withdrawal. The social complexity of middle school is genuinely hard. Academic instruction continues in a blend of general education and resource/specialized settings.

What You Might Notice

- Increasing social isolation as peer expectations become more complex
- Rigid thinking patterns; difficulty with ambiguous or changing expectations
- May mask in general education (appears fine) but 'unravels' at home
- Struggles with open-ended tasks that lack clear right/wrong answers

Student Strengths

- Loyal, consistent, and deeply principled
- Often has advanced knowledge in one or two subject areas
- Strong factual memory and rule-following in structured environments

Recommended Accommodations

- Safe, structured lunch or break option (lunch bunch, library pass)
- Advance notice of all schedule changes
- Explicit rubrics and clear expectations for all assignments
- Access to social emotional support from school counselor
- Reduced written output requirements for tasks requiring open-ended elaboration

Assistive Technology

- Zones of Regulation app for emotional check-ins
- Google Calendar for predictable schedule visualization
- Text-based communication options for self-advocacy (email to teacher)

UDL Strategies

- Provide predictable lesson structures; reduce ambiguity in all tasks
- Use explicit social narratives for middle school social situations
- Create structured peer interaction formats (book clubs by interest, science lab partnerships)

IEP Considerations

- Address bullying and social victimization explicitly in the IEP — it is an educational access issue
- Annual goal: Student will use a regulation strategy before escalating in X out of Y observed opportunities
- Counseling as related service should be on the IEP
- Re-evaluate LRE at each annual IEP — middle school demands may shift appropriate placement

Who to Collaborate With

- School Counselor
- SLP (for social communication)

Family Partnership Tip

If your child says nothing went wrong at school but has a meltdown every evening, that's 'masking' — they're holding it together all day and releasing at home. This is exhausting for them. Let them decompress before asking about school.

High School (9–12)

A student who may be able to participate in many general education courses with supports but needs significant transition planning around independent living, employment, and social connections beyond high school. The gap between intellectual ability and adaptive behavior often widens at this stage.

What You Might Notice

- Participates in some general education courses but requires substantial support
- Social relationships are few but meaningful
- Executive function challenges around complex, multi-step tasks
- Strong knowledge base in areas of interest that can translate to career pathways

Student Strengths

- Reliability, consistency, and depth in areas of genuine interest
- Honest, direct communication style valued in many work environments
- Strong procedural memory for learned routines and systems

Recommended Accommodations

- Explicit transition curriculum addressing daily living, employment, and self-advocacy
- Job shadowing and vocational exploration in areas of interest
- Counseling focused on self-understanding and advocacy
- Clear, explicit instructions and expectations for all tasks
- Check-in support from trusted adult daily

Assistive Technology

- Google Calendar with detailed daily schedule
- Job navigation apps (Jobber, task management tools)
- Social skills programs (PEERS curriculum adapted for this profile)

UDL Strategies

- Connect all academic content to post-school goals and personal interests
- Teach self-advocacy explicitly: how to disclose, how to ask for help
- Build structured social opportunities around shared interests (clubs, interest groups)

IEP Considerations

- Transition plan should include measurable post-secondary goals in all three areas: education/training, employment, independent living
- Summary of Performance must be prepared before exit
- Coordinate with VR (vocational rehabilitation) and adult services before age 21
- Annual goal: Student will independently use a daily schedule app to complete X routine tasks with minimal prompting

Who to Collaborate With

- Transition Specialist
- VR Counselor

Family Partnership Tip

Begin visiting adult services programs and supports now — don't wait until graduation. The transition from school to adult services is the hardest transition of all, and preparation takes years.

Resources: [Autism Speaks — School Support](#) | [CDC — Autism in Schools](#) | [OSEP — Autism & IDEA](#)

Autism - Lower Support Needs (Asperger Profile)

Students with autism requiring lower support (formerly known as Asperger syndrome) are often highly verbal, intellectually capable, and appear to 'pass' in typical classroom settings — yet struggle significantly with social-emotional demands, sensory sensitivity, rigidity of thinking, and the unwritten social rules of school. Their disability is often invisible until a meltdown or crisis makes it suddenly apparent.

<https://fyvr.net/student-personas/autism-lower-support-needs-asperger-profile>

Elementary (K–5)

A highly verbal, often academically advanced student who struggles with peer relationships, flexibility, and the social rules of classroom life. May be seen as a 'little professor' — talks at length about their special interest but misses social cues. Appears 'fine' academically but emotionally fragile.

What You Might Notice

- Socially isolated; approaches peers in ways that are misread or rejected
- Insists on specific routines; becomes distressed by minor changes
- Monologues about special interest topics without reading peer disinterest
- Black-and-white thinking; argues about rules, fairness, and procedures

Student Strengths

- Often academically advanced, especially in areas of special interest
- Exceptional memory for factual information
- Deeply honest and committed to fairness and doing the 'right thing'

Recommended Accommodations

- Advance notice of any routine changes
- Social scripts and explicit teaching of playground/lunchroom social rules
- Flexible grace for minor rule infractions that trigger disproportionate distress in others
- Quiet lunch option or structured social lunch group
- Clear, explicit behavioral expectations with written reminders

Assistive Technology

- Social stories (created with Social Story Creator app)
- Visual schedule for daily routine
- Zones of Regulation emotional check-in tool

UDL Strategies

- Leverage special interests as entry points for all subject areas
- Teach hidden social curriculum explicitly: why groups work the way they do
- Use cooperative learning with very explicit roles and expectations

IEP Considerations

- Even high-functioning students with autism may qualify for and benefit from IEP services
- Social skills goals should be the primary focus, with counseling as a related service
- Annual goal: Student will use a calm-down strategy before escalating in X out of Y situations
- Consider whether a 504 plan is more appropriate if academic impact is minimal

Who to Collaborate With

- School Counselor
- SLP (for social communication and pragmatics)

Family Partnership Tip

After school, give your child 30–60 minutes of decompression time with no demands — they've been working incredibly hard all day to hold it together socially. The explosion after school is not about you.

Middle School (6–8)

A student who may have cruised through elementary school academically but is now struggling intensely with the social complexity of middle school. May be bullied, isolated, or increasingly anxious. Academic performance may decline as emotional and social overwhelm grows. Often misunderstood by teachers as arrogant or defiant.

What You Might Notice

- Increasing anxiety; may be experiencing depression or school avoidance
- Frustration and argumentation with teachers over perceived unfairness
- Social relationships are surface-level or nonexistent; potential for bullying victimization
- Hyperfocus on academics or special interest as an escape from social stress

Student Strengths

- Often highly capable in specific subjects — a genuine expert
- Creative, original thinker who approaches problems differently
- Loyal and deeply genuine in relationships they do form

Recommended Accommodations

- Structured peer group or interest-based club with adult facilitation
- Daily check-in with trusted adult (counselor, case manager)
- Permission to leave stressful situations with a break pass
- Explicit advance notice of group projects, partners, and class changes
- Flexibility on social performance-based tasks (oral presentations with smaller audience)

Assistive Technology

- Private journal or note-taking app for emotional processing
- PEERS social skills program (structured, evidence-based)
- Typed communication options for self-advocacy with teachers

UDL Strategies

- Provide student choice in how they demonstrate social learning

- Use written communication channels for students who struggle verbally in conflict situations
- Reduce public-facing social demands on already-overwhelmed students

IEP Considerations

- Evaluate for co-occurring anxiety — very common in this profile
- Annual goal: Student will identify and use two coping strategies when experiencing social frustration
- Address bullying and peer victimization explicitly as an educational access issue
- Counseling focused on social understanding and emotional regulation is often critical

Who to Collaborate With

- School Counselor
- Outside mental health provider (if anxiety/depression co-occur)

Family Partnership Tip

Connect your student with online communities of autistic teens with shared interests — these connections can be lifelines for students who struggle with in-person peer relationships.

High School (9–12)

A student who is often academically successful but emotionally fragile, socially isolated, and increasingly aware of the gap between themselves and peers. May be highly motivated to understand their own brain and neurology. College is often a strong possibility; self-advocacy and disability disclosure skills are critical.

What You Might Notice

- Academic performance is strong in areas of interest; avoidant in others
- Very few or no peer relationships; may seek adult conversation instead
- Rigidity, perfectionism, and anxiety are significant and growing
- Deep interest in understanding autism and their own neurology

Student Strengths

- Often intellectually gifted in specific domains
- Highly motivated, self-directed learner in areas of passion
- Developing self-awareness and capacity for self-advocacy

Recommended Accommodations

- Extended time on all assessments
- Flexible deadlines on work that requires ambiguous open-ended judgment
- Private space for stress decompression during the day
- Written instructions and expectations for all tasks
- Permission to opt out of certain social activities with an alternative

Assistive Technology

- Grammarly and structured essay templates for written work
- Calendar and task management for executive function support

- Reddit communities and autistic-affirming online spaces for peer connection

UDL Strategies

- Teach self-advocacy through explicit practice and role-play
- Offer independent study options in areas of deep interest
- Build in self-reflection and portfolio-based assessment options

IEP Considerations

- Transition goals must include disability disclosure and self-advocacy in college settings
- Annual goal: Student will independently disclose disability and request accommodations in X simulated post-secondary situations
- Summary of Performance must be strong and specific for college disability services offices
- Consider connecting student with autistic self-advocates and role models as part of transition

Who to Collaborate With

- Guidance Counselor (college planning)
- Mentor with autistic identity/lived experience if possible

Family Partnership Tip

Give your student language for their autism if they don't have it yet — autistic identity can be a source of pride, not shame. Books by autistic authors can be transformative.

Resources: [AANE — Asperger/Autism Profiles](#) | [Understood — Autism Spectrum](#) | [Autism Speaks — IEP Guide](#)

PDA Profile (Pathological Demand Avoidance)

PDA is an autism profile characterized by an extreme avoidance of everyday demands driven by high anxiety rather than willful defiance. PDA students have a profound need for control and autonomy, and typical compliance-based behavioral strategies frequently backfire or escalate situations. Effective support relies on low-demand, collaborative, trust-based relationships.

<https://fyvr.net/student-personas/pda-profile-pathological-demand-avoidance>

Elementary (K–5)

A student who appears to refuse nearly everything — but whose avoidance is anxiety-driven, not defiant. Traditional reward/consequence systems don't work; in fact, they often make things worse. This student needs flexible, collaborative, relationship-based support. They are often perceptive, creative, and socially aware — but overwhelmed.

What You Might Notice

- Refuses demands that seem simple or reasonable, including things they enjoy
- Uses delay, distraction, negotiation, and social excuses to avoid tasks
- Escalates rapidly when pressured or cornered — may have significant meltdowns
- Defies expectations even when they appear calm and willing moments earlier

Student Strengths

- Highly perceptive about social dynamics and others' emotions
- Creative, imaginative, and often has a strong sense of humor
- Deeply empathetic when not dysregulated

Recommended Accommodations

- Reduce language of demand: offer choices and collaboration, not commands
- Give genuine choice and control wherever possible in daily routine
- Avoid confrontational tone and power struggles; use indirect suggestion
- Create a demand-reduced safe space for regulation breaks
- Prioritize the relationship over compliance — trust is the foundation

Assistive Technology

- Flexible, student-directed use of apps and technology as engagement tools
- Visual choice boards that present options without framing as demands

UDL Strategies

- Frame all learning as collaborative and exploratory, not required
- Use student's own projects and interests as the curriculum wherever possible
- Provide ways for students to say no or pause without consequence

IEP Considerations

- PDA is not in DSM-5 as a standalone diagnosis — document under autism (ASD) with PDA profile noted in present levels
- Traditional BIPs based on token economy/compliance will not be effective — document a collaborative, low-demand approach
- Annual goal should focus on self-regulation and communication of needs, not compliance
- The team must understand PDA — educator training is critical before any behavioral intervention

Who to Collaborate With

- School Psychologist (for team education on PDA)
- BCBA trained in PDA-informed approaches

Family Partnership Tip

Read PDA-specific resources (PDA Society, Kristy Forbes) before applying any standard autism or ADHD strategies. PDA requires a fundamentally different approach — and what works can be counterintuitive.

Middle School (6–8)

A student whose avoidance has often been met with escalating consequences that have created a cycle of crisis and relationship breakdown. May have a significant discipline history rooted in behavior that is disability-related. School may feel like a threatening environment.

What You Might Notice

- School refusal or partial attendance due to anxiety
- Escalation when pushed — even in subtle ways (lowered voice, eye contact)
- Uses masking, excuses, and social manipulation to avoid demands
- May be seen as manipulative when they are actually dysregulated and terrified

Student Strengths

- Remarkable social perception and emotional intelligence
- Highly creative and motivated when given autonomy
- Deep loyalty to people who respect their autonomy and trust them

Recommended Accommodations

- Individualized, negotiated school day rather than standard schedule
- One trusted key adult who has primary relationship — not a team of ten
- Flexibility on attendance, location, and timing of all activities
- Completely eliminate punitive behavioral consequences
- Create a written collaborative plan with the student about what school looks like for them

Assistive Technology

- Independent, self-paced learning platforms (Khan Academy, Coursera Jr.)
- Communication apps for student to signal state without verbal demands

UDL Strategies

- Remove overt demand structure entirely where possible
- Gamify and frame all tasks as optional explorations
- Let student lead the direction of learning activities when possible

IEP Considerations

- Manifestation determination is critical — most PDA behaviors ARE manifestations of the disability
- School refusal should be addressed as an anxiety/autism accommodation issue, not a truancy issue
- Annual goal: Student will communicate needs/concerns to a trusted adult using agreed-upon method in X situations
- Team must commit to a low-demand, collaborative approach in writing as an IEP accommodation

Who to Collaborate With

- School Psychologist
- Outside therapist with PDA expertise

Family Partnership Tip

Your intuition that standard approaches don't work for your child is correct. Seek out PDA-specific resources and connect with other PDA families — the mainstream autism advice often makes PDA worse.

High School (9–12)

A student who may be barely attending school, may be home/hospital-based, or may be succeeding in a radically flexible alternative setting. The traditional school environment is often incompatible with severe PDA. Transition planning must center autonomy and meaningful agency over compliance-based employment and education.

What You Might Notice

- Very limited school attendance; school refusal is chronic
- Engages deeply in self-directed projects outside of school
- Significant anxiety and possible co-occurring mental health diagnoses
- Has strong ideas about their own future that may not match conventional pathways

Student Strengths

- Highly capable and often deeply motivated in self-chosen pursuits
- Creative entrepreneur who may find non-traditional paths very successful
- Has developed exceptional self-knowledge about their own needs

Recommended Accommodations

- Individualized or distance-based diploma option
- Portfolio-based or project-based credit earning as an alternative to traditional courses
- Highly flexible attendance and assessment timeline
- One trusted adult relationship with zero power-over dynamic
- Transition plan centered on autonomy, self-employment, or passion-driven career

Assistive Technology

- Independent online learning platforms for self-paced credit completion
- Project management and portfolio tools for alternative assessment documentation

UDL Strategies

- All learning should be self-directed and demand-free wherever legally possible
- Use interest-driven projects as the primary pathway to academic credit
- Honor the student's vision for their post-school life, however unconventional

IEP Considerations

- Consider alternative diploma pathways, independent study programs, or online school
- Transition plan must center on student's own goals and be co-created WITH the student
- Annual goal: Student will independently identify and pursue one post-school interest or skill area
- Document clearly why traditional behavioral approaches are contraindicated for this student

Who to Collaborate With

- Alternative Education Coordinator
- Outside mental health and PDA specialist

Family Partnership Tip

Your student's future may not look like college and traditional employment — and that's okay. Building on their genuine passions in an autonomy-respecting environment is the path forward.

Resources: [PDA Society — Educators](#) | [Autistic UK — PDA Overview](#) | [Understood — Demand Avoidance](#)

Twice-Exceptional Autism

Twice-exceptional (2e) students with autism are simultaneously gifted in some areas and have significant disabilities in others. Their high intellectual ability can mask their autism-related challenges, and their autism can mask or complicate recognition of their giftedness. They are frequently misunderstood, underserved, and at risk of both under-challenge and under-support.

<https://fyvr.net/student-personas/twice-exceptional-autism>

Elementary (K–5)

A student who is reading chapter books at age 6 but melting down over the texture of their socks. Intellectually advanced but emotionally and sensorially overwhelmed. Often misidentified as merely 'gifted' or merely 'behavioral' — rarely as both at once.

What You Might Notice

- Advanced academic performance in some domains alongside significant behavioral challenges
- Intense intellectual interests alongside rigid and inflexible behavior
- Strong factual knowledge but poor social and emotional regulation
- Boredom-driven behavior when content is not sufficiently challenging

Student Strengths

- Exceptional intellectual ability in one or more domains
- Deep curiosity and intrinsic motivation for learning
- Creative, original thinking that surprises and challenges adults

Recommended Accommodations

- Challenge and enrichment in areas of giftedness — do not reduce to average
- Flexible pacing: accelerate in areas of strength, support in areas of challenge
- Sensory accommodations and regulation supports
- Explicit social and emotional skill instruction
- Work with both the gifted program AND special education simultaneously

Assistive Technology

- Advanced content platforms (IXL, Coursera Jr., Art of Problem Solving)
- Regulation apps (Zones of Regulation, MindUP)
- High-interest reading resources at advanced levels (Epic!, project Gutenberg)

UDL Strategies

- Match intellectual challenge to the student's ability — boredom is a sensory and behavioral trigger
- Use student's area of expertise as a teaching resource for the class
- Blend gifted and special education frameworks; neither alone is sufficient

IEP Considerations

- IEP must address BOTH the disability supports AND the gifted needs — neither should be traded for the other
- Annual goal: Student will use a regulation strategy before escalating during non-preferred academic transitions
- Gifted identification should not delay or complicate autism diagnosis and support
- Coordinate between gifted teacher and special education team — they often don't communicate

Who to Collaborate With

- Gifted Education Specialist
- Special Education Teacher

Family Partnership Tip

Advocate loudly for both sides of your child's profile. Schools sometimes say 'but they're so smart' to avoid addressing the autism. A 2e student needs both challenge AND support.

Middle School (6–8)

A student who is simultaneously bored and overwhelmed — some classes are far too easy, others are socially and sensorially unnavigable. Perfectionism and anxiety are often intense. May disengage from education or develop significant anxiety about performance.

What You Might Notice

- Uneven performance: A's in some subjects, failing others — not due to ability but access
- Intellectual arrogance combined with emotional fragility
- Deep frustration when intellectual standards are set too low
- May have given up on school emotionally while retaining strong intellectual ability

Student Strengths

- Genuine intellectual talent that should be actively developed, not just maintained
- Intense curiosity and self-motivated learning outside of school
- Often developing a strong self-concept around their intellectual identity

Recommended Accommodations

- Honors or advanced coursework with autism-related accommodations built in
- Flexible assignment formats that match intellectual level without requiring neurotypical social performance
- Counseling focused on perfectionism, anxiety, and identity
- Sensory and regulation accommodations in all environments
- Permission to engage with passion projects as part of coursework

Assistive Technology

- Advanced research tools and databases
- Regulation tools and sensory supports
- Independent learning platforms aligned with gifted interests

UDL Strategies

- Never reduce academic expectations as a behavioral consequence
- Allow passion-driven projects to earn credit in multiple subject areas
- Use inquiry-based and project-based frameworks that challenge without overwhelming

IEP Considerations

- Both sets of needs must appear in the IEP: gifted and disability
- Annual goal: Student will complete advanced coursework with identified accommodations at X% completion rate
- Address perfectionism and anxiety as barriers — these are disability-related
- Ensure gifted program participation is not removed as a consequence for behavior

Who to Collaborate With

- Gifted Education Coordinator
- School Counselor or Psychologist

Family Partnership Tip

Connect your 2e student with communities of other 2e learners and autistic adults who are thriving intellectually — seeing positive role models who share their profile is invaluable.

High School (9–12)

A student who may be heading toward highly selective universities or research careers while simultaneously struggling to navigate daily high school social life. Self-advocacy is critical. The 2e profile is often an asset in college and career if the student has the tools to manage their needs.

What You Might Notice

- Excels in advanced or AP coursework while struggling with organization and social demands
- May be deeply engaged with a research interest that exceeds the school's capacity to support
- Anxiety and perfectionism may be interfering with performance and wellbeing
- Has a sophisticated self-understanding of their own profile

Student Strengths

- Exceptional intellectual potential — often genuinely gifted in a domain
- Self-directed, passionate researcher or creator in their area of expertise
- Developing self-advocacy skills and disability identity

Recommended Accommodations

- Dual enrollment, independent study, or advanced coursework opportunities
- All autism-related accommodations maintained in advanced courses
- Flexibility in demonstrating mastery (research paper instead of group project)
- Counseling focused on identity, anxiety, and post-secondary planning
- Extended time and sensory accommodations on all standardized tests

Assistive Technology

- Advanced research databases (JSTOR, Google Scholar for students)
- Zotero for research organization
- Productivity and time management tools for executive function

UDL Strategies

- Connect coursework to student's research passion wherever possible
- Allow independent capstone or thesis as an alternative to traditional senior requirements
- Build explicit self-advocacy and disclosure practice into transition planning

IEP Considerations

- Transition plan must address the full profile: intellectual aspirations AND disability support needs
- Annual goal: Student will independently identify and use accommodations in dual enrollment or advanced coursework
- Summary of Performance should highlight both giftedness and disability supports for college disability services
- Consider NAGC resources alongside IDEA resources for a full picture

Who to Collaborate With

- Gifted Program Coordinator
- University disability services outreach

Family Partnership Tip

Help your student research colleges with strong disability services AND strong programs in their area of intellectual passion — these are not mutually exclusive, and the right fit changes everything.

Resources: [NAGC — Twice-Exceptional](#) | [2e Newsletter](#) | [Understood — 2e Students](#)

Emotional & Behavioral

Generalized Anxiety

Generalized Anxiety Disorder (GAD) in school-age children involves persistent, excessive worry about a wide range of topics — school performance, social situations, health, family, and the future. The worry is difficult to control, is out of proportion to actual risk, and causes significant distress and functional impairment in academic and social settings.

<https://fyvr.net/student-personas/generalized-anxiety>

Elementary (K–5)

A student who worries constantly — about grades, friendships, making mistakes, what other kids think of them, and whether they or their family members are safe. Appears overly conscientious, seeks frequent reassurance, and may have physical symptoms (stomachaches, headaches) before school or tests.

What You Might Notice

- Seeks constant reassurance from the teacher ('Is this right? Am I doing okay?')
- Avoids activities with any chance of failure or judgment
- Physical complaints (stomachache, headache) before tests, presentations, or new activities
- Difficulty starting tasks for fear of doing them wrong

Student Strengths

- Often highly conscientious, responsible, and thorough in their work
- Deep care for others; empathetic and attuned to classroom community
- Motivated to do well — anxiety is often the shadow side of genuine engagement

Recommended Accommodations

- Warm, predictable classroom environment with consistent routines
- Advance notice of any changes, assessments, or new activities
- Allow student to take tests in a lower-stakes setting
- Teach and practice coping strategies explicitly
- Limit reassurance-seeking to a structured, brief check-in protocol

Assistive Technology

- Calm or Headspace Kids for guided breathing
- Mood Meter app for emotional identification
- Structured digital checklists to reduce 'did I forget something?' anxiety

UDL Strategies

- Normalize mistakes explicitly and model imperfection in the classroom
- Use low-stakes practice opportunities before any summative assessment
- Build predictable rhythms into the school day — consistency reduces anxiety

IEP Considerations

- Eligibility under Emotional Disturbance (ED) if anxiety significantly impacts educational performance
- Annual goal: Student will use a coping strategy (breathing, grounding) before or during anxiety-provoking situations in X out of Y observed opportunities
- Counseling as a related service is typically warranted
- Coordinate with outside mental health provider if student is receiving therapy

Who to Collaborate With

- School Counselor
- Outside therapist (CBT-trained is ideal for anxiety)

Family Partnership Tip

When your child comes to you with worries, validate the feeling first before problem-solving: 'That sounds really hard. Your worry makes sense.' Then gently introduce perspective and coping.

Middle School (6–8)

A student whose anxiety about social judgment, academic performance, and the future has intensified with the social complexity of middle school. May be a high achiever whose perfectionism is driven by anxiety. Increasingly avoidant of situations that provoke worry — presentations, group work, or any activity with social exposure.

What You Might Notice

- Avoids presentations, group work, or activities involving peer judgment
- Perfectionism interferes with task completion — may not submit work that isn't 'perfect'
- Increasing somatic complaints correlating with tests and social stress
- May be developing social anxiety alongside generalized worry

Student Strengths

- Often the most prepared, organized student in the class
- Strong empathy and interpersonal sensitivity
- High standards and genuine commitment to learning

Recommended Accommodations

- Alternate presentation formats (recorded video instead of live presentation)
- Flexible deadlines for tasks that trigger perfectionism paralysis
- Access to counselor during the day when anxiety peaks
- Test in a smaller, quieter environment
- Advance copy of test format or sample questions to reduce fear of the unknown

Assistive Technology

- Insight Timer or Calm for in-school anxiety management
- Private notes app for journaling anxious thoughts before tests
- Structured to-do list app to reduce 'did I forget something?' spiral

UDL Strategies

- Offer multiple presentation formats; never force a live, high-stakes public performance
- Build in low-stakes rehearsal opportunities before anything summative
- Teach growth mindset and failure normalization explicitly across the curriculum

IEP Considerations

- Re-evaluate if anxiety has escalated — reassess for ED eligibility
- Annual goal: Student will independently use two coping strategies before presentations or tests, as measured by self-report and teacher observation
- Address avoidance patterns in the IEP — avoidance is maintained anxiety
- Counseling goal: Student will practice graduated exposure to anxiety-provoking situations

Who to Collaborate With

- School Counselor
- Outside CBT therapist

Family Partnership Tip

Encourage your student to lean into manageable challenges rather than avoid them — small, successful exposures to anxiety-provoking situations reduce anxiety over time. Avoidance makes anxiety stronger.

High School (9–12)

A student whose anxiety may be driving extreme perfectionism, overcommitment, sleep problems, and increasing physical symptoms. May be a high achiever on the verge of burnout. College application process can be an acute crisis point. Needs both academic accommodations and robust mental health support.

What You Might Notice

- Overextended with activities and AP courses — unable to say no
- Chronic sleep deprivation and stress-related physical symptoms
- Paralyzed by major decisions (college choices, course selections)
- May be masking anxiety with achievement and control

Student Strengths

- Highly capable, organized, and motivated
- Strong sense of responsibility and follow-through
- Has often developed strong self-awareness about their anxiety

Recommended Accommodations

- Extended time on all assessments
- Flexible submission policies for perfectionism-driven delays
- Access to mental health support during the school day
- Permission to reduce course load or extracurricular involvement without stigma
- Test anxiety accommodations: separate room, breaks, read-aloud prompts

Assistive Technology

- Headspace or Calm for daily anxiety management
- Structured planning tools (Notion, Google Calendar) for stress reduction through organization
- Anonymous mental health apps (Woebot, Wysa) for between-session support

UDL Strategies

- Build in explicit mental health literacy across advisory and health curriculum
- Reduce high-stakes single assessments in favor of portfolio-based evaluation
- Model teacher vulnerability and normalize help-seeking from adults

IEP Considerations

- Transition goal: Student will independently identify and use mental health resources in post-secondary settings
- Annual goal: Student will use a self-advocacy strategy to communicate anxiety-related needs to teachers in X out of Y situations
- Ensure accommodations are in place for SAT/ACT and AP exams
- Connect student with college mental health resources before graduation

Who to Collaborate With

- School Counselor
- Outside therapist

Family Partnership Tip

Help your high-achieving teen understand that their worth is not their GPA. Model self-compassion and help them identify what matters most — high achievement driven by fear is exhausting and unsustainable.

Resources: ADAA — Anxiety in Children | Understood — Anxiety at School | Child Mind Institute — Anxiety

School Refusal & Avoidance

School refusal (also called school avoidance or emotionally-based school non-attendance) is a complex pattern of reluctance or refusal to attend school driven by emotional distress — most commonly anxiety, depression, or trauma — rather than truancy or oppositional behavior. These students need compassionate, collaborative support, not punitive attendance enforcement.

<https://fyvr.net/student-personas/school-refusal-and-avoidance>

Elementary (K–5)

A student who cries, complains of illness, or becomes dysregulated every morning before school. May be fine once they arrive but the separation and anticipatory anxiety are debilitating. Parents are often exhausted. School may be triggering due to social fears, academic pressure, a specific teacher relationship, or sensory overwhelm.

What You Might Notice

- Frequent late arrivals, often following physical complaints (stomachache, headache)
- High distress at drop-off — crying, clinging, tantruming
- Once settled, is often able to function but spikes again at the thought of transitions or dismissal
- Multiple visits to the nurse with somatic complaints

Student Strengths

- Strong family attachment and relationship-orientation
- Often highly empathetic and emotionally perceptive
- When regulated, often a thoughtful and engaged learner

Recommended Accommodations

- Warm greeting from a trusted adult at arrival — not just check-in, but genuine connection
- Modified arrival routine: flexible entry point, gradual exposure plan
- Designated calm-down space in school for dysregulation
- School counselor available for daily brief check-in
- Avoid punitive consequences for late arrival — punishment escalates anxiety and avoidance

Assistive Technology

- Visual schedule of the school day to reduce anticipatory anxiety
- Calming apps accessible to student during distress
- Home-school communication app (ClassDojo, Bloomz) for parent reassurance

UDL Strategies

- Build strong predictability and warmth into the classroom environment
- Create a 'special job' that the student is responsible for — builds investment in coming
- Implement a graduated exposure plan collaboratively with student, family, and counselor

IEP Considerations

- School refusal driven by disability (anxiety, autism, trauma) is an IEP issue, not a truancy issue
- Annual goal: Student will enter the school building and transition to their classroom using coping strategies with adult support in X out of Y school days
- Develop a written home-school plan that is consistent and reinforcing
- Avoid referral to truancy court for disability-related attendance — this is a violation of IDEA

Who to Collaborate With

- School Counselor
- Outside therapist (CBT exposure-based therapy)

Family Partnership Tip

Do not enable complete avoidance — but do NOT force with punishment. Work with the school counselor on a gradual, supported re-entry plan. Slow, consistent steps are more powerful than a forced return.

Middle School (6–8)

A student whose avoidance has often escalated from somatic complaints to extended absences, homebound instruction, or online schooling. The longer the avoidance, the harder re-entry becomes. Requires a gradual, compassionate re-entry plan that addresses the underlying anxiety, not just the attendance.

What You Might Notice

- Chronic absences — may have missed weeks or months
- Physical complaints dominate the morning but disappear on weekends
- Behind academically due to absences; falling further behind increases anxiety
- Significant social isolation — has lost peer connections during absences

Student Strengths

- Often comfortable and capable in one-on-one adult interactions
- Frequently has an area of genuine talent or interest that has been nurtured at home
- Motivated to return when a clear, safe, low-stakes plan exists

Recommended Accommodations

- Graduated return-to-school plan with incremental, mastered steps
- One trusted adult point of contact who welcomes student without judgment
- Modified schedule: start with preferred classes or partial days
- Credit recovery plan that doesn't make catching up feel impossible
- Remove or reduce social pressure situations during return phase

Assistive Technology

- Online or hybrid learning tools for partial day during re-entry
- Two-way communication tools between home, school, and counselor
- Self-paced academic platforms for credit recovery

UDL Strategies

- Use differentiated entry points — student returns to what feels manageable, not what's 'required'
- Prioritize belonging and relationship before academics during re-entry
- Implement flexible scheduling and independent study where possible

IEP Considerations

- If absences exceed 10 days due to disability, the IEP team must convene
- Annual goal: Student will attend school for X hours/days per week using a graduated plan with counselor support
- Consider homebound or hospital-based services as a bridge to re-entry
- Attendance records must note disability-related absences separately from truancy

Who to Collaborate With

- School Counselor (primary relationship)
- Outside therapist specializing in anxiety and school refusal

Family Partnership Tip

Every day at home deepens the avoidance cycle. Even very partial attendance (one class, one hour) maintains the connection. Work with the counselor on a plan that feels achievable — not ideal, but achievable.

High School (9–12)

A student who may have been in a school refusal pattern for years and now faces potential credit loss, delayed graduation, or GED pathways. The social and academic cost of years of avoidance is significant. Transition planning must begin immediately alongside any re-entry plan.

What You Might Notice

- Significant credit deficiency due to chronic absences
- May have been in homebound, online, or alternative placement for extended periods
- Anxiety, depression, or trauma are often now multi-layered and complex
- May have given up on traditional school altogether

Student Strengths

- Often has significant self-directed learning and skills developed outside school
- High motivation when given genuine agency over their educational path
- Has often developed sophisticated self-knowledge about their needs

Recommended Accommodations

- Alternative pathways to graduation: online, credit recovery, alternative school
- Flexible school schedule and attendance requirements
- Mental health as a priority before academic acceleration
- No punitive attendance policy applied retroactively
- Individualized transition planning that doesn't require traditional attendance

Assistive Technology

- Online credit recovery platforms
- Telehealth mental health access
- Digital portfolio and independent study tracking tools

UDL Strategies

- Center student agency in all educational planning
- Use interest-driven independent study for credit-earning
- Explore community college dual enrollment as a lower-anxiety alternative

IEP Considerations

- Transition plan must be realistic given current attendance and credit situation
- Annual goal: Student will participate in X post-secondary or community activity using identified coping strategies
- IEP team should consider all alternatives: community school, online, hybrid, independent study
- Prioritize mental health stabilization as the prerequisite to all academic planning

Who to Collaborate With

- Alternative Education Coordinator
- Outside mental health provider

Family Partnership Tip

It's not too late. Your student's future doesn't depend on graduating on a traditional timeline. Focus on mental health stabilization first — a stable, well young adult can always return to education.

Resources: [Child Mind Institute — School Refusal](#) | [NASP — School Refusal](#) | [Understood — School Avoidance](#)

Emotional Dysregulation

Emotional dysregulation refers to difficulty managing the intensity and duration of emotional responses in ways appropriate to the context. Students with significant emotional dysregulation may have explosive outbursts, rapid cycling moods, intense reactions to perceived slights, or difficulty returning to calm after being upset. It can be associated with many diagnoses including ADHD, autism, trauma, bipolar disorder, or borderline personality features.

<https://fyvr.net/student-personas/emotional-dysregulation>

Elementary (K–5)

A student who has intense emotional reactions that are disproportionate to the trigger, has difficulty calming down once upset, and whose outbursts significantly disrupt the classroom. The student is often remorseful after an episode but cannot yet stop the response in the moment.

What You Might Notice

- Explosive outbursts (screaming, crying, throwing materials) over small frustrations
- Long recovery time after emotional episodes — may take 20–30 minutes to calm
- Difficulty accepting redirection or criticism without escalation
- Says hurtful things in the moment of dysregulation that don't reflect typical character

Student Strengths

- Often highly passionate and emotionally invested in their work and relationships
- Quick to feel remorse and reconnect after an episode — genuine empathy
- Creativity and intensity that, when channeled, can be a genuine strength

Recommended Accommodations

- Designated calm-down space in the classroom or hallway
- Co-regulated cool-down routine with an adult
- Teach the Zones of Regulation or similar emotional vocabulary program
- Proactive triggers identification and prevention strategies
- Check-in/Check-out behavioral support with a trusted adult

Assistive Technology

- Zones of Regulation app
- Breathe, Think, Do with Sesame (Sesame Street app for young students)
- Visual schedule to reduce frustration from unpredictability

UDL Strategies

- Use predictable, structured routines — unpredictability is a key trigger
- Build emotional vocabulary into daily morning meeting routines
- Respond to emotional behavior with co-regulation first, instruction second

IEP Considerations

- FBA is required before developing a BIP for behaviors that impede learning
- BIP must include replacement behaviors and proactive antecedent strategies
- Annual goal: Student will use a self-regulation strategy to return to baseline within X minutes of a dysregulation event in Y% of opportunities
- Related services should include school counseling focused on emotion regulation skills

Who to Collaborate With

- School Psychologist (FBA/BIP)
- BCBA or Behavior Specialist

Family Partnership Tip

After a calming episode (NOT during), name what happened and what helped: 'When you went to the calm corner and squeezed the stress ball, it helped you calm down.' This builds metacognitive awareness over time.

Middle School (6–8)

A student whose emotional intensity creates ongoing social conflict with peers and adversarial relationships with teachers. May be seen as 'too much' or difficult to teach. The peer consequences of explosive episodes are significant and deepen isolation and shame. Co-occurring depression or anxiety is common.

What You Might Notice

- Explosive arguments with teachers or peers over perceived unfairness
- Social fallout from emotional episodes — peers avoid or bait the student
- Rapid mood shifts that classmates and teachers find unpredictable
- Self-critical and ashamed after incidents; may have depression cycles

Student Strengths

- Genuine depth of feeling that, when regulated, creates powerful writing and art
- Fierce loyalty and advocacy for those they care about
- When regulated, often perceptive, funny, and charismatic

Recommended Accommodations

- Access to counselor or private space when beginning to escalate
- Environmental modifications to reduce known triggers
- Social-emotional check-in at start of day and after major transitions
- Restorative practices rather than punitive consequences following incidents
- Reduced public exposure during vulnerable moments

Assistive Technology

- Calm or Headspace for daily regulation practice
- Private journal or emotional tracking app
- Collaborative communication tools (email to teacher instead of verbal confrontation)

UDL Strategies

- Teach DBT skills (distress tolerance, emotion regulation) as explicit curriculum
- Use restorative circles to repair relationships after incidents
- Provide private, non-public feedback whenever possible

IEP Considerations

- Updated FBA and BIP should reflect the adolescent presentation
- Annual goal: Student will use a de-escalation strategy to prevent reaching explosive level in X% of identified trigger situations
- Consider whether ED eligibility is appropriate — emotional dysregulation must be documented as having educational impact
- Counseling as related service is essential

Who to Collaborate With

- School Counselor
- Outside therapist (DBT-informed)

Family Partnership Tip

Create a family plan for dysregulation that uses the same language and strategies as school. Consistency across environments is critical — the tools need to work everywhere, not just at school.

High School (9–12)

A student who has often accumulated a significant discipline record driven by disability-related behavioral incidents. Relationships with school adults may be damaged. The student is simultaneously developing identity, experiencing intense emotions, and navigating the demands of high school with limited regulatory skills. At risk for dropout.

What You Might Notice

- Suspension or discipline history that is largely manifestation-related
- Walked out of class or school on multiple occasions
- Teacher relationships are strained; student feels misunderstood and targeted
- Co-occurring substance use, depression, or self-harm concerns possible

Student Strengths

- Deep emotional intelligence — can be a powerful advocate and leader when regulated
- Creative self-expression through art, writing, music
- Genuine desire for authentic relationships and being understood

Recommended Accommodations

- Restorative practices framework for all disciplinary responses
- Designated trusted adult with regular, relationship-first check-ins
- Flexible schedule to allow cool-down periods without academic penalty
- Privacy protection around emotional incidents
- Access to mental health support during the school day

Assistive Technology

- DBT skills apps (DBT Coach, Calm Harm for crisis moments)
- Journaling and creative expression apps
- Secure communication channel with counselor

UDL Strategies

- Use creative outlets as valid academic expression formats
- Build reflection and self-monitoring into all major assignments
- Create explicit mentoring relationships with adults outside the student's crisis team

IEP Considerations

- All discipline incidents must be reviewed for manifestation determination
- Transition plan must include mental health self-management goals
- Annual goal: Student will independently identify when they need to use a coping strategy and use it before crisis in X% of situations
- Coordinate with outside mental health providers — school cannot address this alone

Who to Collaborate With

- School Counselor or Psychologist
- Community mental health provider

Family Partnership Tip

If your teen's emotions feel out of control even to them, DBT (Dialectical Behavior Therapy) is a highly effective evidence-based therapy. Ask their school counselor for a referral.

Resources: [Child Mind Institute — Emotional Regulation](#) | [CASEL — Social-Emotional Learning](#) | [Understood — Emotional Regulation](#)

Trauma-Informed Needs

Students with trauma histories may exhibit behavioral, emotional, and learning patterns that reflect the neurological impact of adverse childhood experiences (ACEs). Trauma affects the developing brain's stress response, executive function, memory, and capacity for trust and safety. These students need relationship-centered, predictable, trauma-informed classroom environments — not punitive behavioral systems.

<https://fyvr.net/student-personas/trauma-informed-needs>

Elementary (K–5)

A student who may be hypervigilant, easily startled, reactive to perceived threat, or alternatively shut down and dissociated. Trust with adults is fragile. Behavioral responses that look defiant or manipulative are often survival responses wired by trauma. Safety and relationship are prerequisites to learning.

What You Might Notice

- Hypervigilant — monitors the room, flinches at sudden sounds or movements
- Difficulty concentrating; may appear to 'zone out' or dissociate
- Explosive or defensive reactions to situations that don't seem high-stakes
- Avoids or resists close relationships with adults; may be indiscriminately friendly or completely avoidant

Student Strengths

- Remarkable resilience and survival skills
- Often perceptive and deeply empathetic when safe
- Creative coping and resourcefulness

Recommended Accommodations

- Trauma-informed classroom: predictable, safe, warm, and free from shame-based discipline
- Avoid public correction, sarcasm, or loud surprises
- Provide a trusted adult relationship — one consistent, warm adult is key
- Avoid forced sharing or disclosure about home life
- Use private, non-confrontational communication for concerns

Assistive Technology

- Calming sensory tools (weighted lap pad, fidgets, noise-canceling headphones)
- Visual schedule for predictability and safety
- Creative expression apps for non-verbal processing

UDL Strategies

- Build predictability: same schedule, same greetings, same transitions every day
- Use narrative and creative arts as low-threat entry points to academic content
- Offer student choice and control wherever possible to restore agency

IEP Considerations

- Trauma alone does not create eligibility — identify the educational impact (emotional disturbance, OHI, SLD) and document clearly
- Avoid diagnosing or labeling trauma in the IEP without family consent and clinical evaluation
- Annual goal should focus on regulation, trust-building behaviors, and academic engagement
- School counselor and outside mental health connection are critical components

Who to Collaborate With

- School Counselor
- Social Worker

Family Partnership Tip

Your child's school behaviors may look like defiance but are survival responses. Share what you know about triggers with the teacher in a private conversation — this context can transform how your child is treated at school.

Middle School (6–8)

A student whose trauma responses are now deeply embedded in behavior patterns. May be in a cycle of crisis, consequence, and re-traumatization from punitive school responses. Academic learning is significantly impaired when the student's nervous system is dysregulated. Relationship is the primary intervention.

What You Might Notice

- Reactive aggression in response to perceived threats (being bumped, looked at wrong)
- Academic performance is wildly inconsistent with the student's actual cognitive ability
- Avoids or tests adult relationships — doesn't believe adults will stay or be safe
- May use substances or engage in risk-taking behaviors outside school

Student Strengths

- Has survived genuinely hard circumstances — strength and resilience are real
- Responds powerfully to authentic connection with an adult who doesn't give up
- Creative self-expression when safe enough to try

Recommended Accommodations

- Eliminate humiliation-based discipline (public shaming, removal in front of peers)
- Use restorative, not punitive, responses to behavioral incidents
- Daily check-in with one consistent trusted adult
- Flexible academic expectations during periods of acute crisis at home
- Mandated reporter and child protective services referrals handled with sensitivity and transparency

Assistive Technology

- Private journaling or audio recording for processing
- Online resources about trauma for student's own understanding (age-appropriate)
- Calming apps available on student device

UDL Strategies

- Use trauma-responsive practices school-wide, not just for this student
- Allow creative and physical expression as valid academic engagement
- Build in student agency and leadership to counteract learned helplessness

IEP Considerations

- Any BIP must be trauma-informed — punitive consequences escalate trauma symptoms
- Annual goal: Student will identify and communicate a safety need to their trusted adult using agreed-upon signals in X out of Y situations
- Coordinate with child welfare and outside mental health systems when appropriate
- Safety planning must be part of the IEP if risk behaviors are present

Who to Collaborate With

- School Social Worker
- Community mental health provider

Family Partnership Tip

If your family has experienced trauma together, trauma-informed family therapy can help all of you learn a common language and strategies. Healing is relational — not just individual.

High School (9–12)

A student who has often aged through the system without adequate support and is now in crisis academically, socially, and emotionally. May be involved with juvenile justice, in foster care, or experiencing homelessness. The school may be the most stable institution in their life — that means something.

What You Might Notice

- Chronic absences, often related to instability at home
- Distrustful of school adults; has been let down repeatedly
- May have a juvenile record or DCF involvement
- Demonstrates significant potential when given the right relationship and environment

Student Strengths

- Remarkable real-world knowledge and competence beyond their years
- Deep empathy and understanding of human suffering
- Strong motivation when they believe a relationship or goal is real

Recommended Accommodations

- Flexible attendance and make-up policies that accommodate system-involved instability
- McKinney-Vento Act rights enforced for homeless/housing-unstable students
- Credit recovery and alternative graduation pathways
- Connection to community resources (food, housing, mental health)
- One adult who genuinely advocates for them in every school context

Assistive Technology

- Free online learning platforms for independent credit recovery
- Telehealth access for mental health
- Remind or text-based communication tools that meet the student where they are

UDL Strategies

- Honor and leverage real-world knowledge as academic content
- Use project-based and community-connected learning
- Center student agency in all educational decision-making

IEP Considerations

- Transition plan must address real-world survival needs alongside post-secondary goals
- Annual goal: Student will access X community or school support services independently using identified strategies
- Coordinate with probation, child welfare, and housing agencies as part of the IEP team
- Document any punitive school practices that may violate IDEA protections for trauma-related behavior

Who to Collaborate With

- School Social Worker
- McKinney-Vento Liaison (if housing-unstable)

Family Partnership Tip

For students in foster care or unstable housing: know your rights under McKinney-Vento and IDEA. Schools must provide records, enrollment, and services regardless of housing status. A social worker can help navigate this.

Resources: [SAMHSA — Trauma-Informed Schools](#) | [NCTSN — Child Trauma in Schools](#) | [ACEs Aware — Educator Resources](#)

Oppositional Defiant Disorder

Oppositional Defiant Disorder (ODD) is characterized by a persistent pattern of angry/irritable mood, argumentative/defiant behavior, and vindictiveness toward authority. It is a clinical diagnosis and is often associated with ADHD, anxiety, depression, or trauma. Effective support requires relational, proactive strategies — not escalating power struggles — and careful examination of underlying unmet needs.

<https://fyvr.net/student-personas/oppositional-defiant-disorder>

Elementary (K–5)

A student who argues about almost every request, refuses to comply with rules, blames others for mistakes, and deliberately annoys others — but who also may be easily frustrated, deeply sensitive to perceived unfairness, and genuinely struggling with emotional regulation. Traditional discipline typically escalates ODD patterns.

What You Might Notice

- Argues about or refuses nearly every adult direction
- Blames peers or teachers for problems ('She made me do it')
- Deliberately bothers classmates to get a reaction
- Explosive reactions when corrected or redirected

Student Strengths

- Strong sense of justice and fair play — when directed positively, this is leadership
- Often highly verbal and can be a persuasive communicator
- Energetic, persistent, and does not give up easily

Recommended Accommodations

- Offer genuine choices wherever possible to reduce power struggles
- Use positive behavior strategies: catch them being good at higher frequency
- Avoid public correction — use private, brief, neutral redirects
- Establish predictable, fair, consistently applied classroom rules
- Build relationship before trying to enforce compliance

Assistive Technology

- Visual choice boards to increase student agency
- Self-monitoring apps for behavioral awareness
- Reward system apps (ClassDojo positive points only)

UDL Strategies

- Frame all requests as choices and invitations, not commands
- Build student leadership and voice into classroom structures
- Use problem-solving conversations (Ross Greene's CPS model) rather than consequence-only approaches

IEP Considerations

- ODD diagnosis alone does not create IDEA eligibility; must document educational impact under Emotional Disturbance or OHI
- FBA is required — identify the function of each behavior before designing a BIP
- BIP must be proactive (antecedent strategies) and not solely consequence-based
- Annual goal: Student will use a problem-solving script when in disagreement with a teacher in X out of Y observed opportunities

Who to Collaborate With

- School Psychologist (FBA)
- School Counselor (for Collaborative Problem Solving or similar approach)

Family Partnership Tip

Ross Greene's book 'The Explosive Child' is written for parents of kids with ODD. His Collaborative Problem Solving approach is evidence-based and respectful. It genuinely works differently than traditional discipline.

Middle School (6–8)

A student whose oppositional patterns have often resulted in escalating discipline, damaged teacher relationships, and a self-fulfilling narrative of being a 'problem student.' The student may have given up on school because they expect rejection and conflict. Co-occurring ADHD, anxiety, or depression is often present.

What You Might Notice

- Has adversarial relationships with most teachers; responds to perceived disrespect with defiance
- Has accumulated a discipline record that precedes them into every class
- Responds well to adults who treat them respectfully — struggles with those who don't
- May show genuine insight in one-on-one conversations but shuts down in group settings

Student Strengths

- Often highly perceptive about social dynamics and adult inconsistency
- Natural leader — other students often follow their lead
- Responds powerfully to genuine respect and trust

Recommended Accommodations

- Begin each day fresh — no carryover of yesterday's conflicts
- Private redirection only — never public confrontation
- Relationship-first approach from each teacher before any compliance expectation
- Involve student in creating classroom norms and problem-solving plans
- Restorative practices after incidents rather than punitive consequences

Assistive Technology

- Restorative circles facilitated through structured apps or scripts
- Private communication tool (email or app) for when verbal communication escalates

UDL Strategies

- Use student voice and choice as primary engagement strategies
- Collaborative Problem Solving (CPS) approach for all significant conflicts
- Build leadership opportunities into academic tasks

IEP Considerations

- Updated FBA if behaviors have escalated or changed
- Annual goal: Student will engage in a problem-solving conversation with teacher using CPS protocol in X out of Y identified conflict situations
- Address the student's negative school identity explicitly — it is an educational barrier
- Counseling must focus on attribution style, perceived fairness, and relationship skills

Who to Collaborate With

- School Counselor (CPS-trained)
- Family (for consistency between home and school)

Family Partnership Tip

Ask the teacher to tell you one specific strength or positive moment from the day. Starting family conversations about school with a positive moment changes the whole dynamic — for everyone.

High School (9–12)

A student whose ODD patterns have often become deeply entrenched and whose school career is in jeopardy. May have a significant suspension or legal record. The power of genuine relationship and respect from one adult cannot be overstated — it can change the trajectory even at this stage.

What You Might Notice

- Openly challenges teacher authority in ways that create classroom crises
- Has been suspended multiple times; may have spent time in alternative placements
- Has genuine insight when given space to reflect without judgment
- Responds to adults who treat them as capable, intelligent people

Student Strengths

- Strong leadership potential — could be extraordinary with the right mentor
- Resilience and self-advocacy skills
- Often entrepreneurial, independent, and capable of generating original ideas

Recommended Accommodations

- Mentoring relationship with a trusted adult outside the student's conflict system
- Restorative rather than punitive responses to every incident
- Genuine voice in their own educational planning
- Alternative credit pathways and schedule flexibility
- Zero punitive zero-tolerance approaches — these have failed and will continue to fail

Assistive Technology

- Anger management and self-monitoring apps
- Alternative communication channels for disagreements with teachers
- Leadership and entrepreneurship platforms

UDL Strategies

- Student-led projects and self-directed learning as primary modalities
- Co-create norms and expectations with student input
- Use restorative practices school-wide — not just for this student

IEP Considerations

- Manifestation determination reviews are critical — most ODD-driven behaviors are disability-related
- Transition goal: Student will independently resolve a workplace or community conflict using problem-solving strategies
- Annual goal: Student will use a structured problem-solving conversation to resolve disagreements with adults in X% of identified situations
- Explore whether long-standing ODD is masking underlying anxiety, ADHD, or trauma — re-evaluate comprehensively

Who to Collaborate With

- Restorative Practices Coordinator
- Outside therapist or community mentor

Family Partnership Tip

The research is clear: punitive approaches make ODD worse. If your student responds to one adult, help the school understand what that adult is doing differently — then scale it.

Resources: [Child Mind Institute — ODD](#) | [CDC — Behavior Disorders in Children](#) | [Understood — ODD at School](#)

Speech & Language

Expressive Language Disorder

Expressive Language Disorder affects a student's ability to produce language — to find words, form grammatically correct sentences, organize ideas verbally or in writing, and communicate effectively. These students understand far more than they can express. They are often misidentified as having low ability when the real issue is an output bottleneck, not comprehension.

<https://fyvr.net/student-personas/expressive-language-disorder>

Elementary (K–5)

A student who understands everything that is said and has rich comprehension but struggles to put words together, uses simple or incorrect sentence structures, has word-finding difficulties ('that thing... you know...'), and produces written and verbal output that doesn't reflect their actual understanding.

What You Might Notice

- Verbal responses are short, simplified, or filled with filler words and pauses
- Word-finding difficulties: pauses, uses 'thing' or 'stuff' frequently
- Written output is significantly shorter and less complex than peers
- Avoids being called on; may shut down when put on the spot verbally

Student Strengths

- Strong comprehension — understands instructions, concepts, and stories well
- Often has good listening skills and benefits from verbal instruction
- Rich inner world; comprehension-based tasks often reveal more than output tasks

Recommended Accommodations

- Allow extra processing time before expecting a verbal or written response
- Accept telegraphic or partial responses without penalizing for grammar
- Offer sentence frames and word banks for all writing tasks
- Do not cold-call — allow opt-in participation
- Accept oral responses or recorded answers as alternatives to writing

Assistive Technology

- Speech-to-text tools (Google Voice Typing)
- Word prediction software (Co:Writer, Clicker)
- Picture-supported vocabulary tools (Boardmaker, SymbolStix)

UDL Strategies

- Provide word banks, vocabulary cards, and sentence starters for all language tasks
- Use visual supports alongside all verbal instruction

- Evaluate comprehension separately from expression — written tests can use multiple choice, matching, or labeling

IEP Considerations

- SLP services are a primary related service for expressive language disorder
- Annual goal: Student will produce grammatically correct sentences with subject-verb-object structure in X out of Y speech therapy prompts
- Language goals should address both oral and written expression
- Academic goals must assess comprehension without penalizing for expressive limitations

Who to Collaborate With

- Speech-Language Pathologist
- Special Education Teacher

Family Partnership Tip

Give your child time to finish their sentences at home — don't fill in words for them. Patience and wait time builds confidence and gives them practice with the hard work of finding their words.

Middle School (6–8)

A student whose expressive language deficit is now particularly visible in academic discussions, presentations, and written assignments that require elaboration. May be perceived as unengaged or low-achieving when they are neither. Social communication is also affected, making peer relationships challenging.

What You Might Notice

- Written assignments lack elaboration, transitions, and complex sentence structures
- Avoids class participation; gives minimal responses when called on
- Narrative and explanatory language lacks organization and detail
- May use slang or very casual register in all contexts due to limited formal language

Student Strengths

- Strong comprehension and content knowledge
- Often excels in visual, mathematical, or hands-on domains
- Has developed strong listening skills and nonverbal communication

Recommended Accommodations

- Graphic organizers and sentence frames for all writing tasks
- Alternative presentation formats (visual, recorded, or written instead of oral)
- Grading rubrics that separate 'knowledge of content' from 'language expression'
- Word prediction and speech-to-text on all devices
- Reduce word count minimums; focus assessment on ideas, not volume

Assistive Technology

- Co:Writer or Ginger for word prediction in writing

- Flipgrid for recorded verbal responses (multiple takes allowed)
- Graphic organizer apps (Inspiration Maps)

UDL Strategies

- Assess content knowledge through multiple modalities: drawing, labeling, oral recording
- Teach academic language explicitly — vocabulary for persuasion, explanation, comparison
- Use think-time protocols before any verbal or written production

IEP Considerations

- Annual goal: Student will use a graphic organizer to produce a structured 3-sentence response to academic questions in X out of Y writing tasks
- SLP goals should address academic language and discourse, not just sentence-level production
- Coordinate SLP goals with ELA teacher to ensure generalization to academic contexts
- Distinguish expressive language disorder from bilingual language development if ELL status is present

Who to Collaborate With

- Speech-Language Pathologist
- ELA Teacher

Family Partnership Tip

Encourage dinner table conversation that requires full sentences — 'Tell me three things that happened today' instead of 'How was school?' Practice in low-stakes, warm contexts builds expressive language over time.

High School (9–12)

A student who has often compensated through strong content knowledge and listening skills but now faces significant barriers in courses requiring verbal argument, formal writing, and presentation. College admission essays and interviews may be particularly challenging. Explicit AT proficiency is critical.

What You Might Notice

- Written work doesn't reflect depth of knowledge; lacks elaboration and formal language
- Oral presentations are minimal and clearly anxiety-provoking
- Struggles with complex academic writing: argument, analysis, synthesis
- Social communication is often affected, impacting peer and teacher relationships

Student Strengths

- Strong listener and content learner in well-supported environments
- Often excellent in visual, mathematical, or technical domains
- Has developed persistence and alternative communication strategies

Recommended Accommodations

- All written work typed with word prediction and spelling support
- Speech-to-text for drafting; editing with additional tools
- Alternative to oral presentation: recorded narration, poster presentation, written report

- Extended time for any timed writing
- Rubrics that assess content knowledge separately from language form

Assistive Technology

- Dragon NaturallySpeaking for drafting
- Grammarly for editing and grammar support
- Co:Writer for academic vocabulary and sentence construction

UDL Strategies

- Allow multimodal final products in all courses
- Teach academic writing structures explicitly (ACES, PEEL, etc.)
- Build in peer review and writing conferences to support revision

IEP Considerations

- Transition plan: student will use identified AT tools for written communication in post-secondary settings
- Annual goal: Student will independently draft a persuasive paragraph using a graphic organizer and AT tools with minimal teacher support
- SLP services may be continued or transitioned to self-directed use of strategies
- Document all accommodations for college applications and disability services

Who to Collaborate With

- Speech-Language Pathologist
- College Guidance Counselor

Family Partnership Tip

Help your student practice using their assistive technology at home for everyday writing tasks — texts, emails, notes. The habit of using tools builds fluency before they need them in high-stakes settings.

Resources: [ASHA — Expressive Language](#) | [ASHA — Language in Schools](#) | [Understood — Expressive Language](#)

Receptive Language Disorder

Receptive Language Disorder affects a student's ability to understand spoken and written language — including words, sentences, directions, and the language of instruction. These students may follow classroom routines so well that their comprehension deficits go unnoticed. When the familiar script breaks down, they become lost. Understanding is not the same as guessing correctly.

<https://fyvr.net/student-personas/receptive-language-disorder>

Elementary (K–5)

A student who appears to understand directions but actually follows the routine rather than processing language in real time. When given a novel instruction, they may look to peers or freeze. Complex sentences, figurative language, and multi-step verbal directions are particularly difficult.

What You Might Notice

- Follows class routines but is lost when the routine breaks
- Needs directions repeated or demonstrated rather than just re-explained
- Difficulty following multi-step verbal instructions
- Takes language literally; misses idioms, jokes, and implied meaning

Student Strengths

- Often has good procedural memory for established routines
- Visually oriented — often understands demonstrations better than verbal explanation
- Can complete known tasks independently when directions are familiar

Recommended Accommodations

- Provide written or visual directions alongside all verbal instructions
- Use simple, direct language; reduce length and complexity of directions
- Allow student to watch a peer model before attempting new tasks
- Check comprehension by asking student to tell you what they are doing, not whether they understand
- Avoid idioms, sarcasm, and implied meaning without explicit clarification

Assistive Technology

- Visual direction cards or task strips for all activities
- Text-to-speech for written instructions (Read&Write;)
- Vocabulary apps with audio and visual support (Vocabulary.com)

UDL Strategies

- Use demonstration + visual + verbal for all new instruction
- Pre-teach vocabulary before every lesson — receptive vocabulary is the foundation
- Use graphic organizers and visual summaries to reinforce comprehension of verbal content

IEP Considerations

- SLP evaluation should include standardized receptive language measures (CELF, TAPS)
- Annual goal: Student will follow 3-step verbal directions without visual support in X out of Y opportunities
- Classroom accommodations must address language of instruction — not just expressive output
- Distinguish receptive language disorder from hearing loss — audiological evaluation is warranted

Who to Collaborate With

- Speech-Language Pathologist
- Audiologist (rule out hearing loss)

Family Partnership Tip

Instead of asking 'Did you understand?', ask your child to 'Show me what you're going to do' or 'Tell me the steps.' This reveals actual comprehension, not just nodding along.

Middle School (6–8)

A student who is increasingly lost in content-area instruction as language becomes more technical, abstract, and rapid. May appear disinterested or low-achieving when they are simply not processing the language of instruction fast enough. Takes notes poorly because they can't keep up with spoken language.

What You Might Notice

- Falls behind in note-taking; cannot keep up with the pace of verbal instruction
- Misinterprets directions; assignments are completed incorrectly despite effort
- Struggles with abstract academic language (analyze, compare, infer, evaluate)
- May copy from peers or guess based on what others are doing

Student Strengths

- Often strong in visual subjects: art, technology, physical education
- Has developed social observation skills as a compensation strategy
- Can demonstrate knowledge through nonverbal or visual tasks

Recommended Accommodations

- Provide all directions in writing as well as verbally
- Pre-teach all academic vocabulary before content lessons
- Reduce verbal instructions; use visual task lists and demonstration
- Allow student to ask for repetition or clarification without social stigma
- Provide teacher notes or outlines so student focuses on understanding, not transcription

Assistive Technology

- Otter.ai for lecture transcription and review
- Vocabulary.com or Quizlet for pre-teaching key terms
- Notability with pre-loaded slides for annotation

UDL Strategies

- Front-load vocabulary for every unit with visual and contextual support

- Use closed captions on all videos and multimedia
- Allow student to read instructions and content independently rather than relying on verbal delivery

IEP Considerations

- Annual goal: Student will identify the meaning of grade-level academic vocabulary words from context with X% accuracy
- Classroom accommodations should include written directions for all assignments as a non-negotiable
- SLP services should include academic language processing, not just social communication
- Consider whether an interpreter or paraphrasing support during testing is warranted

Who to Collaborate With

- Speech-Language Pathologist
- Content area teachers (for vocabulary pre-teaching coordination)

Family Partnership Tip

Go over tomorrow's assignments and any vocabulary tonight — even 10 minutes of previewing what the lesson will be about significantly helps a child with receptive language disorder follow along.

High School (9–12)

A student whose receptive language deficit significantly impairs their ability to learn from lectures, follow complex academic discussions, or interpret multi-step directions on assessments. Has often developed sophisticated compensation strategies. AT and explicit strategy instruction are critical for post-secondary success.

What You Might Notice

- Consistently misinterprets complex assignment directions
- Struggles in lecture-heavy courses; performs better on visual or hands-on tasks
- May have been labeled as inattentive or resistant when the real issue is comprehension
- Academic performance is inconsistent with intellectual ability when language is a barrier

Student Strengths

- Developed strong visual and procedural learning strategies
- Often persistent and hardworking despite significant barriers
- Has developed self-awareness about language needs

Recommended Accommodations

- All test directions in writing; clarification available before testing begins
- Recorded or written versions of all lectures
- Extended time on all assessments
- Simplified language versions of complex directions available
- Access to a paraphraser or SLP support during complex testing contexts

Assistive Technology

- Speechify or Otter.ai for lecture-to-text conversion
- Simplify app or Rewordify for complex text simplification
- Quizlet for vocabulary study with audio

UDL Strategies

- Post all lecture content as slides or notes before class
- Use closed captioning universally in all media
- Teach explicit text and task analysis strategies for decoding complex directions

IEP Considerations

- Transition plan: student will independently use AT tools for language comprehension in post-secondary settings
- Annual goal: Student will independently use a strategy (re-read, simplify, ask) to clarify complex directions before beginning tasks in X% of situations
- Ensure test accommodations address language complexity, not just time
- Document for college disability services the specific nature of the receptive language processing deficit

Who to Collaborate With

- Speech-Language Pathologist
- Guidance Counselor

Family Partnership Tip

Encourage your student to become an expert at asking clarifying questions. 'Can you say that a different way?' and 'Can you write it down?' are powerful self-advocacy tools they'll use for life.

Resources: [ASHA — Receptive Language](#) | [Understood — Receptive Language](#) | [Reading Rockets — Language Disorders](#)

Stuttering & Fluency

Stuttering is a fluency disorder characterized by disruptions in speech production — repetitions, prolongations, and blocks — that can significantly affect communication, academic participation, and social confidence. The emotional and social impact of stuttering is often more impairing than the speech itself. Teachers who create low-pressure, patient communication environments make a profound difference.

<https://fyvr.net/student-personas/stuttering-and-fluency>

Elementary (K–5)

A student who repeats sounds, syllables, or words, prolongs sounds, or gets stuck silently trying to begin a word. The experience is often highly distressing and makes the student avoid speaking in class. Teasing from peers is a significant concern. Teacher attitudes and patience have enormous impact.

What You Might Notice

- Repetitions ('I-I-I want to...'), prolongations ('Ssssnake'), or silent blocks before words
- Avoids oral participation; declines to answer even when they clearly know
- May use circumlocution — substituting easier words to avoid difficult ones
- Increased disfluency under pressure (being called on suddenly, timed tasks)

Student Strengths

- Often thoughtful, determined, and persistent communicators
- Strong listeners and empathizers as a result of their own communication experiences
- Many celebrated communicators and leaders who stutter — the disability does not limit intelligence or leadership

Recommended Accommodations

- NEVER finish the student's words or sentences — this is invalidating and unhelpful
- Maintain natural eye contact and patient body language; don't rush or look away
- Allow alternative participation formats (written response, pre-prepared answer)
- Do not call on the student without warning; use voluntary participation
- Address teasing immediately and privately with a clear classroom norm

Assistive Technology

- Speech-easy or fluency devices (consult SLP) for supported fluency
- Written response tools (chat box, whiteboard app) as participation alternatives
- Audio recording for pre-preparing verbal responses

UDL Strategies

- Create multiple low-pressure ways to participate: written, drawn, pre-recorded
- Model that all communication styles are valued — never rush anyone who is speaking
- Allow rehearsal time for any required verbal participation

IEP Considerations

- SLP services are the primary support for stuttering
- Annual goal should address both fluency techniques AND communicative confidence and participation
- Goals should be co-developed with the student — they know what situations are hardest
- Social-emotional goals may be warranted if anxiety or avoidance is significant

Who to Collaborate With

- Speech-Language Pathologist
- School Counselor (for social confidence)

Family Partnership Tip

The most powerful thing you can do at home is listen fully and patiently. No finishing sentences, no 'relax,' no 'slow down.' Your unhurried attention is the best fluency support there is.

Middle School (6–8)

A student whose stuttering may have increased or shifted in pattern with the social intensity of middle school. Oral presentations, group discussions, and social conversations are highly anxiety-provoking. The student may have become an expert at avoidance. Bullying and social stigma are real concerns.

What You Might Notice

- Refuses or avoids all oral presentation requirements
- Has developed elaborate avoidance strategies (excuses, circumlocution, silence)
- May be teased or bullied about stuttering — or bullied because they're so quiet
- Stuttering severity may fluctuate dramatically based on stress level

Student Strengths

- Often excellent writers who have channeled communication into text
- Perceptive, empathetic, and deeply thoughtful
- Develops genuine resilience and advocacy skills over time

Recommended Accommodations

- Alternative to oral presentations: recorded video (multiple takes), poster board, written report
- Pre-approved voluntary participation — never cold-call
- Allow the student to present to the teacher one-on-one rather than the class
- Work with SLP on which classroom situations to gradually approach
- Address bullying immediately with a clear, enforced classroom community norm

Assistive Technology

- Screencastify or Loom for recording presentations with multiple takes
- Padlet or Google Docs for written contributions to class discussion
- Speech therapy apps (Stamurai, Speech4Good) for practicing outside sessions

UDL Strategies

- Normalize all forms of communication — multimedia, written, visual, and verbal

- Build a classroom culture of patient, attentive listening
- Use partner and small-group settings before whole-class oral tasks

IEP Considerations

- Annual goal should balance fluency skill development with communication confidence and participation
- SLP and counselor should coordinate — both fluency and social-emotional wellbeing are IEP priorities
- Do not require oral presentations without providing an appropriate alternative
- Consider whether the student's avoidance has become functionally limiting — address this specifically

Who to Collaborate With

- Speech-Language Pathologist
- School Counselor

Family Partnership Tip

Look up the National Stuttering Association — they run summer camps and youth conferences for kids who stutter. Meeting others who stutter is transformative for self-acceptance and confidence.

High School (9–12)

A student who has lived with stuttering for their entire school career and has often developed significant avoidance patterns, anxiety, and limited communication participation. College, career, and social life all depend on their developing a healthy relationship with their voice. Self-advocacy and acceptance are key transition goals.

What You Might Notice

- Chooses courses and career paths based on avoidance of speaking rather than genuine interest
- Oral presentations are a genuine barrier to grade-level achievement
- Has internalized negative identity around communication
- May be preparing to hide their stutter in college without appropriate support

Student Strengths

- Often exceptional writers and communicators in text-based formats
- Has developed profound patience and understanding of communication
- Has genuine insight into their own disability and what supports them

Recommended Accommodations

- Alternative formats for all oral assessments
- Extended time for oral tasks if chosen
- Private office hours or 1:1 demonstration as alternative to class presentations
- No cold-calling or surprise verbal demands
- Access to SLP services through high school graduation

Assistive Technology

- Loom or Screencastify for recorded presentations

- Speech therapy apps for ongoing fluency practice
- Written communication tools for professional contexts

UDL Strategies

- Celebrate the student's communication in all its forms
- Connect student with stuttering community and self-advocacy resources
- Build explicit self-advocacy practice: asking for accommodations, explaining stuttering to others

IEP Considerations

- Transition goal: student will disclose stuttering and request communication accommodations in at least two post-secondary settings
- Annual goal: student will use SLP-taught fluency techniques and/or coping strategies in two identified high-challenge speaking contexts per semester
- Summary of Performance should include specific communication accommodations for college disability services
- Connect student with National Stuttering Association adult resources

Who to Collaborate With

- Speech-Language Pathologist
- College Guidance Counselor

Family Partnership Tip

There are adults with significant stutters in every profession — law, medicine, entertainment, politics. Help your student see successful stuttering adults and embrace their voice, not just work to hide it.

Resources: [Stuttering Foundation — Schools](#) | [ASHA — Stuttering](#) | [NSA — Educators](#)

Selective Mutism

Selective Mutism is an anxiety disorder in which a student who is able to speak in some settings (typically home) consistently fails to speak in specific social situations (typically school). It is not stubbornness or shyness — it is an anxiety response that overrides speech. Pressure to speak consistently makes selective mutism worse.

<https://fyvr.net/student-personas/selective-mutism>

Elementary (K–5)

A child who speaks normally at home but is completely silent or whispers only to specific peers at school. May communicate through gestures, nods, or pointing. Teacher may have never heard their voice. The student often appears anxious in situations where speech is expected but is otherwise engaged and participatory in nonverbal ways.

What You Might Notice

- Does not speak at school, or only whispers to one or two peers
- Communicates through nodding, pointing, writing, or pulling others
- Freezes or shows visible distress when asked a direct question
- Participates actively nonverbally — raises hand, completes work, follows directions

Student Strengths

- Often a careful observer and listener — absorbs everything in the classroom
- Strong comprehension and often excellent academic performance
- Has rich verbal communication in safe settings — the capacity for speech is fully intact

Recommended Accommodations

- NEVER pressure or force verbal speech — this entrenches the anxiety
- Accept all nonverbal communication as valid participation
- Build a warm, low-pressure relationship — never make speaking a test
- Use a gradual approach: whisper to one peer → whisper to teacher → quiet voice → normal voice
- Coordinate with SLP and parents — consistent approach across all adults is critical

Assistive Technology

- AAC app (Proloquo2Go, LetMeTalk) as a bridge communication tool
- Written response cards or whiteboard for in-class responses
- Audio recording at home to share at school (bridge between safe and challenging settings)

UDL Strategies

- Create multiple nonverbal participation routes for all students — this benefits everyone
- Never call on this student without prior arrangement and a nonverbal option available
- Use private, low-pressure settings for any communication attempts — not whole-class

IEP Considerations

- Selective mutism may qualify under Emotional Disturbance or Other Health Impairment depending on educational impact
- Annual goal should focus on a gradual increase in communication in targeted settings, not immediate full speech
- SLP and school psychologist should both be involved in treatment planning
- Parents must be central partners — speech at home is the model for generalization

Who to Collaborate With

- Speech-Language Pathologist
- School Psychologist

Family Partnership Tip

Never tell your child 'just talk' at school — they can't control this any more than they can control a physical reflex. Work with the school on a graduated plan and celebrate every tiny communication step.

Middle School (6–8)

A student who has often been silent for years and whose pattern is now deeply entrenched. Peer relationships are affected — peers don't know how to interact with someone who doesn't speak. The student may be increasingly socially isolated and depressed. Academic participation is severely limited by the inability to speak.

What You Might Notice

- Has never spoken in class by this point; silence is the expected norm
- May communicate with only one or two peers; ignored or avoided by others
- Academic participation is entirely nonverbal or written
- May be developing depression or secondary anxiety from prolonged isolation

Student Strengths

- Often academically capable when given nonverbal assessment options
- Observant, thoughtful, and perceptive
- Has found workarounds that demonstrate genuine problem-solving and resilience

Recommended Accommodations

- All participation and assessment in written or nonverbal format
- Individualized plan with SLP: incremental exposure toward verbal communication in targeted settings
- One trusted peer or adult who knows the plan and can bridge communication
- Counseling focused on anxiety, not just speech
- Complete removal of any pressure or consequence for silence

Assistive Technology

- Text-to-speech AAC on phone or tablet as a communication tool
- Email or messaging app for communicating with teachers
- Audio or video recordings for assignment responses

UDL Strategies

- Fully normalize nonverbal participation for this student in all classes
- Create anonymous digital participation options (poll everywhere, chat box) for all students
- Use written warm-up tasks to give this student a voice before any verbal expectation

IEP Considerations

- Annual goal: Student will use an agreed-upon communication mode (written, AAC, gesture) to respond to teacher questions in X out of Y opportunities
- Long-term goal may include speech in small-group or individual settings
- Co-occurring depression or anxiety must be addressed in the IEP or through related counseling
- Involve outside CBT therapist with SM expertise — school-based services may not be sufficient alone

Who to Collaborate With

- SLP (with SM specialization)
- Outside CBT therapist

Family Partnership Tip

Selective mutism specialist therapists exist — search the Selective Mutism Association directory. Standard therapists often don't know how to treat SM effectively; find someone with specific training.

High School (9–12)

A student who has been effectively silent in school for years and now faces graduation, college interviews, and professional environments. The pattern is deeply entrenched. Some students have found ways to thrive nonverbally; others are experiencing escalating depression and isolation. Individualized, compassionate planning is essential.

What You Might Notice

- Graduates with limited speech in school settings but may speak normally outside school
- Has navigated high school through written communication and selected trusted relationships
- May be highly capable but severely limited in expressing that capability verbally
- Post-secondary plans may be constrained by communication anxiety

Student Strengths

- Often excellent writers with sophisticated ideas
- Has developed extraordinary observation and listening skills
- Is often beginning to speak in more settings as the school environment changes

Recommended Accommodations

- Written and recorded communication as the primary academic modality
- Individual exam administration where verbal clarification is provided in writing
- College choice planning that includes communication-friendly environments
- Continued SLP support with goal of communicating in at least one new setting per semester
- Counseling and possibly medication to address underlying anxiety

Assistive Technology

- AAC for professional and social situations where speech is blocked
- Email and text-based professional communication training
- Video introduction or portfolio for college applications in lieu of live interview

UDL Strategies

- All assignments offer written, recorded, or nonverbal formats
- Transition planning uses written and recorded self-advocacy tools
- Build digital presence and communication skills as a strength

IEP Considerations

- Transition goal: Student will communicate their disability and accommodation needs using a written communication tool in two post-secondary settings
- Annual goal: Student will initiate communication with a new adult (outside immediate circle) using agreed-upon modality in X out of Y opportunities
- Summary of Performance must explain the communication profile clearly for college disability services
- Address post-secondary setting selection with student — environment matters enormously

Who to Collaborate With

- School Counselor
- SLP with SM expertise

Family Partnership Tip

Many adults with selective mutism report that college was a turning point — the environment changed and they began speaking more. Don't assume high school is the permanent picture. Keep supporting and keep hope.

Resources: [Selective Mutism Association](#) | [Child Mind Institute — Selective Mutism](#) | [ASHA — Selective Mutism](#)

Social Communication Disorder

Social Communication Disorder (SCD) affects the pragmatic use of language — the social rules of communication including turn-taking, staying on topic, adjusting language to the listener, and interpreting implied meaning. Students with SCD often have normal vocabulary and grammar but use language in ways that are socially awkward or confusing. It is distinct from but often confused with autism.

<https://fyvr.net/student-personas/social-communication-disorder>

Elementary (K–5)

A student who talks too long, interrupts, doesn't adjust language to the listener, misses the point of jokes or indirect communication, and has difficulty maintaining friendships despite genuinely wanting them. May seem rude or odd to peers without understanding why.

What You Might Notice

- Dominates conversations with monologues about their topic
- Interrupts; doesn't wait for natural pauses to contribute
- Misses the point of indirect language: hints, sarcasm, idioms
- Responses in conversation are sometimes irrelevant or off-topic

Student Strengths

- Often has strong vocabulary and structural language skills
- Genuine desire to connect socially — the intent is there even when the execution misses
- Often knowledgeable and enthusiastic about their own interests

Recommended Accommodations

- Explicit instruction in conversational rules (SLP-led social skills group)
- Visual cues or scripts for social situations
- Structured peer interaction formats that scaffold turn-taking
- Private feedback on social communication (never public correction)
- Pre-teach the pragmatic expectations for new social situations

Assistive Technology

- Social stories apps for situational social scripts
- Video self-modeling for reviewing conversational behavior
- Conversation prompt cards for use in social situations

UDL Strategies

- Teach hidden social curriculum explicitly — what conversations are supposed to look like
- Use role-play and video examples to teach pragmatic rules
- Create structured social interaction opportunities with clear roles

IEP Considerations

- SLP services are primary — pragmatic language is an SLP domain
- Annual goal: Student will maintain a two-exchange conversation (listen → respond → ask question) with a peer in X out of Y structured opportunities
- Distinguish SCD from autism spectrum disorder — different profile, different interventions
- Social skills goals should be contextualized — what specific skills in what specific settings?

Who to Collaborate With

- Speech-Language Pathologist
- School Counselor

Family Partnership Tip

Practice conversational turn-taking at home as a game: use a physical object (talking stick or ball) and take exactly one turn each before passing. Make it fun — not a lesson.

Middle School (6–8)

A student whose pragmatic language deficits are increasingly causing social exclusion. The complexity of middle school social interaction (group texts, indirect communication, social subtext) is difficult to navigate. Friendships are often superficial or based on structured contexts. The student may be lonely without fully understanding why.

What You Might Notice

- Excluded from peer groups; has few close friendships
- Talks over others in group discussion; doesn't read the room
- Misinterprets peer communication as hostile when it isn't (or vice versa)
- Struggles in group projects due to communication coordination challenges

Student Strengths

- Often genuine, direct, and honest in communication
- Strong structural language — can write well even if pragmatics are weak
- Often well-liked by adults even when peer relationships are difficult

Recommended Accommodations

- Social skills group focused on pragmatic language (not generic social-emotional learning)
- Structured role assignment in all group work
- SLP-supported analysis of social situations that went wrong
- Private debrief after social situations with trusted adult
- Peer buddy system with a trained, empathetic peer

Assistive Technology

- PEERS curriculum digital supplements
- Video analysis tools for self-reflection on conversational behavior
- Private journaling for processing social situations

UDL Strategies

- Provide explicit debriefs of social situations in safe contexts
- Use literature and film to teach social perspective-taking
- Build social interaction opportunities around shared interests rather than general social exposure

IEP Considerations

- Annual goal: Student will adjust conversation topic or length based on listener cues in X out of Y observed interactions
- SLP group targets should be specific to adolescent social contexts
- Address social isolation as an educational issue — it affects engagement and wellbeing
- Bullying victimization prevention should be addressed if relevant

Who to Collaborate With

- Speech-Language Pathologist
- School Counselor

Family Partnership Tip

Watch a movie together and pause to ask 'What does that character mean by that?' Discussing implied communication in safe, fictional contexts builds real pragmatic language skills.

High School (9–12)

A student whose pragmatic language challenges now affect their ability to navigate job interviews, college admissions, and professional relationships. They may have strong academic skills but struggle to present themselves effectively in social and professional contexts. Explicit coaching and self-advocacy are key.

What You Might Notice

- Job interviews, college interviews, and presentations are particularly challenging
- Professional emails and written communication may be too direct or unclear in pragmatic tone
- Peer social life is limited but student may have one or two solid friendships
- Has developed some self-awareness about communication differences

Student Strengths

- Direct, honest communication style valued in many professional contexts
- Often strong in written and structured communication
- Has navigated real social challenges and developed genuine resilience

Recommended Accommodations

- Explicit coaching for interviews, presentations, and professional communication
- Alternative interview formats where possible (written Q&A, portfolio review)
- Direct feedback on pragmatic communication with scripts for improvement
- Social-professional skills group focused on workplace and college-level interactions
- Continued SLP services through graduation

Assistive Technology

- Interview prep apps and practice platforms
- Email templates for professional communication contexts
- PEERS post-secondary curriculum

UDL Strategies

- Build explicit professional communication instruction into transition curriculum
- Use role-play and video for interview and professional interaction practice
- Develop a personal 'communication style guide' with the student

IEP Considerations

- Transition goal: Student will use identified pragmatic strategies in at least two professional or post-secondary communication contexts
- Annual goal: Student will initiate and maintain a topic-appropriate professional conversation with an adult using X skills from SLP coaching
- Summary of Performance should note specific professional communication contexts needing support
- Link with PEERS high school program or similar if available in district

Who to Collaborate With

- Speech-Language Pathologist
- Transition Coordinator

Family Partnership Tip

Practice interview conversations and professional emails at home — not as tests, but as preparation. Role-play 'Tell me about yourself' and give genuine, specific feedback to help them refine their presentation.

Resources: ASHA — Social Communication | Understood — Social Communication | ASHA — SCD in Schools

Sensory & Physical

Sensory & Physical

Low Vision

Low vision refers to visual impairment that cannot be fully corrected with eyeglasses or contact lenses and that interferes with daily functioning, including reading, mobility, and visual learning tasks. Low vision is distinct from total blindness. Students with low vision need a combination of optical aids, accessible materials, environmental modifications, and instruction in compensatory skills.

<https://fyvr.net/student-personas/low-vision>

Elementary (K–5)

A student who holds books very close to their face, squints or moves to see the board, loses their place while reading, and may bump into things in the classroom or on the playground. May tire quickly from visual tasks. Has glasses but they don't fully correct the impairment.

What You Might Notice

- Holds materials unusually close or moves toward visual displays
- Loses place while reading; skips lines
- Has difficulty seeing the board even from the front of the room
- Fatigues quickly during visual tasks; may rub eyes or report headaches

Student Strengths

- Often develops strong listening and auditory learning skills
- Adaptable and resourceful from experience navigating a visual world
- Often has strong verbal comprehension and memory

Recommended Accommodations

- Large print materials for all worksheets and texts (18-point minimum, as recommended by TVI)
- Preferential seating closest to board and teaching area
- Reduced glare: window shading, matte paper, reduced overhead lighting
- Extended time on all visual tasks
- Verbal description of all visual content on the board or screen

Assistive Technology

- Screen magnification software (ZoomText, built-in OS magnifier)
- CCTV (closed circuit television magnifier) for close reading tasks
- Bookshare or audiobooks for all assigned reading

UDL Strategies

- Verbally describe all visual content: 'The diagram shows a cell with three labeled parts...'
- Offer digital versions of all materials for magnification
- Use high contrast materials: black on white or yellow on black

IEP Considerations

- Teacher of the Visually Impaired (TVI) must be part of the IEP team
- Braille evaluation must be completed even if student uses print — IDEA requires it for blind/VI students
- Annual goal: Student will use [specific AT device] to complete reading tasks independently with X% accuracy
- Orientation and Mobility (O&M;) services may be needed — evaluate

Who to Collaborate With

- Teacher of the Visually Impaired (TVI)
- Orientation and Mobility Specialist

Family Partnership Tip

Request an Assistive Technology evaluation through the IEP team — a TVI can identify the specific AT tools (magnifiers, screen readers, CCTV) best suited to your child's specific vision profile.

Middle School (6–8)

A student who may have been managing adequately in elementary school but now faces increasingly dense, visually complex materials — textbooks, graphs, maps, and PowerPoints — that are challenging without adequate low vision supports. Fatigue is a real barrier to sustained academic engagement.

What You Might Notice

- Struggles with visually dense textbooks and complex diagrams
- Visual fatigue significantly limits sustained academic work
- Difficulty with handwriting at normal size; may use large print or digital tools
- Navigation of the school building between classes may be challenging

Student Strengths

- Often highly audio-oriented learner
- Has developed strong self-advocacy skills
- Independent use of multiple AT tools

Recommended Accommodations

- All materials digitally accessible for magnification
- Audio versions of all textbooks (Bookshare, Learning Ally)
- Extended time with rest breaks for visual fatigue
- Digital note-taking tools rather than handwriting
- Verbal description of all visual content in lectures

Assistive Technology

- ZoomText or Fusion (screen magnification + screen reading)
- CCTV magnifier or portable handheld magnifier
- Bookshare for all textbooks

UDL Strategies

- Provide all visual materials in digital form in advance
- Use audio description as a universal design practice for all media
- Allow rest breaks during visually intensive tasks

IEP Considerations

- Annual goal: Student will independently use [AT tool] to access grade-level reading materials across all classes
- TVI must consult with all content-area teachers
- O&M; services should address independent navigation of the school campus
- Ensure Braille code is maintained even if student primarily uses print/AT — it may become more necessary later

Who to Collaborate With

- Teacher of the Visually Impaired
- All content-area teachers (for accessible materials coordination)

Family Partnership Tip

Make sure your student's TVI meets regularly with their classroom teachers — not just the student. Teachers who have never worked with low vision students need active coaching to provide the right supports.

High School (9–12)

A student who is developing independence with their low vision tools and preparing for post-secondary environments where they will self-manage their access needs. College campus navigation, digital accessibility, and self-advocacy are critical transition skills.

What You Might Notice

- Manages well with established tools and accommodations
- May need support with new, visually complex content areas (chemistry lab, data science)
- Planning for college requires early access planning with disability services
- Career planning should include understanding of workplace accessibility

Student Strengths

- Expert in their own low vision profile and assistive technology
- Strong self-advocacy skills developed over years
- Creative and persistent problem-solver

Recommended Accommodations

- All course materials in accessible digital format
- Extended time on all assessments
- Alternative format for visually heavy laboratory or performance tasks
- Independent navigation of campus and community settings
- Accessible testing accommodations documented for SAT/ACT/AP

Assistive Technology

- JAWS, NVDA, or VoiceOver for screen reading
- Braille display or refreshable Braille if needed
- OCR tools for converting print to digital text

UDL Strategies

- Teach digital accessibility navigation skills as explicit curriculum
- Connect student with blind/low vision professional role models
- Include accessibility advocacy skills in transition curriculum

IEP Considerations

- Transition goal: Student will independently request accessible format materials from college professors using written communication
- Summary of Performance must include specific AT tools and access needs for disability services
- O&M; transition: student will independently navigate college campus and public transit
- Annual goal: Student will independently access all course materials using identified AT without adult prompting

Who to Collaborate With

- Teacher of the Visually Impaired
- Transition Coordinator

Family Partnership Tip

Research college disability services for each school your student is applying to — requirements for documentation and levels of support vary widely. Start the process early.

Resources: AFB — Education & Low Vision | OSEP — Visual Impairment & IDEA | APH — Teachers of the Visually Impaired

Deaf/Hard of Hearing

Students who are deaf or hard of hearing have partial or complete hearing loss that significantly affects educational performance. They may use spoken language, sign language, or both. Access to instruction depends on acoustic environment, communication modality, assistive technology, and the language-accessibility of all materials. Deaf identity and Deaf culture are important considerations in educational planning.

<https://fyvr.net/student-personas/deaf-hard-of-hearing>

Elementary (K–5)

A student who may miss speech in noisy environments, mishear instructions, fatigue from listening effort, or communicate through sign language with an interpreter. Classroom acoustics, teacher voice volume, and background noise all significantly affect their ability to access instruction.

What You Might Notice

- Asks for repetition or seems confused when instructions aren't clearly seen
- Watches lips, faces, and the interpreter closely during instruction
- Fatigues more quickly than hearing peers from the effort of listening
- May miss information shared while they are looking elsewhere

Student Strengths

- Often highly attuned to visual information and nonverbal communication
- Strong in visual learning tasks: diagrams, demonstrations, reading
- May have rich Deaf community connections and bilingual (ASL + English) skills

Recommended Accommodations

- FM system or sound-field amplification to improve signal-to-noise ratio
- Preferential seating facing the teacher, away from HVAC and background noise
- Interpreter services in the student's preferred language (ASL, signed English)
- Captioning for all audio/video content
- Visual and written backups for all verbal instructions

Assistive Technology

- FM hearing loop or sound-field system
- CART captioning for real-time text of spoken instruction
- Video relay services (for DHH communication)

UDL Strategies

- Face the class when speaking; never talk while writing on the board
- Use visual signals (lights, hand signals) instead of auditory signals (bell, verbal warning)
- Caption all multimedia — universally, for all students

IEP Considerations

- Teacher of the Deaf/Hard of Hearing (TODHH) must be on the IEP team
- IDEA requires consideration of the student's communication mode, language, and Deaf community opportunities
- Annual goal may address listening skills, language development, reading, or self-advocacy
- Audiological data (audiogram, aided hearing thresholds) must be current in the IEP

Who to Collaborate With

- Teacher of the Deaf/Hard of Hearing
- Educational Interpreter (if applicable)

Family Partnership Tip

Learn American Sign Language alongside your child if they use it. Families who sign together with their Deaf child have dramatically better outcomes. Even beginner signs matter enormously.

Middle School (6–8)

A student who is navigating both the academic demands and social dynamics of middle school in a world designed for hearing people. The social aspects of middle school (group conversations, background noise, passing periods) are particularly challenging. May be experiencing identity questions about Deaf identity and hearing world membership.

What You Might Notice

- Struggles in group settings where multiple people speak simultaneously
- May miss social-conversational exchanges between peers, leading to social isolation
- Fatigue from listening effort is significant during full school days
- May be exploring Deaf identity and questioning their educational placement

Student Strengths

- Strong visual literacy and spatial reasoning
- Bilingual skills in ASL and English are a genuine cognitive and social asset
- Resilient and adaptive in navigating two worlds

Recommended Accommodations

- Interpreter or CART services in all settings including electives and specials
- Captioning for all videos and announcements
- Note-taking support (peer notes or paraprofessional) so student can focus on instruction
- Quiet working space when auditory fatigue is an issue
- Access to Deaf peers and role models if not available in the school

Assistive Technology

- CART or TypeWell for real-time captioning
- Cochlear implant or hearing aid maintenance plan documented in IEP
- Video phone and communication relay services

UDL Strategies

- Use visual-first instruction: project, demonstrate, caption, then explain verbally
- Teach hearing students basic communication strategies (face the DHH student, reduce background noise)
- Incorporate Deaf history and culture into curriculum — representation matters

IEP Considerations

- Annual goal should address language arts, reading, and self-advocacy
- Communication mode preferences must be honored — do not force speech-only approaches
- Ensure interpreter has appropriate certification and familiarity with academic vocabulary
- Address Deaf identity development as a positive component of the IEP

Who to Collaborate With

- Teacher of the Deaf/Hard of Hearing
- Educational Interpreter

Family Partnership Tip

Connect your teen with the Deaf community — programs, camps, and mentors. Hearing parents raising Deaf children especially need to support their child's access to Deaf adult role models and culture.

High School (9–12)

A student who is preparing for post-secondary life with well-developed communication strategies, strong self-advocacy skills, and a clear understanding of their communication and access needs. Transition planning includes ensuring college access, workplace accommodation, and identity-affirming community.

What You Might Notice

- Has a clear and effective communication profile using preferred mode
- Self-advocates for interpreter, captioning, and acoustic needs in new settings
- May be considering programs specifically designed for Deaf students (Gallaudet, NTID/RIT)
- Navigates both Deaf and hearing worlds with increasing skill

Student Strengths

- Expert self-advocate for communication access
- Bilingual-bicultural skills (ASL + English; Deaf + hearing cultures)
- Strong community connections within the Deaf community

Recommended Accommodations

- All access needs fully documented for college transition
- Self-managed AT and interpreter/CART requests
- Accessible format for SAT/ACT and AP exams
- Career exploration that includes Deaf-friendly and accessible workplaces
- Senior year transition planning including interpreter booking and CART request practice

Assistive Technology

- CART for all college-level coursework
- VideoPhone and ASL-accessible communication platforms
- Captioned telephone services for voice calls

UDL Strategies

- Teach self-advocacy for access needs as explicit curriculum
- Connect student with Gallaudet, NTID, or Deaf post-secondary program resources
- Develop disability disclosure script for employers and professors

IEP Considerations

- Transition plan must include ADA-based post-secondary accommodation planning
- Coordinate with VR for assistive technology funding post-graduation
- Annual goal: Student will independently request and coordinate interpreter services for X types of post-secondary situations
- Summary of Performance must thoroughly document communication access needs

Who to Collaborate With

- Teacher of the Deaf/Hard of Hearing
- VR Counselor

Family Partnership Tip

Explore post-secondary programs specifically designed for Deaf students — Gallaudet University and NTID at RIT offer full access in a Deaf-centered community. This is not settling — it may be exactly right.

Resources: [NAD — Education Rights](#) | [OSEP — Deafness & IDEA](#) | [Hands & Voices — School Supports](#)

Orthopedic Impairment

Orthopedic impairment encompasses a range of physical conditions affecting the musculoskeletal or neuromuscular systems — including cerebral palsy, spina bifida, limb differences, and acquired injuries — that significantly affect educational performance. These students need physical access accommodations, adaptive equipment, and educational adjustments that separate their physical challenges from their intellectual ability.

<https://fyvr.net/student-personas/orthopedic-impairment>

Elementary (K–5)

A student who may use a wheelchair, walker, or prosthetic device, who has limited fine motor control for handwriting, and who tires easily from the physical demands of the school day. Their intelligence and ability to learn are not impaired by their physical condition — but the physical environment and activity demands of school frequently create barriers.

What You Might Notice

- Difficulty with handwriting, cutting, or other fine motor academic tasks
- Slower movement between activities; needs more time for transitions
- Fatigues more easily than peers from physical demands
- May need adult assistance for personal care, materials management, or mobility

Student Strengths

- Academic and intellectual abilities that are completely independent of physical limitations
- Has developed creativity and problem-solving around physical tasks
- Often strong in verbal, analytical, and academic domains

Recommended Accommodations

- Fully accessible physical environment: ramps, wide doors, accessible bathroom
- Keyboard or voice input as alternative to handwriting
- Extra time for transitions between activities and classes
- Adapted physical education with goals appropriate to the student's abilities
- Positioning and seating supports as recommended by PT/OT

Assistive Technology

- Alternative computer access: switch scanning, eye-gaze, voice control
- Adapted keyboards and mice for fine motor limitations
- AAC if speech is also affected

UDL Strategies

- Ensure all physical classroom activities have an adapted equivalent
- Separate physical output requirements from academic knowledge assessment
- Provide adaptive materials (grips, stands, slant boards) universally

IEP Considerations

- PT and OT evaluations must be completed to determine related service needs
- Annual goal: Student will independently manage materials and navigate classroom using [mobility device] with X level of assistance
- Health plan may be needed if student requires nursing care, medication management, or emergency protocols
- Physical access barriers must be documented and resolved as part of IDEA compliance

Who to Collaborate With

- Physical Therapist
- Occupational Therapist

Family Partnership Tip

Ask the school PT to walk the full school day with you — entrance, classroom, lunch, recess, bathroom. Physical accessibility barriers are often invisible to those who don't navigate them.

Middle School (6–8)

A student whose physical needs require coordination across multiple teachers and environments. Middle school schedules and campus navigation add complexity. Self-care and independence are growing priorities. The student's academic ability must be assessed on its own terms — physical needs do not indicate cognitive limitations.

What You Might Notice

- Delayed arrival to class due to physical navigation challenges
- Fatigue accumulates across the day; afternoon performance may suffer
- Fine motor limitations affect note-taking and lab work
- May feel self-conscious about physical differences in social settings

Student Strengths

- Has often developed strong verbal, analytical, and problem-solving skills
- Self-aware and increasingly independent in advocating for physical needs
- Adaptive and resourceful from years of navigating physical challenges

Recommended Accommodations

- Accessible schedule: allow early dismissal from classes for navigation time
- All materials digitally accessible to reduce physical handling demands
- Adapted lab equipment and hands-on activity modifications
- Personal care aide or nursing support as needed
- Rest periods or modified schedule during fatigue peaks

Assistive Technology

- Laptop with voice recognition for all written work
- Adapted science lab equipment
- Mobility devices appropriate to campus navigation demands

UDL Strategies

- Design all activities to have adapted participation alternatives from the start
- Assess knowledge through oral, written, or digital formats — not physical performance
- Partner with PT and OT to adapt hands-on activities without reducing academic rigor

IEP Considerations

- IEP must address academic goals AND physical access, health management, and PE
- Annual goal: Student will independently request and use adaptive equipment in X settings with minimal adult prompting
- Health plan: up-to-date emergency protocols, medication administration, and personnel responsibilities
- Address transition needs: what does independent navigation of the physical environment look like post-school?

Who to Collaborate With

- Physical Therapist
- School Nurse

Family Partnership Tip

Help your child practice asking for what they need in new environments — 'I use a wheelchair, can you show me the accessible route?' This is a life skill they'll need forever.

High School (9–12)

A student who is preparing for a post-secondary life in which they will self-manage their physical access needs in college, the workplace, and the community. ADA self-advocacy, transportation planning, and independent living skills are all critical transition goals alongside academic preparation.

What You Might Notice

- Well-established adaptive strategies and AT use
- Planning for college involves extensive physical accessibility research
- Career planning may need to consider physical environment requirements
- Independent living skills (driving, transportation, personal care) are active planning areas

Student Strengths

- Expert self-advocate for physical access needs
- Strong in intellectual and academic domains when physical barriers are removed
- Resilient and experienced in navigating systems

Recommended Accommodations

- All academic accommodations maintained through graduation
- Transition planning includes physical access assessment of post-secondary environments
- Driver's education alternatives if needed (paratransit, rideshare)
- Independent living curriculum including housing, transportation, and personal care planning
- AT funding and planning for post-graduation through VR

Assistive Technology

- Advanced voice recognition and adaptive access technology
- Communication devices if needed
- Independence-supporting smart home technology planning

UDL Strategies

- Include disability rights and ADA knowledge in transition curriculum
- Connect student with disabled adult role models and organizations (United Spinal, CP Alliance)
- Build self-advocacy for physical access into all post-secondary visits

IEP Considerations

- Transition plan must address all three areas: post-secondary education, employment, and independent living
- Connect with VR by sophomore year for AT funding and employment support
- Annual goal: Student will independently research and request physical access accommodations for X post-secondary environment
- Summary of Performance must include detailed documentation of physical access needs for colleges and employers

Who to Collaborate With

- Transition Coordinator
- VR Counselor

Family Partnership Tip

Contact the Center for Independent Living in your area — they offer transition and independent living resources specifically for young people with physical disabilities, and many are run by people with disabilities themselves.

Resources: OSEP — Orthopedic Impairment & IDEA | AOTA — OT in Schools | Understood — Physical Disabilities

Sensory Processing Disorder

Sensory Processing Disorder (SPD) affects the way the nervous system receives, organizes, and responds to sensory input. Students with SPD may be hypersensitive (overwhelmed by ordinary sensory input), hyposensitive (underresponsive and seeking more input), or both. The school environment — with its noise, lighting, crowds, and unpredictable sensory demands — can be profoundly dysregulating for these students.

<https://fyvr.net/student-personas/sensory-processing-disorder>

Elementary (K–5)

A student who covers their ears, avoids certain textures, refuses to eat lunch in the cafeteria, melts down after recess, or conversely is constantly seeking physical input — crashing into things, touching everything, needing to move. Their behaviors are sensory-driven, not willful, and the school day is exhausting.

What You Might Notice

- Extreme reactions to sounds, textures, smells, or visual stimulation
- Avoids or melts down at sensory-heavy environments: cafeteria, gym, hallways
- Alternatively: constantly seeking movement, crashing into things, touching everything
- Transitions between high-stimulation and low-stimulation environments cause dysregulation

Student Strengths

- Often highly creative and imaginative from a rich internal sensory world
- Passionate and deeply engaged when the sensory environment is comfortable
- Perceptive and detail-oriented

Recommended Accommodations

- Sensory diet developed by OT and embedded throughout the day
- Noise-canceling headphones available for high-stimulation environments
- Flexible seating to meet sensory needs (wobble cushion, standing, floor seating)
- Early dismissal from cafeteria or gym to avoid overstimulating peaks
- Quiet, calm classroom environment: reduced visual clutter, softer lighting where possible

Assistive Technology

- Noise-canceling headphones (Loop, Sony, or Bose Kids)
- Fidget tools (textured, vibrating, resistance-based)
- Weighted blanket or vest (OT-prescribed)

UDL Strategies

- Build sensory movement breaks into the daily schedule universally
- Reduce sensory clutter in classroom: organized, minimal visual environment
- Offer sensory processing-friendly options in all activities (seating, location, materials)

IEP Considerations

- OT evaluation is the primary assessment tool for SPD in schools
- SPD alone may not meet IDEA eligibility criteria — document as part of autism, OHI, or other qualifying condition if applicable
- Annual goal: Student will use a self-selected calming strategy before escalating in X out of Y sensory-challenging situations
- Sensory plan should be written, signed by team, and shared with all staff

Who to Collaborate With

- Occupational Therapist
- Special Education Teacher

Family Partnership Tip

Work with the OT to create a 'sensory diet' for home too. Consistent sensory input routines before and after school reduce the accumulated load — what happens at 6pm is shaped by what happened at 8am.

Middle School (6–8)

A student whose sensory needs can be more covertly managed but whose school day still involves significant sensory regulation challenges. The social pressures of middle school mean they may be less willing to use obvious sensory accommodations in front of peers. Anxiety and sensory issues often intertwine.

What You Might Notice

- Avoids sensory-heavy social situations (lunch, PE, assemblies) using excuses
- Shows increasing anxiety in unpredictable sensory environments
- May have fewer overt meltdowns but now internalizes dysregulation (anxiety, withdrawal)
- Refuses or pushes back on sensory accommodations they perceive as stigmatizing

Student Strengths

- Has developed increasing self-awareness about their own sensory needs
- Motivated to find discreet, socially acceptable strategies
- Creative and determined in finding workarounds

Recommended Accommodations

- Discreet sensory tools: clear earbud headphones, discrete fidgets
- Permission to eat in a quieter location during lunch
- Flexible participation in PE with sensory modifications
- OT check-in for ongoing sensory diet refinement
- All staff trained in sensory needs — inconsistency is counterproductive

Assistive Technology

- Low-profile noise-canceling earbuds (Loop Engage, QuietOn)
- Sensory apps for self-monitoring and calm-down support
- Preference tracking for environmental needs

UDL Strategies

- Offer sensory-friendly alternatives to all high-stimulation activities
- Teach sensory self-awareness as part of social-emotional curriculum
- Design classroom transitions to minimize sudden sensory shifts

IEP Considerations

- Annual goal: Student will independently use a sensory regulation strategy before leaving a sensory-challenging environment in X% of situations
- Address the social dimension of sensory accommodations — peer acceptance of needs is an IEP consideration
- OT consultation with all teachers is essential for school-wide consistency
- Consider counseling to address the anxiety component that often co-occurs

Who to Collaborate With

- Occupational Therapist
- School Counselor

Family Partnership Tip

Help your student identify 1–2 sensory tools they'll actually use in public. Social acceptability matters at this age — a discreet strategy they'll use is better than an effective one they'll hide.

High School (9–12)

A student who has developed significant self-knowledge about their sensory needs and effective strategies. The challenge is now transition to environments — college, work, independent living — where the sensory environment is unknown and self-management is essential. Self-advocacy and planning ahead are the primary skills needed.

What You Might Notice

- Uses sensory strategies independently but may not communicate them well to new adults
- Can identify sensory triggers but may still be surprised by novel environments
- Performance is significantly affected in high-sensory-load settings (assembly, AP exam room)
- Transition environments (college dorm, workplace) are significant sensory unknowns

Student Strengths

- Deep self-knowledge about sensory needs and effective strategies
- Skilled at identifying and setting up preferred sensory environments
- Advocate for their own needs in trusted contexts

Recommended Accommodations

- Separate testing room for standardized tests to control sensory environment
- All sensory accommodations maintained through graduation
- Transition planning includes sensory environment assessment of post-secondary options
- Permission to leave overwhelming environments with a self-management protocol

- Continue OT consultation through graduation

Assistive Technology

- Advanced noise-canceling headphones for focus and regulation
- Smart home/dorm planning tools for controlling sensory environment
- Sensory self-monitoring apps

UDL Strategies

- Teach sensory self-advocacy: naming needs clearly and requesting accommodations
- Practice visiting new environments before expected first use (college visits)
- Include sensory planning in all transition activities

IEP Considerations

- Transition goal: Student will independently assess and request sensory accommodations in two new post-secondary environments
- Annual goal: Student will identify sensory needs in a new environment and independently implement two strategies within X minutes of arrival
- Summary of Performance must include specific sensory needs and proven strategies for college disability services
- Consider how dorm and workplace environments will affect sensory regulation during transition planning

Who to Collaborate With

- Occupational Therapist
- Transition Coordinator

Family Partnership Tip

When visiting college campuses, walk through a dorm, cafeteria, and busy hallway with your student specifically to assess the sensory environment. The 'right' college is the one that's actually accessible to them.

Resources: [STAR Institute — SPD in Schools](#) | [AOTA — Sensory Integration](#) | [Understood — Sensory Processing](#)

Chronic Health Condition

Students with chronic health conditions — including diabetes, epilepsy, severe asthma, cancer, Crohn's disease, lupus, and many others — face educational barriers related to fatigue, pain, medical management, frequent absences, side effects of treatment, and the psychological impact of living with a serious illness. IDEA covers health conditions that adversely affect educational performance under Other Health Impairment (OHI).

<https://fyvr.net/student-personas/chronic-health-condition>

Elementary (K–5)

A student whose health condition requires medical management during the school day, causes unpredictable absences, or creates fatigue and pain that affect concentration and participation. The school must have an individualized health plan and be prepared for medical emergencies. Academic access is equal — with appropriate support.

What You Might Notice

- Frequent or prolonged absences for medical appointments or illness episodes
- Fatigue — visibly tired, especially in the afternoon or post-episode
- Medical management during the school day (glucose checks, medication, rest)
- Difficulty returning to grade-level work after extended absences

Student Strengths

- Has developed remarkable resilience and maturity from managing serious health challenges
- Often has strong connections with trusted adults due to need for support
- Highly motivated to succeed academically when health allows

Recommended Accommodations

- Individualized Health Plan (IHP) developed with school nurse — separate from IEP but referenced in it
- 504 Plan or IEP for academic accommodations related to health impact
- Flexible attendance and make-up policies for medical absences
- Access to nurse's office or rest area as needed
- Modified workload during recovery periods

Assistive Technology

- Medical management apps (glucose monitoring integration, seizure diary)
- Remote learning tools for extended absences
- Audio recordings of lessons missed during absences

UDL Strategies

- Provide digital access to all materials for home-based learning during absences
- Prioritize essential learning standards when student has limited capacity
- Avoid penalizing absences with automatic grade reductions

IEP Considerations

- OHI is the typical IDEA eligibility for chronic health conditions
- IHP and emergency action plan must be in place before the student arrives at school
- Annual goal: Student will manage academic participation during X episodes of health impact using agreed-upon accommodations
- Coordinate with medical team (physician, nurse) for academic impact documentation

Who to Collaborate With

- School Nurse
- Student's medical team

Family Partnership Tip

Share your child's medical management plan in writing with every teacher, the school nurse, and the office. Don't assume the school will coordinate internally — your proactive sharing saves critical time.

Middle School (6–8)

A student whose chronic health condition intersects with the social and academic demands of middle school. Absences affect peer relationships and academic standing simultaneously. Medical management may be increasingly private and independent. The psychological impact of being different or dealing with serious illness is significant.

What You Might Notice

- Falling behind peers academically due to cumulative absences
- Social isolation linked to absences and 'being different'
- Fatigue and pain directly affecting concentration and output during the school day
- Anxiety or depression related to managing a chronic illness during adolescence

Student Strengths

- Extraordinary maturity and perspective from living with serious health challenges
- Strong relationship with trusted adults who understand their condition
- Academic motivation that persists even through illness

Recommended Accommodations

- Formal catch-up plan for every significant absence
- Homework load reduction during recovery periods (not zero — proportional)
- Medical pass for nurse access without classroom interruption
- Private medical management (insulin, EpiPen) without social stigma or exposure
- Counseling for the psychological impact of chronic illness

Assistive Technology

- Learning management system access for remote work during extended absences
- Medical tracking apps integrated with school health plan
- Communication tools for teacher-student coordination during absences

UDL Strategies

- Provide recorded lectures and digital materials for all absent students universally
- Allow flexible timelines for all work during health-related absences
- Build peer note-sharing systems so absences don't mean missing everything

IEP Considerations

- Annual goal: Student will independently communicate health needs and academic impact to teachers using agreed-upon self-advocacy strategies
- 504 or IEP must address both physical access and academic recovery needs
- Mental health support should be integrated — chronic illness and depression co-occur frequently
- Consider whether homebound services are needed during extended medical absences

Who to Collaborate With

- School Nurse
- School Counselor

Family Partnership Tip

Help your student practice saying what they need: 'I'm having a difficult health day and may need to rest for 10 minutes before the exam.' Teaching them to self-advocate now builds a skill they'll use for life.

High School (9–12)

A student who is managing a chronic health condition while also planning for college and career. The transition to a less supported environment — where they will manage their own medical care and academic accommodations — requires intentional planning. Resilience and self-management skills are often genuine strengths.

What You Might Notice

- Mostly independent in medical management but may need school backup
- Planning for college requires medical provider documentation and disability services coordination
- Fatigue or pain management affects participation in demanding AP or college-level coursework
- Has often developed exceptional time management to make the most of 'good health' days

Student Strengths

- Expert in their own health management and academic self-advocacy
- Mature, goal-oriented, and capable of communicating complex needs
- Has often developed deep empathy and life wisdom from health experiences

Recommended Accommodations

- All prior academic accommodations maintained through graduation
- Transition planning includes medical provider documentation for college disability services
- Flexible coursework and attendance policies for health-related absences
- College planning that considers proximity to medical care
- Health insurance and medical continuity planning as part of transition

Assistive Technology

- Telehealth for medical appointments that don't require in-person attendance
- Health management apps that support independent medical self-management
- Online coursework as a backup during extended health episodes

UDL Strategies

- Design all courses with remote/flexible access as a universal feature
- Build health-related self-advocacy into transition curriculum
- Honor the student's expertise in their own condition and management

IEP Considerations

- Transition plan must include health management as an independent living goal
- Annual goal: Student will independently obtain and communicate medical documentation for accommodation requests in two post-secondary settings
- Ensure health insurance continuity is part of transition planning — coverage changes at 18/26 depending on state
- Summary of Performance must include specific academic accommodations related to health condition for college disability services

Who to Collaborate With

- School Nurse
- Transition Coordinator

Family Partnership Tip

Work with your student's medical team to prepare a clear, brief written summary of their condition and academic needs — this document will be requested by every college disability services office they contact.

Resources: OSEP — OHI & IDEA | School Nurse — Chronic Illness | Understood — Chronic Illness

Intellectual & Developmental

Intellectual & Developmental

Mild Intellectual Disability

Mild Intellectual Disability (ID) is characterized by significant limitations in both intellectual functioning (IQ approximately 55–70) and adaptive behavior, originating before age 18. Students with mild ID can achieve meaningful academic skills, particularly in literacy and functional math, with high-quality, individualized instruction. They benefit from concrete, experiential learning and explicit skill instruction across academic and functional domains.

<https://fyvr.net/student-personas/mild-intellectual-disability>

Elementary (K–5)

A student who is learning to read, write, and count but significantly below grade-level expectations. Academic skills develop more slowly and require more repetition, concrete examples, and systematic instruction. Social development is often on par with peers or slightly delayed. This student is in the classroom, learning, and belongs here.

What You Might Notice

- Academic skills significantly below grade level in all areas
- Requires significantly more repetition and practice to acquire and retain skills
- Follows classroom routines but needs adult support for complex or novel tasks
- Social interactions are generally appropriate but may be delayed compared to peers

Student Strengths

- Often eager to please, socially connected, and genuinely kind
- Responds well to positive reinforcement and meaningful relationship
- Can achieve meaningful literacy and numeracy skills with appropriate instruction

Recommended Accommodations

- Modified curriculum at the student's instructional level, not just reduced grade-level content
- Extended time on all tasks
- Concrete, hands-on learning materials for all concepts
- Visual supports for all instructions and expectations
- Systematic instruction: explicit teaching, modeling, guided practice, independent practice

Assistive Technology

- AAC if expressive language is limited
- Text-to-speech for reading tasks
- Visual schedule and task-organizing apps

UDL Strategies

- Teach at the student's instructional level — just-right challenge, not failure or boredom

- Use task analysis for all complex skills — break into small, learnable steps
- Embed instruction in natural, meaningful activities and real-world contexts

IEP Considerations

- Annual goals must be aligned to the student's present level of performance, not grade-level standards
- Academic, communication, adaptive behavior, and social skills goals are all relevant
- Extended School Year (ESY) should be considered — regression risk is real
- Transition planning should begin early and be referenced in every IEP by age 14 or earlier

Who to Collaborate With

- Special Education Teacher
- SLP (if communication needs are present)

Family Partnership Tip

The most important thing you can do is practice real-world skills at home: cooking, shopping, telling time, following a schedule. These functional skills are as important as reading, and you teach them best.

Middle School (6–8)

A student whose academic skills may be at a 2nd–4th grade level. Inclusion in general education settings with appropriate support is a legal right and a social necessity. Functional academic skills (reading signs, managing money, completing job applications) become increasingly important alongside academic content.

What You Might Notice

- Reads and writes at a primary level; uses modified grade-level content
- Follows classroom routines and participates with support
- Engages socially with peers — often popular, friendly, and well-liked
- Benefits significantly from structured peer interaction and cooperative learning

Student Strengths

- Social warmth and genuine desire to belong — often well-liked by peers
- Learns practical and functional skills well in real contexts
- Reliable, consistent, and hardworking in structured environments

Recommended Accommodations

- Modified assignments aligned to grade-level themes at accessible skill level
- Paraprofessional support with planned fading in general education
- Functional reading embedded in all subjects: reading menus, signs, forms
- Alternative assessment: portfolio, demonstration, oral response
- Continued access to general education classes with supports

Assistive Technology

- Text-to-speech and adapted digital content
- Simple word processing with word prediction

- Apps for functional math (money, time, measurement)

UDL Strategies

- Adapt all general education tasks to provide access to the same concepts at a different level
- Use peer buddies and cooperative learning structures for inclusion
- Connect all academic learning to real-world applications

IEP Considerations

- IEP must address functional academics alongside academic skills
- Annual goal: Student will read and follow X-step functional directions in community settings with Y level of support
- Social skills and peer interaction goals are as important as academic goals
- Begin formal transition planning by age 14: identify post-school goals

Who to Collaborate With

- Special Education Teacher
- General Education Teachers (co-planning modified participation)

Family Partnership Tip

Help your student build self-care and independence skills every day — making their own lunch, doing laundry, managing their morning routine. These are IEP transition skills you can practice at home.

High School (9–12)

A student for whom transition planning is now the primary focus of the IEP. Employment, independent or supported living, community participation, and daily life skills are the curriculum. Academic skills are functional and applied. The student's vision for their future — not a standardized curriculum — drives the program.

What You Might Notice

- Participating in community-based vocational experiences
- Developing daily living skills: cooking, transportation, money management
- Has genuine interests and preferences that should drive career and life planning
- Needs support but is developing meaningful independence in familiar settings

Student Strengths

- Strong procedural memory for learned routines
- Genuine social warmth; builds relationships in workplace and community
- Has interests and preferences that are the foundation of transition planning

Recommended Accommodations

- Community-based instruction as part of school program
- Job site training and supported employment preparation
- Self-determination curriculum instruction
- Functional academics in applied contexts

- Daily living skills instruction across all settings

Assistive Technology

- Visual schedule and task management apps for independence
- Money management apps
- GPS navigation and transit apps for community independence

UDL Strategies

- All instruction is functional, community-referenced, and student-centered
- Use self-determination frameworks: student leads their own IEP meeting
- Center student's vision, choices, and preferences in all planning

IEP Considerations

- Transition plan must address all three IDEA areas: education/training, employment, independent living
- Coordinate with VR, DDS, and adult services before graduation
- Student must be invited to their IEP and their preferences must be documented and honored
- Annual goal: Student will complete X job tasks at the work site with Y level of prompting

Who to Collaborate With

- Transition Specialist
- VR Counselor

Family Partnership Tip

Visit your regional DDS (developmental disability services) office now — before graduation. Understanding what adult services your student qualifies for, and getting on waitlists, cannot wait until the last minute.

Resources: AAIDD — Intellectual Disability | OSEP — Intellectual Disability & IDEA | Understood — Intellectual Disability

Moderate Intellectual Disability

Moderate Intellectual Disability involves significant limitations in cognitive functioning (IQ approximately 40–55) and adaptive behavior. Students with moderate ID require intensive, individualized instruction across all domains. Academic learning focuses on functional literacy and numeracy. Communication, daily living, community participation, and vocational skills are primary educational domains. With appropriate support, these students can achieve meaningful independence and community integration.

<https://fyvr.net/student-personas/moderate-intellectual-disability>

Elementary (K–5)

A student who communicates with support (verbal, AAC, or sign), follows structured routines with adult guidance, and is learning foundational functional skills — recognizing words in the environment, counting objects, following picture schedules. Inclusion in general education settings for social learning and community membership is legally required and developmentally important.

What You Might Notice

- Limited verbal communication; may use AAC, sign, or picture exchange
- Follows structured routines but requires consistent adult support
- Learning pre-academic skills: letter identification, number recognition, matching
- Participates in general education through adapted, meaningful roles

Student Strengths

- Responds deeply to consistent, caring adult relationships
- Can learn routines and functional skills with systematic instruction
- Social-emotional warmth; genuine engagement with peers and adults

Recommended Accommodations

- Individualized, functional curriculum focused on real-world skills
- AAC system or alternative communication consistently available
- Visual schedule and structured daily routine
- Intensive adult support with systematic fading toward independence
- Meaningful participation in general education (adapted roles, peer modeling)

Assistive Technology

- AAC device (Proloquo2Go, TouchChat) for communication
- Interactive visual schedule apps
- Cause-and-effect and switch-access apps for engagement

UDL Strategies

- All instruction through concrete materials and natural learning opportunities
- Use incidental teaching and natural environment teaching alongside structured sessions
- Peer-mediated interventions for inclusive social learning

IEP Considerations

- Communication goals are typically the highest priority at this level
- Annual goal: Student will use AAC device to make a request or comment in X out of Y natural opportunities across three settings
- ESY services are almost always appropriate — regression during summers is significant
- OT, SLP, and PT may all be needed; coordinate so the student isn't pulled from class all day

Who to Collaborate With

- SLP (AAC specialist)
- Occupational Therapist

Family Partnership Tip

Use the same AAC system your child uses at school, at home, all the time. Language develops through use across all environments — consistency between home and school is the most powerful intervention.

Middle School (6–8)

A student whose program centers on functional academics, communication, social skills, and daily living skills. Participation in general education may be for elective classes, lunch, and structured social activities. Academic content is highly modified and functional.

What You Might Notice

- Communicates through AAC, simple speech, or sign; messages are functional and increasing
- Participates in structured social activities with peers
- Is learning practical academic skills: sight words for functional reading, basic money handling
- Benefits greatly from structured job sampling and vocational exploration

Student Strengths

- Often has strong visual memory for logos, signs, and familiar objects
- Social engagement and warmth that builds genuine community
- Learns and generalizes skills in natural environments with systematic support

Recommended Accommodations

- Special day class as primary placement with inclusion in electives and social settings
- Functional academic curriculum with community-referenced content
- All communication supported at all times
- Structured community-based instruction beginning in middle school
- Personal care support as needed with dignity and privacy

Assistive Technology

- High-tech AAC updated with relevant vocabulary
- Adapted iPads and apps for functional skill practice
- Video modeling apps for daily living and social routines

UDL Strategies

- All learning is embedded in functional, community-referenced activities
- Use systematic instruction (most-to-least prompting, time delay) for all skill acquisition
- Peer tutoring and circles of friends programs for social inclusion

IEP Considerations

- Transition planning should begin at age 14 or earlier; person-centered planning is ideal
- IEP must address communication, daily living, social skills, and functional academics
- Annual goal: Student will independently complete X functional daily living tasks using visual task card with minimal prompting
- Coordinate with family about home skill generalization — IEP skills must transfer to real life

Who to Collaborate With

- Special Education Teacher
- All related service providers (SLP, OT, PT)

Family Partnership Tip

Ask about community-based instruction opportunities — learning to buy groceries, use public transportation, or order food in a restaurant are IEP skills as important as reading.

High School (9–12)

A student whose educational program is fully transition-focused: community-based vocational training, daily living skills, communication in real settings, and supported community membership. The school's job is to prepare this student for the most independent, meaningful, and fulfilling adult life possible.

What You Might Notice

- Participates in job training sites and vocational experiences
- Community navigation and daily living skills are active instruction areas
- Communication continues to develop with AAC and natural speech
- Family and school are planning for adult services and supported living

Student Strengths

- Reliable, consistent, and eager to contribute in structured work settings
- Genuine community connections and social relationships
- Has a vision for their future that should drive all planning

Recommended Accommodations

- Community-based instruction for majority of the school program
- Supported employment job coaching
- Independent living skill instruction across settings
- Continued AAC and communication support
- Person-centered transition planning with student at the center

Assistive Technology

- AAC for community and workplace communication
- Task management and visual schedule for independence
- GPS and transit apps for community navigation

UDL Strategies

- All activities are functional, person-centered, and community-referenced
- Student participates in their own IEP meeting to the fullest extent possible
- Families are equal partners in all transition planning

IEP Considerations

- Student must be invited to their own IEP meeting and actively participate
- Coordinate with DDS, VR, and supported employment providers before graduation
- Annual goal: Student will complete a multi-step work task at job site with X level of prompting
- Summary of Performance and comprehensive documentation needed for adult services eligibility

Who to Collaborate With

- Transition Specialist
- DDS and VR representatives

Family Partnership Tip

Person-centered planning is the gold standard — it asks 'What does YOUR child want for their life?' Bring your family's knowledge of your child's dreams, preferences, and personality to every transition meeting.

Resources: AAIDD — Intellectual Disability | OSEP — Intellectual Disability & IDEA | ARC — Education Resources

Down Syndrome

Down syndrome is a chromosomal condition (Trisomy 21) that causes intellectual disability, characteristic physical features, and may include associated health conditions. Educational needs vary widely — some students with Down syndrome read novels; others need intensive functional instruction. Visual learning, social strengths, and music are common assets. All students with Down syndrome deserve high expectations, inclusion, and rich educational opportunities.

<https://fyvr.net/student-personas/down-syndrome>

Elementary (K–5)

A student who learns well through visual and hands-on methods, has strong social skills, and has a genuine personality and interests that shape their educational experience. Academic development is typically below grade level but highly variable. Literacy and math skills are emerging with appropriate instruction. This student belongs in the general education classroom.

What You Might Notice

- Academic skills developing below grade level with significant individual variation
- Strong social interest; genuinely engages with peers and adults
- Visual learner who benefits from pictures, demonstrations, and hands-on materials
- May have speech that is difficult to understand; SLP is a critical support

Student Strengths

- Warm, socially engaged, and eager to connect
- Often has strong memory for visual and routine-based content
- Music, visual arts, and movement are often genuine strengths

Recommended Accommodations

- Visual supports for all instruction: picture cards, visual schedules, illustrated text
- Modified curriculum aligned to student's instructional level
- Speech-language therapy to support communication clarity and language development
- Peer buddy systems and structured peer interaction in general education
- High expectations maintained — do not cap learning based on the diagnosis

Assistive Technology

- AAC if verbal communication is significantly limited
- Text-to-speech for reading tasks
- Visual learning apps aligned to curriculum

UDL Strategies

- Use visual, auditory, and kinesthetic instruction for all concepts
- Build on social strengths — cooperative learning is a natural fit
- Teach literacy explicitly using systematic, phonics-based approaches

IEP Considerations

- NEVER set expectations based solely on the diagnosis — assess the individual
- Annual goals should reflect the student's actual learning trajectory, not diagnostic assumptions
- SLP, OT, and potentially PT services are common related services
- Include health monitoring needs (cardiac, thyroid, vision) in health plan and IEP as needed

Who to Collaborate With

- SLP (for communication and language development)
- OT (for fine motor and visual-motor skills)

Family Partnership Tip

Seek out high expectations for your child. Many students with Down syndrome learn to read, graduate, hold jobs, and live in the community. Connect with the National Down Syndrome Society for family resources and research.

Middle School (6–8)

A student who has developed genuine academic skills, meaningful friendships, and personal interests. Literacy and functional math are active skills. Inclusion in general education with appropriate support is both a right and a genuine benefit — for this student and for their peers. Transition planning is beginning.

What You Might Notice

- Reading and writing at an individualized level; range is wide
- Socially central — often a genuine and beloved member of the classroom community
- Participates in general education classes with modified materials and paraprofessional support
- Is developing self-advocacy and personal identity

Student Strengths

- Social intelligence and genuine warmth that creates community
- Persistent, hard-working, and motivated with appropriate challenge
- Often has a strong sense of humor and self

Recommended Accommodations

- Modified general education participation with adapted materials
- Paraprofessional support with systematic fading
- Continued SLP for language, articulation, and academic vocabulary
- Functional academics embedded in general education content
- Social-emotional support and peer relationship facilitation

Assistive Technology

- Text-to-speech and word prediction for written work
- Calculator and visual math tools
- Communication support if needed

UDL Strategies

- Peer-mediated instruction: structured peer tutoring benefits both students
- Use same-age social activities with support rather than separate peer groups
- Leverage music, art, and drama as academic entry points

IEP Considerations

- Annual goals must continue to reflect ambitious, achievable expectations
- Begin transition planning by age 14: interests, employment ideas, daily living goals
- Health monitoring should be current (annual thyroid, vision, audiology, cardiac follow-up)
- Address co-occurring sleep apnea or other health conditions that affect school performance

Who to Collaborate With

- Special Education Teacher
- School Counselor (social-emotional and transition)

Family Partnership Tip

Build friendships outside of school too — Special Olympics, community recreation, and interest-based clubs are not just fun; they are critical for building the social network your child will depend on as an adult.

High School (9–12)

A student for whom transition planning is now the central focus. Some students with Down syndrome attend college (inclusive PSE programs). Others prepare for supported or competitive employment and community living. The vision is a rich, connected, meaningful adult life — and the IEP team's job is to plan that with the student and family.

What You Might Notice

- Participating in vocational training and community-based experiences
- Has personal goals for work, relationships, and living situation
- Participates actively in their own IEP meeting when supported
- Developing independence in daily tasks with appropriate supports

Student Strengths

- Genuine personality, clear preferences, and a vision for their life
- Established social network and community connections
- Demonstrated work ethic and reliability in structured settings

Recommended Accommodations

- Transition-focused IEP with student-centered goals
- Job training, community-based instruction, and vocational experiences
- Daily living skills instruction in natural settings
- Continued SLP and OT consultation as needed
- Self-determination skills curriculum (student leads their own planning)

Assistive Technology

- Apps for independent task management and daily living
- Communication tools for workplace contexts
- GPS and transit apps for community navigation

UDL Strategies

- Person-centered planning as the transition framework
- Self-determination curriculum explicitly taught and practiced
- All planning centers the student's own goals and values

IEP Considerations

- Research inclusive post-secondary education (PSE) programs — many exist specifically for students with ID
- Coordinate with VR and DDS well before graduation
- Student should be the primary voice in their transition planning — teach them to speak for themselves
- Annual goal: Student will identify and communicate two personal post-school goals using self-advocacy skills

Who to Collaborate With

- Transition Specialist
- DDS and VR representatives

Family Partnership Tip

Look into Think College (thinkcollege.net) — a database of inclusive post-secondary education programs for students with intellectual disability. College is not off the table for your student.

Resources: [NDSS — Education Advocacy](#) | [NDSS — Educator Resources](#) | [Understood — Down Syndrome](#)

Developmental Delay

Developmental Delay is an IDEA eligibility category for children ages 3–9 (extended to age 13 in some states) who show delays in physical, cognitive, communication, social-emotional, or adaptive development that require special education services. It is an early childhood category that allows schools to serve young children who need support without forcing premature diagnostic labels.

<https://fyvr.net/student-personas/developmental-delay>

Elementary (K–5)

A young student (typically K–3) who is developing more slowly across multiple domains — language, motor skills, cognitive skills, social-emotional skills, or all of the above. They may have a specific diagnosis or the delay may not yet be fully characterized. Early intervention and high-quality special education are critical to improving long-term outcomes.

What You Might Notice

- Developmental milestones (language, motor, social) that lag significantly behind same-age peers
- May have been in early intervention (Part C services) before kindergarten
- Learning is emerging but requires more time, repetition, and concrete support
- May have difficulty communicating needs; may use alternative communication

Student Strengths

- Young children's brains are highly plastic — early intervention has the greatest effect
- Often has social engagement and desire to connect even when language is delayed
- Responds strongly to warm, consistent adult relationships

Recommended Accommodations

- Small group or individual instruction for all foundational skills
- Visual supports, routines, and concrete materials throughout the day
- AAC if language is significantly delayed
- Close collaboration between special education, general education, and related services
- Frequent data collection on goal progress to guide instructional decisions

Assistive Technology

- AAC for early communicators
- Cause-and-effect apps for switch-access learning
- Visual schedule and choice boards

UDL Strategies

- Use play-based learning as the primary instructional modality for young children
- Embed skill instruction in all daily routines — transitions, meals, specials
- Use peer modeling and naturalistic teaching

IEP Considerations

- Developmental Delay eligibility requires re-evaluation before age 9 or 13 (state-dependent)
- Annual goals should span all developmental domains, not just one
- Coordinate transition from Part C to Part B services carefully — no service gap is acceptable
- Parent involvement is legally required and practically essential at this age

Who to Collaborate With

- Early Childhood Special Educator
- SLP, OT, PT (typically all involved)

Family Partnership Tip

You are your child's first and most important teacher. Reinforce what they're learning at school through play, daily routines, and consistent language at home. Ask the teacher for specific activities to do together.

Middle School (6–8)

By middle school, the Developmental Delay eligibility category has typically expired and been replaced by a more specific diagnosis (intellectual disability, autism, SLD, etc.). If a student in middle school was previously identified under Developmental Delay, their current IEP should reflect a more specific eligibility determination based on comprehensive re-evaluation.

What You Might Notice

- Student has likely been re-evaluated and has a more specific eligibility determination
- Academic skills may span a wide range depending on the nature of earlier delays
- Social and adaptive skill development varies widely
- Student benefits from continued intensive, individualized instruction

Student Strengths

- Has often made significant gains from early intervention and continuous support
- Brings diverse skills and perspectives to the learning community
- Often has made genuine academic and social progress that early assessments couldn't predict

Recommended Accommodations

- Accommodations should reflect the student's current re-evaluated eligibility and needs
- Modified academic content at the student's actual instructional level
- Continued related services as indicated by current assessment data
- Strong transition planning beginning at age 14
- Functional academics integrated with modified general education content

Assistive Technology

- Tools matched to current communication and academic needs
- Visual supports and organizational tools
- Adaptive technology for any identified sensory, motor, or communication needs

UDL Strategies

- Match instruction to the student's current profile, not their early history
- Use the student's strengths and interests as the motivational anchor
- Provide multiple access routes to all content and assessment

IEP Considerations

- Review the eligibility determination — it should reflect current comprehensive evaluation data
- Annual goals should be ambitious and based on the student's current performance, not early prognosis
- Transition planning under IDEA must begin by age 16 (many states start earlier)
- Celebrate growth from baseline rather than comparing to grade-level norms

Who to Collaborate With

- Special Education Teacher
- School Psychologist (for re-evaluation)

Family Partnership Tip

Request a comprehensive re-evaluation if your student hasn't had one recently. Needs change — the supports that were right at age 6 may not be right at 12. New data leads to better planning.

High School (9–12)

Developmental Delay is not a valid eligibility category at the high school level — students should have a specific eligibility determination. However, this persona represents students who had early developmental delays and are now in high school with significant support needs. Transition is the primary focus.

What You Might Notice

- Student has a current eligibility determination based on comprehensive re-evaluation
- Needs across academic, functional, communication, and social domains are significant but vary
- Transition planning is urgent and central
- Student has genuine preferences and emerging self-determination skills

Student Strengths

- Has made a genuine journey of growth from early childhood to adolescence
- Has a history of overcoming predictions and demonstrating individual potential
- Often has meaningful relationships and community connections

Recommended Accommodations

- Transition-focused IEP addressing post-school education, employment, and independent living
- Community-based instruction and vocational experiences
- Daily living skills in natural settings
- Self-determination curriculum and student-led IEP participation
- Coordination with adult services beginning at least 2 years before graduation

Assistive Technology

- Tools matched to current communication and independence needs

- Community navigation and daily life management apps
- Vocational task management supports

UDL Strategies

- Person-centered planning for transition
- Self-determination as the central instructional framework
- Student voice and choice in all daily decisions as preparation for adult life

IEP Considerations

- Transition goals must reflect the student's actual post-school vision, not generic defaults
- Coordinate with VR, DDS, and community adult programs before graduation
- Annual goal: Student will participate in their own IEP meeting by presenting X goals and preferences
- Summary of Performance is a critical document — prepare it thoughtfully and in plain language

Who to Collaborate With

- Transition Specialist
- Adult services agencies

Family Partnership Tip

Start visiting adult day programs, supported employment providers, and supported living options now — not after graduation. The right fit takes time to find, and waitlists are long.

Resources: CDC — Developmental Delays | OSEP — Early Intervention & IDEA | Understood — Developmental Delay

Intellectual Giftedness with Co-occurring Disability

Twice-exceptional (2e) students who are intellectually gifted with a co-occurring disability (other than autism or learning disability) present unique profiles where high cognitive ability coexists with significant challenges related to ADHD, anxiety, physical disability, chronic health, or other conditions. Their giftedness is often masked by their disability and vice versa, leading to chronic underidentification and underservice.

<https://fyvr.net/student-personas/intellectual-giftedness-with-co-occurring-disability>

Elementary (K–5)

A student who astonishes adults with their insights, vocabulary, and depth of thinking in one moment, and then frustrates them completely in the next when the disability-related challenge emerges. The gap between their highest and lowest performance is striking. Teachers may see one side or the other — rarely both at once.

What You Might Notice

- Remarkable verbal reasoning or creative production in areas of strength
- Significant challenges in areas affected by the co-occurring disability
- Uneven profile: high complexity in some tasks, significant struggle in others
- May underperform on standardized measures because disability suppresses expression of ability

Student Strengths

- Exceptional intellectual ability in one or more domains
- Intense curiosity, creativity, and depth of thinking
- Has developed sophisticated understanding of their own learning profile

Recommended Accommodations

- Challenge and enrichment for areas of giftedness — never remove rigor as a response to disability
- Disability-specific accommodations for the co-occurring condition
- Flexible grouping: sometimes with gifted peers, sometimes with grade-level, sometimes in pull-out for disability support
- Psychological safety to try and fail without losing access to challenge
- Gifted program participation alongside IEP services — never an either/or choice

Assistive Technology

- AT matched to the specific disability (e.g., text-to-speech for reading disability, voice recognition for motor disability)
- Advanced content platforms for intellectual engagement
- Regulation tools if emotional dysregulation co-occurs

UDL Strategies

- Design activities with a wide range of entry and exit points — accessible floor, high ceiling
- Use open-ended, project-based learning that accommodates diverse approaches
- Never lower intellectual ceiling because of disability

IEP Considerations

- Both sets of needs must appear explicitly in the IEP
- Annual goals address the disability — not the giftedness (giftedness is served through the gifted program)
- Coordinate between gifted program and special education — they rarely communicate without explicit structure
- Cognitive evaluation must assess ability separately from disability-affected performance

Who to Collaborate With

- Gifted Education Specialist
- Special Education Teacher

Family Partnership Tip

Be a fierce advocate for both sides of your child's profile. Push for challenge AND support simultaneously. Any school that says they can't do both needs to be educated — best practices are clear.

Middle School (6–8)

A student whose profile is increasingly misread — teachers see either the giftedness (and wonder why they're failing) or the disability (and underestimate their potential). Perfectionism, anxiety, and frustration are common. May be disengaging from education because neither system is working.

What You Might Notice

- Deep engagement with chosen topics alongside avoidance of disability-affected domains
- Frustration when intellectual ability is 'wasted' on boring or unchallenging content
- May have anxiety or depression linked to the experience of simultaneous giftedness and disability
- Social complexity from being 'different' in multiple directions simultaneously

Student Strengths

- Exceptional depth of knowledge and original thinking in areas of passion
- Developing self-awareness about their unique profile
- Creative, non-traditional problem-solving

Recommended Accommodations

- Intellectual challenge protected: access to advanced coursework with disability accommodations
- Disability accommodations applied consistently in all settings including gifted program
- Counseling for identity, perfectionism, and the stress of navigating two worlds
- Choice in how mastery is demonstrated — accommodates disability while honoring giftedness
- Advocacy from adults who understand 2e and take both sides seriously

Assistive Technology

- Advanced research and creation tools for intellectual engagement
- Disability-specific AT for the co-occurring condition
- Regulation and self-management tools

UDL Strategies

- Complex, open-ended tasks that allow students to go as deep as they can
- Multiple pathways to demonstrate mastery
- Social-emotional curriculum that explicitly addresses 2e identity

IEP Considerations

- Annual IEP goals address the disability, but the team must actively advocate for intellectual challenge
- Address the gifted-disability intersection explicitly in present levels
- Both gifted and special education staff should attend the IEP meeting
- Consider whether the student's underperformance reflects untreated disability, inadequate challenge, or both

Who to Collaborate With

- Gifted Education Coordinator
- School Counselor

Family Partnership Tip

Find a community of other 2e families — SENG (Supporting Emotional Needs of the Gifted) and 2eNewsletters are good starting points. You are not alone, and your child is not broken — just complex.

High School (9–12)

A student who, with the right support, is capable of extraordinary things after graduation — and who, without it, may be at significant risk of failure, dropout, or underachievement well below their potential. Self-advocacy, AT proficiency, and identity development are the most critical transition outcomes.

What You Might Notice

- May be earning As in advanced courses while failing others due to disability access
- Has often internalized conflicting messages about their ability and worth
- Is developing a complex post-secondary vision that honors their potential
- Has genuine intellectual gifts that should drive their future planning

Student Strengths

- Exceptional potential in specific intellectual domains
- Developing complex self-understanding and self-advocacy
- Has navigated an unusually difficult educational experience with real resilience

Recommended Accommodations

- All disability accommodations maintained in all courses including advanced
- Flexible demonstration of mastery in all courses
- College planning that identifies programs strong in both intellectual challenge AND disability services
- Counseling focused on identity and post-secondary readiness
- Recognition of their full potential in transition planning — no ceiling

Assistive Technology

- Advanced disability-specific AT proficiency for post-secondary independence
- Research and creation tools appropriate to intellectual level
- Self-management tools for executive function or emotional regulation

UDL Strategies

- Honor the full complexity of the student's profile in all planning
- Build self-advocacy for both disability accommodations AND intellectual opportunities
- Connect with 2e college programs and communities

IEP Considerations

- Transition goal: Student will independently identify and request both disability accommodations AND intellectual challenge opportunities in post-secondary settings
- Summary of Performance must clearly articulate both the disability support needs AND the intellectual strengths
- Annual goal: Student will use identified AT tools independently to access and produce in all courses with minimal adult prompting
- College planning should include schools with strong honors programs AND strong disability services

Who to Collaborate With

- Gifted Program Coordinator
- Guidance Counselor

Family Partnership Tip

Your student's potential is not defined by what school has been hard. Help them see themselves through the lens of their gifts AND their hard-won resilience — that combination is powerful.

Resources: [NAGC — Twice-Exceptional](#) | [Understood — Gifted with Disabilities](#) | [2e Newsletter](#)

Complex & Intersectional

Traumatic Brain Injury

Traumatic Brain Injury (TBI) is an IDEA-specific eligibility category for students who have sustained an acquired brain injury from an external physical force. TBI can affect cognition, language, memory, attention, reasoning, abstract thinking, problem-solving, sensory, perceptual, and motor abilities, psychosocial behavior, physical functions, and speech. The educational impact is highly individualized and may change over time during recovery.

<https://fyvr.net/student-personas/traumatic-brain-injury>

Elementary (K–5)

A student who was previously typically developing but, following a TBI from a car accident, sports injury, or fall, now has significant changes in memory, attention, behavior, and academic performance. Classmates and teachers often don't understand why the 'same' student is so different now. School re-entry after TBI is a critical juncture.

What You Might Notice

- Significant memory difficulties — forgets recently learned material
- Fatigue that comes on rapidly and is not related to effort or motivation
- Behavioral or emotional changes that are new since the injury
- Performance that is inconsistent and doesn't match prior academic history

Student Strengths

- Prior academic knowledge and skills are often preserved even when new learning is affected
- Motivation to return to prior functioning can be strong
- Has typically-developing peers who knew them before and can provide social support

Recommended Accommodations

- Reduced school day and gradual return to full schedule
- Rest breaks when fatigue emerges — this is medically necessary, not laziness
- External memory aids: written schedules, checklists, digital reminders
- Reduced homework and workload during recovery phase
- Avoid penalizing for cognitive changes that are neurological, not behavioral

Assistive Technology

- Google Calendar and reminder apps for memory support
- Recorded lessons for review when fatigue prevented learning
- Note-taking apps and organizational tools

UDL Strategies

- Reduce cognitive load: chunk information, repeat key concepts, allow longer processing time

- Use external supports universally: visual schedules, checklists, worked examples
- Allow flexible pacing — recovery is not linear

IEP Considerations

- TBI is a specific IDEA eligibility — ensure this category is used, not just 'Other Health Impairment'
- IEP must be developed collaboratively with the student's medical team (neurologist, neuropsychologist)
- Annual goals should reflect the dynamic nature of TBI recovery — goals may need mid-year revision
- Track both recovery gains AND areas of persistent difficulty over time

Who to Collaborate With

- School Psychologist (neuropsychological consultation)
- Student's medical/rehabilitation team

Family Partnership Tip

Share the neuropsychological evaluation from the hospital with the school — this is the most detailed map of your child's brain injury and should drive every IEP decision. Don't let it sit in a folder.

Middle School (6–8)

A student navigating TBI recovery in the complex environment of middle school. Cognitive fatigue, memory difficulties, and behavioral changes may be misread as laziness or attitude. Peers may not understand the changes. The student may have a clear memory of who they were before the injury, which adds to psychological distress.

What You Might Notice

- Fatigue significantly limits participation, especially later in the day
- Memory and organizational deficits that look like inattention
- Emotional regulation challenges that are new post-injury
- Social difficulties linked to personality or communication changes

Student Strengths

- Prior academic and social knowledge that was not erased by the injury
- Often highly motivated to recover and reconnect with prior self and peers
- Support network from pre-injury relationships

Recommended Accommodations

- Flexible schedule with rest periods built in
- External organizational supports for all classes
- Memory aids: recorded instructions, written summaries, daily review
- Counseling to address identity and adjustment to post-injury self
- Collaborative academic recovery plan that is realistic about the timeline

Assistive Technology

- Otter.ai for lecture recording

- Digital planner with reminders
- Text-to-speech for reading tasks if visual fatigue is an issue

UDL Strategies

- Reduce cognitive demands on every task: shorter chunks, more supports, more time
- Allow all assignments with external memory supports (notes, outlines) on all tests
- Use frequent, brief check-ins to monitor fatigue and comprehension

IEP Considerations

- TBI IEPs must be revisited more frequently than annual cycle if recovery is active
- Annual goals should include both cognitive skill recovery goals AND academic content goals
- Counseling as a related service is typically warranted
- Ongoing neuropsychological monitoring should inform IEP revisions

Who to Collaborate With

- School Psychologist
- Neurologist/Neuropsychologist (ongoing)

Family Partnership Tip

TBI recovery is a marathon, not a sprint. Ask the school to revise the IEP every 6 months during the active recovery phase — annual review is not frequent enough when the student is changing rapidly.

High School (9–12)

A student who may have had a TBI years ago with persistent cognitive effects, or one in active recovery from a recent injury. Academic performance, memory, and executive function are affected in ways that will follow the student into college and career. Self-advocacy and documentation are critical.

What You Might Notice

- Ongoing memory and executive function challenges
- Fatigue management is a daily reality
- Self-awareness about TBI-related challenges is developing
- Post-secondary planning must account for persistent cognitive effects

Student Strengths

- Has developed extraordinary resilience and self-knowledge from recovery
- Many cognitive strengths are preserved
- Motivated to succeed and reconnect with pre-injury potential

Recommended Accommodations

- All accommodations current and documented for post-secondary transition
- Reduced course load during fatigue-heavy periods
- External memory supports for all academic tasks
- Career planning that accounts for cognitive strengths and vulnerabilities

- Counseling for identity, adjustment, and future planning

Assistive Technology

- Comprehensive organizational and memory support apps
- Text-to-speech and voice recording tools
- Smart devices with calendar, reminders, and task management integration

UDL Strategies

- Build explicit cognitive load management strategies into all course work
- Allow repeated exposure to content for memory consolidation
- Use portfolio-based assessment to document learning over time

IEP Considerations

- Transition plan must be realistic about ongoing TBI-related cognitive challenges
- Coordinate with neurologist for current documentation for college disability services
- Annual goal: Student will independently use memory and organizational supports in X post-secondary contexts
- Summary of Performance must include TBI-specific cognitive profile from current neuropsychological data

Who to Collaborate With

- Neuropsychologist
- Transition Coordinator

Family Partnership Tip

Keep a current neuropsychological evaluation — colleges require documentation that is no more than 3–5 years old for TBI accommodations. Plan evaluations accordingly.

Resources: BIAA — Brain Injury in School | OSEP — TBI & IDEA | CDC — TBI in Schools

Multiple Disabilities

Multiple Disabilities is an IDEA eligibility category for students who have simultaneous impairments — such as intellectual disability combined with physical impairment, or autism combined with blindness — that cause educational needs so severe that they cannot be accommodated in a program designed for any one disability alone. These students require highly individualized, coordinated, multidisciplinary support across every aspect of their school day.

<https://fyvr.net/student-personas/multiple-disabilities>

Elementary (K–5)

A student with two or more significant co-occurring disabilities who requires intensive support across communication, mobility, learning, and daily life. No single description fits — these students are defined by their complexity and individuality. The IEP team must bring all disciplines together around the unique person.

What You Might Notice

- Needs intensive support across multiple domains simultaneously
- Communication is complex and often requires multiple modalities
- Physical, cognitive, sensory, and behavioral needs interact with each other
- Participation in all settings requires individualized planning and adaptation

Student Strengths

- Has developed individualized and unique ways of engaging with the world
- Responds meaningfully to familiar people, routines, and sensory preferences
- Has a life, a personality, and preferences that matter and should drive educational planning

Recommended Accommodations

- Highly individualized educational program developed by full multidisciplinary team
- All physical access, communication, positioning, and sensory needs addressed in the IEP
- Full-day paraprofessional or health aide support as needed
- Meaningful participation in school community — not just physical presence
- Family as central, essential partners in daily planning

Assistive Technology

- High-tech and low-tech AAC for communication across settings
- Switch-access and alternative computer access
- Adaptive equipment for physical access and mobility

UDL Strategies

- Design all instruction from the student's current abilities, preferences, and priorities outward
- Use multi-sensory instruction that meets the student where they are
- Embed every skill in naturally occurring, meaningful activities

IEP Considerations

- IEP team must include SLP, OT, PT, special educator, general educator, family, and others as needed
- Annual goals must address communication as a primary priority
- ESY services are virtually always appropriate
- LRE must be individually determined — most restrictive is not always appropriate, but exclusion is never acceptable

Who to Collaborate With

- Full multidisciplinary team: SLP, OT, PT, BCBA, school nurse, vision/hearing specialists as appropriate
- Family as central team members

Family Partnership Tip

You are the most important team member in every IEP meeting. You know things about your child that no specialist ever will. Bring your full knowledge — their preferences, their communication, what works at home.

Middle School (6–8)

A student in middle school with multiple disabilities whose program requires ongoing coordination across all systems. Social belonging, communication expansion, and increasing self-direction are priorities alongside functional academic skills and daily living. The student is growing and changing, and the IEP must keep pace.

What You Might Notice

- Program requires intensive coordination across multiple staff and disciplines
- Social participation in age-appropriate settings is a priority requiring active facilitation
- Communication is functional but can always expand — never assume a communication ceiling
- The student is developing preferences, relationships, and a personal identity

Student Strengths

- Has a unique personality and genuine preferences that should drive all planning
- Responds to and benefits from meaningful, consistent relationships
- May have developed significant skills in specific areas that are not always visible in school

Recommended Accommodations

- All prior accommodations reviewed and updated annually with current data
- Communication system updated with age-appropriate, relevant vocabulary
- Expanded community-based instruction for generalization
- Social facilitation in age-appropriate environments
- Student's preferences actively solicited and honored in all planning

Assistive Technology

- AAC updated to reflect the student's age, interests, and environment
- Adaptive equipment updated for growth and changing needs
- Technology for communication, learning, and recreation

UDL Strategies

- Presume competence — never assume the student cannot learn or communicate
- Use person-centered planning to identify the student's vision and priorities
- Facilitate genuine peer relationships, not just proximity

IEP Considerations

- Annual goals must reflect current assessment data across all domains
- Begin formal transition planning — by age 14 in most states
- Student's participation in their own planning must be supported and documented
- Health plan must be current with all medical management needs

Who to Collaborate With

- Full multidisciplinary team
- Transition Specialist (beginning)

Family Partnership Tip

Person-centered planning tools (like MAPS or PATH) give your family and child a structured way to describe the life you're working toward. Ask the transition team about using these before formal IEP meetings.

High School (9–12)

A student for whom transition planning is the full educational program. The goal is the most meaningful, supported, and self-determined adult life possible — and that looks different for every individual. Coordination with adult services is urgent and complex.

What You Might Notice

- Full educational program focused on transition to adult life
- Vocational, daily living, communication, and community skills are the curriculum
- Student has a life vision that should drive every planning decision
- Family and adult services system coordination is intensive and ongoing

Student Strengths

- The student is a full person with genuine preferences, relationships, and a future
- Has demonstrated learning and growth across many years of intensive support
- Families have developed deep expertise in supporting their child

Recommended Accommodations

- Community-based instruction in real environments for all skill areas
- Supported employment and vocational exploration
- Daily living skills across all environments
- Communication support maintained and expanded
- Person-centered planning and student-led IEP participation

Assistive Technology

- Communication and independence technology appropriate to adult environments

- Adaptive equipment for work and community settings
- Smart home and assistive technology planning for adult living

UDL Strategies

- Everything is individualized and person-centered — there is no generic curriculum here
- Student's choices and preferences are the primary instructional driver
- Community and family integration are embedded in all activities

IEP Considerations

- Coordinate with DDS, VR, supported living, and supported employment providers urgently
- Student must lead their own IEP meeting to the fullest extent of their ability
- Summary of Performance is a life-planning document — make it vivid, person-centered, and useful
- Plan for the IDEA exit carefully — at age 22, school services end; adult services must be in place

Who to Collaborate With

- Full transition team including adult service providers
- Family (indispensable)

Family Partnership Tip

If your child is approaching 22 without adult services in place, escalate immediately. Contact your state DDS office, parent information center, and a disability rights organization. This is a legal rights issue, not just a planning issue.

Resources: [OSEP — Multiple Disabilities & IDEA](#) | [TASH — Inclusive Education](#) | [Understood — Multiple Disabilities](#)

Deaf-Blindness

Deaf-Blindness is an IDEA-specific eligibility category defined as combined hearing and visual impairments that cause such severe communication and other developmental and educational needs that they cannot be accommodated in special education programs solely for children with deafness or children with blindness. Interveners and one-on-one specialized communication support are often essential to learning.

<https://fyvr.net/student-personas/deaf-blindness>

Elementary (K–5)

A student who has significant combined vision and hearing loss — who may have some usable vision and/or hearing, or may be functionally deaf and blind. Communication and access to the world are the foundational challenges. Every teacher needs specialized training to work with this student effectively.

What You Might Notice

- Limited access to visual and auditory information in the environment
- Communication is often tactile or requires specialized methods (tactile sign, object symbols)
- Navigation of the physical environment requires consistent, specialized support
- Learning requires an intervener who bridges the student to the world

Student Strengths

- Often tactile learners with strong sense of touch and kinesthetic memory
- Can develop meaningful communication and relationships with the right support
- Has a unique way of perceiving and engaging with the world that deserves respect

Recommended Accommodations

- One-on-one intervener who is trained in tactile communication
- All materials in tactile or high-contrast accessible format
- Orientation and mobility instruction throughout the school day
- Consistent, predictable routine to support environmental navigation
- Regular consultation with deaf-blind specialist

Assistive Technology

- Tactile AAC systems (object symbols, tactile sign)
- Braille displays and refreshable Braille
- Auditory and tactile environmental cues

UDL Strategies

- All instruction is tactile, experiential, and contextualized — no incidental learning possible
- Intervener mediates all learning by bridging sensory input
- Social inclusion requires active adult facilitation — it does not happen passively

IEP Considerations

- The National Center on Deaf-Blindness (NCDB) is the primary professional resource for this population
- An intervener (not just an aide) is an evidence-based, recommended support
- Annual goals must address communication as the absolute primary priority
- The IEP team must include specialists with deaf-blind expertise — general special education training is not sufficient

Who to Collaborate With

- Deaf-Blind Specialist
- Orientation and Mobility Specialist

Family Partnership Tip

Contact your state's Deaf-Blind Technical Assistance project (every state has one, coordinated through the NCDB). They provide free consultation, resources, and support specifically for families of children with deaf-blindness.

Middle School (6–8)

A student with deaf-blindness whose communication system and educational program are individualized and complex. The school day requires consistent implementation by a trained intervener who understands the student's unique communication and sensory profile. Academic content is accessed through highly adapted means.

What You Might Notice

- All learning mediated through tactile and residual sensory channels
- Communication is specialized and not always immediately readable by untrained observers
- Social participation requires active facilitation and planning
- Has genuine preferences, relationships, and engagement with the world when fully supported

Student Strengths

- Deep and meaningful engagement when communication and access are optimized
- Resilient, curious, and genuinely engaged with trusted adults and peers
- Unique perspective and embodied knowledge

Recommended Accommodations

- Trained intervener present at all times during the school day
- All materials adapted for tactile and residual vision/hearing access
- Consistent communication system used by all team members
- Orientation and mobility throughout the building and community
- Family training and home-school consistency

Assistive Technology

- Refreshable Braille display for students who read Braille
- Screen magnification and audio for students with residual vision/hearing
- Specialized tactile AAC tools

UDL Strategies

- All learning is direct, tactile, and concrete — no assumption of incidental access
- Use meaningful real-world activities as the primary learning context
- Communication system drives every instructional decision

IEP Considerations

- Annual goals must reflect the student's unique communication profile
- Transition planning should begin by age 14 — adult services for deaf-blind individuals are specialized
- IEP team should be small, skilled, and consistent — turnover is devastating for this population
- Consult state deaf-blind project at every IEP cycle

Who to Collaborate With

- Deaf-Blind Specialist
- TVI (Teacher of the Visually Impaired) and TODHH (Teacher of the Deaf/HH)

Family Partnership Tip

Push for your child's intervener to receive formal training in deaf-blind intervening through programs like TRACES or similar. An untrained aide is not the same as a trained intervener — the difference is enormous.

High School (9–12)

A student for whom transition planning to supported adult life is the educational program. Community participation, employment, meaningful relationships, and as much independence as possible are the goals. The support network this student will need for life must be actively built during the high school years.

What You Might Notice

- Program is fully transition-focused
- Communication and mobility continue as primary skill areas
- Vocational exploration requires significant adaptation and support
- Family and adult services planning is intensive and ongoing

Student Strengths

- Established communication system and meaningful relationships
- Has a life history, preferences, and a personality that should drive all planning
- The people who love and support this student are a strength to be honored

Recommended Accommodations

- Transition IEP focused on post-school participation and quality of life
- Community-based instruction with trained intervener support
- Supported employment exploration adapted to sensory and communication profile
- Daily living skills in real environments
- Person-centered planning with family and student at the center

Assistive Technology

- Technology appropriate to adult community and work settings
- Communication devices for use in community settings
- Smart home and assistive technology for independent living support

UDL Strategies

- Everything is person-centered, individualized, and community-referenced
- Families are equal partners — not consultants, but central team members
- Support is built around the student's vision, not a generic program

IEP Considerations

- Deaf-blind adult services are specialized and the waitlists are long — begin coordinating now
- Contact the Helen Keller National Center (HKNC) for transition resources and post-secondary planning
- Annual goal: Student will communicate preferences and needs in community/vocational settings using established communication system
- Summary of Performance must be rich, person-centered, and practically useful for adult service providers

Who to Collaborate With

- Deaf-Blind Specialist
- Helen Keller National Center (HKNC) transition consultant

Family Partnership Tip

Contact the Helen Keller National Center (HKNC) — they provide comprehensive adult services for individuals who are deaf-blind and have a transition program. This is a lifeline for families planning for adulthood.

Resources: OSEP — Deaf-Blindness & IDEA | NCDB — National Center on Deaf-Blindness | TSBVI — Deaf-Blind Resources

English Language Learner with Disability

English Language Learners (ELLs) who also have disabilities face a doubly complex situation: their disability-related challenges must be distinguished from the normal process of second language acquisition, and they are entitled to both ELL services (Title III/ESSA) and disability services (IDEA/504). These students are often underidentified for disabilities because language difference is used to explain all challenges, or over-identified because language difference is misread as disability.

<https://fyvr.net/student-personas/english-language-learner-with-disability>

Elementary (K–5)

A student who is simultaneously learning English and navigating a disability. The school must ensure both sets of needs are served simultaneously, not sequentially. Distinguishing what is language acquisition from what is disability requires careful evaluation in the student's home language and coordination between ELL and special education staff.

What You Might Notice

- Challenges in both English language acquisition and other developmental domains
- Difficulty that persists in the home language as well as English (indicator of possible disability beyond language)
- Need for both ELL language supports and disability-specific accommodations
- Potential for cultural and family misunderstanding of disability in the school context

Student Strengths

- Multilingual skill is a genuine cognitive and cultural asset — not a deficit
- Rich home language and cultural knowledge that enriches the classroom
- Resilience from navigating multiple challenging systems

Recommended Accommodations

- Both ELL services (language instruction) AND IEP services — never one at the expense of the other
- Evaluations conducted in the student's home language by bilingual evaluators
- All disability accommodations provided regardless of English proficiency level
- Family communication in the home language, including all IEP documents translated
- Cultural responsiveness in all educational planning

Assistive Technology

- Translation tools and bilingual digital content
- AAC systems with home language vocabulary
- Bilingual text-to-speech tools

UDL Strategies

- Use bilingual instructional materials — home language is a cognitive scaffold, not a crutch
- Involve bilingual staff or community members as instructional partners
- Value and build on home language and cultural knowledge in all content areas

IEP Considerations

- OCR and IDEA guidance is clear: ELL students cannot be denied IEP services due to language status
- Evaluation must be in the home language — English-only evaluation is legally insufficient for ELL students
- IEP meeting must have a qualified interpreter; family must receive all documents in their language
- Annual goals must address disability, not language acquisition alone

Who to Collaborate With

- ELL/ESL Teacher
- Bilingual School Psychologist (for evaluation)

Family Partnership Tip

You have a right to have all school documents, including the IEP, translated into your home language. You also have the right to a qualified interpreter at every meeting. Request these services in writing.

Middle School (6–8)

A student who may have been in U.S. schools for several years, whose English proficiency is increasing, but whose disability-related challenges are now clearer as language ceases to be the primary explanation. Academic gaps may be wide due to years of under-identification or insufficient services.

What You Might Notice

- English proficiency is improving but disability-related challenges persist
- Academic gaps that exceed what language acquisition alone would explain
- May have experienced delayed identification that left a long service gap
- Cultural navigation and identity development add complexity to an already complex profile

Student Strengths

- Bilingual and bicultural — genuine cognitive and social assets
- Resilient navigator of multiple complex systems
- Often has strong family and community connections

Recommended Accommodations

- Both ELL language development support AND IEP services
- Culturally responsive instruction and materials
- Bilingual family engagement — not just translation, but genuine partnership
- Disability accommodations applied in both ELL and general education settings
- Cultural mediation when family needs support navigating the disability services system

Assistive Technology

- Bilingual AT (Spanish-English text-to-speech, dual-language word prediction)
- Translation-integrated disability tools
- Bilingual communication boards or AAC vocabulary

UDL Strategies

- Honor the student's home language as a scaffold for academic learning
- Use culturally responsive texts and examples in all content areas
- Provide translanguaging opportunities — using both languages to understand and express

IEP Considerations

- Annual goals must be written in plain language accessible to the family
- ELL teacher should be at the IEP meeting or provide input in writing
- Do not exit student from ELL services prematurely to serve disability — both are rights
- Address the cultural context of disability with the family — understand their framework

Who to Collaborate With

- ELL/ESL Teacher
- Bilingual family liaison or community member

Family Partnership Tip

Disability services in schools are a right under U.S. federal law — for every child, regardless of immigration status. Do not hesitate to request an evaluation if you believe your child has learning challenges beyond language.

High School (9–12)

A student with strong home language literacy and an emerging English profile who also has a disability. Transition planning must account for immigration and legal status in post-secondary planning, the student's specific disability, and their bilingual/bicultural assets. College and career planning must be genuinely inclusive of this student's full context.

What You Might Notice

- Managing disability while also developing English proficiency for post-secondary contexts
- Post-secondary options may be complicated by immigration status — must be addressed compassionately and honestly
- Bilingual skills are a genuine labor market and academic asset
- Family may have strong hopes and specific visions for the student's future

Student Strengths

- Bilingual proficiency is a competitive advantage in virtually every career field
- Cultural and linguistic capital that enriches every setting
- Has navigated extraordinary complexity with remarkable resilience

Recommended Accommodations

- All disability accommodations maintained through graduation
- Bilingual post-secondary options explored: community college, bilingual workforce training
- Immigration-sensitive transition planning — in collaboration with family
- Home language assessment for any standardized testing where possible
- Culturally responsive counseling and transition support

Assistive Technology

- Bilingual AT tools appropriate to the disability and career context
- Translation-accessible digital learning tools
- Bilingual professional communication templates

UDL Strategies

- Leverage home language as an academic and professional asset in all transition planning
- Connect to bilingual career pathways and mentors
- Honor the student's full identity in all planning

IEP Considerations

- Transition plan must address the student's realistic post-secondary context, including language and legal factors
- Annual goal: Student will use identified disability accommodations AND language supports to access post-secondary training or employment
- Summary of Performance should be translated and explain accommodations in the family's language
- Coordinate with immigration-sensitive community organizations as part of transition team

Who to Collaborate With

- ELL/ESL Teacher
- Community liaison or immigration-informed social worker

Family Partnership Tip

Your child's home language is an asset, not an obstacle. In many careers — healthcare, education, social services, business — bilingualism opens more doors than it closes. Help your student see this as a strength.

Resources: [Colorín Colorado — ELL with Disabilities](#) | [OSEP — ELL & IDEA](#) | [NCELA — ELL Resources](#)

Twice-Exceptional: Gifted with Learning Disability

Twice-exceptional (2e) students who are intellectually gifted with a co-occurring learning disability present a complex profile where high cognitive potential exists alongside significant processing deficits. These students are frequently misidentified — either as simply gifted (because ability masks disability) or simply disabled (because disability suppresses expression of ability). Neither label alone is correct, and neither serves them.

<https://fyvr.net/student-personas/twice-exceptional-gifted-with-learning-disability>

Elementary (K–5)

A student who reads complex chapter books at home but can't decode unfamiliar words at school (or vice versa). Writes elaborate stories verbally but cannot get them onto paper. Understands abstract math concepts but cannot recall multiplication facts. The profile is internally inconsistent in ways that confuse teachers.

What You Might Notice

- Remarkably high performance in some domains alongside significant deficits in others
- Oral language ability that dramatically exceeds written output
- Frustration when intellectual ability is assumed absent because of the disability-related failure
- Creative, elaborate thinking that cannot be expressed through affected channels

Student Strengths

- Genuine intellectual giftedness in specific domains
- Creative, original thinking that can be explosive when given the right outlet
- Highly motivated when working in areas of strength

Recommended Accommodations

- Gifted programming participation is NOT contingent on disability support receiving priority first
- Disability accommodations applied consistently in gifted program AND general education
- Alternative methods for demonstrating knowledge that bypass the specific disability
- Explicit instruction in compensatory strategies for disability-affected skills
- Both challenge AND support — never one at the expense of the other

Assistive Technology

- AT specific to the learning disability (text-to-speech, speech-to-text, word prediction)
- Advanced content platforms for intellectual engagement at appropriate challenge level
- Organizational and executive function tools

UDL Strategies

- High ceiling + appropriate scaffold = 2e success
- Use open-ended tasks that allow genius AND allow accommodation of disability
- Never reduce intellectual demand as a disability accommodation

IEP Considerations

- IEP must explicitly describe the gifted profile alongside the disability profile
- Gifted teacher must attend or provide written input to the IEP meeting
- Annual goal addresses the SLD — not the giftedness
- Cognitive evaluation data must include both full-scale IQ and index/subtest analysis

Who to Collaborate With

- Gifted Education Specialist
- Special Education Teacher (LD specialist)

Family Partnership Tip

Request testing that assesses BOTH cognitive ability AND processing — ask for index scores, not just a full-scale IQ. The full-scale number averages out the peaks and valleys and can hide both the giftedness and the disability.

Middle School (6–8)

A student who is increasingly frustrated by a system that sees only one half of their profile. The gifted program may have rejected them due to grades; the special education team may be focused only on remediation. They need adults who see and hold the whole picture.

What You Might Notice

- Uneven grades — As in some subjects, Fs in others, for entirely different reasons
- Intellectually bored and disability-frustrated simultaneously — a potent mix
- May be developing anxiety, perfectionism, or a negative academic identity
- Has strong opinions about how they learn best — listen to them

Student Strengths

- Original intellectual ideas that should be developed and nurtured
- Has developed significant self-knowledge about their learning
- Strong motivation in areas of genuine intellectual challenge

Recommended Accommodations

- Advanced coursework with disability accommodations built in
- Disability-specific AT in every class including honors/advanced
- Flexible demonstration formats: no student should fail because of the disability channel, not the content
- Counseling for identity and perfectionism
- 2e-informed adults on the team who understand the full profile

Assistive Technology

- Text-to-speech and speech-to-text for affected skill areas
- Advanced research tools for intellectual areas
- Organization and executive function tools

UDL Strategies

- Open-ended project options that allow wide entry points
- Multiple ways to demonstrate mastery — never one-size assessment
- Teach to the top of the profile while supporting the bottom

IEP Considerations

- Annual goal addresses the SLD — annual enrichment plan addresses the giftedness
- Both plans should be coordinated and should reference each other
- Evaluate for co-occurring ADHD or anxiety — very common in this profile
- Gifted coordinator and special education case manager should meet regularly

Who to Collaborate With

- Gifted Coordinator
- Special Education Case Manager

Family Partnership Tip

Find an evaluator who is specifically familiar with twice-exceptional students — they will know how to identify both the giftedness and the LD in the same evaluation, and why that distinction matters for educational planning.

High School (9–12)

A student who may be academically failing in some courses while excelling in others — and for whom the gap is explained entirely by whether the disability is being accommodated. College is very often in the picture; the challenge is navigating both a strong application and a detailed accommodations plan simultaneously.

What You Might Notice

- AP or honors performance alongside failure in courses requiring the affected skills unaided
- College application process is complicated: the student has both extraordinary strengths and genuine needs
- Has often developed a sophisticated understanding of their own profile
- Can articulate exactly what helps and what doesn't — listen carefully

Student Strengths

- Exceptional intellectual potential in one or more domains
- Has developed extraordinary advocacy skills and self-knowledge
- Has navigated a genuinely difficult educational experience with persistence and creativity

Recommended Accommodations

- All SLD accommodations in all courses including advanced and AP
- College planning that identifies schools strong in both academic program AND disability services
- Extended time and AT accommodations on all standardized tests (apply early)
- Self-advocacy practice for both giftedness and disability disclosure
- Transition counseling that honors both the intellectual potential and the support needs

Assistive Technology

- Comprehensive AT toolkit proficiency before graduation

- Advanced disability-specific tools (Dragon, Snap&Read, etc.)
- Research and creation tools appropriate to intellectual level

UDL Strategies

- Portfolio-based and project-based assessment allows full expression of both giftedness and accommodation of disability
- Teach self-advocacy for both AT use and academic challenge
- College essays are an opportunity to narrate this complex story authentically

IEP Considerations

- Transition plan must honor both sides: ambitious post-secondary goals AND specific accommodation needs
- Summary of Performance must articulate the full 2e profile — gifted and disabled — clearly
- Annual goal: Student will independently use AT tools and request accommodations in all dual enrollment or advanced coursework
- College Board/ACT accommodations applications should reference both the giftedness and the disability data

Who to Collaborate With

- Guidance Counselor (college planning)
- Special Education Case Manager

Family Partnership Tip

Your student's college essay may be the most powerful place to tell the 2e story — the journey of navigating giftedness and disability simultaneously often produces extraordinary young people with extraordinary things to say.

Resources: [NAGC — Twice-Exceptional](#) | [2e Newsletter](#) | [Understood — 2e Gifted with LD](#)

Chronic Health Conditions

Chronic Health Conditions

Sickle Cell Disease

Sickle Cell Disease (SCD) is an inherited blood disorder in which red blood cells become rigid and crescent-shaped, blocking blood flow and causing episodes of severe pain called vaso-occlusive crises. It can also cause stroke, organ damage, fatigue, and increased susceptibility to infection. Students with SCD may have frequent absences and unpredictable medical episodes that require flexible, well-coordinated school support.

<https://fyvr.net/student-personas/sickle-cell-disease>

Elementary (K–5)

A young student who is often cheerful and social but experiences sudden painful episodes that require immediate nurse visits or early dismissal. May miss school frequently and struggle to maintain pace with peers due to fatigue and pain.

What You Might Notice

- Sudden complaints of severe joint, chest, or abdominal pain requiring immediate nurse referral
- Fatigue and low stamina; needs to rest after minimal physical activity
- Frequent absences, sometimes in clusters around illness or weather changes
- Difficulty concentrating after returning from absences or pain episodes
- Reluctance to participate in outdoor or physically demanding activities

Student Strengths

- Strong emotional resilience and adaptability developed through navigating illness
- Close family relationships that provide consistent support and advocacy
- Empathy and social awareness from lived experience with chronic illness

Recommended Accommodations

- Immediate access to the school nurse at any time without requiring a hall pass
- Unlimited water and snack access throughout the day to prevent dehydration
- Flexible attendance and makeup work policy documented in health plan
- Seating away from drafts, air conditioning vents, or cold environments
- Reduced or modified physical education participation on high-risk days

Assistive Technology

- Tablet or Chromebook for completing makeup work remotely during hospitalization
- Text-to-speech tools (Read&Write;) for when fatigue makes sustained reading difficult
- School-to-home communication platform (ClassDojo, Remind) for continuity during absences

UDL Strategies

- Provide recorded or video versions of key lessons for makeup during absences

- Use flexible grouping so student can join collaborative activities at any point
- Offer multiple ways to demonstrate learning (oral, drawn, written) based on daily capacity

IEP Considerations

- Student is typically eligible under Other Health Impairment (OHI); a 504 Plan may be appropriate if academic performance is not significantly affected but health needs require accommodation
- Present levels should document typical absence patterns and their academic impact; document baseline performance after return from crisis
- Health plan (IHP) should be developed with school nurse and family detailing crisis protocol, pain management steps, and hospitalization communication procedures
- Accommodation language: flexible attendance and makeup deadlines, immediate nurse access, environmental modifications (temperature, hydration)

Who to Collaborate With

- School Nurse (primary coordinator of health plan)
- Special Education Teacher or 504 Coordinator
- School Counselor (social-emotional support during and after medical episodes)

Family Partnership Tip

Share a written crisis action plan with the classroom teacher at the start of each school year — include early warning signs of a pain episode, steps you want the school to take, and the best way to reach you. Update it if the student's treatment regimen changes.

Middle School (6–8)

A student managing increasing academic complexity while dealing with unpredictable health episodes. May be embarrassed about health needs and reluctant to use accommodations in front of peers. Academic gaps from repeated absences may be widening.

What You Might Notice

- Leaves class suddenly and frequently for nurse visits, sometimes returning the same period
- Patchy knowledge base due to gaps from accumulated absences across grade levels
- Visible fatigue mid-day, especially following a recent crisis or hospitalization
- Hesitation to disclose pain to teachers to avoid drawing peer attention
- Difficulty with extended writing or fine motor tasks during active pain periods

Student Strengths

- Sophisticated self-awareness about health and body signals
- Mature perspective and strong problem-solving developed through managing chronic illness
- Motivation to keep up with peers; often highly self-directed when feeling well

Recommended Accommodations

- Discreet exit pass system allowing nurse visits without classroom announcement
- Extended time on all assessments, with option to resume across sessions

- Digital copies of all textbooks and materials for home/hospital access
- Homework and project deadlines adjusted following absence without academic penalty
- Access to a quiet, warm rest space during the school day

Assistive Technology

- Google Classroom or Canvas for remote assignment submission and continuity
- Dragon NaturallySpeaking or Google Voice Typing for extended writing during pain episodes
- Telehealth-compatible device setup if district supports hospital-homebound instruction

UDL Strategies

- Post all lesson materials, notes, and assignments in the LMS so student can access from home
- Offer asynchronous options for participation (discussion board, video response) during absences
- Build explicit self-advocacy skills: help student practice communicating their needs to each teacher

IEP Considerations

- IEP or 504 should include a Health Plan developed in collaboration with school nurse, specifying school-day management steps and emergency contacts
- Present levels should document cumulative academic impact of absences including gap analysis by content area
- Annual goals may address academic skill gaps (e.g., reading fluency, math computation) directly caused by missed instruction
- Document homebound instruction eligibility and trigger conditions (e.g., hospitalization exceeding 5 consecutive days)

Who to Collaborate With

- School Nurse
- 504 Coordinator or Special Education Case Manager
- Hospital Liaison or Homebound Instruction Coordinator

Family Partnership Tip

Encourage your child to keep a simple pain log (rating, time, trigger if known) and share it with the school nurse weekly. Patterns can help the team anticipate difficult periods and proactively adjust academic expectations.

High School (9–12)

A teenager managing SCD increasingly independently but facing high-stakes academic consequences from absences and fatigue. Self-advocacy and transition planning are critical; the student needs to understand their rights and how to navigate disability services in post-secondary settings.

What You Might Notice

- Missing credit hours due to frequent medically-related absences; at risk of course failure despite demonstrated understanding
- Fatigue-related performance drop in afternoon classes or during high-demand exam weeks
- Reluctance to disclose SCD to new teachers each semester; navigates health needs quietly

- Anxiety about standardized testing schedules conflicting with health episodes
- Strong verbal performance when feeling well but inconsistent written output

Student Strengths

- Deep self-knowledge and advocacy skills built over years of navigating the medical system
- Genuine motivation to succeed academically despite systemic barriers
- Ability to inspire peers through resilience and authentic perspective

Recommended Accommodations

- Attendance flexibility with a clear makeup plan that cannot result in course failure for medically-excused absences
- Option to complete assessments across multiple sessions or on an alternate day following crisis
- Priority scheduling for classes that matter most to graduation or college plans
- Written accommodation letter to all teachers at the start of each semester
- Access to notes, slides, and recordings for any missed instruction

Assistive Technology

- Notability or OneNote for digital note-taking accessible on any device
- Telehealth or hospital homebound instruction tools during extended hospitalizations
- College Board/ACT accommodation documentation tools for standardized testing

UDL Strategies

- Explicitly teach post-secondary self-disclosure and accommodation request processes
- Allow flexible assignment formats so student can complete work during high-capacity windows
- Use project-based learning where work can be distributed over time rather than concentrated on single-day tests

IEP Considerations

- Transition planning must address post-secondary disability services registration; help student practice self-disclosure language for college disability offices
- 504 or IEP should explicitly protect against course failure or GPA reduction due solely to medically-excused absences
- Consider College Board or ACT accommodation requests early (spring of junior year at latest) — SCD qualifies under medical documentation
- Annual transition goals may include: student will independently contact teachers and disability services to arrange accommodations in X out of Y opportunities by [date]

Who to Collaborate With

- Transition Coordinator
- School Nurse and 504 Coordinator
- School Counselor (graduation planning and college access)

Family Partnership Tip

Help your teen gather and organize their medical documentation now — diagnosis letters, treatment records, physician statements — so they are ready to register with disability services when they reach college or vocational training.

Cystic Fibrosis

Cystic Fibrosis (CF) is a genetic condition that causes the body to produce thick, sticky mucus that clogs the lungs and digestive system. Students with CF require daily respiratory therapies, enzyme supplements with meals, and frequent medical appointments that affect school attendance. While cognitive ability is unaffected, fatigue, chronic cough, and the physical burden of treatment significantly impact stamina and participation.

<https://fyvr.net/student-personas/cystic-fibrosis>

Elementary (K–5)

A child who appears engaged and capable but tires easily and may need to leave class for airway clearance or medication. A persistent, wet cough is common and can be socially uncomfortable; classmates may be curious or reactive.

What You Might Notice

- Persistent, productive cough that is louder and more frequent than a typical cold
- Needs to leave class one or more times per day for airway clearance treatments
- Slower to eat lunch; needs time to take enzyme capsules before eating
- Fatigue after physical activity; may need to sit out part of PE
- Frequent absences for pulmonary clinic appointments or respiratory illnesses

Student Strengths

- Strong routine-following skills from years of adhering to daily treatment schedules
- Mature sense of responsibility and self-care for their age
- Typically strong verbal reasoning and curiosity about science and how things work

Recommended Accommodations

- Scheduled daily breaks for airway clearance; communicated privately to avoid stigma
- Extra time for lunch to allow for enzyme administration
- Unlimited access to water to support mucus clearance
- Flexible attendance and makeup work policy for medical appointments and illness
- Reduced exposure to respiratory irritants (perfumes, cleaning chemicals, dusty environments)

Assistive Technology

- Tablet or Chromebook for remote access to assignments during absences or hospitalizations
- Voice-to-text tools for writing when fatigue or coughing limits sustained work
- Communication platform (Seesaw, Remind) for home-school continuity during illness

UDL Strategies

- Record instructional videos or use class content in the LMS so student can catch up remotely
- Offer flexible participation options so student can contribute from a rest position or at low-energy moments
- Normalize wellness breaks as part of the classroom culture for all students

IEP Considerations

- Eligible for OHI under IDEA; a 504 Plan is often appropriate when health needs affect school access but academic performance is within grade level
- Individualized Health Plan (IHP) must be developed with the school nurse and CF care team; include airway clearance schedule, enzyme protocol, and emergency respiratory plan
- Accommodation language: scheduled nurse visits, attendance flexibility, extended makeup deadlines, environmental modifications
- Annual goals are rarely needed for CF itself; focus any goals on addressing skill gaps from missed instruction

Who to Collaborate With

- School Nurse (IHP coordinator)
- 504 Coordinator
- Physical Education Teacher (modified activity plan)

Family Partnership Tip

At the start of each year, provide the classroom teacher and nurse with a one-page summary of your child's daily schedule — when treatments happen, what the cough means, and what to do if breathing seems harder than usual. Teachers feel more confident when they know what to expect.

Middle School (6–8)

A student who manages most CF routines independently but is beginning to feel self-conscious about the cough and treatment schedule in a social middle school environment. May minimize symptoms to avoid attention, which can delay intervention.

What You Might Notice

- Suppressed or muffled cough in classroom, followed by more intense coughing in hallways
- Visible fatigue in afternoon classes, especially after a respiratory flare
- Eating slowly or skipping lunch to avoid taking enzymes publicly
- Increased absences during cold and flu season; misses clusters of consecutive days
- Difficulty sustaining attention during extended coughing episodes or after treatment breaks

Student Strengths

- High capacity for self-monitoring and routine management
- Strong intrinsic motivation to participate fully when health allows
- Often excels in areas of personal interest where engagement drives through fatigue

Recommended Accommodations

- Private, designated space for airway clearance without peer awareness
- Option to eat in a low-scrutiny environment (classroom, nurse's office) to manage enzyme routine
- Digital access to all class materials for home or hospital completion
- Extended time and flexible deadlines during and following respiratory illness
- Pass to leave any class without explanation for airway clearance or coughing management

Assistive Technology

- Google Classroom or Canvas for assignment submission from home or hospital
- Noise-canceling headphones to support focus during congested or high-cough periods
- Dragon NaturallySpeaking or Google Voice Typing for writing tasks when fatigue is high

UDL Strategies

- Provide asynchronous alternatives for participation so student can engage on good-health days
- Use chunked assignments with multiple submission windows rather than single-day tests
- Create private communication channel between student and teacher for daily check-ins

IEP Considerations

- 504 or IEP should include updated IHP from the CF care team at the start of each school year; nurse must have current emergency protocols
- Document cumulative academic impact of absences in present levels; address specific content gaps with targeted intervention if needed
- Consider whether student qualifies for homebound instruction during extended hospitalizations (often 10+ days)
- Accommodation language should explicitly address attendance policies to prevent failing grades due solely to medically-excused absences

Who to Collaborate With

- School Nurse
- 504 Coordinator or Special Education Case Manager
- School Counselor (body image and social stigma around cough)

Family Partnership Tip

Work with your child to develop a brief, age-appropriate script they can use if classmates ask about the cough — something true and simple, like 'I have a lung condition and this is just how my lungs clear themselves.' Rehearsing it reduces anxiety.

High School (9–12)

A teenager with strong self-management skills who may be managing increasing treatment complexity as lung function changes. College and career planning must account for CF management. Self-advocacy for post-secondary accommodations is a critical skill to develop now.

What You Might Notice

- Intermittent absences for IV antibiotic courses or pulmonary rehab; sometimes weeks at a time
- Visible fatigue and decreased stamina; may request schedule modifications during exacerbations
- Highly capable during high-health periods but performance drops sharply during illness
- Occasionally anxious about CF's impact on college plans, career goals, and independence
- May downplay symptoms to appear 'normal' among peers, delaying accommodation use

Student Strengths

- Sophisticated health self-management transferable to adult medical independence
- Exceptionally mature perspective on time, priorities, and what matters academically
- Strong advocacy skills when supported to use them

Recommended Accommodations

- Attendance and credit completion flexibility during hospitalizations and IV antibiotic courses
- Opportunity to complete all coursework and assessments remotely during extended illness
- Priority or modified schedule to allow medical appointments without missing critical instruction
- Access to testing accommodations on SAT, ACT, and AP exams based on medical documentation
- Written semester accommodation letter to all teachers before the first day of each semester

Assistive Technology

- Telehealth-compatible laptop or tablet for hospital-homebound academic continuity
- PDF annotation tools (Adobe, Notability) for completing assignments during medical stays
- College Board Services for Students with Disabilities documentation tools

UDL Strategies

- Explicitly teach post-secondary accommodation processes, including how to register with college disability offices
- Use mastery-based or flexible grading approaches where possible to reflect knowledge, not stamina
- Offer project-based alternatives to single-sitting tests during health-fragile periods

IEP Considerations

- Transition section of IEP or 504 must address post-secondary disability services registration; include goal for student to self-disclose and request accommodations independently
- Document how chronic absences have affected cumulative learning; plan credit recovery or alternative pathways if needed for graduation
- Ensure College Board/ACT accommodation requests are submitted well in advance; CF medical documentation typically meets the evidentiary standard
- Annual transition goal: student will independently contact post-secondary disability services and submit accommodation documentation in preparation for enrollment

Who to Collaborate With

- Transition Coordinator
- School Nurse and 504 Coordinator
- School Counselor (college planning with health considerations)

Family Partnership Tip

Connect your teen with CF Foundation's young adult resources and peer networks — hearing from young adults with CF who are thriving in college or careers can be more motivating than anything school staff can say. This is also a good time to practice having medical conversations independently at clinic visits.

Resources: Cystic Fibrosis Foundation — School | CDC — Cystic Fibrosis | NASN — CF in Schools

Type 1 Diabetes

Type 1 Diabetes (T1D) is an autoimmune condition in which the pancreas produces little or no insulin, requiring the student to manage blood glucose through insulin administration and continuous monitoring. Blood sugar fluctuations — both high (hyperglycemia) and low (hypoglycemia) — directly affect cognition, attention, and behavior. T1D requires immediate, unrestricted access to food, glucose tablets, and self-monitoring devices at all times.

<https://fyvr.net/student-personas/type-1-diabetes>

Elementary (K–5)

A child who may need to stop any activity at any moment to check blood sugar or eat. Behavioral changes — sudden irritability, crying, confusion, or lethargy — are often signs of low blood sugar rather than a behavioral issue. Teachers must recognize these signs quickly.

What You Might Notice

- Sudden mood shift — irritability, tearfulness, or unusual silliness without identifiable cause (possible hypoglycemia)
- Difficulty concentrating; glazed expression or slowed responses even when alert moments before
- Frequently leaves class to visit the nurse for blood glucose checks or insulin dosing
- Needs to eat or drink immediately during class without waiting for an appropriate break
- Shaking, pallor, or sweating — physical signs of low blood sugar requiring immediate response

Student Strengths

- Highly attuned to body signals and internal states — a form of sophisticated self-awareness
- Strong sense of responsibility developed through early management of health routines
- Often develops empathy for peers facing health challenges

Recommended Accommodations

- Unrestricted access to fast-acting glucose (juice box, glucose tabs) in the classroom at all times
- Permission to eat or drink during instruction without asking
- Immediate nurse access without a hall pass
- Designated private space for insulin pump checks or CGM alerts
- Do not penalize for any behavior that occurs during a blood sugar event; document and review with nurse

Assistive Technology

- Continuous Glucose Monitor (CGM) such as Dexcom G7 — alarm alerts teacher and student
- Insulin pump (e.g., Omnipod, Tandem t:slim) — wearable, managed by student and nurse
- Smartwatch paired to CGM (e.g., Apple Watch) for discreet glucose monitoring during class

UDL Strategies

- Normalize snacking during class for all students to reduce stigma around T1D food access
- Build predictable routines so student can anticipate transitions and plan blood sugar management
- Provide visual schedules and timers to help student self-manage monitoring within the school day

IEP Considerations

- T1D qualifies under OHI; most students are served under a 504 Plan unless academic performance is also affected
- A Diabetes Medical Management Plan (DMMP) from the student's physician and an Individual Health Plan (IHP) from the school nurse are required by federal law (IDEA and Rehabilitation Act Section 504)
- 504 accommodations must include: unrestricted food/water access, immediate nurse access, no academic penalty for T1D-related behavior or missed instruction, private space for device management
- Classroom teacher must be trained on hypoglycemia recognition and emergency glucagon protocol by the school nurse at the start of each year

Who to Collaborate With

- School Nurse (IHP and DMMP coordinator)
- 504 Coordinator
- School Counselor (managing social anxiety around T1D)

Family Partnership Tip

At the start of each school year, schedule a brief meeting with the classroom teacher to walk through your child's CGM alarms and what each alert means — most teachers are eager to help but have never seen a CGM before. Bring a laminated one-page emergency guide to leave on the teacher's desk.

Middle School (6–8)

A student who is beginning to manage T1D more independently but may be inconsistent in checking glucose during social situations to avoid peer attention. Blood sugar fluctuations may manifest as attitude, distraction, or social withdrawal that teachers misinterpret as behavioral issues.

What You Might Notice

- Periodic inattention, foggy responses, or emotional reactivity that resolves after eating
- Checking phone discreetly during class — likely monitoring CGM app, not social media
- Avoidance of nurse visits or eating in class due to peer embarrassment
- Excuses to leave class that are actually blood sugar management needs
- Energy crashes in late morning or mid-afternoon corresponding with glucose fluctuations

Student Strengths

- Data-driven thinking: students with T1D often become sophisticated health data interpreters
- Mature executive functioning from years of self-management decisions
- Resilience and self-reliance in navigating complex, high-stakes health situations

Recommended Accommodations

- Permission to use smartphone/smartwatch to monitor CGM during class without challenge
- Unrestricted access to snacks and glucose at desk — no need to explain or ask permission
- Private, low-key exit option for insulin management or nurse visits
- Flexibility on assessments if a blood sugar event occurs during testing

- Makeup time or reschedule option for any assessment interrupted by a T1D event

Assistive Technology

- CGM integrated with smartwatch for discrete glucose monitoring
- Insulin pump with smartphone app management (Omnipod 5, Control-IQ)
- Google Classroom or Canvas for assignment submission on high-variability health days

UDL Strategies

- Discuss privately with the student what support looks like to them — avoid calling attention to T1D in class
- Allow flexible seating near the door for easy, low-profile exits
- Build in check-in moments during long class periods so student can self-regulate without interrupting instruction

IEP Considerations

- 504 Plan should include explicit language permitting smartphone use for CGM monitoring — this is a medical device, not a distraction
- Present levels should note if glucose fluctuations have caused documented academic impact (missed assessments, inconsistent performance patterns)
- Ensure all subject teachers — not just homeroom — receive copies of the 504 accommodations and understand T1D alert recognition
- If student is experiencing significant academic gaps, evaluate for additional learning support alongside the health plan

Who to Collaborate With

- School Nurse
- 504 Coordinator
- All subject-area teachers (requires annual briefing)

Family Partnership Tip

Review CGM data weekly with your child to spot patterns — maybe 2nd period is consistently high, or 6th period low before lunch. Sharing this trend data with the school nurse can lead to proactive schedule or snack adjustments rather than reactive crisis responses.

High School (9–12)

A teenager who is largely self-sufficient in T1D management but faces additional pressure from high-stakes academics, athletic participation, and social contexts where managing diabetes feels stigmatizing. Transition planning should address independent medical management in college or the workplace.

What You Might Notice

- Highly variable performance — excellent on days with stable glucose, significantly impaired on high- or low-glucose days
- Occasionally distracted or anxious before major tests due to fear of blood sugar unpredictability
- May request retakes or extensions following documented T1D events

- Sophisticated self-advocate who can clearly articulate their needs if teachers are receptive
- May be managing an increasing complexity of insulin regimen alongside academic demands

Student Strengths

- Exceptional self-monitoring and real-time decision-making skills
- Strong intrinsic motivation to succeed despite unpredictable health variables
- Well-prepared for adult medical self-management with appropriate transition support

Recommended Accommodations

- Permission to reschedule any high-stakes assessment affected by a documented T1D event
- Unrestricted CGM device and insulin pump access throughout school day and during exams
- Accommodation letter provided to each teacher at semester start without requiring student to disclose details
- Extended time on standardized tests (SAT, ACT, AP) when supported by medical documentation
- Flexible attendance policy for endocrinology appointments without academic penalty

Assistive Technology

- Closed-loop insulin delivery system (Omnipod 5, Medtronic 780G) for automated management
- CGM with cloud sharing for parent/guardian or nurse remote monitoring
- College Board documentation tools for SSD accommodation requests

UDL Strategies

- Teach explicit self-advocacy language for post-secondary contexts — how to request test accommodations, disclose to a professor, or contact student health services
- Use flexible assessment formats that reduce the high-stakes, single-day-performance model
- Invite student to lead their own 504 meeting — a key self-advocacy milestone

IEP Considerations

- Transition goals should include independent management of medical appointments, medication refills, and post-secondary disability service registration
- 504 should include testing accommodation language suitable for college disability offices; prepare documentation packet early in junior year
- If student is heading to college, connect them with JDRF's T1D on Campus resources and the university disability services office before senior year
- Annual transition goal: student will independently schedule and attend two endocrinology appointments and manage prescription renewals during the academic year

Who to Collaborate With

- Transition Coordinator
- School Nurse and 504 Coordinator
- School Counselor (college planning with health management considerations)

Family Partnership Tip

Before senior year, practice having your teen handle one medical appointment entirely on their own — scheduling, speaking with the doctor, picking up prescriptions. Independence in medical self-management is as important as academic preparation for the transition to college.

Epilepsy & Seizure Disorders

Epilepsy is a neurological disorder characterized by recurrent, unprovoked seizures caused by abnormal electrical activity in the brain. Seizures vary widely — from brief absence episodes (staring, blinking) to generalized tonic-clonic events with loss of consciousness. Medications affect alertness and cognition; seizures themselves can cause temporary memory loss, confusion, and fatigue. Teachers must recognize seizure types, respond appropriately, and support the student's return to learning afterward.

<https://fyvr.net/student-personas/epilepsy-and-seizure-disorders>

Elementary (K–5)

A young child who may have brief absence seizures that are frequently missed or mistaken for daydreaming, or more visible convulsive seizures that require immediate teacher response. Post-seizure (postictal) confusion and fatigue are common and may last 30 minutes to several hours.

What You Might Notice

- Brief, repetitive staring episodes with unresponsiveness and eye blinking — possible absence seizures
- Sudden limpness, falling, or convulsive movements requiring immediate first aid
- Post-seizure confusion, disorientation, or deep fatigue lasting 30–60 minutes
- Difficulty following multi-step instructions; memory gaps for content taught during or after a seizure
- Slowed processing or drowsiness that may be a medication side effect

Student Strengths

- Often develops strong social connections and support networks in the classroom
- Cognitive strengths in non-affected areas often remain fully intact
- Families tend to be highly engaged advocates for their child's educational needs

Recommended Accommodations

- First seizure response plan posted in the classroom, reviewed by all school staff who interact with the student
- Immediate nurse notification for any seizure event; 911 protocol clearly defined in the health plan
- Designated safe seating away from hard edges, stairs, or high surfaces
- Extended recovery time after a seizure before resuming academic tasks
- Flexible makeup policy for content missed during or after seizure events

Assistive Technology

- Seizure alert wearable (Empatica Embrace2) for automated caregiver/nurse notification
- Text-to-speech tools to support learning when medication affects reading stamina
- Tablet with recorded lessons for catching up on missed instruction

UDL Strategies

- Provide all instructional content in multiple formats so gaps from seizure episodes can be filled asynchronously
- Use visual schedules and consistent classroom routines to reduce cognitive load for a student with variable attention

- Pre-teach vocabulary and concepts before new units so student has multiple exposures to key material

IEP Considerations

- OHI eligibility is most common; evaluate for comorbid learning or processing challenges which are more prevalent in students with epilepsy
- Seizure Action Plan (SAP) developed by the student's neurologist and coordinated by the school nurse must be on file and reviewed with staff annually
- IEP or 504 accommodations must address: postictal recovery time, makeup policies, seizure-safe environment, and medication administration by nurse
- If seizures are frequent, assess academic impact on memory and executive function — referral for neuropsychological evaluation may be appropriate

Who to Collaborate With

- School Nurse (SAP coordinator and medication administration)
- Special Education Teacher or 504 Coordinator
- School Psychologist (neuropsychological impact assessment if indicated)

Family Partnership Tip

Give each of your child's teachers a laminated copy of the seizure action plan and walk them through it at the start of the year. Include a simple first aid summary: 'time the seizure, protect the head, do not restrain, call the nurse at X minutes.' Most teachers feel much less anxious with a clear protocol in hand.

Middle School (6–8)

A student who may have well-controlled seizures but still experiences cognitive side effects from medication and social anxiety about having a seizure at school. Memory and executive function challenges may be increasing as academic demands grow.

What You Might Notice

- Reports of brief confusion or memory blanks — may have had a subtle seizure during class
- Difficulty retaining information over multiple class periods; inconsistent recall of previously learned material
- Significant fatigue following seizures or medication adjustments
- Social withdrawal or anxiety about being seen having a seizure at school
- Processing speed and short-term memory below what verbal ability suggests

Student Strengths

- High social intelligence and strong peer relationships when seizures are well-controlled
- Genuine determination to participate fully in school life despite health uncertainty
- Often strong in visual and hands-on learning contexts

Recommended Accommodations

- Preferential seating away from lab equipment, sharp surfaces, and high-traffic areas
- Flexible assessment scheduling — ability to reschedule if a seizure or postictal period occurs on test day
- Extended time on assignments and assessments due to processing speed impact of medication

- Note-taking support (peer notes, teacher-provided outlines) to address memory gaps
- Private, low-key recovery space after a seizure event before returning to class

Assistive Technology

- Smart pen (Livescribe) or digital note-taking to capture class notes for later review
- Read&Write; or Kurzweil 3000 for text support when medication slows reading processing
- Digital homework platforms so assignments can be submitted from home on difficult days

UDL Strategies

- Provide recorded lessons or slide-with-audio formats for students who miss or mentally check out during seizures
- Use spaced repetition review strategies to address epilepsy-related memory consolidation challenges
- Establish a private signal system so the student can alert the teacher discreetly before or after a seizure

IEP Considerations

- IEP or 504 should address cognitive side effects of anti-epileptic medications (AEDs) as well as the seizures themselves — document impact on memory, processing, and attention
- If neuropsychological testing has been completed, present levels should reflect those findings specifically
- Determine whether student qualifies for extended time or other testing accommodations on state assessments based on processing speed data
- Document seizure-safe environment requirements across all classes including gym, science lab, and shop

Who to Collaborate With

- School Nurse
- Special Education Teacher or 504 Coordinator
- School Psychologist (if cognitive impact requires evaluation)

Family Partnership Tip

Keep a brief seizure log and share the pattern with the school at the start of each semester — noting time of day, duration, and recovery time helps teachers understand what to expect and prepare for. If medication recently changed, alert the nurse and main teacher so they can watch for side effects.

High School (9–12)

A teenager working toward independence in seizure management while navigating high-stakes academics. Driving restrictions, employment limitations, and college planning all intersect with epilepsy. Self-advocacy for post-secondary accommodations is essential.

What You Might Notice

- Variable academic performance that correlates with seizure frequency and medication changes
- Anxiety about seizure occurrence during high-visibility situations: presentations, exams, athletic events
- May underreport seizures to preserve social reputation or avoid restrictions
- Cognitive fatigue affecting sustained performance in block-schedule or long-period classes
- Strong verbal performance inconsistent with written test scores — likely processing speed impact

Student Strengths

- Self-awareness and health advocacy skills built over years of medical management
- Strong intrinsic motivation to succeed academically and socially
- Often demonstrates exceptional perseverance and problem-solving

Recommended Accommodations

- Testing accommodations: extended time, breaks, ability to reschedule following a seizure
- Attendance flexibility for neurology appointments and post-seizure recovery days
- Safety accommodations in lab, PE, and vocational/shop classes documented in writing
- Accommodation letters distributed to all teachers at semester start without requiring student to explain medical history
- Option to take tests in small-group setting to reduce anxiety and provide safer environment

Assistive Technology

- Seizure alert wearable (Embrace2, Garmin Health) for independent monitoring
- Note-taking and voice recording apps (Otter.ai, Notability) for class capture
- College Board SSD documentation tools for SAT/ACT/AP accommodations

UDL Strategies

- Explicitly teach the post-secondary accommodation request process; role-play conversations with disability services offices
- Use portfolio and project-based assessment as an alternative or supplement to timed tests
- Build flexibility into curriculum delivery so student can access content during stable health windows

IEP Considerations

- Transition planning must address restrictions associated with epilepsy (driving, certain occupations) and how the student plans to navigate them
- Ensure SAP and accommodation documentation is updated and ready to transfer to post-secondary disability services or employers
- Annual transition goal: student will self-identify two disability services contacts at their planned post-secondary institution and initiate accommodation registration by [date]
- Consider whether Extended School Year (ESY) services are warranted if seizure frequency has caused significant academic regression

Who to Collaborate With

- Transition Coordinator
- School Nurse and 504 Coordinator
- Vocational Rehabilitation Counselor (if seizure-related restrictions affect career planning)

Family Partnership Tip

When your teen is planning for college, contact the university disability office before acceptance — many have specific processes for neurological conditions and can walk you through documentation requirements early. Having the documentation ready prevents a gap in accommodations during the critical first semester.

Resources: [Epilepsy Foundation — School](#) | [CDC — Epilepsy in Schools](#) | [NASN — Seizure Management](#)

Childhood Cancer & Treatment

Children and adolescents undergoing cancer treatment — including chemotherapy, radiation, and surgery — experience significant physical side effects (fatigue, nausea, immunosuppression) and cognitive effects often called 'chemo brain,' including memory and attention difficulties. School re-entry and continuity of learning during treatment require flexible, compassionate, and coordinated support. Survivors may face long-term cognitive and physical late effects that continue after treatment ends.

<https://fyvr.net/student-personas/childhood-cancer-and-treatment>

Elementary (K–5)

A child who may be returning from extended absences for treatment or receiving ongoing outpatient chemotherapy while trying to maintain school attendance. Physical appearance may change due to hair loss, weight changes, or a port/PICC line. Peers will notice and react.

What You Might Notice

- Extended and unpredictable absences for treatment cycles, hospitalizations, or immune-compromised recovery periods
- Fatigue that is disproportionate to visible activity — chemotherapy causes bone-deep tiredness
- Difficulty concentrating or remembering things recently learned — chemo brain effects
- Heightened emotional sensitivity, anxiety, or tearfulness related to illness and treatment
- Physical appearance changes that may prompt peer questions, teasing, or social exclusion

Student Strengths

- Remarkable resilience and perspective on what matters — often inspires empathy in the whole class
- Close family relationships that provide strong advocacy
- Typically maintains strong motivation to reconnect with peers and school routines

Recommended Accommodations

- Homebound instruction during immune-compromised periods (when absolute neutrophil count is low)
- Flexible attendance and makeup deadlines with no academic penalty for treatment-related absences
- Reduced daily workload during active treatment; prioritize essential learning over volume
- Hand sanitizer stations, and right to wear a mask without classroom comment
- Permission to rest in the nurse's office or designated space as needed during the school day

Assistive Technology

- Laptop or tablet for hospital and homebound instruction continuity
- Video conferencing (Google Meet, Zoom) for virtual class attendance during hospitalization
- Text-to-speech and audiobooks for periods when reading concentration is limited

UDL Strategies

- Record all key lessons and post slides to an LMS so student can access from hospital or home
- Use read-aloud and audio alternatives to reduce cognitive demand during chemo brain periods

- Engage classmates in positive, inclusive communication about the student's absence through a class card, video, or shared activity

IEP Considerations

- Eligible for OHI; hospital homebound services (required under IDEA) must be activated immediately upon diagnosis — do not wait for formal IEP meeting; interim services can begin within days
- IEP team should include a hospital liaison or homebound teacher; present levels must account for educational disruption, not just pre-illness baseline
- Accommodation language: unlimited absences, makeup deadlines, reduced workload during treatment, re-entry transition plan developed collaboratively with family
- After treatment ends, schedule a re-evaluation to assess for late cognitive effects (attention, memory, processing speed) which may warrant ongoing services

Who to Collaborate With

- Hospital Liaison or Homebound Instruction Teacher
- School Nurse and 504/IEP Coordinator
- School Counselor (peer re-entry support and grief/anxiety)

Family Partnership Tip

Ask your child's oncology social worker if the hospital has a school re-entry program — many pediatric cancer centers have school liaison staff who will communicate directly with teachers and help prepare classmates for your child's return. This makes re-entry far smoother than doing it alone.

Middle School (6–8)

A student who may be in active treatment or in remission, navigating academic recovery after significant missed instruction. Social dynamics at this age — appearance changes, absence from peer milestones — can be as challenging as the academic catch-up.

What You Might Notice

- Significant academic gaps across multiple subjects from extended absences
- Attention and short-term memory difficulties — chemo brain can persist months after treatment
- Fatigue that peaks in the afternoon or after physically or cognitively demanding periods
- Social re-entry difficulties: peer relationships have shifted during absence, social anxiety
- Emotional volatility or withdrawal not typical of the student's pre-illness personality

Student Strengths

- Often has a clear sense of purpose and motivation that peers who haven't faced adversity lack
- Strong relationships with family and select close friends who provided support during treatment
- Frequently develops empathy and leadership potential through the experience

Recommended Accommodations

- Formal re-entry transition plan developed before return, including modified workload and a schedule for catching up

- Extended time on all assessments reflecting ongoing processing speed and memory impacts
- Access to all class materials digitally for continued use during partial or full absences
- Permission to leave class for medical needs without drawing attention
- Flexible lunch arrangement if eating in a large cafeteria is medically or socially difficult

Assistive Technology

- Smart pen (Livescribe) for capturing class notes when attention and memory are affected
- Read&Write; or NaturalReader for text-to-speech support during low-concentration periods
- Google Classroom for submitting work asynchronously on low-energy days

UDL Strategies

- Create an explicit, written academic recovery plan with the student that prioritizes the most critical content first
- Build in weekly check-ins between student and case manager or counselor to adjust academic load in real time
- Provide graphic organizers and scaffolded note-taking tools to support chemo brain-related memory challenges

IEP Considerations

- If not already on an IEP, conduct a 60-day re-evaluation after return from extended treatment to assess current functioning — do not rely on pre-diagnosis data
- IEP or 504 should include a formal re-entry plan with academic recovery milestones that are compassionate and realistic — not grade-retention-threatening
- Consider OT or PT evaluation if treatment has affected fine motor skills or physical stamina
- Ensure all subject teachers receive a shared, brief, private summary of the student's current needs — not the medical history, just the functional impact

Who to Collaborate With

- School Counselor (social re-entry and mental health)
- Special Education Case Manager or 504 Coordinator
- Occupational Therapist (if treatment affected fine motor skills)

Family Partnership Tip

Before your child returns to school, request a brief 'school re-entry meeting' with the counselor, case manager, and key teachers. Having a written plan — workload for the first two weeks, makeup timeline, who the student goes to if they feel overwhelmed — removes a huge amount of anxiety for both the student and the family.

High School (9–12)

A teenager who may have survived cancer or be managing ongoing treatment, now facing the high-stakes transition to graduation and post-secondary life. Late effects of treatment — cognitive, physical, or emotional — may require long-term accommodation planning.

What You Might Notice

- Persistent processing speed and working memory challenges classified as late effects of chemo or radiation
- Anxiety about the future, including recurrence, career, and whether they can 'keep up' with peers
- Highly motivated but exhausted; may push past physical limits to maintain academic standing
- May be managing ongoing surveillance appointments that require schedule flexibility
- Strong verbal skills that may mask underlying cognitive processing deficits on written tasks

Student Strengths

- Extraordinary resilience, maturity, and purpose that distinguishes them in college essays and interviews
- Deep self-knowledge and a nuanced understanding of their own learning needs
- Highly developed empathy and interpersonal skills

Recommended Accommodations

- Extended time and breaks on all major assessments — cognitive late effects are documented disabilities
- Flexible attendance for ongoing surveillance appointments, blood draws, and follow-up visits
- Credit recovery or alternative pathway to graduation if cancer caused missed credits
- Accommodation letters for SAT, ACT, and AP exams based on neuropsychological documentation
- Permission to adjust course load in collaboration with family during periods of high medical demand

Assistive Technology

- Dragon NaturallySpeaking or Google Voice Typing for extended writing tasks
- Otter.ai for lecture transcription to support memory and note-taking
- College Board SSD documentation and online accommodation request tools

UDL Strategies

- Work with student to identify their high-capacity times and schedule the most cognitively demanding work for those windows
- Use oral exams or project-based assessment alongside written tests to capture knowledge across multiple formats
- Explicitly address survivorship: connect student with peer support or counseling for post-treatment anxiety and identity work

IEP Considerations

- Transition plan must account for cancer survivorship — many survivors need post-secondary medical follow-up documented in transition services
- Neuropsychological evaluation is strongly recommended to document late effects for post-secondary disability services; initiate referral by the end of sophomore year
- Annual transition goal: student will independently register with college disability services and submit neuropsychological documentation by [date]
- Discuss and document whether Vocational Rehabilitation (VR) services are appropriate if late effects affect career preparation

Who to Collaborate With

- Transition Coordinator
- School Psychologist (neuropsychological evaluation referral)
- School Counselor (survivorship planning and college access)

Family Partnership Tip

Request a copy of your child's neuropsychological evaluation from the hospital if one was completed during treatment — many pediatric cancer centers perform these routinely. Sharing it with the school's psychologist can unlock accommodations that are much harder to get without documentation.

Resources: [Children's Oncology Group — School Re-entry](#) | [CancerCare — School Support](#) | [NCI — Children with Cancer at School](#)

Inflammatory Bowel Disease

Inflammatory Bowel Disease (IBD) — including Crohn's disease and ulcerative colitis — causes chronic inflammation of the gastrointestinal tract, resulting in abdominal pain, diarrhea, urgency, and fatigue. Flares are unpredictable and can be triggered by stress, diet, or infection. Students with IBD need private, immediate, and unrestricted restroom access at all times; denial of this accommodation can cause genuine physical harm and severe anxiety.

<https://fyvr.net/student-personas/inflammatory-bowel-disease>

Elementary (K–5)

A child who may need to leave class urgently and frequently for the bathroom, especially during flares. IBD-related symptoms can cause shame and anxiety; the student may be reluctant to disclose why they need to leave. Fatigue and anemia associated with IBD can also affect stamina and concentration.

What You Might Notice

- Requests to use the restroom frequently, urgently, and sometimes immediately after just returning
- Visible discomfort — holding the stomach, squirming, appearing pale or distressed
- Fatigue and low energy disproportionate to physical activity level
- Anxiety before transitions, meals, or activities away from familiar restroom access
- Reluctance to go on field trips, attend after-school activities, or eat in the cafeteria

Student Strengths

- High tolerance for discomfort and strong ability to self-regulate in challenging situations
- Typically highly motivated to participate when health is stable
- Strong family advocacy and close parent-teacher communication

Recommended Accommodations

- Unconditional, immediate restroom access at any time without needing to ask or explain
- Pass that does not require teacher permission or a visible sign-out process
- Spare clothing kept with the nurse or in a private locker in case of accident
- Seating near the classroom door for discreet exit
- Flexible attendance policy for IBD flares, infusion appointments, and GI procedures

Assistive Technology

- Tablet or Chromebook for completing work during medical absences or recovery days
- LMS access (Seesaw, Google Classroom) for work continuity from home
- Text-to-speech tools when fatigue limits sustained reading

UDL Strategies

- Normalize frequent movement breaks in the classroom so restroom exits are less conspicuous
- Provide a private, non-verbal signal system the student can use to exit discreetly
- Post all assignments digitally so students can keep up during absences without extra requests

IEP Considerations

- IBD typically qualifies under OHI; a 504 Plan is most appropriate unless academic performance is significantly affected
- The single most critical accommodation is unrestricted restroom access — this must be written explicitly in the 504, not treated as informal
- IHP developed with the school nurse should include dietary restrictions, medication timing, flare protocol, and infusion schedule
- If the student experiences school avoidance or anxiety related to IBD, refer to school counselor for assessment

Who to Collaborate With

- School Nurse
- 504 Coordinator
- School Counselor (IBD-related anxiety and school avoidance)

Family Partnership Tip

Ask your gastroenterologist for a brief letter stating the medical necessity of immediate restroom access — schools are legally required to honor this under Section 504, but a physician's note makes the conversation faster and removes any ambiguity. Keep a copy in your child's school file.

Middle School (6–8)

A student who is increasingly embarrassed about IBD symptoms and may hide the condition from peers and new teachers. The combination of social shame, physical pain, and unpredictability can create significant school avoidance. Academic impact from absences is accumulating.

What You Might Notice

- Leaves class frequently for the bathroom with visible anxiety about doing so
- Appears pained or tense during class; fidgets or holds stomach
- Reluctance to eat in the cafeteria or skips lunch to avoid symptom triggers
- Increasing school avoidance, particularly during flares or after a bathroom-related incident at school
- Fatigue and anemia-related cognitive dulling, especially in the morning

Student Strengths

- Mature self-awareness and health management skills
- Strong relationships with adults who understand their condition
- Often highly motivated to engage academically when symptoms are controlled

Recommended Accommodations

- Private, non-verbal exit system for restroom use — student does not need to announce or ask
- Spare clothing in a discreet, private location at school
- Access to a semi-private restroom near the classroom if available
- All assignments available digitally for home/hospital completion during flares

- Flexible deadline policy during documented flares without academic penalty

Assistive Technology

- LMS (Canvas, Google Classroom) for submitting work during absence
- Google Voice Typing for extended writing when fatigue is high
- Smart phone with parental access to symptom tracking app to support health management

UDL Strategies

- Meet privately with the student to co-design their accommodation implementation — giving them control reduces anxiety
- Provide all instruction in recorded or async format during flares so student doesn't fall behind
- Address school avoidance proactively with the counselor before it becomes entrenched

IEP Considerations

- 504 accommodation language must explicitly protect against punitive attendance policies during medically-excused absences for IBD flares or infusion therapy
- If IBD has caused significant academic gaps, assess whether an IEP or intensive tutoring is warranted in addition to the 504
- Document any anxiety or school avoidance as a secondary concern requiring counseling support in the plan
- Ensure the student's 504 explicitly addresses all teachers — not just homeroom — with language about restroom access and absence flexibility

Who to Collaborate With

- School Nurse
- 504 Coordinator
- School Counselor (social-emotional support and school avoidance intervention)

Family Partnership Tip

If your child is starting to avoid school, address it early rather than letting it build. A brief meeting with the school counselor to co-create a 'bad-symptom school plan' — what classes to attend, who to check in with, where to go if symptoms spike — can make school feel safer even on hard days.

High School (9–12)

A teenager who has developed significant self-management skills but may be facing increasing academic consequences from chronic absences and the emotional toll of managing a stigmatized, painful condition. Transition planning must address IBD management in post-secondary environments.

What You Might Notice

- Academic performance that correlates with IBD flare cycles — strong during remission, sharply reduced during flares
- Anxiety or hypervigilance about proximity to bathrooms in any new environment
- May be managing biologic infusions or immunosuppressant medications with significant side effects
- Reluctance to discuss IBD with teachers; prefers written accommodation letters over verbal disclosure

- Occasional depression or frustration about the chronic, unpredictable nature of the condition

Student Strengths

- Exceptional self-advocacy skills when supported and practiced
- Strong understanding of their body and triggers — transferable health self-management
- Motivation to plan and prepare carefully, given experience with unpredictable symptoms

Recommended Accommodations

- Full attendance flexibility without academic penalty for medically-documented IBD flares, infusions, and colonoscopies
- Option to complete all major assessments remotely or on a rescheduled date following a flare
- Discreet restroom access in all settings — including during standardized testing
- Accommodation letter submitted to each teacher without requiring student to explain their medical history
- Note-taking support during extended absences — peer notes or teacher-provided materials

Assistive Technology

- Notability or OneNote for comprehensive note capture on good-health days
- LMS remote access for assignment submission during absence or infusion days
- College Board SSD documentation tools for testing accommodation requests

UDL Strategies

- Discuss with student which accommodation implementation strategies preserve their dignity — let them lead
- Use project-based or portfolio assessment to reduce single-day performance dependency
- Help student develop language to self-disclose IBD to future employers or professors at an appropriate level of detail

IEP Considerations

- Transition planning must address IBD management in college — identifying campus health services, disability office registration, and how to communicate needs to roommates and professors
- Ensure standardized testing accommodations (restroom access during exam, extended time if processing is affected by pain) are documented and submitted to College Board/ACT early
- Annual transition goal: student will independently contact college disability services and submit medical documentation for accommodation registration by [date]
- If IBD has caused significant social-emotional impact, include a mental health or counseling transition goal

Who to Collaborate With

- Transition Coordinator
- School Nurse and 504 Coordinator
- School Counselor (transition planning and mental health)

Family Partnership Tip

Before your teen leaves for college, help them identify the closest restrooms to all of their likely classrooms — this one practical step reduces enormous anxiety for students with IBD entering a new campus. Many students also benefit from contacting the disability office before orientation so accommodations are in place from day one.

Resources: [Crohn's & Colitis Foundation — Schools](#) | [CDC — IBD](#) | [NASN — IBD in Schools](#)

Migraines

Migraines are a neurological condition involving recurring, often debilitating headaches accompanied by nausea, light sensitivity (photophobia), sound sensitivity (phonophobia), and sometimes visual disturbances (aura). Migraine attacks can last hours to days and are unpredictable. Environmental triggers common in schools — fluorescent lighting, noise, strong smells, dehydration, stress, and screen use — can precipitate episodes. During and after attacks, students may be completely unable to function academically.

<https://fyvr.net/student-personas/migraines>

Elementary (K–5)

A child who may experience sudden, severe headaches that require immediate removal to a quiet, dark space. Migraine in children can present differently than in adults — sometimes manifesting primarily as nausea, abdominal pain, or pallor. The child may not be able to articulate what is happening but will become visibly distressed.

What You Might Notice

- Sudden complaints of severe headache, often preceded by complaints of seeing spots or blurry vision (aura)
- Sensitivity to classroom lighting — squinting, shading eyes, requesting to close blinds
- Nausea or vomiting accompanying headache, requiring nurse visit
- Pallor, quietness, and withdrawal from peer interaction at onset
- Inability to concentrate, follow directions, or maintain task engagement during migraine onset

Student Strengths

- Often highly attentive and engaged when symptom-free — migraines do not affect cognitive capacity
- Typically develops strong self-awareness and the ability to identify their own early warning signs
- Families tend to be proactive communicators about the student's triggers and needs

Recommended Accommodations

- Immediate access to a quiet, dimly lit rest space (nurse's office) at the first sign of a migraine
- Permission to wear sunglasses or a hat brim indoors on high-risk days or during fluorescent light exposure
- Flexible makeup policy for any work missed during migraine episodes
- Reduced or eliminated scented materials (markers, soaps) near the student's workspace
- Cold pack or dark space access per nurse protocol during active episodes

Assistive Technology

- Tablet with blue-light filter or night mode for screen time on sensitive days
- Text-to-speech tools to reduce eye strain during recovery from a migraine
- Colored overlays or screen tinting software (f.lux) to reduce visual trigger exposure

UDL Strategies

- Provide lesson materials in audio or video format for makeup during post-migraine recovery days
- Reduce reliance on fluorescent overhead lighting where possible — use natural light or lamps
- Normalize rest breaks as part of the classroom culture to reduce stigma around nurse visits

IEP Considerations

- Chronic, medically-diagnosed migraine qualifies as OHI under IDEA; a 504 Plan is appropriate if no academic skill deficits are present
- Accommodation language must include: immediate rest area access, lighting modifications, scent-free workspace, flexible makeup deadlines
- If migraine frequency is high (more than 4 per month), document attendance impact and consider evaluation for secondary learning challenges
- Nurse must have a written migraine action plan including approved medications and dosing authorized by the prescribing physician

Who to Collaborate With

- School Nurse (medication and action plan)
- 504 Coordinator
- School Counselor (if anxiety or school avoidance is contributing)

Family Partnership Tip

Keep a brief migraine log for your child — date, time, duration, any possible trigger (food, sleep, weather, stress). After a few months, patterns often emerge that you and the school can address proactively — for example, scheduling the most visually demanding tasks for the time of day migraines are least likely.

Middle School (6–8)

A student who may be experiencing migraines more frequently as academic and social stress increases. Fluorescent lights, screen time, and noisy cafeterias are all common school triggers. The student may resist using accommodations to avoid appearing different from peers.

What You Might Notice

- Requesting to leave class or go to the nurse with regular frequency, especially during high-stimulation periods
- Visibly photosensitive — wearing hoodies pulled low, closing eyes, or avoiding bright screens
- Underperforming on assessments taken while managing a developing migraine
- Academic gaps from absences during multi-day migraine cycles
- Increased irritability or social withdrawal at migraine onset — often misread as attitude

Student Strengths

- Strong self-awareness about triggers and early warning signs
- Typically highly capable and motivated during symptom-free periods
- Developing self-advocacy skills through necessity

Recommended Accommodations

- Discreet pass to rest area without requiring public explanation
- Permission to wear tinted glasses or low-brim hat indoors without comment
- Flexible seating away from windows and strong overhead lights

- All tests and major assignments reschedulable following a documented migraine day
- Reduced homework load or extended deadlines during migraine recovery periods

Assistive Technology

- F.lux or screen dimmer software on all school-issued devices
- Text-to-speech (Read&Write;) to reduce visual strain during recovery
- Noise-canceling headphones for use in overstimulating environments as a preventive measure

UDL Strategies

- Offer digital, low-contrast materials as an alternative to bright white printed pages
- Allow student to complete work in a low-stimulation environment (library, empty classroom) when needed
- Reduce timed, single-sitting high-stakes assessments in favor of flexible-window alternatives

IEP Considerations

- 504 accommodation language should explicitly address lighting modifications across all classrooms — a student-specific accommodation, not a whole-classroom requirement
- If migraine frequency is causing significant attendance impact, document this in present levels and create a clear makeup framework
- If student experiences anxiety about migraines at school, address this as a comorbid concern with counseling services
- Review accommodations at the start of each semester — migraine patterns often shift with puberty, academic load, and stress

Who to Collaborate With

- School Nurse
- 504 Coordinator
- School Counselor (stress management and anxiety)

Family Partnership Tip

If your child's neurologist prescribes a 'rescue medication' for school hours, ensure the nurse has a signed authorization form with exact dosing instructions. Many parents don't realize that even OTC medications like ibuprofen must be formally authorized to be administered at school.

High School (9–12)

A teenager managing chronic migraines in a high-pressure academic environment. Stress, sleep disruption, screen overload, and intense studying are all known triggers. The student needs robust accommodations that allow fair demonstration of knowledge even when migraines interfere with exam schedules.

What You Might Notice

- Missing or underperforming on high-stakes tests due to migraine onset on exam day
- Wearing sunglasses or low-brim hats in brightly lit classrooms — a coping strategy, not defiance
- Requesting to reschedule tests or deadlines following documented migraine episodes
- Significant anxiety about the unpredictability of migraines during finals or standardized testing

- Highly capable during symptom-free periods, creating an inconsistent performance pattern

Student Strengths

- Sophisticated understanding of their neurological triggers and prevention strategies
- Strong intrinsic motivation to achieve academically despite frequent obstacles
- Developing self-advocacy skills that will serve them well in post-secondary settings

Recommended Accommodations

- Ability to reschedule any major assessment within a defined window following a documented migraine
- Tinted screen filters, dimmer software, and flexible lighting on all school devices
- Permission to wear tinted glasses or sit away from windows without requiring explanation to each teacher
- Accommodation letters to all teachers at semester start covering exam rescheduling and absence flexibility
- Testing in a low-stimulation environment — reduced noise and fluorescent lighting

Assistive Technology

- F.lux or Iris screen dimming software on all devices
- Otter.ai or Notability for note-taking when screen exposure needs to be minimized
- College Board SSD documentation tools for extended time or testing environment accommodations

UDL Strategies

- Offer project-based or distributed-assessment alternatives to single-day, single-sitting exams
- Work with student to identify their lowest-migraine-risk times of day for scheduling critical tasks
- Explicitly teach post-secondary self-advocacy: how to request rescheduling from a professor, how to register with disability services

IEP Considerations

- Chronic migraine with documented frequency (neurologist records) qualifies for College Board/ACT testing accommodations — initiate documentation request in junior year
- 504 or IEP accommodation language should explicitly state the right to reschedule standardized and classroom assessments following documented migraine events
- Transition goal: student will independently communicate accommodation needs to two post-secondary instructors or disability services contacts by [date]
- If migraines are significantly affecting GPA or graduation timeline, discuss credit recovery options and document academic impact in present levels

Who to Collaborate With

- Transition Coordinator
- 504 Coordinator and School Nurse
- School Counselor (stress and anxiety management)

Family Partnership Tip

Encourage your teen to bring a single-page 'migraine accommodation letter' from their neurologist summarizing the diagnosis, frequency, and functional impact — this is the most efficient way to establish credibility quickly with new teachers each semester and to support College Board accommodation requests.

Severe Allergies & Anaphylaxis

Severe allergies, including food allergies, environmental allergies, and insect venom allergies, can cause anaphylaxis — a life-threatening immune response requiring immediate epinephrine injection. Students with severe allergies must have epinephrine auto-injectors (EpiPens) accessible at all times, and all staff who interact with them must be trained to recognize anaphylaxis and administer emergency medication. Even trace allergen exposure in school environments can be dangerous.

<https://fyvr.net/student-personas/severe-allergies-and-anaphylaxis>

Elementary (K–5)

A child who depends entirely on adults for allergy safety and emergency response. The student may feel anxious about food at school events, field trips, and birthday celebrations. Clear, consistent protocols and a school culture of inclusion are essential to their safety and social participation.

What You Might Notice

- Refusal to eat certain foods; anxiety around food-sharing, birthday treats, or cafeteria trays
- Hives, swelling, or rash appearing after contact with an allergen
- Distress or throat tightening complaints requiring immediate nurse response
- Anxiety about classroom craft materials, adhesives, or art supplies containing allergens
- Reluctance to attend field trips or school events where food safety cannot be guaranteed

Student Strengths

- Often highly self-aware about their allergy and able to clearly identify unsafe items
- Typically has strong family support and very consistent home management routines
- Develops early executive functioning skills from managing a serious, life-impacting condition

Recommended Accommodations

- Epinephrine auto-injector (EpiPen) kept accessible to trained staff in the classroom and in the student's own bag (per physician and school policy)
- Allergen-safe zone at lunch table; classmates in proximity informed of nut-free or relevant allergen-free policy without singling out the student
- Teacher trained annually on anaphylaxis recognition and EpiPen administration by school nurse
- Advance notice of any food-related classroom activity (parties, science experiments) so safe alternatives can be arranged
- Allergen-free craft and art supply substitutions when possible

Assistive Technology

- EpiPen or Auvi-Q auto-injector — primary life-safety device, not optional
- Medical alert wristband with allergen information for emergency responders
- School nurse communication app for parental notification during any allergen exposure event

UDL Strategies

- Develop a classroom culture of food-allergy awareness and inclusion — avoid using food as reward

- Provide allergen-free alternatives for all classroom activities involving food or craft materials
- Use social stories or class discussions about allergies appropriate to the age level to build peer understanding without stigma

IEP Considerations

- Severe, life-threatening allergies qualify under OHI for a 504 Plan; the Anaphylaxis Emergency Action Plan (AEAP) developed by the student's allergist and school nurse is the cornerstone document
- 504 must include: EpiPen access location, trained staff names, allergy avoidance protocols, field trip protocols, substitute teacher communication plan
- Allergy avoidance protocols should be reviewed and re-signed by all responsible staff at the start of each year and whenever the student's class or environment changes
- If the student shows significant anxiety about allergies (avoidance, hypervigilance, refusal to eat), refer to school counselor to assess for food allergy-related anxiety

Who to Collaborate With

- School Nurse (AEAP coordinator and medication administration trainer)
- 504 Coordinator
- School Counselor (allergy-related anxiety)

Family Partnership Tip

Provide the school with at least two EpiPens at the start of every school year — check the expiration dates before drop-off. Keep one in the nurse's office and one in a labeled kit that travels with the student to all settings including PE, field trips, and after-school activities.

Middle School (6–8)

A student who is beginning to manage their allergy more independently but may make riskier decisions in social situations to fit in. The pressure to eat 'normally' with peers is high at this age. Teachers should be aware that a student who dismisses concern about a food item may be downplaying known risk.

What You Might Notice

- Self-carries EpiPen but may not always have it accessible at lunch or during field trips
- Social risk-taking around food in peer settings — may eat something uncertain to avoid drawing attention
- Anxiety about eating in unfamiliar environments (field trips, class parties, food labs in FACS class)
- Possible reluctance to self-administer EpiPen in front of peers even in an emergency
- Vigilance fatigue — may be less careful about reading labels after years of constant monitoring

Student Strengths

- Significant allergy management experience and knowledge of their allergens
- Growing capacity to self-advocate and communicate allergy needs to adults
- Strong attention to environmental detail and risk assessment

Recommended Accommodations

- Permission to carry self-use EpiPen at all times; no requirement to leave it only with the nurse

- Teacher awareness of allergy in every class, including FACS, science, and PE
- Advance notice of food-related activities in any class so student can prepare safe alternatives
- Separate seating or table option in cafeteria if allergen cross-contact is a concern
- No academic penalty for leaving class to address a potential allergic reaction — no delayed response

Assistive Technology

- EpiPen or Auvi-Q carried by student per physician and school policy
- Allergy alert app on student phone for cross-reference of ingredient lists in the cafeteria
- Medical alert wristband updated with current allergen information

UDL Strategies

- Incorporate food allergy awareness into health, FACS, and science curricula to build schoolwide empathy
- Role-play scenarios where student practices saying 'I can't eat that, I have a severe allergy' in low-stakes settings
- Review all food lab and science lab materials for allergen content before the student participates

IEP Considerations

- 504 must travel with the student to all new teachers, environments, and extracurricular activities — not just homeroom
- Field trip protocol must be explicitly documented: who carries the EpiPen, who is trained on the trip, what happens if epinephrine is administered
- If student is showing vigilance fatigue or social risk-taking around allergens, address this directly in counseling — not as a discipline issue
- Ensure FACS and science teachers are included in allergy training — many school food and chemical allergen exposures happen in these settings

Who to Collaborate With

- School Nurse
- 504 Coordinator
- FACS and Science Teachers (food and chemical allergen exposure risk)

Family Partnership Tip

Have an honest conversation with your middle schooler about vigilance fatigue — the exhaustion of always being careful. Validate how tiring it is, and together identify one or two simplifying strategies (like a go-to safe lunch they never have to question) that reduce daily cognitive load.

High School (9–12)

A teenager who is nearly or fully independent in allergy management but is preparing for environments — college, work, travel — where no one knows their history and no nurse is seconds away. Self-advocacy, recognition of early reaction signs, and willingness to self-administer epinephrine are critical skills to develop now.

What You Might Notice

- Confident self-management in familiar settings; less practiced in unfamiliar social or travel situations
- May have experienced a delayed or near-miss reaction due to social pressure or assumption about ingredient safety
- Anxiety about college dining halls, study abroad, internships, and other environments without established safety plans
- Strong self-advocate in medical settings but may not have practiced advocating in social situations
- May underestimate the probability of a reaction in low-risk-seeming situations

Student Strengths

- Deep knowledge of their allergens and extensive experience with avoidance
- Capable of communicating needs clearly to food service providers, medical staff, and peers
- Strong self-regulation and risk-assessment skills

Recommended Accommodations

- Unrestricted self-carry of EpiPen and antihistamines at all times — including during standardized testing
- Access to private space to address a reaction or administer medication without peer audience
- Written allergy protocol on file that all teachers and support staff can reference
- Flexibility to leave any class immediately if a reaction begins — no delay, no explanation required
- Support for college transition: allergy accommodation documentation for campus dining services

Assistive Technology

- Auvi-Q (voice-guided EpiPen) for self-administration under stress
- Allergy alert medical ID bracelet or digital medical ID (Apple Health, Google Health)
- Food scanning apps (Fig, Yummly allergy filter) for college dining and travel food safety

UDL Strategies

- Role-play post-secondary scenarios: how to tell a college roommate, how to speak to a restaurant server, how to handle a reaction when alone
- Encourage student to lead their own 504 meeting and explain their allergy management to school staff — practice for real-world self-advocacy
- Connect student with FARE (Food Allergy Research and Education) young adult resources for peer support

IEP Considerations

- Transition planning should include documentation package for college disability office or campus health: physician AEAP, EpiPen prescription, accommodation letter
- Ensure standardized testing accommodations address unrestricted access to medication during exam
- Annual transition goal: student will independently contact college dining services and disability office to arrange allergen-safe meal plan and accommodations before enrollment
- If student has experienced significant allergy-related anxiety, include a mental health transition goal and referral to community support

Who to Collaborate With

- Transition Coordinator
- School Nurse and 504 Coordinator
- School Counselor (transition planning and anxiety)

Family Partnership Tip

Before college move-in, help your teen set up two things independently: a meeting with campus dining to review their allergens, and a registration appointment with the disability or health office. Doing this before the semester starts, rather than after a reaction, makes the conversation much easier and the safety plan much more robust.

Resources: [FARE — Food Allergy in Schools](#) | [CDC — Food Allergies at School](#) | [NASN — Anaphylaxis in Schools](#)

Physical & Motor

Juvenile Idiopathic Arthritis

Juvenile Idiopathic Arthritis (JIA) is a chronic autoimmune condition causing joint inflammation in children and adolescents. Symptoms include joint pain, stiffness (especially in the morning), swelling, and fatigue. JIA is unpredictable — students may have periods of remission followed by sudden flares. Fine motor tasks like handwriting, keyboard use, and opening containers can be difficult, and sustained sitting or walking may cause significant pain.

<https://fyvr.net/student-personas/juvenile-idiopathic-arthritis>

Elementary (K–5)

A child who may appear stiff and slow to get moving in the morning, struggle with fine motor tasks like writing and cutting, and need to move, stand, or rest during the day. Pain is real and not visible — a student sitting still and 'looking fine' may be managing significant discomfort.

What You Might Notice

- Slow, stiff movement at the start of the day — especially after sitting for extended periods
- Difficulty gripping pencils, scissors, or manipulatives; handwriting becomes worse as fatigue sets in
- Reluctance to participate in physical education or recess on high-pain days
- Swollen or visibly inflamed joints in fingers, wrists, or knees on flare days
- Fatigue disproportionate to activity level; afternoon energy crash

Student Strengths

- Strong verbal communication and problem-solving skills not affected by JIA
- Often highly empathetic and attuned to others' physical and emotional experiences
- Typically motivated to participate fully on good-pain days

Recommended Accommodations

- Adapted grip pencils, pens, and fine motor tools to reduce joint strain
- Access to keyboard or voice typing as an alternative to handwriting
- Permission to stand, sit on a cushion, or change positions throughout the day
- Extra time for all written tasks to account for slower handwriting
- Modified PE participation on flare days; written or observational alternative

Assistive Technology

- Pencil grips and ergonomic writing tools (slant boards, triangular pencils)
- Chromebook or iPad with keyboard for writing tasks
- Speech-to-text (Google Voice Typing) for writing when hand pain is acute

UDL Strategies

- Incorporate regular movement breaks and posture shifts for all students — normalizes the student's need to move
- Offer oral, drawn, or typed alternatives to hand-written work
- Use visual and manipulative-light alternatives in math and science to reduce grip-dependent tasks

IEP Considerations

- JIA qualifies under OI (Orthopedic Impairment) or OHI under IDEA; a 504 Plan is appropriate for physical access accommodations when academic performance is on grade level
- Physical Therapy (PT) evaluation is strongly recommended — PT can provide school-based treatment, classroom positioning recommendations, and adapted PE programming
- Occupational Therapy (OT) evaluation is recommended to assess fine motor needs and recommend assistive tools for writing and classroom tasks
- Accommodation language should include: keyboard alternatives to handwriting, extended time, positioning supports, modified PE, and early dismissal from class for joint movement breaks

Who to Collaborate With

- Physical Therapist (positioning, mobility, adapted PE)
- Occupational Therapist (fine motor, writing tools)
- 504 Coordinator or Special Education Case Manager

Family Partnership Tip

On bad-pain mornings, let the school know before your child arrives so the PE teacher and classroom teacher can quietly have a plan ready — it's much easier to modify an activity before it starts than to pull a child out mid-class. A brief text or email to the nurse is often enough.

Middle School (6–8)

A student dealing with the dual challenge of JIA symptoms and the intense social self-consciousness of middle school. They may hide pain to avoid seeming different, skip movement breaks to avoid peer comment, and push through writing-heavy assignments despite hand pain. Academic demand for handwriting has increased significantly.

What You Might Notice

- Handwriting becomes illegible or painful during extended writing assignments — quality deteriorates over time
- Avoidance of class participation in activities requiring physical demonstration or fine motor work
- Slow class transitions — needs extra time moving between rooms or opening lockers
- Visible joint swelling or reported stiffness, especially on cold or weather-change days
- Fatigue after high-load academic days; may appear disengaged in afternoon classes

Student Strengths

- Often highly organized and planful — compensates for physical limitations with strong executive function
- Typically develops strong verbal communication and self-advocacy skills

- Motivated to succeed academically; does not use JIA as an excuse

Recommended Accommodations

- Keyboarding as the primary written output tool in all classes — no expectation of handwriting
- Extended time on all handwritten or typed tasks and assessments
- Permission to use a rolling backpack or locker close to main classes to reduce carrying load
- Elevator or ramp access for all floor transitions; schedule classes on accessible floors where possible
- Modified locker assignment (lower locker with a combination accessible to affected joints, or keypad lock)

Assistive Technology

- Voice typing (Dragon NaturallySpeaking, Google Voice Typing) for long writing tasks
- Ergonomic keyboard and mouse for computer-based work
- Digital note-taking apps (Notability, OneNote) to replace binder and notebook systems that require fine motor manipulation

UDL Strategies

- Provide typed or digital note templates to reduce the volume of handwriting required
- Allow oral alternatives to written tests for demonstrating knowledge on high-flare days
- Reduce unnecessary paper-based tasks — digitize as much as possible to reduce grip demands

IEP Considerations

- IEP or 504 should explicitly address physical access needs across the building — not just classroom accommodations
- PT and OT services should be included if they are needed to support the student's access to educational activities; note that educational PT/OT focuses on school-function, not medical treatment
- Document cumulative academic impact if hand pain and fatigue have caused significant written output reduction; consider whether writing-based goals are appropriate or if adapted output methods should be the norm
- Ensure accommodation language is updated if disease activity changes — JIA can worsen or improve; annual review is essential

Who to Collaborate With

- Physical Therapist
- Occupational Therapist
- 504 Coordinator or Special Education Case Manager

Family Partnership Tip

Request a joint visit from both the school OT and PT at the start of each year — together they can walk the student's full daily schedule and identify every physical barrier (lockers, stairs, chairs, desks, PE requirements) rather than addressing issues one at a time as they arise.

High School (9–12)

A teenager with JIA preparing for post-secondary environments that may be less physically accessible or supportive than their K-12 setting. Self-advocacy for accommodation in college, workplace, and daily life is critical. Writing-intensive academic demands are at their peak.

What You Might Notice

- Handwriting-intensive courses (AP exams, essay tests, lab reports) causing visible pain and performance decline
- Joint pain managing during heavy course load, internships, or sports participation
- Navigating a complex medication regimen (biologics, DMARDs) that may cause fatigue or immune side effects
- Self-conscious about physical differences — especially in competitive or physically active peer environments
- Highly capable academically but written output does not reflect actual knowledge

Student Strengths

- Strong self-management and medical advocacy skills
- Sophisticated understanding of their functional needs and communication of accommodation requests
- Often develops strong written voice through necessity of dictation and voice-first writing practices

Recommended Accommodations

- Keyboarding or voice typing for all written work, including AP exams (with College Board approval)
- Extended time on all assessments — fine motor fatigue causes slowing across extended writing periods
- Physical access accommodations across all buildings in the schedule
- Attendance flexibility for rheumatology and infusion appointments without academic penalty
- Permission to carry a reduced load or access an elevator without a formal pass system

Assistive Technology

- Dragon NaturallySpeaking for hands-free writing in extended work
- Ergonomic laptop stand, split keyboard, and vertical mouse for computer-based workstations
- College Board SSD paperwork and documentation tools for AP exam accommodation

UDL Strategies

- Offer oral or recorded alternatives to handwritten exams on high-flare days
- Work with student to pre-plan high-writing-demand periods (finals, AP exams) with targeted pacing strategies
- Explicitly teach post-secondary accommodation request language: disclosing JIA to disability services, requesting ergonomic furniture in a dorm or classroom

IEP Considerations

- Transition planning must address physical access needs at post-secondary institutions — not just academic accommodations; include ADA considerations for building access, dormitory modifications, and campus navigation
- College Board/ACT accommodation request for typed responses (in place of handwriting) is strongly supported by JIA documentation — initiate early in junior year
- Annual transition goal: student will independently contact campus disability services to arrange computer-based testing and ergonomic accommodations before enrollment
- Consider Vocational Rehabilitation referral if JIA affects career path planning — VR can fund ergonomic workplace equipment

Who to Collaborate With

- Transition Coordinator
- Occupational Therapist (ergonomic workstation and transition planning)
- Vocational Rehabilitation Counselor

Family Partnership Tip

Help your teen practice dictating written work using voice typing before they leave for college — it takes several weeks to become fluent, and the time to learn is before they're in a high-pressure freshman semester. Dragon NaturallySpeaking offers a free trial; Google Voice Typing is free and works in any document.

Resources: [Arthritis Foundation — School](#) | [CARRA — JIA Resources](#) | [NASN — JIA in Schools](#)

Ehlers-Danlos Syndrome

Ehlers-Danlos Syndrome (EDS) is a group of heritable connective tissue disorders causing joint hypermobility, skin fragility, and autonomic dysfunction. The most common subtype in school settings is hypermobile EDS (hEDS), which involves joint instability, chronic pain, fatigue, and frequent joint subluxations or dislocations. EDS is often invisible — students may look fine but be managing significant pain, dizziness (dysautonomia/POTS), brain fog, and fatigue. It is frequently misdiagnosed, and students may encounter skepticism from adults.

<https://fyvr.net/student-personas/ehlers-danlos-syndrome>

Elementary (K–5)

A young child who may have difficulty with fine motor tasks, tires easily, and may have joints that 'pop out' during play or handwriting. They may have been labeled as clumsy or inattentive. Pain is real but often invisible, and the child may not have the vocabulary to accurately describe it.

What You Might Notice

- Unusual joint flexibility that causes instability — may trip, fall, or injure easily during normal activities
- Complaints of hand or wrist pain during writing tasks; grip may be unusually weak
- Fatigue disproportionate to activity; may need to lie down or rest after brief exertion
- Dizziness or lightheadedness when standing up quickly — a sign of dysautonomia (POTS)
- Skin that bruises easily; minor injuries that take longer to heal

Student Strengths

- Often highly perceptive and intellectually curious, with strong verbal and conceptual reasoning
- Develops strong empathy and awareness of others' physical and emotional experiences
- Frequently becomes an expert in understanding their own body and communicating needs

Recommended Accommodations

- Seating that provides joint support — chair with armrests, lumbar support, and appropriate height
- Reduced or eliminated handwriting requirements; keyboard access as primary writing tool
- Permission to lie down or use a rest position during class if pain or fatigue becomes unmanageable
- Modified PE and recess participation; no high-impact, joint-stress activities without clearance
- Extra time for all tasks requiring fine motor control

Assistive Technology

- Pencil grips and triangular pencils to reduce grip strain on hypermobile joints
- iPad or Chromebook with on-screen or external keyboard as handwriting alternative
- Speech-to-text tools (Google Voice Typing) for writing when hand joints are painful

UDL Strategies

- Offer consistent alternatives to handwriting across all subjects — do not require students to 'earn' keyboard access
- Provide seated alternatives to floor activities during morning meeting, carpet time, and group work

- Normalize rest breaks and positioning changes as part of classroom routines

IEP Considerations

- EDS may qualify under OI (Orthopedic Impairment) or OHI depending on which system is most affected; evaluate holistically as both physical and health-related impacts are common
- OT and PT evaluations are strongly recommended: OT for fine motor, adaptive writing tools, and sensory processing; PT for joint stability, safe movement, and adapted PE programming
- If EDS-related brain fog or fatigue is affecting learning, evaluate for learning-based goals in addition to physical accommodations
- Accommodation language should explicitly include: rest space access, positioning modifications, keyboard alternatives, no-high-impact PE without clearance, and attendance flexibility for medical appointments

Who to Collaborate With

- Occupational Therapist
- Physical Therapist
- School Nurse (health plan for joint subluxation first aid protocol)

Family Partnership Tip

Share any PT or OT notes from your child's outside providers with the school therapists — school-based therapists can only address educational function, but knowing the full clinical picture helps them make better recommendations. Bring a brief written summary to the IEP or 504 meeting rather than relying on verbal communication in the moment.

Middle School (6–8)

A student managing increasing pain and fatigue while navigating the social and academic demands of middle school. EDS is often misunderstood or minimized by adults who cannot see the pain; the student may face skepticism and feel unheard. Brain fog from pain and dysautonomia may be mistaken for inattention or apathy.

What You Might Notice

- Inconsistent performance — high capability on low-pain days, significant difficulty on high-pain or high-fatigue days
- Difficulty sustaining handwriting across extended written assignments; hand 'gives out' visibly
- Dizziness or flushing when moving between classrooms, especially up stairs or in warm environments
- Requests to sit during activities that other students do standing
- Brain fog — difficulty retrieving words, following multi-step instructions, or maintaining focus during pain

Student Strengths

- Strong verbal intelligence and conceptual thinking typically unaffected by EDS
- Deeply motivated to succeed academically when physical support allows them to demonstrate knowledge
- Developing strong self-awareness and capacity to articulate complex needs to adults

Recommended Accommodations

- Keyboarding as the default written output method — no expectation of handwriting in any class

- Elevator and ramp access throughout the building; schedule classes on accessible floors
- Permission to stand, shift positions, or use a stability cushion during class
- Extended time on all tasks and assessments; accommodation language addresses brain fog as well as fine motor
- Flexible attendance for physical therapy, rheumatology, and cardiology appointments

Assistive Technology

- Ergonomic keyboard and mouse for computer-based work to reduce joint strain
- Voice typing (Dragon NaturallySpeaking, Google Voice Typing) for extended writing tasks
- Digital note-taking platform (OneNote, Notability) to replace binder-based organization

UDL Strategies

- Reduce unnecessary handwriting demands across all subjects — prioritize digital-first workflows
- Allow oral or recorded responses as alternatives to written tests on high-symptom days
- Provide written agendas and lesson outlines to compensate for brain fog-related memory difficulties

IEP Considerations

- EDS often co-occurs with anxiety, depression, or ADHD — conduct a comprehensive evaluation rather than attributing all learning challenges to the physical diagnosis alone
- IEP or 504 must address both physical and cognitive symptoms; extended time should be grounded in processing speed data if available, not just assumed
- PT and OT services should be reviewed annually as EDS severity can fluctuate significantly; adapt services to current functional level
- Ensure all teachers — not just homeroom — receive and acknowledge the accommodation plan; EDS students often face pushback from teachers who cannot see the disability

Who to Collaborate With

- Physical Therapist
- Occupational Therapist
- School Psychologist (comprehensive evaluation for co-occurring learning or emotional needs)

Family Partnership Tip

If your child encounters skepticism from a teacher or administrator about invisible pain, the most effective tool is a letter from their physician or specialist that specifically describes functional limitations in school terms — not medical jargon, but 'this student cannot sustain handwriting for more than 5 minutes without pain' or 'this student may need to sit or lie down without advance notice.' Concrete, functional language is harder to dismiss.

High School (9–12)

A teenager with EDS who is managing high-stakes academic demands while dealing with chronic pain, fatigue, and often a complex medication and therapy regimen. They need robust accommodations and strong transition planning, as EDS may require significant workplace and post-secondary environment modifications.

What You Might Notice

- Highly variable performance that frustrates both student and teachers; inconsistency is the hallmark of EDS
- Writing-intensive courses triggering significant hand and wrist pain; typed work is qualitatively better than handwritten
- Brain fog during pain peaks making recall, articulation, and multi-step reasoning temporarily difficult
- Anxiety about managing EDS in college or the workplace, where support structures may be less robust
- Strong verbal ability and conceptual thinking that rarely gets to shine through pain-limited written output

Student Strengths

- Often becomes an expert communicator about complex medical needs — a genuine transferable skill
- Strong analytic and verbal reasoning when physical barriers to output are removed
- Demonstrated resilience and persistence in the face of chronic, often misunderstood illness

Recommended Accommodations

- Typed or voice-dictated output for all written work, including AP exams and standardized tests
- Extended time (1.5x–2x) on all assessments — addresses both fine motor slowing and brain fog
- Physical access accommodations across all settings including gym, lab, and extracurricular spaces
- Attendance flexibility for PT, pain management, and specialist appointments without academic penalty
- Permission to adjust posture, use a recliner or prone position for work during severe flares

Assistive Technology

- Dragon NaturallySpeaking for hands-free, extended dictation in academic writing
- Ergonomic workstation: adjustable desk, split keyboard, vertical mouse, monitor arm
- College Board SSD documentation and request tools for typed testing accommodation

UDL Strategies

- Discuss with the student their high-capacity and low-capacity patterns and help design a schedule that clusters high-demand tasks in peak-function windows
- Use oral defense or recorded presentation as an alternative to written essays when appropriate
- Explicitly practice post-secondary self-advocacy: how to request ergonomic furniture in a dorm, how to explain EDS brain fog to a professor, how to register with disability services

IEP Considerations

- Transition planning must address post-secondary physical access (ADA accommodations for dormitories, ergonomic classrooms, elevator access) as well as academic accommodations
- College Board/ACT accommodation request for typed responses is strongly supported by EDS documentation — initiate process in spring of junior year
- Vocational Rehabilitation referral is appropriate if EDS will affect career environment or require workplace modifications (ergonomic equipment, schedule flexibility for medical appointments)
- Annual transition goal: student will independently identify and contact disability services and campus health at their post-secondary institution and initiate accommodation registration by [date]

Who to Collaborate With

- Transition Coordinator
- Occupational Therapist (ergonomic workstation and transition planning)
- Vocational Rehabilitation Counselor

Family Partnership Tip

Before your teen leaves for college, have them practice assembling their own accommodation documentation packet — diagnosis letters, functional limitation descriptions, physician statements — without your help. Knowing they can do this independently is both practically important and emotionally empowering for students who have spent years depending on parents to advocate for them.

Resources: [EDS Society — School Support](#) | [Dysautonomia International — Schools](#) | [Understood — Chronic Pain & School](#)

Spina Bifida

Spina bifida is a neural tube defect in which the spinal column does not close completely during fetal development, resulting in varying degrees of lower-body paralysis, reduced sensation, and bladder/bowel differences. Most students with myelomeningocele (the most common form) use wheelchairs or forearm crutches and have typical or near-typical cognitive ability, though many also experience associated conditions such as hydrocephalus that can affect executive function and processing speed. Early and consistent support from physical therapists, occupational therapists, and school nurses is essential.

<https://fyvr.net/student-personas/spina-bifida>

Elementary (K–5)

A student who uses a wheelchair or forearm crutches full-time and requires adult assistance with catheterization on a medical schedule throughout the school day. May have strong verbal skills and grade-level reading but may need extra time on fine-motor tasks such as cutting, writing, and manipulating small materials.

What You Might Notice

- Requires scheduled nurse visits for intermittent catheterization (every 2–3 hours); may be away from class briefly and predictably
- Difficulty manipulating small objects, scissors, or pencils due to reduced hand strength or motor coordination
- Needs wide aisle access and extra time navigating between classroom areas
- May fatigue more quickly than peers during transitions or long physical activities
- May struggle with tasks requiring spatial reasoning or multi-step processing if hydrocephalus is present

Student Strengths

- Often highly verbal with strong vocabulary and storytelling ability
- Demonstrates persistence and problem-solving from years of navigating physical environments
- Builds strong relationships with adults and peers through consistent communication

Recommended Accommodations

- Accessible seating at front or side of classroom with clear path to exit
- Provide a second set of materials (e.g., books, supplies) at desk height to minimize reaching
- Allow extra time for written tasks and fine-motor activities
- Schedule predictable nurse breaks into the daily routine without academic penalty
- Provide a copy of class notes or graphic organizer to reduce writing load

Assistive Technology

- Adaptive grip pencils or weighted pens to support handwriting
- iPad or tablet with stylus as an alternative writing surface
- Voice typing (Google Voice Typing or iOS Dictation) for longer written responses

UDL Strategies

- Offer multiple means of action and expression: oral responses, drawings, or dictation as alternatives to handwriting

- Use visual schedules and predictable routines to reduce cognitive load during transitions
- Provide manipulatives and hands-on learning activities adapted for use from a seated position

IEP Considerations

- Present levels should document fine-motor performance (e.g., grip strength, handwriting speed/legibility) and physical stamina, based on OT and PT assessments
- Annual goal example: Student will complete written assignments of 3+ sentences using adaptive grip or voice typing with no more than 2 adult prompts in 4 of 5 opportunities as measured by classroom data
- Health plan (within the IEP or as a separate school health plan) must detail catheterization schedule, trained staff, privacy procedures, and emergency protocols
- Placement: general education with pull-out for OT/PT services; note need for accessible route to all instructional spaces including specials and cafeteria

Who to Collaborate With

- Physical Therapist (PT) — mobility, positioning, and accessibility recommendations
- Occupational Therapist (OT) — fine-motor adaptations and assistive technology
- School Nurse — medical management plan and catheterization training for staff

Family Partnership Tip

Share the student's catheterization schedule and any new medical equipment with the school nurse at the start of each school year. A quick written summary of what the student can do independently versus what requires adult assistance helps staff provide the right level of support — not too much and not too little.

Middle School (6–8)

A student who is increasingly aware of physical differences from peers and may be self-conscious about catheterization, wheelchair use, or mobility aids. Academically may be on grade level but shows inconsistent performance on timed tests or multi-step tasks. Is developing independence skills and should be learning to self-advocate with teachers.

What You Might Notice

- May need extra time between classes due to longer navigation routes; arrives slightly late to class
- Avoids drawing attention to catheterization schedule; may delay leaving class and risk a health incident
- Written assignments may be shorter or slower to complete than verbal ability suggests
- May withdraw from group activities that require physical mobility (science labs, PE alternatives)
- Shows awareness of social differences; may report frustration at inaccessible activities or spaces

Student Strengths

- Developing strong self-awareness and the ability to articulate personal needs
- Often empathetic and socially skilled from navigating complex environments
- Demonstrates creativity in finding workarounds and alternative approaches

Recommended Accommodations

- Provide a pass to leave 3–5 minutes early for class transitions

- Allow laptop or tablet use for all written assignments in place of handwriting
- Ensure all lab and project stations are accessible at wheelchair height; adapt lab partners accordingly
- Eliminate unnecessary physical barriers in the room (clear paths at all times)
- Discuss catheterization schedule privately and matter-of-factly; never publicly reference it

Assistive Technology

- Chromebook with Google Docs and voice typing enabled
- Text-to-speech browser extension (Read&Write; for Google) for reading support if processing is affected
- Digital planner app (e.g., Notability or Google Keep) for organizational support

UDL Strategies

- Provide choice in assignment format — written, oral, recorded video, or visual poster
- Build in self-reflection and goal-setting components to foster self-advocacy
- Use flexible grouping so the student participates in hands-on activities in an adapted role

IEP Considerations

- Present levels should address academic performance in core subjects AND functional performance related to self-advocacy and health management
- Annual goal example: Student will independently notify the nurse for scheduled catheterization without prompting on 9 of 10 school days as tracked by nurse log
- IEP or 504 should explicitly address accessible pathway requirements, including specials, gym, cafeteria, and off-campus field trips
- Begin documenting self-advocacy skills as a transition-related goal — students should practice requesting accommodations directly with teachers

Who to Collaborate With

- Physical Therapist (PT) — reassess accessibility needs and adaptive PE participation
- School Counselor — social-emotional support for body image, peer relationships, and identity
- Special Education Case Manager or 504 Coordinator

Family Partnership Tip

Practice having your student call ahead to ask about accessibility before field trips or community activities. This real-world self-advocacy skill builds confidence and gives school staff a model to follow.

High School (9–12)

A student who manages most of their medical needs independently and is focused on course rigor, college or career planning, and social participation. May have processing-speed or executive-function challenges that require accommodations on high-stakes tests. Physical accessibility in lab settings, electives, and off-campus activities requires ongoing coordination.

What You Might Notice

- Manages catheterization independently; may still need a private space and extra time away from class

- Executive function challenges may appear as difficulty prioritizing multi-week assignments or managing deadlines
- May be anxious about college accessibility and whether chosen colleges can meet needs
- Strong verbal participant in discussions but written work may be slower or shorter
- May advocate assertively or conversely may underreport barriers to avoid being seen as 'different'

Student Strengths

- Highly developed self-awareness and self-management skills
- Strong verbal reasoning and advocacy ability when properly supported
- Lived expertise in navigating systems that can translate to leadership and career strengths

Recommended Accommodations

- Extended time on all tests (1.5x–2x) to account for processing speed and physical fatigue
- All coursework accessible digitally; avoid handwritten-only submission policies
- Ensure college-level AP and standardized testing accommodations are documented well in advance
- Private space for health management during the school day without academic penalty
- Accessible seating and setup in all classrooms, labs, and school-hosted events

Assistive Technology

- Dragon NaturallySpeaking or built-in OS dictation for extended writing tasks
- Digital note-taking app (Notability, OneNote) synced across devices
- Google Calendar or task management app for executive-function support with long-term assignments

UDL Strategies

- Provide assignment scaffolding (milestone check-ins) for multi-week projects to support executive function
- Build explicit college transition and disability self-advocacy content into coursework where applicable
- Offer alternative demonstration of mastery: oral defense, video presentation, or portfolio

IEP Considerations

- Transition plan (required by age 16 under IDEA) must address post-secondary education (disability services registration), independent living goals, and health self-management
- Annual goal example: By end of year, student will independently contact two post-secondary institutions to request disability services information and identify accommodation request processes
- Document all accommodations in a format that can be shared with college disability services offices; include psychoeducational evaluation if processing speed or executive function deficits are documented
- Review eligibility category annually — student may qualify under both OI (Orthopedic Impairment) and OHI (Other Health Impairment) depending on hydrocephalus-related cognitive impact

Who to Collaborate With

- Transition Coordinator — post-secondary planning and disability services outreach
- School Counselor — college application process and mental health support
- Physical Therapist (PT) — transition recommendations for college or workplace accessibility

Family Partnership Tip

Before senior year, help your student request their complete IEP and evaluation records — they will need this documentation when registering with college disability services. Contact the college's disability services office together so your student hears the process firsthand.

Resources: [Spina Bifida Association — School](#) | [OSEP — Spina Bifida & IDEA](#) | [AOTA — OT for Spina Bifida](#)

Osteogenesis Imperfecta

Osteogenesis imperfecta (OI), sometimes called brittle bone disease, is a genetic connective tissue disorder characterized by fragile bones that fracture easily, often with minimal trauma. Severity ranges widely — some students have a few fractures per year while others have had hundreds. OI does not affect intelligence; most students are cognitively typical. The primary school concerns are physical safety, pain management, absences due to fractures and recovery, and social-emotional wellbeing. Teachers play a critical role in creating a safe physical environment and a normalized, matter-of-fact approach to the student's differences.

<https://fyvr.net/student-personas/osteogenesis-imperfecta>

Elementary (K–5)

A small-statured student who may use a wheelchair, crutches, or walk independently depending on severity. May have missed significant school time due to fractures and hospitalizations. Cognitively on track but may have gaps in curriculum coverage from absences. Peers may be overly cautious or avoid contact, leading to social isolation.

What You Might Notice

- Moves carefully and deliberately; avoids crowded hallways, rough play, and sudden contact
- Visibly tenses when peers are nearby or during transitions with physical jostling
- May have casts, splints, or braces on rotating limbs throughout the year
- Returns after absences needing academic catch-up; may have incomplete assignments from hospitalization periods
- May self-limit physical activity beyond what is medically necessary due to anxiety about re-injury

Student Strengths

- Strong verbal and intellectual curiosity developed through extensive time reading, listening, and exploring independently
- Mature problem-solving and self-awareness from navigating complex medical situations
- Often highly creative and imaginative from periods of independent learning at home

Recommended Accommodations

- Assign a safe, low-traffic path to and from the classroom; avoid placing the student near high-traffic areas
- Provide a second set of books and materials to minimize carrying heavy backpack loads
- Develop a clear re-entry plan after each absence including a no-penalty make-up window
- Avoid any activities involving physical contact, rough play, or falling risk without explicit medical clearance
- Seat the student away from doors and high-traffic aisles to reduce bump risk

Assistive Technology

- iPad or lightweight laptop to avoid heavy textbook carrying
- Voice typing or speech-to-text tools if arm fractures affect handwriting
- Audiobooks (Bookshare, Learning Ally) for extended reading to reduce physical page-turning demands

UDL Strategies

- Provide digital versions of all materials so the student can access content during home recovery
- Use flexible grouping and seated alternatives for all hands-on activities
- Build in regular check-ins using a private signal system so the student can communicate pain or discomfort discreetly

IEP Considerations

- Present levels must address current academic standing, impact of absences on curriculum coverage, and physical functioning based on current medical status and physician input
- Health plan (within IEP or separate) must document fracture emergency protocol, staff trained in safe handling, and evacuation procedures for wheelchair users
- Homebound instruction eligibility: document the process for transitioning to and from home instruction during recovery periods to ensure continuity of FAPE
- Annual goal example (if cognitive impact is documented): Student will complete grade-level math assignments with no more than 1 week delay following documented medical absences, using digital materials, in 4 of 5 occurrences as tracked by teacher log

Who to Collaborate With

- School Nurse — emergency fracture protocol, safe handling training for all staff
- Occupational Therapist (OT) — adaptive equipment and fine-motor support during and after fractures
- Physical Therapist (PT) — safe mobility plan and adaptive PE accommodations

Family Partnership Tip

Provide the school nurse with a written one-page 'what to do if a fracture occurs' protocol at the start of the year, including who to call first, whether 911 should be called, and how the student prefers to be positioned while waiting for help. Update it after any major medical change.

Middle School (6–8)

A student who is increasingly aware of the social visibility of their condition and may be navigating a complicated mix of wanting independence from adult supervision while needing physical safety supports. Academic performance may be inconsistent due to chronic absences. Social participation in sports and unstructured peer activities is limited.

What You Might Notice

- Hesitates before crowded activities like hallway passing periods, assemblies, or lunch
- May appear withdrawn or reluctant to join group activities due to fear of contact
- Written work may lag when arm fractures occur; oral or digital alternatives are needed
- Shows frustration when repeatedly singled out for special handling procedures
- May overcompensate socially by being highly verbal or entertaining to maintain peer connections

Student Strengths

- Intellectual curiosity and depth of knowledge from extensive reading and independent learning
- Emotional intelligence and empathy developed through navigating complex healthcare and social situations

- Creative thinking and adaptability in problem-solving

Recommended Accommodations

- Allow early dismissal from class for transitions to avoid crowded hallways
- Ensure all digital submission options are available so paper-based work is never the only option
- Develop a private, agreed-upon re-entry plan for each return after hospitalization
- Adapt all lab and physical activities with a seated or observational alternative and meaningful academic role
- Discuss pain management needs privately; allow brief breaks as needed

Assistive Technology

- Chromebook or tablet for all written work; voice typing enabled
- Google Classroom or school LMS access from home for continuity during absences
- Digital textbooks (VitalSource, Bookshare) to eliminate heavy book carrying

UDL Strategies

- Offer asynchronous alternatives (recorded lectures, written discussion boards) for days when the student attends remotely
- Provide choice boards for demonstrating mastery that include non-physical options
- Use student-directed goal setting to help the student articulate their own learning needs

IEP Considerations

- Present levels should explicitly document the effect of chronic absences on academic performance across content areas
- 504 may be sufficient if cognitive functioning is typical — document accommodations including attendance flexibility, make-up policies, and accessible physical environment
- If an IEP is warranted under OI or OHI, goals should address self-advocacy: student will identify and request needed accommodations with the teacher at the start of each new unit in 3 of 4 opportunities
- Ensure all staff who interact with the student (specials, PE, lunch) receive the safe-handling and emergency plan annually

Who to Collaborate With

- School Counselor — social-emotional support for identity, peer relationships, and anxiety about re-injury
- 504 Coordinator or Special Education Case Manager
- Physical Therapist (PT) — adaptive PE plan and safe activity guidelines

Family Partnership Tip

Ask the school to identify one consistent contact person (such as the school counselor or case manager) who can coordinate re-entry plans and communicate with all teachers after each absence. A single point of contact reduces the student's burden of re-explaining their situation to every teacher individually.

High School (9–12)

A student who manages their condition with increasing independence and is focused on college or career planning. Chronic absences may have created cumulative gaps in specific subjects. Social participation in school activities may be limited but the student has likely developed strong coping skills and a clear sense of personal identity. Transition planning must address post-secondary disability services and independent living.

What You Might Notice

- Self-manages most physical safety needs but may need reminders to communicate with new staff
- Written output may be slower or affected during fracture recovery periods
- May advocate strongly for themselves or, conversely, disengage when barriers feel too high
- Attendance record shows patterns of multi-day absences with academic recovery afterward
- May research accessibility at colleges and express anxiety about post-secondary transitions

Student Strengths

- Exceptional self-advocacy and self-knowledge from years of navigating medical and educational systems
- Strong verbal communication and persuasive skills
- High resilience and ability to adapt goals and strategies in response to changing circumstances

Recommended Accommodations

- Extended time on all tests and major assignments; flexible attendance policy with make-up plan in writing
- All course materials available digitally, including textbooks, before the unit begins
- Ensure testing rooms and all instructional spaces (including elective classrooms and career labs) are physically accessible
- Document all accommodations in a format compatible with college disability services requirements
- Allow recording of lectures and use of digital note-taking tools in all classes

Assistive Technology

- Dragon NaturallySpeaking or OS dictation for extended writing during fracture recovery
- Laptop with digital textbooks and note-taking apps (OneNote, Notability)
- Remote learning platform access (Google Meet, Zoom) for continuing coursework during hospitalizations

UDL Strategies

- Design units with parallel remote-access options so the student can maintain learning continuity during hospitalizations
- Include self-advocacy and disability rights content in advisory, health, or English coursework
- Offer portfolio-based assessment as an alternative to timed exams where content knowledge is the goal

IEP Considerations

- Transition plan must address: post-secondary disability services registration, physical accessibility at chosen colleges/worksites, and health self-management in independent living
- Annual transition goal example: Student will complete online inquiry forms to two post-secondary disability services offices and schedule one informational meeting by the end of the school year
- Document all accommodations and their rationale in a summary of performance for post-secondary use
- Review whether homebound instruction eligibility and procedures are clearly understood by all parties for senior year hospitalizations

Who to Collaborate With

- Transition Coordinator — post-secondary planning and disability services outreach
- School Counselor — college application support and mental health
- School Nurse — updated emergency and safe-handling plan for all new staff each year

Family Partnership Tip

Before graduation, help your student compile a 'disability services packet' — current IEP or 504, recent evaluations, accommodation history, and physician documentation — so they are ready to submit to college disability services offices promptly. The earlier they register, the earlier accommodations are in place.

Resources: OIF — [Osteogenesis Imperfecta](#) | OIF — [School & OI](#) | NASN — [OI in Schools](#)

Limb Difference

Limb difference refers to the congenital absence, partial development, or acquired loss of one or more limbs. It is not a disease or injury requiring ongoing medical treatment in most cases; students with congenital limb differences have often developed highly adaptive, self-sufficient ways of completing tasks from an early age. Intelligence and academic ability are not affected. The primary school considerations are physical access to materials and activities, normalizing the use of adaptive tools, and supporting a healthy, confident identity — both in how the student sees themselves and how their peers respond.

<https://fyvr.net/student-personas/limb-difference>

Elementary (K–5)

A student who completes most classroom tasks independently using adaptive techniques developed over their lifetime. May use a prosthetic device some or all of the time, or may prefer to work without one. Tends to be highly resourceful and creative; the challenge is usually peer curiosity or staring rather than academic difficulty.

What You Might Notice

- Completes tasks such as writing, cutting, and manipulating materials using adaptive strategies (mouth, feet, modified grip) that may look different but are effective
- Peers may stare, ask intrusive questions, or treat the student as a novelty — which is visible and affects the student's comfort
- May need slightly more time to set up materials or transition to new activities
- Occasionally expresses frustration when activities are designed without any adaptive pathway
- Participates confidently in most activities but may disengage from specific tasks that are genuinely not accessible

Student Strengths

- Exceptional problem-solving and resourcefulness from adapting to a world designed for two-limbed individuals
- Confidence and ease in their own identity when supported by a normalizing school environment
- Strong social awareness and ability to educate peers with age-appropriate information when they choose to

Recommended Accommodations

- Provide adaptive scissors, grip tools, or stabilizing devices (dycem mat, clamps) as needed for art and fine-motor tasks
- Allow extra setup time at the beginning of activities without drawing attention to it
- Consult with the student about which adaptations they want — do not assume; they are the expert on their own body
- Address peer curiosity proactively (with the student's input and permission) using inclusive education practices
- Ensure PE and specials classes have adapted versions of activities; consult adapted PE specialist if needed

Assistive Technology

- Adaptive keyboard and mouse options (one-handed keyboard, trackball, head mouse) if upper limb difference affects typing
- Voice typing (Google Voice Typing, iOS Dictation) as an alternative to keyboarding
- Mounting systems and stabilizing equipment (book holders, tablet stands) for independent access to materials

UDL Strategies

- Design activities from the start with multiple physical pathways (e.g., students may type, write, or dictate — not because of a disability but as standard choice)
- Use Universal Design principles for all classroom materials so adaptation is built in, not added on
- Create a classroom culture where diverse bodies and tools are visible and normalized through books, posters, and discussion

IEP Considerations

- Many students with limb differences do not require an IEP if adaptive strategies are sufficient — evaluate whether 504 is more appropriate unless cognitive or academic needs are also present
- If an IEP is developed, present levels should document functional performance rather than deficits: what the student can do independently, with adaptive tools, and with minimal support
- OT consultation should be documented to assess and recommend adaptive equipment; OT services may not need to be direct if the student is already highly adaptive
- Accommodation language for 504: adapted materials, extra setup time, alternative tool use permitted, adapted PE participation

Who to Collaborate With

- Occupational Therapist (OT) — adaptive equipment assessment and recommendations
- Adapted Physical Education (APE) Specialist — PE activity modifications
- School Counselor — support for social-emotional wellbeing and peer education

Family Partnership Tip

Ask your child what they want their teacher to tell classmates — if anything. Some children prefer that the teacher address peer questions directly in a matter-of-fact way; others prefer to answer questions themselves. Give your child control over this narrative from the start.

Middle School (6–8)

A student who is highly self-sufficient but increasingly navigating adolescent peer dynamics, social media, and identity in a body that differs visibly from peers. May be developing a strong disability identity and disability pride, or may be in a more private phase of figuring out how they want to present their limb difference to others. Academic support is usually minimal; social-emotional support may be more important.

What You Might Notice

- Completes academic tasks independently with adaptive tools; rarely requires direct academic assistance
- May be self-conscious about using adaptive tools in front of peers; may attempt tasks in less effective ways to avoid drawing attention

- Responds to intrusive questions from peers with varying degrees of tolerance — sometimes patient, sometimes frustrated
- Engages confidently in activities they can adapt; disengages quietly from activities with no accessible pathway
- May use humor as a social strategy, which teachers can misread as not needing support

Student Strengths

- Strong sense of identity and self-determination when that identity is affirmed by school staff
- Adaptive expertise that translates into creative and lateral thinking across academic tasks
- Peer leadership potential, especially in inclusive and disability-aware school environments

Recommended Accommodations

- Ensure all lab, art, and technology stations are set up so adaptive tools can be used without special requests
- Allow use of voice-to-text, dictation, or adaptive keyboard in all classes without requiring a separate ask each time
- Avoid class activities that single out or highlight physical differences (e.g., hand-tracing art projects) without offering an alternative
- Check in privately — not publicly — if you notice the student struggling with a specific task
- Include disability representation in classroom texts and discussions to normalize diverse bodies

Assistive Technology

- One-handed keyboard or adaptive mouse if upper limb difference is present
- Voice typing via Chromebook or tablet for extended writing
- Screen-recording or video tools for demonstrating learning in non-written formats

UDL Strategies

- Embed adaptive options into assignment design so the student does not need to request them separately
- Use project-based learning with student-directed roles so students can assign themselves to tasks that match their abilities
- Build in student choice for how final products are demonstrated: written, oral, visual, or multimedia

IEP Considerations

- 504 Plan is typically the appropriate vehicle; review annually and ensure all teachers and elective staff receive a copy
- If social-emotional needs are significant, consider including school counseling services in the 504 or IEP
- Present levels should address both academic performance (typically grade-level) and functional/social performance
- Goal example if self-advocacy is a focus: Student will identify and use one adaptive tool independently in each new class setting within the first two weeks, as reported by teacher and student self-report

Who to Collaborate With

- School Counselor — social-emotional support, identity development, peer education
- Occupational Therapist (OT) — annual equipment check and transition to new tools
- 504 Coordinator — plan coordination across all subject-area teachers

Family Partnership Tip

Connect your student with limb-difference communities and social media accounts featuring adaptive athletes, artists, and professionals. Seeing adults with limb differences thriving in a range of careers is one of the most powerful supports you can provide during adolescence.

High School (9–12)

A student with a well-established adaptive skill set who is preparing for post-secondary life. Academic performance is typically on par with peers. The most important school-related needs are ensuring adaptive technology is in place for college coursework and career exploration, and that the student has practiced self-advocating with institutions and employers.

What You Might Notice

- Manages all academic tasks independently with established adaptive strategies
- May advocate assertively for accessible spaces and tools or may avoid drawing attention to needs
- Participates fully in academic discussions; demonstrating learning in writing or typed format is preferred over handwriting-dependent tasks
- May research careers, colleges, and workplace accessibility independently
- May encounter non-adapted elective or CTE environments (auto shop, culinary arts, construction tech) that require proactive consultation

Student Strengths

- Highly developed self-advocacy and communication skills
- Creative, flexible thinking from a lifetime of adapting standard approaches
- Often has a clear career vision connected to interests that are not limited by limb difference

Recommended Accommodations

- Ensure all CTE, vocational, and elective classrooms have been consulted with OT for adaptive access before the student enrolls
- Allow all written work to be submitted digitally; typing and voice input are standard tools, not special accommodations
- Provide extra time on hands-on assessments where physical setup takes longer
- Ensure all standardized test accommodations (SAT, AP, ACT) are documented and applied for well in advance
- Include adaptive technology access in any school-sponsored work experience or internship placements

Assistive Technology

- Dragon NaturallySpeaking or OS-level dictation for extensive writing
- Adaptive hardware (one-handed keyboard, foot mouse, mouth-controlled pointer) based on individual need
- Video-based portfolio tools (Flipgrid, Adobe Express) to demonstrate skills in non-handwriting formats

UDL Strategies

- Offer portfolio and project-based alternatives to timed pen-and-paper assessments
- Include transition and career exploration curriculum that explicitly addresses disability disclosure and workplace accommodation requests
- Build in mentorship or informational interview opportunities with professionals who have visible disabilities

IEP Considerations

- Transition plan must address: post-secondary education (disability services registration, adaptive technology needs), employment (workplace accommodation awareness), and independent living
- Summary of Performance (SOP) should detail adaptive technology currently in use and functional impact on written output
- Annual transition goal example: Student will complete a job-shadow or informational interview in a field of interest and identify two workplace adaptations that would support their success
- Review whether all standardized testing accommodations have been applied for and approved; keep documentation for college admissions disability services packets

Who to Collaborate With

- Transition Coordinator — career exploration, vocational rehabilitation referral (pre-ETS services)
- Occupational Therapist (OT) — transition assessment and updated equipment recommendations
- School Counselor — college application process and disclosure decisions

Family Partnership Tip

Encourage your student to contact the disability services office at their top college choices before they apply — this normalizes self-advocacy and helps them make an informed college choice based on how well the institution supports students with physical differences.

Resources: [Amputee Coalition](#) — [Pediatric](#) | [AOTA](#) — [Adaptive Equipment in Schools](#) | [Understood](#) — [Physical Disabilities](#)

Dwarfism

Dwarfism describes a group of medical conditions resulting in short stature, typically defined as an adult height of 4 feet 10 inches or under. The most common form, achondroplasia, is a skeletal dysplasia affecting bone growth. Most individuals with dwarfism are cognitively typical and academically on par with peers; the primary school concerns are physical access to the built environment, social-emotional wellbeing, and peer education to prevent bullying and staring. Some students may have associated orthopedic concerns (spinal stenosis, frequent ear infections, hearing loss) that require health monitoring.

<https://fyvr.net/student-personas/dwarfism>

Elementary (K–5)

A cognitively typical student who participates fully in academic activities but may need physical modifications to standard furniture and classroom materials. May need a step stool at the sink or water fountain, a lower desk or footrest, and a modified chair. Peers may stare, compare heights, or ask inappropriate questions — proactive peer education from the teacher (with family permission) is often the most important support.

What You Might Notice

- Requires a step stool or adapted furniture to reach standard-height desks, sinks, cubbies, and interactive whiteboards
- May tire more easily during long walks or physical activities due to shorter stride length and joint differences
- Peers may draw attention to height differences, leading to visible discomfort or withdrawal
- Participates fully in academic and verbal activities; no cognitive difference from peers
- May have more frequent ear infections or mild hearing loss — watch for signs of hearing difficulty

Student Strengths

- Fully participates in academic, creative, and social activities with appropriate physical access supports
- Often socially skilled and confident when the environment is respectful and welcoming
- Demonstrates early self-advocacy when adults model appropriate language and normalize the student's physical needs

Recommended Accommodations

- Provide an ergonomically appropriate chair, footrest, and desk that allow feet to rest flat
- Place a step stool at the sink, water fountain, and any needed height-adjusted areas
- Lower coat hooks, cubbies, and supply areas to an accessible height
- Ensure the student has full visual access to the board and interactive displays from their seated position
- Address inappropriate peer comments and questions consistently and matter-of-factly as a class community norm

Assistive Technology

- Adjustable-height desk or table (manually or electronically adjustable)
- Wireless presentation remote or tablet mirror so the student does not need to reach the interactive whiteboard

- Hearing amplification system (FM system or sound-field system) if hearing loss is documented

UDL Strategies

- Ensure all classroom materials and displays are at a height accessible to all students — this benefits all learners
- Use flexible seating arrangements so the student is never inadvertently blocked from view
- Incorporate diverse body representation in classroom books and visual materials

IEP Considerations

- Most students with dwarfism are served under a 504 Plan, not an IEP, unless an additional disability (e.g., hearing loss, learning disability) is present
- 504 documentation should address: furniture modifications, physical access to all school spaces, PE adaptations, and a protocol for addressing height-related comments
- If hearing loss is identified, add audiological testing and hearing accommodations to the plan
- Health plan should note any spinal stenosis precautions, activity restrictions, or signs of neurological compression to watch for

Who to Collaborate With

- School Nurse — health monitoring for ear infections, hearing, and any orthopedic precautions
- Occupational Therapist (OT) — ergonomic assessment and furniture modification recommendations
- School Counselor — peer education and social-emotional support

Family Partnership Tip

At the start of each school year, send a brief written note to your child's teacher explaining preferred language (e.g., 'dwarfism' or 'little person' — ask your child what they prefer), what physical modifications are needed, and your wishes regarding peer education. Many teachers want to do the right thing but do not know where to start.

Middle School (6–8)

A student who is cognitively on track and fully participates in academic activities but navigates significant social-emotional challenges during adolescence, when peer attention to physical appearance intensifies. The student may be dealing with staring, comments, or social media attention. Physical access to the school environment (lockers, bathrooms, lab stations) continues to need attention. May begin experiencing more significant joint pain or fatigue.

What You Might Notice

- May show signs of social withdrawal or irritability connected to peer interactions about height
- Navigates physical environment with extra effort — reaching lockers, climbing stairs with shorter stride, reaching lab materials
- Academic performance is on grade level; any dip in grades is more likely social-emotional than cognitive
- May be the target of jokes or become hypersensitive to any body-related comments
- May advocate for themselves clearly or may internalize frustration to avoid standing out

Student Strengths

- Academic competence and ability to participate fully in all content-area learning
- Strong sense of self when school environment actively affirms dignity and belonging
- Developing self-advocacy skills; can clearly articulate physical access needs when asked

Recommended Accommodations

- Ensure locker is at accessible height or assigned a lower locker; provide a combination lock that can be operated from a lower position
- Lab stations and maker-space tables should be adjustable or have a suitable-height workspace identified in advance
- Allow extra transition time if physical fatigue is a factor during long school days
- Establish a clear, consistent anti-bullying response specifically addressing comments about stature
- Provide private check-in time with school counselor as a scheduled, destigmatized support

Assistive Technology

- Adjustable standing/sitting desk for the primary classroom
- Wireless presentation clicker or tablet mirroring for any presentation activities requiring board access
- FM system or preferential seating near speaker/teacher if hearing loss is present

UDL Strategies

- Create assignments that celebrate diverse forms of expertise, including physical adaptability and creative problem-solving
- Facilitate class discussions on diverse bodies and disability representation in age-appropriate contexts
- Allow flexible participation in PE with adapted activities designed with the student, not just for the student

IEP Considerations

- 504 Plan review should explicitly address middle school transition: new lockers, new building layout, multiple teachers, lab environments, and PE
- If social-emotional impact on academic performance is documented, consider adding school counseling services
- Annual 504 review should include direct student input about what is and is not working
- If hearing loss has been identified, ensure hearing accommodations (FM system, preferential seating, captioning for videos) are explicitly listed

Who to Collaborate With

- School Counselor — social-emotional support, peer relationships, and anti-bullying response
- Physical Therapist (PT) — adaptive PE participation and fatigue management
- 504 Coordinator — annual plan review and new-environment access planning

Family Partnership Tip

Consider connecting your student with the Little People of America (LPA) Youth programs or regional chapters. Peer connections with other adolescents with dwarfism — particularly through teen programs — can be powerfully normalizing during the middle school years.

High School (9–12)

A student who is academically capable and is preparing for post-secondary education or employment. Physical access to all school spaces, including CTE labs and career-focused electives, must be verified. Social-emotional wellbeing may be strong or may require continued support depending on school climate. Transition planning should address workplace accessibility and disability disclosure.

What You Might Notice

- Participates fully in academic coursework at grade level; no cognitive accommodations needed
- Physical access may become an issue in specialized CTE or vocational settings not originally designed with accessibility in mind
- May begin discussing disability disclosure decisions for college and employment
- May need accommodations on standardized tests for extended time if joint pain or fatigue affects test endurance
- Social-emotional wellbeing is generally positive with appropriate school support but remains vulnerable to public attention (e.g., social media)

Student Strengths

- Academically strong; brings full intellectual capacity to all coursework
- Well-developed self-advocacy and communication skills
- Clear awareness of accessibility needs and able to communicate them to new environments

Recommended Accommodations

- Assess all CTE, vocational, and elective classrooms for physical accessibility before enrollment and implement modifications proactively
- Provide extended time on standardized tests if fatigue or joint discomfort is documented by a physician
- Ensure college visits and career exploration activities are to physically accessible locations
- Document physical access accommodations clearly in a summary of performance for post-secondary use
- Address any workplace accommodation needs in transition planning explicitly

Assistive Technology

- Adjustable workstation in career and technical education classrooms
- Hearing assistive technology (FM system, captioning apps) if hearing loss is present
- Ergonomic tools (adapted handles, extended-reach devices) for shop, lab, or vocational tasks

UDL Strategies

- Design transition curriculum that includes disability disclosure role-play and Americans with Disabilities Act (ADA) rights education
- Offer college campus accessibility visits as part of transition activities
- Incorporate career exploration that exposes students to professionals with dwarfism across diverse fields

IEP Considerations

- Most students will have a 504 rather than an IEP; ensure the 504 is updated to reflect high school and transition needs
- If an IEP exists, transition plan (required by age 16) must address: post-secondary education (accessibility of campus, housing), employment (workplace accommodation process), and independent living (accessible housing considerations)
- Summary of Performance should document all accommodations, physical access modifications used, and student's self-advocacy ability
- Vocational Rehabilitation referral is appropriate; VR can fund adaptive equipment and worksite modifications for employment

Who to Collaborate With

- Transition Coordinator — college and career planning with accessibility lens
- Vocational Rehabilitation (VR) Counselor — pre-employment transition services
- School Counselor — disability disclosure decisions and social-emotional support

Family Partnership Tip

Before graduation, research the ADA workplace accommodation request process with your student. Knowing their rights — and practicing how to request a modified workstation or accessible tools — is a practical life skill that makes the workplace transition much smoother.

Resources: LPA — [Little People of America Education](#) | [MAGIC Foundation — School](#) | [Understood — Physical Access in Schools](#)

Neurological & Other Health

Tourette Syndrome

Tourette Syndrome (TS) is a neurological disorder characterized by repetitive, involuntary movements and vocalizations called tics. Tics can be motor (eye blinking, head jerking, shoulder shrugging) or vocal (throat clearing, sniffing, words, or phrases), and they wax and wane in frequency and intensity over time. Coprolalia — the involuntary utterance of obscene words — occurs in only a minority of individuals with TS and is often overrepresented in popular portrayals. Most students with TS are of typical intelligence; many also have co-occurring ADHD or OCD that significantly affect academic performance. Stress and fatigue typically worsen tics.

<https://fyvr.net/student-personas/tourette-syndrome>

Elementary (K–5)

A student who may be newly diagnosed and still developing awareness of their tics. Tics may be most noticeable during quiet, focused tasks such as reading or testing. Peers may giggle or imitate tics, and the student may be embarrassed or confused by their own behavior. Co-occurring ADHD (present in up to 60% of students with TS) is often the greater academic challenge.

What You Might Notice

- Repetitive movements or sounds that the student cannot voluntarily suppress for long periods (eye blinking, throat clearing, shoulder jerking)
- Tics increase during periods of stress, fatigue, or high-pressure activities like tests or oral reading
- May appear distracted or off-task due to tic suppression effort, which is mentally exhausting
- Peers react with laughter, questions, or imitation — student appears embarrassed or frustrated
- If ADHD co-occurs: difficulty starting tasks, sustaining attention, and following multi-step directions

Student Strengths

- Often highly creative and divergent thinkers with strong pattern recognition
- Empathetic and perceptive, often acutely aware of classroom social dynamics
- Persistent and resilient from navigating a neurologically complex world from a young age

Recommended Accommodations

- Allow the student to step out briefly when tics are at peak intensity — a pre-arranged, discreet exit pass
- Provide a low-pressure alternative seating option (e.g., near the door, slightly away from dense peer seating) if preferred
- Never draw attention to tics in front of the class; treat tics as medically expected and unremarkable
- Extend time on tests and timed tasks; tic suppression takes cognitive effort that reduces academic output
- If ADHD co-occurs: reduce task length, chunk directions, and provide visual timers

Assistive Technology

- Voice typing for written tasks during high-tic periods when fine-motor tics affect handwriting

- Noise-canceling headphones to reduce environmental triggers and help with focus (especially if ADHD co-occurs)
- Visual schedule app to support daily routine predictability

UDL Strategies

- Build predictable routines with clear visual cues to reduce anxiety-triggered tic surges
- Offer multiple response modes (oral, written, drawn, typed) so tics do not prevent participation
- Use flexible seating so the student can move slightly without disrupting peers

IEP Considerations

- Eligibility is typically under OHI (Other Health Impairment); if ADHD co-occurs, document both in present levels
- Present levels should describe tic type, frequency, and functional impact — particularly during testing and written tasks
- Annual goal example (if ADHD co-occurs): Student will begin independent work tasks within 3 minutes of instruction 4 of 5 days per week as tracked by teacher observation log
- IEP or 504 must include: private space for tic release, extended time, and clear language prohibiting disciplinary action for tic behaviors

Who to Collaborate With

- School Psychologist — evaluation and co-occurring ADHD/OCD assessment
- School Counselor — peer education support and social-emotional wellbeing
- Special Education Teacher or 504 Coordinator — accommodation coordination

Family Partnership Tip

Ask the school to designate a safe, private space where your child can go briefly when tics are intense — a trusted adult's classroom, the counselor's office, or a quiet hallway spot. Knowing the escape hatch exists reduces anxiety, which in turn often reduces tic frequency.

Middle School (6–8)

A student who is increasingly aware of peer perception and may be actively suppressing tics during school — which is exhausting and often causes a 'rebound' of more intense tics after school. Academic performance may be inconsistent due to the effort of suppression and co-occurring ADHD or OCD. Social dynamics around tics are more complex and more consequential in middle school than in elementary school.

What You Might Notice

- Tics may appear milder during class because the student is suppressing them — but the student is exhausted
- Written work may be slower or less organized during high-stress periods; tics affect handwriting quality
- May avoid reading aloud or oral presentations due to vocal tics emerging under performance pressure
- Social isolation or self-consciousness; may have experienced bullying about tics
- If OCD co-occurs: may spend excessive time on homework, erasing and rewriting, or checking work

Student Strengths

- Deep intellectual interests and strong engagement when content connects to personal passions
- Sophisticated understanding of neurodiversity and ability to educate peers when given the right support
- Creative, non-linear thinkers who often generate original ideas in project-based work

Recommended Accommodations

- Allow typed or voice-recorded assignments as a standard option — not a special request — for all written work
- Provide a private, pre-arranged exit option with no questions asked during peak tic periods
- Avoid cold-calling for oral reading; allow opt-in participation for read-alouds
- Schedule tests during lower-stress times of day when possible; allow breaks during long tests
- Address tic-related bullying as a disability rights issue under school non-discrimination policy

Assistive Technology

- Chromebook with voice typing enabled for all written work
- Text-to-speech tools for reading-heavy assignments if ADHD affects comprehension under tic suppression load
- Smartwatch or discreet timer app for self-monitoring during timed tasks

UDL Strategies

- Offer alternative project formats that reduce public performance pressure (recorded video, podcast, visual portfolio)
- Build in choice for independent versus collaborative work so the student can self-regulate social demands
- Teach and model self-advocacy language — give the student scripts for telling teachers about their needs

IEP Considerations

- Present levels should document both tic impact and any co-occurring ADHD or OCD impact separately, with specific effect on academic tasks
- If OCD co-occurs and causes significant assignment completion challenges, add a goal targeting task completion efficiency
- IEP or 504 should explicitly prohibit disciplinary action for tic behaviors and include staff training requirement
- Annual goal example: Student will self-initiate use of approved tic-break procedure without adult prompting on 4 of 5 school days as tracked by student log and teacher confirmation

Who to Collaborate With

- School Counselor — social-emotional support and anti-bullying advocacy
- School Psychologist — OCD/ADHD co-occurring assessment and CBT referral if needed
- Special Education Case Manager — IEP/504 coordination across content-area teachers

Family Partnership Tip

Coordinate with the school on a 'tic release schedule' — brief, private breaks between classes where your student can let tics out fully without social observation. This reduces post-school rebound tics at home and lowers overall fatigue.

High School (9–12)

A student who has typically developed significant awareness and management strategies for their tics. Academic performance may be strong or may reflect the cumulative toll of years of suppression fatigue and co-occurring ADHD. Self-advocacy is critical as the student prepares for post-secondary settings where they must disclose and request accommodations independently.

What You Might Notice

- Manages tics more discreetly but may show visible fatigue by the end of the school day
- Written work quality may drop under timed conditions or during high-stress exam periods
- Strong verbal and discussion skills; may underperform on written assessments relative to actual knowledge
- May have clear self-understanding about triggers and management strategies
- May be anxious about college — specifically, whether tics will be accepted and accommodated

Student Strengths

- Highly developed self-knowledge and metacognitive awareness
- Creative, lateral thinking that translates well to college and career contexts
- Advocacy experience that can translate into leadership in disability rights and neurodiversity spaces

Recommended Accommodations

- Extended time on all tests and major assignments (1.5x minimum; more if fatigue is documented)
- Private testing space to reduce suppression pressure and allow tic release without peer observation
- All written assignments accepted in typed or voice-recorded format
- Ensure standardized test accommodations (SAT, AP, ACT) are applied for and approved well in advance
- Flexible attendance policy for days with severe tic episodes; homebound work option

Assistive Technology

- Dragon NaturallySpeaking for extended writing during high-tic periods
- Noise-canceling headphones for focus during independent work and testing
- Digital task management app (Todoist, Google Tasks) for executive function support if ADHD co-occurs

UDL Strategies

- Offer oral exams or recorded video alternatives to timed written tests where content knowledge is the goal
- Build explicit self-advocacy skill instruction into advisory, health, or college preparation coursework
- Create opportunities for students to demonstrate mastery through portfolio or project-based capstones

IEP Considerations

- Transition plan must address post-secondary disability services registration and self-advocacy for accommodation requests with professors
- Annual goal example: Student will independently submit documentation and complete the disability services intake process at one post-secondary institution by end of senior year
- Summary of Performance should describe tic impact on timed writing and testing, current accommodations in use, and student's self-management strategies
- If ADHD or OCD co-occurs, ensure those are separately addressed in the transition plan with specific college-readiness goals

Who to Collaborate With

- Transition Coordinator — post-secondary planning and disability services preparation
- School Psychologist — updated evaluation documentation for college disability services
- School Counselor — mental health support and college transition anxiety

Family Partnership Tip

Help your student practice the exact language they will use when disclosing Tourette Syndrome to a college professor or employer. Role-playing a brief, matter-of-fact disclosure ('I have Tourette Syndrome — you may notice some involuntary movements or sounds; these are neurological and not disruptive to my work') removes the anxiety of finding the words in the moment.

Resources: [TAA — Tourette in Schools](#) | [CDC — Tourette Syndrome](#) | [Child Mind Institute — Tourette](#)

Fetal Alcohol Spectrum Disorder

Fetal Alcohol Spectrum Disorder (FASD) is an umbrella term for a range of lifelong brain-based disabilities caused by prenatal alcohol exposure. FASD affects the central nervous system and can result in difficulties with memory, cause-and-effect reasoning, impulse control, abstract thinking, understanding social cues, and adaptive behavior. Most individuals with FASD have average or near-average intelligence as measured by IQ tests, but significant functional gaps in daily living and executive function often lead to academic and behavioral challenges that appear inconsistent or confusing to educators. FASD is a hidden disability — students typically do not have visible physical markers.

<https://fyvr.net/student-personas/fetal-alcohol-spectrum-disorder>

Elementary (K–5)

A student who may appear inconsistent — doing well some days and struggling significantly on others. May have strong verbal recall but limited understanding of what words mean in context. Is often kind and socially motivated but struggles with social consequences and cause-and-effect in relationships. Responds well to visual structure, routine, and patient repetition.

What You Might Notice

- Difficulty connecting actions to consequences; may repeat the same rule-breaking behavior after multiple corrections
- Strong verbal language that masks limited comprehension — can repeat instructions but not act on them correctly
- Difficulty with transitions; meltdowns or shutdown when routine changes without warning
- Short-term memory challenges: forgets multi-step directions given verbally; benefits from written or visual reminders
- Socially eager but struggles with turn-taking, reading social cues, and peer conflict resolution

Student Strengths

- Warm, affectionate, and socially motivated — genuinely wants to connect with peers and adults
- Strong visual memory for familiar experiences, people, and places
- Creative and imaginative; often excels in hands-on, visual, or artistic tasks

Recommended Accommodations

- Use visual schedules with picture supports for all daily routines and transitions
- Give directions one step at a time; have student repeat back the step before acting
- Provide consistent, predictable routines — warn of any changes in advance with a visual timer
- Break academic tasks into the smallest possible chunks with frequent check-ins
- Use concrete, hands-on materials for all abstract concepts; avoid relying on verbal explanation alone

Assistive Technology

- Visual schedule app (ChoiceWorks, First Then Visual Schedule) for daily routine support
- Text-to-speech tools for reading tasks to bypass decoding barriers

- Audio recording of directions so the student can replay them independently

UDL Strategies

- Use multi-sensory instruction: visual, auditory, and hands-on elements in every lesson
- Provide graphic organizers and visual maps for all reading and writing tasks
- Build in frequent, low-stakes practice of skills in varied contexts to support generalization

IEP Considerations

- FASD often qualifies under OHI (Other Health Impairment) or Developmental Disability depending on the evaluation; present levels should document executive function, adaptive behavior (using Vineland or ABAS), and academic performance
- Annual goal example: Student will follow a 3-step task routine using a visual schedule with no more than 1 adult verbal prompt in 4 of 5 opportunities as measured by weekly observation log
- Behavior Intervention Plan (BIP) should be needs-based and trauma-informed — many students with FASD have experienced adverse childhood experiences (ACEs); avoid punitive behavior management
- Generalization is a specific challenge for FASD: goals should include plans to practice skills in multiple settings, not just one classroom

Who to Collaborate With

- School Psychologist — comprehensive evaluation including executive function and adaptive behavior
- Special Education Teacher — intensive instruction and IEP management
- School Counselor — social skills support and trauma-informed practices

Family Partnership Tip

Share your child's FASD diagnosis with the school in writing, including any neuropsychological evaluation that has been done. Many FASD-related behaviors are misunderstood as willful defiance when they are actually neurological. Providing the 'brain-based' framing helps teachers respond with support rather than discipline.

Middle School (6–8)

A student who faces increasing academic and social demands that exceed executive function and abstract reasoning capacity. May be labeled as unmotivated or manipulative when behavior is actually neurologically driven. The gap between verbal ability and functional performance often widens in middle school, where independent work and social complexity increase dramatically.

What You Might Notice

- Cannot reliably generalize skills learned in one class to another class or context
- Written work is significantly below verbal ability; essays and multi-step projects are extremely difficult
- Social conflicts are frequent; difficulty understanding others' perspectives or predicting social consequences
- Organization and time management are very weak — locker, binder, and schedule management require structured adult support
- Responses to correction may appear immature (laughing, denying, repeating the same behavior)

Student Strengths

- Genuine social interest and desire to be included — a strong motivator for skill development when used well
- Often excels in hands-on, concrete learning tasks and vocational-type activities
- Strong long-term factual memory for preferred topics and experienced events

Recommended Accommodations

- Provide a structured homework and organizational system with daily adult check-in (not child-dependent)
- Use preferential seating near a supportive peer and away from high-distraction areas
- Break all multi-day assignments into daily tasks with specific due dates and adult check-ins
- Use social narratives (written or visual) to pre-teach appropriate behavior in new social situations
- Reduce written output requirements: allow voice recording, graphic organizers, or oral demonstration of knowledge

Assistive Technology

- Google Docs with voice typing for all written assignments
- Google Classroom for organized, centralized assignment access
- Organizational app (MyHomework, iStudiez) with parent/teacher co-management

UDL Strategies

- Use project-based learning with concrete, real-world tasks that connect to the student's daily experience
- Provide social scripts and role-play for common challenging social situations
- Offer alternative assessment formats: oral interviews, video demonstrations, or visual presentations

IEP Considerations

- Present levels must address executive function, adaptive behavior, and academic performance separately — do not rely solely on IQ scores
- Social skills goals are often as important as academic goals: Student will use a pre-taught problem-solving script in peer conflicts with no more than 2 adult prompts in 3 of 4 observed opportunities
- Behavior goals must be strengths-based and function-based — identify the unmet need driving behavior, not just the behavior itself
- Ensure all teachers (not just the special education teacher) receive FASD-specific training; behaviors will not be managed by punishment

Who to Collaborate With

- Special Education Case Manager — IEP management and teacher coordination
- School Psychologist — adaptive behavior reassessment and BIP support
- School Social Worker — family support, trauma-informed care, and community resource connections

Family Partnership Tip

Ask the school to create a single daily communication method — a brief check-in log, app, or notebook that travels between home and school. Consistent home-school communication about what happened that day helps you reinforce (in concrete, simple language) the same skills the school is teaching.

High School (9–12)

A student who is approaching transition age with significant functional gaps between academic ability and daily living independence. Abstract coursework (algebra, literary analysis, chemistry) is typically very challenging. Transition planning is extremely important and must be concrete, goal-oriented, and connected to supported employment and independent living supports rather than solely focused on academic achievement.

What You Might Notice

- Struggles significantly with abstract academic content in core subjects; concrete, applied learning is much more accessible
- Executive function challenges (planning, task initiation, self-monitoring) affect all coursework and daily organization
- Social judgment continues to lag developmental peers; may be vulnerable to peer pressure or exploitation
- May set unrealistic post-secondary goals without support to calibrate based on functional skill levels
- Benefits greatly from structured vocational exploration and hands-on career preparation

Student Strengths

- Motivated by concrete, real-world tasks and vocational learning that connects to daily life
- Loyal, dedicated, and often highly reliable in structured, supported work environments
- Develops strong attachments to mentors and supportive adults who provide consistent expectations

Recommended Accommodations

- Modify academic content to applied/functional level where appropriate with IEP team agreement
- Use a planner system with daily adult oversight — independent planning is rarely fully effective
- Extend time on all tests; allow oral responses and alternative assessment formats
- Provide explicit, concrete instruction on ADA rights, workplace expectations, and safety in community settings
- Avoid sarcasm, idioms, and abstract language in instruction — use literal, concrete communication

Assistive Technology

- Voice typing and text-to-speech across all devices for reading and writing support
- Video modeling tools for teaching workplace and daily living procedures
- Smart home or phone reminders for daily scheduling and self-management routines

UDL Strategies

- Connect all academic content to functional, real-world applications (budgeting, job applications, community navigation)
- Use video modeling and visual step-by-step guides for all procedural learning
- Provide peer mentoring and adult mentoring connections in vocational and community settings

IEP Considerations

- Transition plan (required by age 16) must be highly individualized and functional: post-secondary education goals should reflect actual adaptive skill levels, not aspirational ones
- Vocational Rehabilitation referral is critical — VR can fund supported employment, job coaching, and pre-employment transition services
- Annual transition goal example: Student will complete a 5-day unpaid work experience in a vocational area of interest with job coach support and receive a positive performance review in 3 of 5 skill areas
- Consider extended eligibility for special education services to age 21 if needed; document progress toward functional independence goals annually

Who to Collaborate With

- Transition Coordinator — post-secondary planning, VR referral, and community agency connections
- Job Coach or Vocational Educator — supported employment exploration
- School Social Worker — independent living planning and adult services connections (DDA, regional center)

Family Partnership Tip

Connect with your state's Developmental Disabilities Administration (DDA) or equivalent agency before your student turns 18. Many supports and housing or employment services have long wait lists — starting the eligibility process early is one of the most important transition steps for students with FASD.

Resources: [FASD United — Education](#) | [NOFAS — School & FASD](#) | [CDC — FASD](#)

PTSD

Post-Traumatic Stress Disorder (PTSD) is a mental health condition that can develop after exposure to traumatic events such as abuse, violence, neglect, accidents, loss, or community trauma. In the classroom, PTSD does not look like a clinical diagnosis — it looks like a student who startles easily, shuts down under pressure, struggles to trust adults, or whose behavior and performance shift dramatically without obvious cause. Trauma-informed teaching — prioritizing safety, consistency, predictability, and relationship — is the foundation of effective support. Educators are not therapists, but how teachers respond to trauma-related behavior has a profound effect on whether students heal and learn.

<https://fyvr.net/student-personas/ptsd>

Elementary (K–5)

A student whose behavior and academic performance may be highly inconsistent. On calm, structured days, the student may be engaged and successful; on days with disruptions, absences, or sensory triggers, they may shut down, act out, or become clinically unavailable for learning. The student may have experienced neglect, abuse, domestic violence, community trauma, or sudden loss — and may have limited language to describe their internal experience.

What You Might Notice

- Strong startle response to unexpected sounds, touch, or sudden movement; may freeze, cry, or lash out
- Hypervigilance — frequently scanning the room, watching the door, unable to settle into independent work
- Avoidance or shutdown when certain topics arise (family, home, particular subjects or activities)
- Emotional dysregulation that appears disproportionate to the trigger — large reactions to small frustrations
- Difficulty trusting adult authority; may test limits or withdraw from help-seeking

Student Strengths

- Often highly attuned to social and emotional cues in the environment — a strength that can support empathy and peer relationships when safe
- Demonstrates remarkable resilience and survival skills developed through adversity
- When trusting relationships are established, shows capacity for deep engagement and loyalty

Recommended Accommodations

- Establish a predictable, consistent daily routine and warn of any changes as far in advance as possible
- Create a designated safe space in or near the classroom where the student can go when dysregulated (a calm corner, a quiet chair with sensory items)
- Use a private non-verbal check-in system (a feelings card, a color scale) at the start of each day
- Avoid public correction, sarcasm, or raised voices; use calm, private redirects
- Do not require the student to discuss home or family in class activities; offer neutral alternatives

Assistive Technology

- Calming apps (Calm, Headspace for Kids, Breathe2Relax) for self-regulation during escalation
- Visual schedule app to support predictability and reduce transition anxiety

- Weighted lap pad or sensory fidget tools (with OT guidance) to support self-regulation at the desk

UDL Strategies

- Build a predictable structure into every lesson: consistent opening, activity, and closing routines
- Use choice-giving throughout the day to provide safe experiences of agency and control
- Incorporate movement, creative arts, and hands-on activities to engage multiple regulatory pathways

IEP Considerations

- Eligibility under IDEA: emotional disturbance (ED) is the most common category, though OHI is also possible; consult the school psychologist to determine eligibility based on documented educational impact
- Any behavior plan (BIP) must be trauma-informed: punitive consequences for trauma responses are ineffective and re-traumatizing. The BIP should identify triggers, sensory needs, and de-escalation strategies
- Annual goal example: Student will independently use a pre-taught calming strategy (e.g., deep breathing, going to the calm corner) within 2 minutes of an identified trigger in 4 of 5 observed opportunities as measured by teacher log
- Mental health services coordination: IEP team should document referral to school-based mental health services or outside therapist and include how educational support connects to therapeutic goals

Who to Collaborate With

- School Counselor or School-Based Mental Health Provider — therapeutic support and trauma-informed behavior planning
- School Psychologist — evaluation and BIP development
- School Social Worker — family connection, mandatory reporting obligations, and community resource navigation

Family Partnership Tip

If you are a caregiver of a child with PTSD, you do not need to share the details of the trauma with the school — only what helps the teacher respond appropriately. A simple note such as 'My child is working with a therapist on managing stress; the most helpful thing at school is predictable routines and calm responses to big emotions' is often enough to change how a teacher responds.

Middle School (6–8)

A student who may have developed better verbal and cognitive tools for understanding their trauma history but whose nervous system still responds to perceived threats in the environment. Adolescent social dynamics and autonomy needs intersect with trauma in complex ways — the student may resist authority, avoid vulnerability, or use control behaviors to manage fear. Academic performance may be inconsistent but genuine cognitive capacity is often intact.

What You Might Notice

- Difficulty with tasks that require vulnerability (sharing writing, presenting, working in groups with peers they do not trust)
- Apparent disengagement or defiance that occurs consistently in specific contexts (a particular room, topic, or adult interaction style)

- May arrive at school dysregulated from home events and be unable to access learning for part of the day
- May engage in avoidance behaviors (going to the bathroom, nurse, or counselor) to escape triggering situations
- Academic work is inconsistent — strong when regulated, significantly impaired when dysregulated

Student Strengths

- When safety is established, demonstrates insightful thinking and strong empathy
- Often highly creative and expressive through art, music, writing, or other non-verbal modes
- Developing greater capacity for self-reflection and use of coping language when supported by a trusted adult

Recommended Accommodations

- Develop a written regulation plan with the student: what triggers them, what helps, who they go to, and what the re-entry plan looks like
- Provide a predictable, non-judgmental check-in at the start of each school day with one consistent trusted adult
- Allow flexible work locations and quiet zones for high-stress moments
- Accept late or incomplete work with a private make-up plan — do not use public shame or grade deduction as motivation
- Avoid confrontational or power-assertive redirection strategies; de-escalate privately and calmly

Assistive Technology

- Private journaling app (Day One, Google Docs private folder) as an emotional processing outlet
- Music or nature sound app for self-regulation during transitions
- Digital planner for organizational support when executive function is disrupted by hyperarousal

UDL Strategies

- Provide structured choice throughout the day to build safe experiences of agency and predictability
- Use project-based learning with meaningful, real-world topics that allow personal connection without forced disclosure
- Build in relationship-building classroom routines (community circles, restorative check-ins) that are consistent and predictable

IEP Considerations

- Any BIP must be developed with trauma-informed principles; consult a trauma-informed behavior specialist if available
- Functional Behavior Assessment (FBA) should be conducted to identify the function and triggers of dysregulation — not just to categorize behavior
- Annual goal example: Student will identify a regulated state using a self-monitoring tool and independently implement one coping strategy before escalating in 3 of 4 observed opportunities as tracked by counselor and teacher log
- Coordinate IEP goals with the student's outside therapist if there is a release of information — shared language and strategies across settings is highly effective

Who to Collaborate With

- School Counselor or Trauma-Informed Mental Health Provider — therapeutic support
- School Social Worker — family coordination and community agency referrals

- Special Education Case Manager — IEP/504 coordination and teacher training on trauma-informed practices

Family Partnership Tip

Ask the school if they use a trauma-informed or restorative practices approach. If the school's discipline practices rely heavily on punitive consequences (detention, suspension, removal), ask what alternatives are available for your student. Connecting the school team to the language your child's therapist uses can make home-school strategies more consistent.

High School (9–12)

A student who may have developed significant coping skills and insight but whose trauma history continues to affect daily functioning, relationships, and academic engagement. The transition to adulthood adds stress, and unprocessed trauma can resurface during high-stakes moments. Self-advocacy, disclosure decisions, and post-secondary planning must all be approached with a trauma-informed lens.

What You Might Notice

- Avoids high-stakes academic situations (essays, oral presentations, group projects) that feel vulnerable or out of control
- Attendance may be irregular during periods of stress or trauma re-activation
- May over-rely on a single trusted adult; transfers poorly to new teachers or environments
- When dysregulated, may walk out, shut down, or engage in conflict rather than asking for help
- May have clear insight into their own patterns and benefit from being treated as a partner in their support plan

Student Strengths

- Often highly self-aware and emotionally articulate when trusting relationships exist
- Demonstrates extraordinary resilience and the ability to reframe adversity into purpose
- Strong motivation when school connects to meaningful goals (career, independence, community impact)

Recommended Accommodations

- Create a formalized safety plan in partnership with the student, counselor, and family for how to handle high-stress periods
- Allow flexible attendance with make-up options for days the student is unable to attend due to mental health
- Ensure all major tests and presentations have low-stakes preparation components and an alternative format option
- Assign the student a consistent, trusted advocate (counselor, case manager, mentor) who checks in weekly
- Honor the student's boundaries about personal disclosure — never require sharing of personal trauma in academic writing without a private opt-out option

Assistive Technology

- Crisis text or school-based mental health app (e.g., Crisis Text Line resources, Sandy Hook Promise Say Something app) for awareness
- Mindfulness app (Insight Timer, Calm) for self-regulation
- Digital portfolio tools to demonstrate academic achievement without timed, high-pressure conditions

UDL Strategies

- Offer alternative assessment formats (portfolio, oral interview, project) as a standard option for all students
- Incorporate mental health literacy content into advisory, health, or English coursework
- Build in structured transition planning conversations about mental health self-advocacy in post-secondary settings

IEP Considerations

- Transition plan must address mental health self-management: how to access mental health services in college or the community, how to request accommodations, and crisis planning
- Annual transition goal example: Student will identify one post-secondary mental health resource (campus counseling center, community therapist) and complete an intake appointment by end of senior year
- Summary of Performance should describe current mental health supports, regulation strategies in use, and documented impact of PTSD on academic functioning without clinical over-disclosure
- Ensure any mental health-related attendance or late work accommodation is clearly stated in 504 or IEP for all teachers and supported by administration

Who to Collaborate With

- School Counselor or School-Based Mental Health Provider — ongoing therapeutic support and crisis planning
- Transition Coordinator — post-secondary planning with mental health lens
- School Social Worker — community agency connections for adult mental health services

Family Partnership Tip

Help your student identify at least one adult at their post-secondary institution (college counseling center, trusted professor, RA) they can reach out to before a crisis escalates. Practicing who to call and what to say — in advance, when regulated — makes it much more likely they will actually reach out when they need help.

Resources: NCTSN — [Trauma in Schools](#) | SAMHSA — [Trauma-Informed Approach](#) | Child Welfare — [Trauma-Informed Schools](#)

Albinism

Albinism is a group of inherited conditions characterized by reduced or absent melanin production, affecting skin, hair, and eye pigmentation. The most educationally significant effect of albinism is visual impairment — nearly all individuals with albinism have some degree of vision loss due to nystagmus (involuntary eye movement), reduced visual acuity, photophobia (light sensitivity), and foveal hypoplasia (underdevelopment of the central retina). Most students with albinism do not have cognitive differences; academic impact is specifically related to visual access and the physical and cognitive demands of compensating for vision loss. Albinism also carries significant social-emotional dimensions, as the condition is visually distinctive.

<https://fyvr.net/student-personas/albinism>

Elementary (K–5)

A student who may have low visual acuity (often in the 20/100–20/400 range) that requires large print, preferential seating, and magnification tools to access classroom materials. Nystagmus causes involuntary eye movements that can make sustained reading tiring. The student may hold books very close to their face, tilt their head to use a preferred field of vision, or squint in bright environments. Skin and eye sensitivity to sunlight requires management during outdoor activities.

What You Might Notice

- Holds materials very close to the face or uses an unusual head tilt to focus — these are compensatory strategies, not signs of inattention
- Squints or avoids glare in brightly lit rooms or near windows
- Tires more quickly during sustained reading tasks due to the effort of nystagmus compensation
- May miss details on the board, projected slides, or small print materials even from the front row
- Skin is sensitive to fluorescent light and outdoor UV exposure; may prefer covered or shaded areas

Student Strengths

- Strong auditory learning and listening comprehension from developing compensatory pathways
- Demonstrates persistence and adaptability from navigating a visually demanding world
- Often highly verbal and socially connected when the environment is welcoming

Recommended Accommodations

- Seat the student in the front row or the position of their choice based on their preferred visual field
- Provide all printed materials in large print (18pt minimum; confirm preferred size with student and TVI)
- Reduce glare at the student's workspace: avoid seating near bright windows; use non-glare screen covers
- Allow the student to access a magnified copy of board content via tablet or document camera feed
- Provide extra time on all reading and visually demanding tasks to account for fatigue

Assistive Technology

- Video magnifier (CCTV or handheld magnifier) for printed materials
- iPad or tablet with built-in zoom and display contrast settings for digital access
- Screen magnification software (ZoomText, built-in OS magnification) for computer-based tasks

UDL Strategies

- Provide all content in digital format so the student can control font size and contrast
- Use high-contrast visual materials throughout the classroom — black on white or white on black, never light gray on white
- Offer audio alternatives for all visual-heavy materials (audiobooks, text-to-speech)

IEP Considerations

- Eligibility: Visual Impairment (VI) including Blindness under IDEA; Teacher of the Visually Impaired (TVI) services are required
- Present levels must include a functional vision assessment (FVA) conducted by the TVI and learning media assessment (LMA) to determine primary learning channels
- Annual goal example: Student will independently access grade-level printed materials using a tablet magnification tool in 4 of 5 opportunities as measured by TVI and classroom teacher observation
- Orientation and Mobility (O&M;) services should be evaluated — particularly for navigation of large or new school environments

Who to Collaborate With

- Teacher of the Visually Impaired (TVI) — primary specialist; direct instruction and accommodation implementation
- Orientation and Mobility (O&M;) Specialist — building navigation and outdoor travel
- School Nurse — photosensitivity management, sun protection plan for outdoor activities

Family Partnership Tip

Ask the school's Teacher of the Visually Impaired (TVI) to visit your child's classroom at the beginning of each year to brief the teacher on the specific accommodations your child needs. A 15-minute TVI classroom visit does more to set up appropriate support than any written plan alone.

Middle School (6–8)

A student who is managing visual access with increasing independence but may be navigating significant social and identity challenges related to the visibility of albinism — including hair and skin color differences and the visible use of assistive devices such as monoculars or magnifiers. May be experiencing bullying or social isolation. Academic demands are increasing, and visual fatigue becomes a more significant factor with longer reading assignments.

What You Might Notice

- Uses monocular, magnifier, or tablet zoom for board and distance access but may avoid doing so in front of peers
- Shows fatigue after extended reading-heavy periods; may rub eyes, tilt head, or lose focus
- Navigates digitally well when materials are accessible; struggles with printed handouts, worksheets, or physical textbooks
- Social self-consciousness about appearance and assistive technology use; may underreport visual access needs

- Photophobia visible in hallways, cafeteria, and outdoor settings; may wear tinted lenses or a hat

Student Strengths

- Strong auditory memory and listening skills built from compensation strategies
- Developing sophisticated self-advocacy about visual access needs in the right environment
- Often creative and intellectually engaged when visual access barriers are removed

Recommended Accommodations

- Ensure all teacher-produced materials are provided digitally before class — never paper-only in middle school
- Use high-contrast, large-format projections with sufficient time on each slide for the student to read
- Provide a printed seating accommodation card that all subject-area teachers receive at the start of the year
- Allow the student to use tinted lenses indoors if prescribed; dim lighting near the student's desk as needed
- Ensure PE and outdoor activities include a sun protection plan (shaded rest areas, hat policy, sunscreen application time)

Assistive Technology

- Monocular telescope for distance viewing (board, sports scoreboard, navigating hallways)
- Laptop or Chromebook with high-contrast mode and built-in magnification for all coursework
- Text-to-speech browser extension (Read&Write; for Google) for extended reading assignments

UDL Strategies

- Adopt a universal digital-first materials policy so the student is not singled out for requesting print alternatives
- Use multimedia and audio versions of content as standard options for all students
- Create classroom norms around assistive technology that normalize the use of magnifiers and adaptive devices

IEP Considerations

- TVI consultation and direct service hours should be reviewed as academic demands increase; extended time for all reading-heavy assessments is typically warranted
- Social-emotional impact of albinism should be assessed at each IEP meeting; counseling services may be appropriate
- Annual goal example: Student will independently request large-print or digital alternative materials from new teachers at the start of each semester in 2 of 2 opportunities as measured by TVI and student self-report
- Ensure TVI accompanies student on school-wide standardized testing to confirm accommodation implementation

Who to Collaborate With

- Teacher of the Visually Impaired (TVI) — direct instruction, material preparation, and teacher consultation
- School Counselor — social-emotional support and anti-bullying advocacy
- Orientation and Mobility (O&M;) Specialist — transition to larger building, field trips, and community travel

Family Partnership Tip

Work with your child's ophthalmologist to provide the school with an updated vision report before each school year begins. The TVI and 504 or IEP team need current acuity measurements and any new prescriptions or device recommendations to ensure accommodations stay accurate.

High School (9–12)

A student who has developed strong visual access strategies and is largely independent in managing their disability at school. The primary needs are ensuring accommodations are applied consistently across all teachers and preparing for post-secondary self-advocacy. Visual fatigue during high-stakes testing (AP exams, SAT) and in intensive reading coursework continues to require documented accommodation.

What You Might Notice

- Manages visual tools independently and advocates for access to digital materials
- Performance on visually demanding, timed exams may not reflect actual knowledge due to visual fatigue
- May need to communicate accommodation needs to many new teachers each year — this is a significant self-advocacy burden
- Participates fully in class discussions; reading-aloud or board-focused tasks require accessible alternatives
- Career exploration and college planning should include institutions with strong disability services

Student Strengths

- Exceptional self-advocacy and ability to communicate visual access needs clearly
- Strong auditory processing and listening skills that are assets in lecture-based college settings
- Resilient and strategic, with a well-developed adaptive toolkit

Recommended Accommodations

- All course materials must be available digitally before or on the day of class — no paper-only policies
- Extended time (1.5x–2x) on all timed assessments, especially reading-heavy ones
- Preferential seating with glare-free lighting in every classroom
- Ensure SAT, AP, and ACT accommodations are applied for and approved with adequate lead time
- Provide a summary of accommodations the student can hand directly to new teachers at the start of each semester

Assistive Technology

- Laptop with ZoomText or SuperNova for full screen magnification with reading capability
- Digital textbooks (VitalSource, Bookshare, Kindle) with adjustable contrast and font size
- Optical magnifiers for printed materials not available in digital format

UDL Strategies

- Design assessments that accept audio-recorded or typed responses with access to digital source materials
- Incorporate disability rights and self-advocacy content into advisory or English coursework
- Provide mentorship connections with professionals with visual impairments or albinism in career fields of interest

IEP Considerations

- Transition plan must address: post-secondary disability services registration with VI documentation, technology needs for college (campus-specific software, adaptive lab equipment), and career exploration
- Annual transition goal example: Student will complete the disability services intake process at one post-secondary institution and identify two specific accommodations that transfer from high school
- Summary of Performance (SOP) must include TVI's current assessment, functional vision status, learning media access preferences, and current assistive technology in use
- Vocational Rehabilitation referral (pre-ETS) is strongly recommended — VR can fund advanced assistive technology, college support, and vocational assessments

Who to Collaborate With

- Teacher of the Visually Impaired (TVI) — transition assessment and post-secondary AT recommendations
- Vocational Rehabilitation (VR) Counselor — AT funding, job exploration, and pre-employment services
- Transition Coordinator — college planning with accessibility focus

Family Partnership Tip

Before graduation, help your student obtain a comprehensive low vision evaluation from a low vision specialist (not just a general ophthalmologist). This evaluation documents the specific technology and accommodations that will be most effective in college or work, and is often required by post-secondary disability services offices.

Resources: [NOAH — Albinism & Education](#) | [AFB — Low Vision in Schools](#) | [APH — Visual Impairment Resources](#)

Cortical Visual Impairment

Cortical Visual Impairment (CVI) is the leading cause of visual impairment in children in developed countries. Unlike most eye conditions, CVI is a brain-based visual disorder — the eyes may be structurally typical but the brain has difficulty processing and interpreting visual information. CVI is caused by neurological damage or disruption (often from hypoxia, prematurity, brain injury, or infection) and is characterized by highly specific visual behaviors: strong preference for color, movement, and familiar objects; difficulty in visually complex environments; visual fatigue; and inconsistent visual response. CVI occurs on a spectrum from very low vision to near-typical vision with subtle processing challenges; students at higher CVI functional levels may appear to have typical vision until academic demands reveal the deficit.

<https://fyvr.net/student-personas/cortical-visual-impairment>

Elementary (K–5)

A student whose visual functioning may appear inconsistent — looking at a preferred toy readily but struggling to find materials on a cluttered desk. The student may see better in familiar, controlled environments and worse in visually complex settings like a busy classroom display or a playground. Many students at higher CVI levels are not yet diagnosed; teachers may notice the visual behaviors described below without understanding they are neurological.

What You Might Notice

- Responds more reliably to brightly colored, moving, or familiar objects than to printed black-and-white materials
- Struggles in visually complex environments — a cluttered desk, a wall covered in displays, a busy worksheet
- May look away from materials when touching or handling them — using peripheral rather than central vision
- Visual response is inconsistent: looks at something one moment, cannot locate it the next
- May become visually fatigued quickly; performance declines after sustained visual tasks

Student Strengths

- Strong tactile and kinesthetic learner — explores and understands the world through touch and movement
- Often has strong auditory memory and benefits greatly from verbal instruction and audiobooks
- When environmental modifications reduce visual complexity, demonstrates clear engagement and learning

Recommended Accommodations

- Reduce visual clutter on the desk and in the direct learning environment — use a clear work area with only the materials currently needed
- Provide materials in the student's preferred color (often red or yellow for early CVI) with simple, high-contrast backgrounds
- Use movement to attract attention: gently move materials slightly to activate the student's visual attention
- Provide verbal descriptions of all visual content — do not assume the student is seeing what is presented
- Allow extra processing time for all visual tasks; never rush a visual response

Assistive Technology

- iPad with CVI-specific color and background settings (dark mode, preferred color highlighting)

- Light box for backlighting materials to increase visual saliency
- Video magnifier with color contrast and background control for printed materials

UDL Strategies

- Pair all visual materials with tactile and auditory components to provide multiple access channels
- Use CVI-adapted instructional materials: single object per field, preferred color, controlled background
- Provide digital materials that allow the student to control color, contrast, and font size

IEP Considerations

- Eligibility: Visual Impairment (VI) including Blindness under IDEA; requires TVI with CVI-specific training and experience — not all TVIs have this specialization, so families should confirm expertise
- CVI Range assessment (by Christine Roman-Lantzy, or equivalent) should be conducted and documented in present levels — not just traditional visual acuity measurements
- Annual goal example: Given materials presented in the student's preferred color with a reduced background, student will visually locate a single target object within 10 seconds in 4 of 5 trials as measured by TVI observation
- AT assessment must specifically address CVI needs: device lighting, color settings, and environmental control — not just standard low-vision equipment

Who to Collaborate With

- Teacher of the Visually Impaired (TVI) with CVI specialization — essential; primary specialist for material adaptation and instruction
- Orientation and Mobility (O&M;) Specialist — environmental navigation with CVI-specific considerations
- Occupational Therapist (OT) — visual-motor integration and tactile learning support

Family Partnership Tip

Ask specifically whether your child's TVI has CVI training and experience. CVI requires a different approach than other visual impairments — a TVI trained only in print-based or braille low-vision approaches may not have the skills to create the right learning environment. CVI Now (at Perkins School for the Blind) has family resources and a list of CVI-trained specialists.

Middle School (6–8)

A student who has typically been navigating CVI for years with some interventions but who may be at a higher functional CVI level where the visual challenges are less obvious but still significantly affect academic performance in complex visual environments. Textbooks, multi-column worksheets, cluttered digital interfaces, and visually dense classroom walls all increase processing demands. The student may have developed strong auditory compensatory strategies.

What You Might Notice

- Navigates simple, familiar environments well but struggles in new or visually complex spaces (science labs, libraries, field trips)
- Written work on paper may be less complete or accurate than the same work done orally or digitally

- Fatigue increases significantly during periods of intense visual demand (standardized testing, visual note-taking)
- May appear to 'not pay attention' when looking at visually complex projections or slides with many elements
- Performs significantly better when auditory input accompanies visual content

Student Strengths

- Strong auditory and verbal processing — typically excellent in discussion, lecture, and audio-based learning
- Develops strong memory strategies to compensate for inconsistent visual access
- When visual environment is controlled and simplified, demonstrates full comprehension and creative thinking

Recommended Accommodations

- Provide all instructional slides and handouts digitally with clean, uncluttered formatting
- Reduce classroom wall displays near the student's working area to decrease visual noise
- Allow use of colored overlays, iPad dark mode, or preferred contrast settings on all devices
- Pair all visual presentations with verbal narration — never rely on visuals alone to convey key information
- Schedule brain and visual rest breaks during visually intensive tasks

Assistive Technology

- iPad or laptop with customized display settings (preferred color, dark mode, high contrast)
- Text-to-speech tools for all reading to reduce visual processing load
- Video magnifier with color and contrast control for printed handouts

UDL Strategies

- Adopt a digital-first materials policy with simple, clean formatting for all teacher-produced content
- Use audio narration, video captions, and verbal instruction as primary channels rather than visual-only
- Offer oral response or recorded video as alternatives to visually demanding written tasks

IEP Considerations

- CVI Range reassessment should occur at least every 2–3 years; functional level often increases with appropriate instruction and may decrease under environmental stress
- Present levels should describe current CVI Range phase, visual behaviors in the academic environment, and specific subject-area impacts
- Annual goal example: Given a digital text with student-selected contrast settings and a simplified layout, student will independently access and summarize grade-level content in 3 of 4 assigned reading tasks per month as measured by product assessment
- All teachers, not just the TVI, must understand CVI; IEP should include a professional development requirement or TVI consultation schedule for general education teachers

Who to Collaborate With

- Teacher of the Visually Impaired (TVI) with CVI specialization — direct service and teacher consultation
- Assistive Technology Specialist — CVI-specific device and software configuration
- School Counselor — social-emotional support and peer awareness of visual differences

Family Partnership Tip

Create a one-page 'visual access guide' with your student's TVI that explains what CVI means in practical terms for teachers: what the student can see, what they cannot, and what changes to make in the classroom right now. Request that this guide is shared with every teacher at the start of each year.

High School (9–12)

A student at a higher CVI functional level who may have near-typical visual function in simple, controlled environments but who experiences significant visual fatigue and processing challenges during the complex, visually dense demands of high school. AP coursework, standardized testing, and visually complex STEM environments can expose CVI-related challenges that are less visible in lower-demand settings. Transition planning must address technology needs for post-secondary settings.

What You Might Notice

- Performs well in verbal and discussion-based assessments but struggles with timed visual tests
- Complex diagrams, charts, multi-column layouts, and dense visual media cause fatigue and reduced accuracy
- STEM and technical coursework with heavy visual component may be significantly more challenging than verbal/humanities coursework
- Manages technology confidently but may not have optimal CVI-specific settings configured on school devices
- May not have current CVI documentation — some students receive the diagnosis late or have outdated assessments

Student Strengths

- Highly developed auditory processing and verbal reasoning
- Strong metacognitive awareness of visual limitations and environmental needs
- Resilient and strategic in accessing content through multiple channels

Recommended Accommodations

- Extended time (1.5x–2x) on all visually demanding assessments; allow breaks during extended testing
- All course materials provided in digital format with clean, accessible formatting before or on class day
- Ensure SAT, AP, and ACT accommodations are applied for, including extended time and digital formats
- Allow student-configured display settings (contrast, color, zoom) on all school-issued devices
- Science lab diagrams and technical drawings must be provided in accessible format — consult TVI for adaptation

Assistive Technology

- Screen reader (JAWS, NVDA, or VoiceOver) as a backup channel for dense visual text
- Text-to-speech with synchronized highlighting (Kurzweil 3000) for textbook and reading-heavy coursework
- Digital microscope or camera attachment for science labs to reduce close visual demands

UDL Strategies

- Use audio-first instruction design: verbal explanations accompany all visual presentations as a standard practice
- Provide alternative assessment formats (oral exam, audio recording, verbal defense) to allow demonstration of knowledge without visual processing barriers

- Build explicit post-secondary self-advocacy instruction into advisory and transition programming

IEP Considerations

- Transition plan must address: college AT needs (identify specific software and hardware for college setting), disability services registration, and career exploration with CVI-informed job analysis
- Current CVI Range assessment and functional vision assessment are required for college disability services packets — begin this documentation process in junior year
- Annual transition goal example: Student will configure CVI-specific accessibility settings on their personal device and demonstrate their use to the college disability services coordinator before the first day of college
- Vocational Rehabilitation referral is strongly recommended — VR can fund advanced AT equipment and college supports specifically for VI students

Who to Collaborate With

- Teacher of the Visually Impaired (TVI) with CVI specialization — transition assessment and AT documentation
- Vocational Rehabilitation (VR) Counselor — AT funding and career planning
- Transition Coordinator — post-secondary planning with VI-specific knowledge

Family Partnership Tip

Request an updated functional vision assessment from the TVI in junior year — this document, along with the CVI Range score and current AT recommendations, is what college disability services offices need to provide appropriate technology and accommodations from day one. An outdated assessment from elementary school will not be sufficient.

Resources: [CVI Now — Educators](#) | [APH — CVI Resources](#) | [Perkins — CVI in Schools](#)

Unilateral Hearing Loss

Unilateral hearing loss (UHL) refers to hearing loss in one ear while the other ear has normal or near-normal hearing. It is sometimes called single-sided deafness (SSD) when the affected ear has profound loss. Because students with UHL can hear with their good ear, the condition is often overlooked in school settings, but research consistently shows that children with UHL are at elevated academic risk: they have significantly more difficulty understanding speech in noise, locating sound sources, and maintaining focus in noisy classroom environments compared to peers with bilateral normal hearing. Fatigue from the extra effort of listening is a major and underrecognized impact.

<https://fyvr.net/student-personas/unilateral-hearing-loss>

Elementary (K–5)

A student who seems to hear normally in quiet one-on-one interactions but struggles in the noisy, reverberant environment of a typical classroom. May miss instruction delivered from the wrong side, mishear words in group work, or appear inattentive during noisy activities. Listening fatigue may be evident by the end of the school day as irritability, disengagement, or declining performance. The student may not be able to identify or explain why they are struggling.

What You Might Notice

- Frequently asks for repetition or says 'what?' — especially when the teacher is speaking from the student's impaired-ear side
- Turns their head to position their good ear toward the speaker — may appear to be looking away or not paying attention
- Struggles to follow group discussions, particularly in noisy settings (cafeteria, gym, outdoor recess)
- Appears more tired or irritable by afternoon; listening fatigue is real and cumulative
- May miss parts of oral directions that were delivered from the impaired side or across a noisy room

Student Strengths

- Typically develops strong visual attention and lip-reading skills as compensatory strategies
- Full cognitive ability and academic potential — hearing in the good ear is functional in quiet conditions
- Responds well to simple environmental modifications that have immediate positive effect on access

Recommended Accommodations

- Seat the student so their good ear faces the primary instruction area and the teacher's typical speaking position
- Use an FM system or sound-field amplification system in the classroom — even mild benefit significantly reduces fatigue
- Face the student when speaking and ensure clear line of sight for visual cues and lip reading
- Reduce classroom noise where possible: carpet, sound-absorbing panels, tennis balls on chair legs
- Provide a written summary of oral directions for complex multi-step tasks

Assistive Technology

- FM system (e.g., Roger Touchscreen mic or similar) — teacher-worn microphone transmitting directly to a receiver worn by the student
- Sound-field classroom amplification system for whole-class benefit
- CROS hearing aid or bone-anchored hearing aid (BAHA) — fitted by audiologist; routes sound from impaired ear to good ear

UDL Strategies

- Pair all verbal instruction with visual supports: text on board, written directions, visual demonstrations
- Use a talking stick or visual turn-taking tool in group discussions so the student can track the speaker
- Provide printed or digital copies of read-aloud texts so the student can follow along visually

IEP Considerations

- Many students with UHL qualify under IDEA's Hearing Impairment category; others are better served by a 504 Plan if academic performance is on track with accommodations alone
- Educational Audiologist consultation is essential — they can conduct a classroom acoustics assessment and recommend the appropriate FM or amplification system
- Present levels should document audiological findings, classroom listening behavior, and any documented academic impact
- Annual goal example (if IEP): Student will accurately follow 3-step oral directions given from across the classroom in 4 of 5 trials as measured by Educational Audiologist and teacher log

Who to Collaborate With

- Educational Audiologist — audiological assessment, FM system fitting and management, classroom acoustics
- Teacher of the Deaf and Hard of Hearing (TDHH) — consultation and direct service if warranted
- 504 Coordinator or Special Education Case Manager

Family Partnership Tip

Ask for a classroom acoustics assessment from the school's Educational Audiologist or a contracted audiologist. Background noise levels in typical classrooms are often 3–4 times higher than what is recommended for students with hearing loss. A simple report with specific recommendations can justify modifications that immediately improve your child's access.

Middle School (6–8)

A student who has typically been managing UHL for years but is now facing increasingly complex listening environments — multiple teachers, noisy hallways, group projects, science labs, and social dynamics that depend heavily on casual conversation in loud settings. Listening fatigue is cumulative and may be at its peak in middle school due to longer, noisier, and more socially demanding school days. The student may be self-conscious about using hearing devices.

What You Might Notice

- Misses portions of instruction in noisy class discussions, group work, and lab settings

- Performance gaps appear specifically in auditory-heavy tasks (listening comprehension, following oral lectures without notes)
- Resists using the FM system or hearing aid in front of peers due to self-consciousness
- Appears more fatigued or disengaged in the afternoon — after a full day of compensatory listening effort
- May miss calls in the hallway, arrival of class announcements over intercom, or group conversations at lunch

Student Strengths

- Developing self-awareness of listening environment needs — can often identify what helps and articulate it
- Strong visual processing skills from years of compensatory attention to lip movement and visual cues
- Full academic capacity — performance reflects listening access, not cognitive ability

Recommended Accommodations

- Allow the student to choose their seat in every class based on optimal good-ear positioning toward the teacher
- All teachers must use the FM system consistently, including during class discussions and lab instructions
- Provide printed or digital notes to supplement oral lectures in all content areas
- Schedule a check-in with the school's audiologist or case manager if the FM system is not being used consistently by all teachers
- Create a private, non-judgmental way for the student to signal when they missed something without embarrassment

Assistive Technology

- Roger FM or Phonak Roger Select system for multi-speaker classroom environments
- Captions on all classroom video content (YouTube auto-captions minimum; professional captions preferred)
- Note-taking app (OneNote, Notability) for organizing teacher-provided notes alongside class materials

UDL Strategies

- Adopt a caption-on policy for all classroom video as a universal practice — it benefits all learners
- Use visual turn-taking and discussion structures (numbered heads, talking chips) in group work
- Provide written agendas and learning objectives at the start of each class to reduce reliance on purely oral orientation

IEP Considerations

- Annual review should assess whether the FM system is being used consistently by all teachers — document teacher compliance as a service delivery metric
- If fatigue is significantly affecting afternoon academic performance, consider scheduling high-demand classes in the morning
- Annual goal example: Student will independently set up and confirm FM system connection with each teacher within the first 5 minutes of class in 4 of 5 school days as tracked by student self-report and teacher confirmation
- Social-emotional impact of self-consciousness about hearing devices should be assessed; school counseling services may be appropriate

Who to Collaborate With

- Educational Audiologist — FM system management, troubleshooting, and teacher training
- School Counselor — support for self-consciousness about hearing devices and peer relationships

- Teacher of the Deaf and Hard of Hearing (TDHH) — direct instruction or consultation if warranted

Family Partnership Tip

At the start of each school year, schedule a brief meeting (even 15 minutes) with each of your student's core teachers to demonstrate how the FM system works, how to put the microphone on, and what to do when it malfunctions. Teachers who have been shown how to use the equipment in person are far more likely to use it consistently.

High School (9–12)

A student who is managing UHL largely independently but who faces the most acoustically complex environment of their school career — large lecture-style classes, career and technical education labs, after-school activities, and the social demands of being a teenager in loud social settings. Self-advocacy is critical. Transition planning must address post-secondary accommodations and the student's right to request captioning, FM systems, and accessible seating at colleges.

What You Might Notice

- Manages FM system and hearing technology independently but may not consistently advocate for its use when teachers forget
- Performance in large classes, auditoriums, or noisy CTE labs may drop relative to smaller or quieter settings
- May experience fatigue that appears as disengagement or mood changes by the end of intensive school days
- Strong academic performance when listening access is in place; inconsistencies reflect access barriers, not ability
- May be researching post-secondary accommodations and anticipating whether FM systems will be available in college

Student Strengths

- Well-developed self-advocacy skills for requesting listening environment modifications
- Strong visual and written processing skills from years of supplementing auditory input
- Full academic ability — final performance with appropriate accommodations matches cognitive capacity

Recommended Accommodations

- Ensure FM system is available and used in every course including CTE, electives, and advisory
- All standardized testing (AP, SAT, ACT) should be conducted in a quiet, controlled acoustic environment — document this as a formal accommodation
- Provide captioning for all multimedia instructional content in every class
- Allow flexible seating in all classes based on optimal acoustic position
- Document all accommodations in a portable format for submission to college disability services

Assistive Technology

- Roger MultiMic or table-mounted microphone for roundtable discussion environments
- CART (Communication Access Realtime Translation) — real-time captioning for large assemblies, presentations, or CTE labs

- Captioning apps (Otter.ai, Live Caption on Android, Live Captions on iOS) for informal settings without FM access

UDL Strategies

- Adopt captioning on all video as a school-wide norm — not a one-student accommodation
- Build disability self-advocacy curriculum into health or advisory coursework for all students
- Offer flexible assessment formats that do not rely solely on oral/aural performance for content knowledge

IEP Considerations

- Transition plan must address post-secondary accommodations: how to register with college disability services, how to request FM system or CART at college, and ADA rights in the workplace
- Annual transition goal example: Student will independently contact the disability services office at one post-secondary institution, request FM equipment availability information, and document the response by end of junior year
- Summary of Performance must include: audiological report, current assistive hearing technology in use, classroom accommodations in effect, and student's self-advocacy skills related to hearing access
- Vocational Rehabilitation may fund advanced hearing technology (CROS aids, BAHA) for college and employment settings — begin VR referral by junior year

Who to Collaborate With

- Educational Audiologist — transition assessment, updated audiological documentation, and college AT recommendations
- Vocational Rehabilitation (VR) Counselor — hearing technology funding and pre-employment transition services
- Transition Coordinator — post-secondary planning with hearing access focus

Family Partnership Tip

Help your student practice a short, confident disclosure statement they can use with college professors: 'I have hearing loss in one ear and use an FM system in class — here is the microphone, you just clip it on and it transmits to my hearing device.' Having a practiced, matter-of-fact script removes the anxiety of asking and significantly increases the chance teachers will comply consistently.

Resources: [ASHA — Unilateral Hearing Loss](#) | [Hands & Voices — Unilateral HL](#) | [OSEP — Hearing Impairment & IDEA](#)